

Chapter 1 Issues and Trends in Pediatric Nursing

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse has provided a presentation to middle school children about nursing as a career. One of the children stated that he wanted to be a pediatric nurse, because he wants to take care of babies and little “kids.” What would be an appropriate response to this middle school–aged child?
1. “Nursing is a general practice; all nurses must know how to take care of all ages of patients.”
 2. “As a pediatric nurse, you would get to care for babies and little kids as well as children through their teenage years.”
 3. “What you are describing is actually a neonatal nurse. Neonatal nurses work with newborn babies, toddlers, and preschool-age children.”
 4. “Being a pediatric nurse requires a master’s degree, so be sure to work hard in school!”
- _____ 2. The medical-surgical float nurse is assigned to the pediatric unit for the first time and states that the skills are “the same as general nursing since children are little adults with smaller bodies.” What component of pediatric nursing contraindicates this statement?
1. Pediatric nursing involves care based on the developmental level of the patient.
 2. Pediatric nursing involves the patient’s family in the plan of care.
 3. Pediatric nursing is based on prevention of infectious diseases.
 4. Pediatric nursing involves cultural sensitivity in patient care.
- _____ 3. The culturally sensitive pediatric nurse is aware of the changes in the racial population of the United States. Which of the following reflects the historical changes in the projected demographics of the U.S. pediatric population in 2020?
1. The American Indian and Alaska Native population will continue to decrease.
 2. Less than half of the children will represent less than half of the population.
 3. The Asian children will reflect no change from 2015 into 2020.
 4. Non-Hispanic children who represent two or more races will increase in 2020.
- _____ 4. The nurse at a family clinic is providing care to an adolescent patient diagnosed with diabetes. The patient has verbalized discomfort with the feelings of isolation when unable to drink alcohol with their friends. Which of the following would be the most appropriate response to this statement?
1. “Drinking alcohol is not only illegal but dangerous for your blood sugar control and overall well-being.”
 2. “You can still drink alcohol, in moderation, if you control the sugar spike with more insulin.”
 3. “Prepare for alcohol intake with more water and less carbohydrates before drinking in order to slow sugar spikes.”
 4. “It is understandable that you want to be included, but perhaps we can work out a plan on how to avoid those situations.”
- _____ 5. Which statement best describes the common core concepts of the different models of pediatric nursing?

1. All models focus on the open communication between the child and nurse.
2. All models focus on importance of family relationship to the child.
3. All models focus on safeguarding the child's dignity during care.
4. All models focus on coordination of care needs that arise during illness.

_____ 6. Which of the following tasks is associated with the pediatric medical home care model of pediatric nursing?

1. The nurse refers the family to a childhood cancer support group prior to discharge from the hospital.
2. The nurse includes a simple explanation of the procedure to the child based upon the child's developmental level.
3. The nurse includes the family's cultural celebrations and observations while providing care to the child.
4. The nurse develops the plan of care with the child and family as the focal point of the interventions.

_____ 7. The pediatric nurse begins a research project on 8-year-old patients with the medical diagnosis of pediatric acute onset neuropsychiatric syndrome, PANS. The family has consented to the research, but as the nurse begins to administer the intervention, the patient refuses to cooperate. What is the appropriate response to this refusal?

1. The nurse should discuss with the family the options of performing the intervention without the child's knowledge.
2. The nurse should explain to the child that this is necessary and promise a reward at the end of the research project.
3. The nurse should refuse to perform the intervention until the child consents to the intervention and project.
4. The nurse should discuss the pros and cons of the research project with the child and focus on the outcomes.

_____ 8. A nurse is providing relationship-based care to a pediatric patient. Which of the following is a challenge the nurse is likely to encounter while using this model of care?

1. Maintaining a sterile field for a procedure with a restless toddler
2. Communicating with the blended family on regular basis
3. Treating children with antibiotic-resistant illnesses
4. Caring for a larger number of children with autism spectrum disorders and childhood depression

_____ 9. The pediatric nurse desires to expand her career options to become the hospital's nurse educator of pediatrics. Which of the following is necessary for this nurse to achieve this career goal?

1. The nurse must earn a minimum of a master's degree in nursing.
2. The nurse must earn a minimum of a doctorate degree in nursing.
3. The nurse must implement and develop research in nursing practice.
4. The nurse must implement a plan to obtain certification in pediatrics.

_____ 10. The school nurse is developing a teaching session for the population of children in a middle school. The nurse has determined that the greatest need for the overall health of the children is promotion of healthy foods. Which of the following interventions is most likely to be effective?

1. Remove all unhealthy snacks from the vending machines.

2. Have a poster contest on healthy eating options.
3. Survey the student population during lunch breaks.
4. Engage the students in healthy choices with cooking lessons.

- _____ 11. The nurse recognizes the needs of professional development to provide safe and effective care to the pediatric population by attending which of the following courses?
1. Procedure with rationale to administer HPV vaccines to the middle school population
 2. Study review of the incidence of bullying and intimidation in the elementary school population
 3. Demographic study of the U.S. population with regard to cultural diversity and practices
 4. How to provide individual health care education for the children without parental interference
- _____ 12. Which of the following continuing education courses would best enhance the pediatric nurse's practice in relationship to current trends?
1. Evidence-based practice on improving antibiotic adherence with education to the family unit
 2. Evidence-based practice on proper administration of antibiotics for children with autoimmune diseases
 3. How to enhance communication with children in regards to adherence to antibiotic use
 4. How to decrease instances of exacerbation of infections of children with autoimmune diseases with antibiotics
- _____ 13. The pediatric nurse is developing interventions for a school-aged child based on the nursing diagnosis of *impaired body image related to BMI of 37 and poor food choices as evidenced by patient's mother's food diary for the family and child's statements of food preferences*. Which of the following would be an appropriate nursing intervention?
1. Provide the child and family with information from MyPlate.gov for healthy food options.
 2. Increase the child's activity by incorporating the child's friends into a plan of healthy exercise options.
 3. Explain the importance of balancing diet and exercise to prevent obesity to the child and family.
 4. Provide information on the increased risk for potential disease processes such as diabetes and hypertension.
- _____ 14. The nurse is developing interventions for a transgender adolescent based on the nursing diagnosis *decreased self-esteem related to transgender identity as evidenced by statements of feeling "worthless, different, and alone."* What would be the most appropriate intervention based on the diagnosis?
1. Give positive feedback on personal choices as appropriate.
 2. Provide gender-affirming care to transgender youth.
 3. Provide information on support groups for patient and family.
 4. Assess risk factors for depression and suicidal thoughts.

- _____ 15. The pediatric nurse is providing care to an 8-year-old client who has Down syndrome and wants to ensure he is providing evidence-based care. Which of the following resources would best offer evidence-based practice guidelines related to the care of this patient?
1. *Journal of Specialists in Pediatric Nursing*
 2. *Journal of Child and Adolescent Psychiatric Nursing*
 3. *National Database of Nursing Quality Indicators*
 4. *American Journal of Maternal/Child Nursing*

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 16. The nurse must obtain data for a routine assessment of the 8-month-old patient. Which of the following are part of that assessment? *Select all that apply.*
1. Measure the head circumference and record results.
 2. Enter height and weight in the growth chart.
 3. Assess bilateral hand grasp strength and function.
 4. Input the Tanner scale results in the chart.
 5. List most recent immunizations in the chart.
- _____ 17. Which of the following examples accurately represent the unique role of the pediatric nurse? *Select all that apply.*
1. The nurse developed an intervention that correlated with the patient's particular cultural practice.
 2. The nurse shared information on the progress and interventions with the family and patient.
 3. The nurse engaged in one-to-one conversations with the family members as well as the patient.
 4. The nurse requested the family to leave during invasive procedures to maintain privacy.
 5. The nurse developed interventions based on the knowledge of human growth and development.
- _____ 18. The nurse is developing a health fair for children aged 5 and 6 years. Which of the following would be appropriate topics for this population based on current trends? *Select all that apply.*
1. How to deal with bullying on the internet
 2. How to prevent the flu with hand washing
 3. How to choose and eat healthy foods
 4. How to recognize suicidal thoughts
 5. Ways to stay active and stay healthy
- _____ 19. The school nurse is providing care to the student population in the urban elementary school. The nurse notes that the children do not feel safe playing outside during recess due to the recent violent incidents in the surrounding area. What are some disease processes that the nurse may notice due to this inactivity? *Select all that apply.*
1. The general population is at higher risk for obesity.
 2. The students are at higher risk for increased bullying.
 3. The students are at a higher risk for learning disabilities.

- 4. The students are at higher risk for diabetes mellitus.
- 5. The students are at higher risk for earlier onset of puberty.

- _____ 20. The pediatric nurse is providing care to a 5-year-old patient for speech impediments that were caused by repeated incidences of otitis media. Which of the following is an example of current issues regarding this type of medical problem? *Select all that apply.*
- 1. Increased admissions based on environmental risk factors, such as smoking
 - 2. Increased admissions based on knowledge deficits due to language barriers
 - 3. Increased awareness of childhood health issues from social media and Internet
 - 4. Shift in the focus of medical care from disease treatment to disease prevention
 - 5. Deficient knowledge that has negatively impacted the adherence to the antibiotic regimen

Chapter 1 Issues and Trends in Pediatric Nursing

Answer Section

MULTIPLE CHOICE

1. ANS: 2

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 1. Define pediatric nursing.

Page: 3

Heading: Introduction to Pediatric Nursing

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Cognition

Difficulty: Moderate

	Feedback
1	This is incorrect. Nurses can specialize in a number of areas. Pediatrics is one of these areas.
2	This is correct. Pediatric nursing care requires the nurse to use assessment and evaluation tools that are unique to infants, children, and adolescent populations, not just babies and young children.
3	This is incorrect. This is not accurate. Neonatal nurses typically provide care to infants. They would not provide care to toddlers or preschool-age children.
4	This is incorrect. A master's degree is not required to be a pediatric nurse. However, advanced degrees and additional certification are available for pediatric nurses.

PTS: 1

CON: Cognition

2. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 6. Describe the roles of the pediatric nurse.

Page: 3

Heading: Introduction to Pediatric Nursing > Nursing Care for Children Versus Care for Adults

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is correct. Pediatric nursing involves not only general nursing care and planning but also the developmental level of the patient. The history of pediatric nursing describes how the role has evolved into a specialty.

2	This is incorrect. Although this is a component of pediatric nurses, it does not answer the concern regarding the “little adults” comment.
3	This is incorrect. Although this is a component of pediatric nurses, it does not answer the concern regarding the “little adults” comment.
4	This is incorrect. Although this is a component of pediatric nurses, it does not answer the concern regarding the “little adults” comment.

PTS: 1 CON: Safety

3. ANS: 2

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 4. Describe the projected racial population changes in the United States.

Page: 5

Heading: Racial Makeup of American Children

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Culture

Difficulty: Moderate

	Feedback
1	This is incorrect. Although this is projected to occur, it is not of historical significance.
2	This is correct. This will be the first time in history that less than 50% of the children aged 0 to 17 will be white non-Hispanic.
3	This is incorrect. Although this is projected to occur, it is not of historical significance.
4	This is incorrect. Although this is projected to occur, it is not of historical significance.

PTS: 1 CON: Culture

4. ANS: 4

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 3. Describe the differences between nursing care of infants, children, and adolescents and care of the adult population.

Page: 5

Heading: Role of the Pediatric Nurse

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Safety; Self

Difficulty: Moderate

	Feedback
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1	This is incorrect. This is not a solution to the patient's issues. The patient reports feeling left out due to the disease process. This does not address the issue in a developmentally appropriate manner to promote the optimal physical, psychological, and social well-being.
2	This is incorrect. This encourages unsafe health habits and does not address the issue with the disease process and the adolescent's need for inclusion.
3	This is incorrect. This is not therapeutic communication, nor is it safe for the adolescent. It does not care in a developmentally appropriate manner to promote the optimal physical, psychological, and social well-being of the adolescent.
4	This is correct. Helping the adolescent develop anticipatory guidance facilitates independence and healthy responses to peer pressure.

PTS: 1 CON: Safety | Self

5. ANS: 2

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 5. Identify models of care applied to pediatric nursing.

Page: 5

Heading: Models of Care in Pediatric Nursing

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. This omits the necessity of family relationships in the care and development of the child.
2	This is correct. This is a common core concept in family-centered care, relationship-based care, and pediatric medical home care.
3	This is incorrect. This is a concept based on patient relationship-based care.
4	This is incorrect. This is a component of pediatric medical home care.

PTS: 1 CON: Patient-Centered Care

6. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 5. Identify models of care applied to pediatric nursing.

Page: 5

Heading: Models of Care in Pediatric Nursing > Pediatric Medical Home

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
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1	This is correct. The pediatric medical home care model coordinates care and includes referrals for community services for nonmedical needs.
2	This is incorrect. This is a core concept of relationship-based care.
3	This is incorrect. This is a core concept of cultural sensitivity and family-centered care.
4	This is incorrect. This is a core concept of relationship-based care.

PTS: 1 CON: Family Dynamics

7. ANS: 3

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 6. Describe the roles of the pediatric nurse; 8. identify current issues and trends in pediatric nursing practice, education, and research.

Page: 6

Heading: Issues in Pediatric Nursing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Communication, Legal

Difficulty: Moderate

	Feedback
1	This is incorrect. This could be considered assault and battery against the child. Children aged 7 years and older can assent or dissent to be in a clinical trial.
2	This is incorrect. This would be considered bribery, and per the IRB guidelines, a clinical trial should not promise any direct rewards.
3	This is correct. Children aged 7 years and older can assent or dissent to be in a clinical trial.
4	This is incorrect. The 8-year-old child may not have the faculties or cognitive development to comprehend “pros and cons” of a research project.

PTS: 1 CON: Communication | Legal

8. ANS: 2

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 6

Heading: Issues in Pediatric Nursing

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
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1	This is incorrect. This is a fundamental issue in nursing care, not a relationship issue.
2	This is correct. Maintaining communication with families and caregivers has become more challenging due to the dynamics of current families.
3	This is incorrect. There has been an increase in the number of antibiotic-resistant illnesses. This is a trend that has influenced nursing roles and responsibilities, not relationship-based care.
4	This is incorrect. This is a trend that has influenced nursing roles and responsibilities, not relationship-based care.

PTS: 1 CON: Patient-Centered Care

9. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 7. Identify the different fields of pediatric nursing and the education required for each.

Page: 6

Heading: Role of the Pediatric Nurse > Where Pediatric Nurses Work

Integrated Processes: Nursing Process

Client Need: Physiological Integrity

Cognitive Level: Comprehension [Understanding]

Concept: Professionalism

Difficulty: Moderate

	Feedback
1	This is correct. Pediatric nurses with master's degree preparation are often employed as nurse educators. Many academic settings require nursing instructors to have a nursing doctorate or be actively working to obtain a doctorate.
2	This is incorrect. Pediatric nurses with master's degree preparation are often employed as nurse educators. Many academic settings require nursing instructors to have a nursing doctorate or be actively working to obtain a doctorate. (Keyword: minimum)
3	This is incorrect. Although nurses with doctorates may develop and implement research programs for hospitals, this is not necessary to obtain this position.
4	This is incorrect. Although nurses may obtain certification in pediatrics, it is often not the minimum requirement for nurse educator.

PTS: 1 CON: Professionalism

10. ANS: 4

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 7

Heading: Trends in Pediatric Nursing Practice > Obesity

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. This would only prevent the students from buying unhealthy foods via the vending machine, not help them make choices.
2	This is incorrect. Although this may help with a competition, it may not facilitate lifestyle changes.
3	This is incorrect. Although this may help assess the student's lunchtime eating habits, it would not help with the full picture of the problem.
4	This is correct. Engaging the students in healthy options would promote active learning and new skills.

PTS: 1 CON: Health Promotion

11. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 7

Heading: Trends in Pediatric Nursing Practice > Education

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Safety; Health Promotion

Difficulty: Moderate

	Feedback
1	This is correct. This is an example of shifting the focus of medical/nursing care from disease treatment to disease prevention.
2	This is incorrect. Although this is a safety issue, it would not increase knowledge of the nurse as far as care provision in pediatric nursing practice.
3	This is incorrect. This may enhance the knowledge base for certain cultural practices, but it does not indicate how this would enhance practice.
4	This is incorrect. This contraindicates the nursing model of relationship-based nursing care.

PTS: 1 CON: Safety | Health Promotion

12. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 7

Heading: Trends in Pediatric Nursing Practice > Education

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control
Cognitive Level: Application [Applying]
Concept: Health Promotion
Difficulty: Moderate

	Feedback
1	This is correct. Evidence-based practice may facilitate medication adherence if provided to the family unit. This may help prevent future antibiotic-resistant diseases.
2	This is incorrect. Although this includes key words such as evidence-based practice and proper administration of antibiotics to children, lack of family involvement may prevent adherence of proper administration of medications.
3	This is incorrect. Although this may help with adherence for the child, the trend is to incorporate the family unit in the child's care.
4	This is incorrect. Although the trend in nursing is disease prevention, this would not be a feasible option, as overuse of antibiotics leads to resistance.

PTS: 1 CON: Health Promotion

13. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing
Chapter Learning Objective: 6. Describe the roles of the pediatric nurse.
Page: 7
Heading: Trends in Pediatric Nursing Practice > Obesity
Integrated Processes: Nursing Process
Client Need: Physiological Integrity: Reduction of Risk Potential
Cognitive Level: Analysis [Analyzing]
Concept: Health Promotion
Difficulty: Moderate

	Feedback
1	This is correct. Providing information on a reliable resource for both the child and family may facilitate adherence and comprehension.
2	This is incorrect. This may incorporate the child's adherence in exercise due to peer pressure, but it would not facilitate adherence within the family unit.
3	This is incorrect. Current trends incorporate the child's family in the plan of care for better adherence. There is no indication that the child would make healthy choices without the support of the family.
4	This is incorrect. Giving information on the increased risk would only inform not educate on healthy choices for disease prevention, which is a current trend in pediatric nursing.

PTS: 1 CON: Health Promotion

14. ANS: 3

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 7

Heading: Trends in Pediatric Nursing Practice > Lesbian, Gay, Bisexual, and Transgender Youth

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Self

Difficulty: Moderate

	Feedback
1	This is incorrect. Giving positive feedback may help at first, but this does not address the isolation statement.
2	This is incorrect. Although it is appropriate to provide gender-affirming care to the patient, it may not address the isolation statement.
3	This is correct. By locating support for both the patient and family, the patient may feel less isolated and develop coping skills from peers.
4	This is incorrect. This may be an appropriate intervention for a general diagnosis; it does not address the isolation statement.

PTS: 1

CON: Self

15. ANS: 1

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 9. Identify evidence-based resources that are available for pediatric nurses.

Page: 8

Heading: Resources for Evidence-Based Pediatric Nursing Practice

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Evidence-Based Practice

Difficulty: Moderate

	Feedback
1	This is correct. This may include evidence-based resources for special instances of Down syndrome.
2	This is incorrect. This is a developmental issue, not a psychiatric diagnosis.
3	This is incorrect. This may provide insight on quality indicators on care; it is not the most appropriate resource for the special needs client.
4	This is incorrect. Although Down syndrome is often diagnosed early in life, this would not cover the issues for the growing child/adult.

PTS: 1

CON: Evidence-Based Practice

MULTIPLE RESPONSE

16. ANS: 1, 2, 5

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 2. Describe the difference between nursing care of infants, children, and adolescents.

Page: 3

Heading: Introduction to Pediatric Nursing > Nursing Care for Children Versus Care for Adults

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1.	This is correct. The pediatric nurse monitors growth and development, including physical maturation.
2.	This is correct. The pediatric nurse monitors growth and development, including physical maturation.
3.	This is incorrect. This is an assessment that is not appropriate for an infant, as the infant would not be able to follow directions for this assessment.
4.	This is incorrect. The Tanner scale is used to assess the onset of puberty. It is not an appropriate assessment for an infant.
5.	This is correct. The pediatric nurse assessment includes listing of required immunizations in preparation for administering the appropriate ones.

PTS: 1

CON: Growth and Development

17. ANS: 2, 3, 5

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 5. Identify models of care applied to pediatric nursing.

Page: 5

Heading: Role of the Pediatric Nurse

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1.	This is incorrect. This is not unique to pediatric nursing; this is a general nursing skill.
2.	This is correct. The family-centered care model of pediatric nursing incorporates the family in the plan of care as the focal point of the child's well-being.
3.	This is correct. The family-centered care model of pediatric nursing incorporates the family in the plan of care as the focal point of the child's well-being.
4.	This is incorrect. This would be a part of adult nursing practice, whereas the pediatric nurse model of care incorporates the family as part of the care of the pediatric nurse.

5	This is correct. The pediatric nurse must be mindful of the growth and development of the pediatric patient.
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PTS: 1 CON: Family Dynamics

18. ANS: 1, 2, 3, 5

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 2. Describe the differences between nursing care of infants, children, adolescents, and adults.

Page: 5

Heading: Trends in Pediatric Nursing Practice > Obesity

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1.	This is correct. There is a trend of bullying on the internet that can influence a child's overall health.
2.	This is correct. Young children are able to use gross motor skills and some reasoning for proper hand washing.
3.	This is correct. This may empower the child to eat healthy items to prevent obesity and the associated disease processes.
4.	This is incorrect. Although there is a trend of a higher need of mental health services, small children may not be able to describe suicidal thoughts.
5	This is correct. This may help facilitate a life skill that they may carry forward to prevent obesity and the disease processes associated with it.

PTS: 1 CON: Health Promotion

19. ANS: 1, 4

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 6

Heading: Trends in Pediatric Nursing Practice > Disease prevention

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Safety; Growth and Development

Difficulty: Moderate

	Feedback
1.	This is correct. Inactivity increases the incidences of obesity.
2.	This is incorrect. Although this is a safety issue, it a behavioral issue, not a disease process.

3.	This is incorrect. Although inactivity may lead to distractive (bored) behavior, it is not a disease process.
4.	This is correct. Along with obesity, inactivity may increase the likelihood of diabetes mellitus.
5.	This is incorrect. There is no correlation to inactivity and earlier onset of puberty.

PTS: 1 CON: Safety | Growth and Development

20. ANS: 1, 2, 5

Chapter: Chapter 1 Issues and Trends in Pediatric Nursing

Chapter Learning Objective: 8. Identify current issues and trends in pediatric nursing practice, education, and research.

Page: 7

Heading: Trends in Pediatric Nursing Practice

Integrated Processes: Communication and Documentation

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1.	This is correct. Trends in pediatric nursing practice have been affected by environmental risk factors.
2.	This is correct. Language barriers directly affect adherence and understanding of the care. When the family-centered care model is used in pediatric nursing, if there is a deficit in communication, care can be negatively impacted.
3.	This is incorrect. There is no correlation stated regarding social media and patient care.
4.	This is incorrect. Although this is a current trend in nursing, it does not affect the rationale for the patient's speech impediment at this time.
5.	This is correct. Deficit knowledge directly affects adherence and understanding of the care.

PTS: 1 CON: Health Promotion

Chapter 2 Standards of Practice and Ethical Considerations

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. Which of the following behaviors serves as an example of the nurse directly upholding the American Nurses Association's (ANA's) *Code of Ethics for Nurses*?
1. The nursing union develops a safe patient-to-nurse ratio.
 2. The nurse joins a professional specialty organization.
 3. The nurse maintains sterile technique during procedures.
 4. The nurse develops a plan of care for a community-wide illness.

- _____ 2. Which of the following behaviors serves as an example of the nurse directly upholding the ANA *Code of Ethics for Nurses*?
1. The nursing union develops a safe patient-to-nurse ratio.
 2. The nurse joins a professional specialty organization.
 3. The nurse maintains sterile technique during procedures.
 4. The nurse develops a plan of care for a community-wide illness.
- _____ 3. The emergency department nurse is providing care to a 5-year-old child with a repeated exacerbation of otitis media. When it becomes apparent that the parents smoke and refuse to prevent the child's exposure to secondhand smoke, the nurse voices frustration to the family that they are causing the child's otitis media. Which of the provisions of the ANA *Code of Ethics for Nurses* has the nurse failed to follow?
1. The nurse promotes, advocates for, and strives to protect the health, safety, and rights of the patient.
 2. The nurse practices with compassion and respect for dignity, unrestricted by the nature of health problems.
 3. The nurse promotes, advocates for, and strives to protect the health, safety, and rights of the patient.
 4. The patient's right to self-determination must be upheld, justified only when justified by law.
- _____ 4. The nurse is providing discharge teaching to the family of a hospitalized 9-year-old patient, recently diagnosed with asthma. The family states that they feel overwhelmed with the treatment plans and cannot perform the necessary nebulizer treatments. Which nursing intervention best reflects the professional performance aspect of pediatric nursing standards?
1. The nurse arranges for a home care respiratory therapy consultation to teach the family proper techniques in the home setting.
 2. The nurse contacts the physician to delay the patient's discharge from the hospital until the family is able to perform treatments.
 3. The nurse collects comprehensive data pertinent to the patient's situation by reviewing the home situation.
 4. The nurse evaluates progress toward the attainment of outcomes by having the family perform return demonstrations.
- _____ 5. The nurse recognizes that the American Nurses Association characterizes which fundamental nursing practice within the *Pediatric Nursing Standards of Practice and Professional Performance for Nurses*?
1. Nursing utilizes evidence-based practice for the rationale of interventions.
 2. Nursing complies with the physician orders to maintain scope of practice.
 3. Nursing maintains the institutional review boards that monitor care.
 4. Nursing facilitates the rules of laws within the health-care continuum.
- _____ 6. The nurse understands that the nursing profession has standards of care that promote the covenant between patient and nurse. Which of the following is an example of this covenant?
1. The nurse adheres to strict guidelines regarding medication administration and procedures.
 2. The nurse develops a care plan with mutually developed goals and interventions with the patient.

3. The nurse attends continuing education courses to maintain competency in skills.
4. The nurse considers each patient's cultural practices without bias or judgment.

_____ 7. The home care nurse is assessing the home environment for the pediatric patient. The nurse notes that the family is unable to perform some of the physical therapy exercises safely. What is the most appropriate initial nursing action?

1. Consult with the physical therapist to arrange more educational sessions.
2. Consult with the health-care provider for admission to a rehabilitation center.
3. Assess the exercises per the worksheet provided by the physical therapist.
4. Assess the family's understanding of a safe of the physical therapy exercises.

_____ 8. The nurse is providing care to an adolescent patient with cystic fibrosis in the acute care setting. The patient tells the nurse, "You are my favorite because you always listen to me, not like the *other* nurse who hates me." Which response by the nurse is most appropriate?

1. "I will report the other nurse immediately, and you won't have to see her again."
2. "If you have any concerns, I can ask the charge nurse to talk to you, but we all work as a team."
3. "That's nice of you to say. I will try hard to keep you happy while here as my patient."
4. "I understand. She has been reported by the other staff members for playing favorites."

_____ 9. The nurse is providing care to a child in the home care setting. The nurse often provides the mother with advice that varies from how to perform the child's medical care to marital advice. The nurse has stated that this is her favorite patient and has made this child's home the last stop of her shift to use her free time for visits. What is the relationship status between the nurse and family?

1. This is a therapeutic relationship based on family-centered care, with a focus on empathy.
2. This is not a therapeutic relationship, as the nurse has become enmeshed within the family.
3. This is a therapeutic relationship, with the nurse focusing on the well-being of the whole family.
4. This is not a therapeutic relationship, as the nurse has not included the whole family in the care.

_____ 10. The nurse is assessing an adolescent patient in the emergency department after she was attacked at a party. The patient refuses to give her parent's contact information and states, "They will kill me when they find out that I went to the party." What is the nurse's best response?

1. "They will understand. Let me call them to at least get the insurance information."
2. "Since you are a minor, we need their consent to legally treat you."
3. "We will treat you, and your parents do not have to know that you are here."
4. "I need to contact the social worker, as we have a duty to report this attack to your family and the authorities."

_____ 11. The nurse is assessing an adolescent patient in the emergency department with a suspected intentional overdose. After given treatment, the patient refuses to give her parent's contact information and states, "It is none of their business." What is the nurse's best response?

1. "Now that you are safe, let me help you develop a plan to prevent a recurrence."

2. "I'm sorry, but I'm obligated to tell them. Let me help you develop a plan for their possible responses."
3. "Although most of your health care is confidential, we are obligated to notify them."
4. "I think you should tell them, but we will honor your confidentiality and not disclose the reason for your visit."

- _____ 12. The nurse is preparing a 13-year-old patient for a full physical examination by the primary care provider (PCP). The patient asks that her mother leave the room because of the sensitive nature of the examination. What is the best course of action to be taken by the nurse to protect the interested parties?
1. Ask the mother to leave the room, and allow the patient to discuss her concerns privately with the PCP.
 2. Allow the mother to stay in the room, and explain that she has the right to witness any care provided by the PCP.
 3. Act as a chaperone and remain with the patient and mother in the room during the full examination.
 4. Reassure the mother that the nurse will remain as a chaperone while she leaves the room.
- _____ 13. The nurse is providing care to the mechanically ventilated 4-year-old patient who is believed to be the victim of abuse from the parents. The parents refuse to permit removal of the ventilator. The nurse believes this is because the parents are afraid that they will be charged with murder if the child dies. What is the recourse for the health-care team in this ethical dilemma?
1. Provide the treatment even in the face of undue burden of resources.
 2. Provide the parents with their financial burden of this care.
 3. Provide information to the courts for legal involvement.
 4. Provide support to the parents even if the team would choose differently.
- _____ 14. The nurse is preparing an 8-year-old patient for a cancer protocol that has a significant amount of risk. The parents have provided informed consent for this trial, but the patient has refused to partake in this research. What is the most appropriate course of action for this situation?
1. Treatment is carried forth, despite the patient's dissent.
 2. Treatment must be delayed until the parents and patient agree.
 3. Treatment is delayed for review by the ethics committee.
 4. Treatment is canceled until the patient assents.
- _____ 15. The nurse is providing care to a hospitalized 1-year-old patient. The nurse locks the exits per protocol to prevent the removal of the child from the unit. The grandparents become angry about this, because they want to take the infant outside for "some fresh air." What is the appropriate measure that the staff take to promote patient safety at this time?
1. Explain that the child is not to leave the floor without express permission from the parents.
 2. Explain to the grandparents that this is a measure to prevent abduction of the patient.
 3. Permit the grandparents to take the child after they sign a waiver for any possible injuries incurred off the unit.

4. Permit the grandparents a limited amount of time off the unit, due to treatment schedules.
- ____ 16. The nurse is providing care to a hospitalized pediatric patient, and the patient is refusing all pain medication despite signs of obvious distress. The patient tells the nurse, "All you nurses lied, and the shots hurt." What is the most appropriate nursing measure at this time?
1. Provide the medication in a cup of juice.
 2. Have the parents administer the medication.
 3. Explain not all medications are shots.
 4. Admit to the patient that the shots hurt but explain they help.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- ____ 17. Which of the following nursing actions is an example of a function of the *Code of Ethics for Nurses* directly relating to the health-care environment? *Select all that apply.*
1. The nurse cowrote a policy on nurse:patient ratio for the unit.
 2. The nurse includes the extended family in discharge instructions.
 3. The nurse joined a local professional organization that shapes community policy.
 4. The nurse enrolled in a basic American Sign Language course.
 5. The nurse joins the safety and infection control committees.
- ____ 18. Which of the following are examples of a nurse adhering to the pediatric standards of practice? *Select all that apply.*
1. The nurse educates the parents on how to administer insulin to their 9-year-old child.
 2. The nurse asks the 3-year-old child to describe their level of pain with a Faces or Oucher scale.
 3. The nurse refuses to divulge medical information to the parents of a pregnant adolescent.
 4. The nurse provides extra free supplies to the family of an underinsured ill child.
 5. The nurse uses evidence-based practice to maintain competency of new practices.
- ____ 19. The nurse is providing care to a toddler recently diagnosed with asthma. The parents have become verbally aggressive with staff and have challenged many of the unit rules. Which of the following may have contributed to the family's challenging behavior? *Select all that apply.*
1. The parents are both working professionals in leadership roles and fear loss of control.
 2. The family has researched asthma on the Internet and does not agree with the treatment protocol.
 3. The staff members have maintained therapeutic communication with the patient and family.
 4. The nurse has developed a mutual set of goals and interventions with the family.
 5. The staff members have provided a medically trained translator to promote understanding in care.

- _____ 20. Which of the following interventions are examples of a family-focused strategy? *Select all that apply.*
1. Coordinating care with other departments to lessen sleep disruptions
 2. Providing storage and prep of meals from home for family members
 3. Stipulating unilateral visiting rules for all patients and their families
 4. Involving the parents in nursing change of shift reports and care plans
 5. Providing dedicated family space with basic needs for rooming in parents

Chapter 2 Standards of Practice and Ethical Considerations

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 1. Identify and describe sources of standards of practice relevant to the day-to-day practice of pediatric nurses

Page: 13

Heading: Box 2-1: The Nine Provisions of the ANA *Code of Ethics for Nurses*

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment

Cognitive Level: Analysis [Analyzing]

Concept: Ethics

Difficulty: Moderate

	Feedback
1	This is correct. Provisions 5 and 6 of the ANA code describe how the nurse establishes and improves health-care environments and conditions of employment conducive to quality of health care.
2	This is incorrect. Although this may facilitate duties to self and others, it is not a direct effect from the <i>Code of Ethics for Nurses</i> .
3	This is incorrect. This is a fundamental nursing skill. It is not a direct effect from the <i>Code of Ethics for Nurses</i> .
4	This is incorrect. Although the nurse practice involves developing plans of care, it may not involve a <i>Code of Ethics for Nurses</i> stance.

PTS: 1

CON: Ethics

2. ANS: 1

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 1. Identify and describe sources of standards of practice relevant to the day-to-day practice of pediatric nurses

Page: 13

Heading: Box 2-1: The Nine Provisions of the ANA *Code of Ethics for Nurses*

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment

Cognitive Level: Analysis [Analyzing]

Concept: Ethics

Difficulty: Moderate

	Feedback
1	This is correct. Provisions 5 and 6 of the ANA code describe how the nurse establishes and improves health-care environments and conditions of employment conducive to quality of health care.

2	This is incorrect. Although this may facilitate duties to self and others, it is not a direct effect from the <i>Code of Ethics for Nurses</i> .
3	This is incorrect. This is a fundamental nursing skill. It is not a direct effect from the <i>Code of Ethics for Nurses</i> .
4	This is incorrect. Although the nurse practice involves developing plans of care, it may not involve a <i>Code of Ethics for Nurses</i> stance.

PTS: 1 CON: Ethics

3. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 5. Identify the nine provisions of the *Code of Ethics for Nurses* and relate them to practical situations encountered in the day-to-day practice of pediatric nursing.

Page: 13

Heading: Box 2-1: The Nine Provisions of the ANA *Code of Ethics for Nurses*

Integrated Processes: Communication and Documentation

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is incorrect. Although the nurse realizes that the secondhand smoke may be a contributing factor to the child's otitis media, the nurse did not fail to comply to this code of ethics at this time.
2	This is correct. The nurse failed to practice with compassion and respect for the dignity of the patient (and family).
3	This is incorrect. The nurse attempted to maintain the overall health of the patient (and family) regarding the smoking.
4	This is incorrect. This is not an issue at the time, as this is a minor child.

PTS: 1 CON: Family Dynamics

4. ANS: 1

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 1. Identify and describe standards of practice relevant to the day-to-day practice of pediatric nurses.

Page: 14

Heading: Box 2-2: Pediatric Nursing Standards: Professional Performance

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Ethics

Difficulty: Moderate

	Feedback
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1	This is correct. This would involve communication with the family regarding their concerns. The nurse will collaborate with the respiratory therapist to arrange a home visit for further teaching. The nurse considers the overall safety of the patient and family by using available resources to deliver home care. The nurse is being an active advocate for the patient and family by providing more resources and coordinating help at home.
2	This is incorrect. Although the nurse is being an advocate, there are no data to support delaying the child's discharge (standard #17).
3	This is incorrect. This is a component of practice and should be ongoing. It may facilitate better understanding of the family's fears or concerns, but it is not defined as a professional performance component of the standards (standard #1).
4	This is incorrect. This is a component of practice and should be ongoing. It may facilitate better understanding of the family's fears or concerns but is not defined as a professional performance component of the standards (standard #6).

PTS: 1 CON: Ethics

5. ANS: 1

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 3. List the six standards of practice and 11 standards of professional performance highlighted in *Pediatric Nursing: Scope and Standards of Practice* and discuss associated measurement criteria.

Page: 14

Heading: Box 2-2: Pediatric Nursing Standards of Practice and Professional Performance

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Ethics

Difficulty: Difficult

	Feedback
1	This is correct. This is an example of the recognition of the <i>Pediatric Nursing Standards of Practice and Professional Performance</i> as a living document that changes with scientific knowledge. Standard 5 states, "The nurse owes the same duties to self as to others, including the responsibility to preserve integrity and safety, to maintain competence, and to continue person and professional growth." Additionally, standard 7 states, "The nurse participates in the advancement of the profession through contributions to practice, education, administration, and knowledge development."
2	This is incorrect. The nurse will comply with physician orders as an advocate for both the patient and nursing profession. This is not based on the <i>Pediatric Nursing Standards of Practice and Professional Performance</i> .
3	This is incorrect. The institutional review board is a source of standards that monitors and regulates research. However, this is not directly based on the <i>Pediatric Nursing Standards of Practice and Professional Performance</i> .
4	This is incorrect. This relates to nursing standards and scope of practice, but it is not the <i>Pediatric Nursing Standards of Practice and Professional Performance</i> .

PTS: 1 CON: Ethics

6. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 2. Identify and elaborate on the key themes related to pediatric nursing standards of practice.

Page: 14

Heading: Pediatric Nursing Scope and Standards of Practice > Role of Therapeutic Relationships

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Ethics

Difficulty: Moderate

	Feedback
1	This is incorrect. This is safe nursing practice and may promote trust between the nurse and patient, but it does not directly promote the covenant.
2	This is correct. This develops a level of trust and empathy between the patient and nurse. Key word: mutual goal and interventions.
3	This is incorrect. although this may increase the nurse's competence it does not directly affect the trust between the patient and nurse.
4	This is incorrect. Although this may facilitate respect from the nurse, it may not promote trust between the nurse and patient.

PTS: 1 CON: Ethics

7. ANS: 4

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 14

Heading: Box 2-2: Pediatric Nursing Standards of Practice and Professional Performance

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Coordinated Care

Cognitive Level: Analysis [Analyzing]

Concept: Collaboration; Culture

Difficulty: Moderate

	Feedback
1	This is incorrect. This intervention may be appropriate if the patient's family needed more education and therapy sessions, but it is not appropriate in this situation.
2	This is incorrect. Although this may be an appropriate intervention if the patient's family is unable to perform the home exercise program as set by the therapist, it is not an appropriate initial nursing intervention in this situation.
3	This is incorrect. Although it would be an appropriate intervention for further assessment, this is not appropriate in this situation.

4	This is correct. The appropriate initial nursing intervention is to assess the family's understanding of the problem.
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PTS: 1 CON: Collaboration | Culture

8. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 2. Identify and elaborate on the key themes related to pediatric nursing standards of practice.

Page: 15

Heading: Safe and Effective Nursing Care: Clinical Pearl

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. This is unprofessional and facilitates splitting of the staff. The patient is chronically ill and may have repeated hospital admissions.
2	This is correct. Acknowledgment of the patient's statement and referring the matter to a neutral party may prevent splitting of the staff members. This can avert adversarial responses from the patient.
3	This is incorrect. This is not therapeutic and strengthens the patient's splitting behavior.
4	This is incorrect. This is not therapeutic, strengthens the patient's splitting behavior, and is not proper to discuss other staff members to patients.

PTS: 1 CON: Patient-Centered Care

9. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 16

Heading: Safe and Effective Nursing Care: Clinical Pearl

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Ethics

Difficulty: Moderate

	Feedback
1	This is incorrect. There may be some empathy, but the nurse has become enmeshed with the care and is neither goal directed nor family focused.
2	This is correct. This nurse has become enmeshed with the family without regard for professional boundaries.

3	This is incorrect. Although the pediatric nursing model is family centered, this appears more nurse centered than family centered.
4	This is incorrect. Although this is not a therapeutic relationship, the rationale is not correct.

PTS: 1 CON: Ethics

10. ANS: 4

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 20

Heading: Developmentally Appropriate Care > Child Development, Privacy, and Confidentiality > Practical Limitations and Legal Exceptions Regarding Confidentiality

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Safety; Legal

Difficulty: Moderate

	Feedback
1	This is incorrect. This is not therapeutic communication, and it negates the patient's concerns.
2	This is incorrect. The patient may receive emergency treatment without parental consent.
3	This is incorrect. The parents may receive a bill from the emergency department for the visit. It may not provide information on why the patient was in the emergency department.
4	This is correct. The nurse has an obligation as a mandated reporter to notify the appropriate authorities. Often the social worker will fill a report. The nurse must verify that the patient's record contains documentation of this report.

PTS: 1 CON: Safety | Legal

11. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 20

Heading: Developmentally Appropriate Care > Child Development, Privacy, and Confidentiality > Practical Limitations and Legal Exceptions Regarding Confidentiality

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Communication; Legal

Difficulty: Moderate

	Feedback
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1	This is incorrect. This does not address the fact that the health-care provider has a mandated obligation to report self-harm to the authorities or parents.
2	This is correct. When health-care professionals have a reasonable concern that a patient may harm himself or herself, they are required by law to break confidentiality. The child should be given the option to participate in telling the parents, but ultimately the health-care provider is mandated to report the patient's self-harm.
3	This is incorrect. Although the health-care provider is mandated to report self-harm, the best option would be to prepare the patient for the fallout of the disclosure.
4	This is incorrect. This does not address the health-care provider's mandated obligation to report self-harm.

PTS: 1 CON: Communication | Legal

12. ANS: 4

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 20

Heading: Developmentally Appropriate Care > Child Development, Privacy, and Confidentiality > Use of Chaperones

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Due to the sensitive nature of the examination, there should be a qualified chaperone to protect both the patient and practitioner.
2	This is incorrect. Due to the sensitive nature of the examination, the child has the right to privacy.
3	This is incorrect. The patient may be hesitant to discuss sensitive information with the mother present, and the patient has requested privacy at the beginning of the examination.
4	This is correct. Due to the sensitive nature of the examination, there should be a qualified chaperone to protect both the patient and practitioner.

PTS: 1 CON: Patient-Centered Care

13. ANS: 3

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 21

Heading: Pediatric Decision Making > Best Interest: Who Gets to Decide?

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Ethics

Difficulty: Moderate

	Feedback
1	This is incorrect. This would negate the best intent of the child. The parents are making decisions based on the interests of themselves rather than the child and there is an apparent conflict of interest.
2	This is incorrect. This course of action is not in the best intent of the child. When parents are making health-care decisions based on the interests of themselves rather than the child or have a conflict of interest, their parental decision-making rights may be challenged legally.
3	This is correct. The parents are putting their self-interests ahead of the interests of the child and there is a clear conflict of interest. In this case, the parent's decision-making rights may be challenged legally.
4	This is incorrect. This would negate the best intent of the child. The parents are making decisions based on the interests of themselves rather than the child and there is an apparent conflict of interest.

PTS: 1

CON: Ethics

14. ANS: 4

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 7. Differentiate consent, permission, and assent, and discuss how promoting the best interests of children in the issue of consent differs from obtaining informed consent from a competent adult.

Page: 23

Heading: Critical Component: Consent, Assent, and Permission

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment, Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Ethics; Legal

Difficulty: Moderate

	Feedback
1	This is correct. Although the patient has refused, the parents have provided permission for the care of the patient.
2	This is incorrect. The treatment may be delayed for further discussion, but ultimately, the parents have given permission to proceed.
3	This is incorrect. Unless the parents do not agree, there is no call for an ethics board review at this time.
4	This is incorrect. Although the patient has refused to assent, the parents have provided informed consent to the treatment.

PTS: 1

CON: Ethics | Legal

15. ANS: 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 1. Identify and describe sources of standards of practice relevant to the day-to-day practice of pediatric nurses.

Page: 25

Heading: Safety in the Pediatric Setting > National Patient Safety Goals

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. This is a safety measure to prevent abduction of the patients by noncustodians.
2	This is correct. This is a standard safety measure to prevent abduction by noncustodians.
3	This is incorrect. Unless the parents have given express permission, the noncustodial grandparents may not take the child off the unit.
4	This is incorrect. Unless the parents have given express permission, the noncustodial grandparents may not take the child off the unit.

PTS: 1

CON: Safety

16. ANS: 4

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing and discuss relevant principles, duties, rights, and virtues.

Page: 27

Heading: Truth-Telling Standards and Pediatric Nursing

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. The key terms are truth and veracity. This would essentially constitute a lie by masking the medication from the child.
2	This is incorrect. This would provide a chance for the child to distrust the parents.
3	This is incorrect. Although this would help with the administration of a pain medication, it does not address the issue of veracity or truth telling to the patient.
4	This is correct. Telling the truth may help rebuild the lost trust. "In general, children prefer the truth and may develop distrust for health-care providers when just one health care provider deceives them."

PTS: 1

CON: Safety

MULTIPLE RESPONSE

17. ANS: 1, 3, 5

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 5. Identify the nine provisions of *Code of Ethics for Nurses* and relate them to practical situations encountered on a day-to-day practice of pediatric nursing.

Page: 13

Heading: Box 2-1: The Nine Provisions of the ANA *Code of Ethics for Nurses*

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1.	This is correct. This is provision #6, which states the nurse participates in improving health-care environments and conditions of employment.
2.	This is incorrect. Although this is an example of family-centered care, it is not a component of the <i>Code of Ethics for Nurses</i> .
3.	This is correct. This could indirectly influence the workplace environment by shaping social policy. This relates to provision #9.
4.	This is incorrect. Although this would facilitate communication and cultural sensitivity, it does not directly influence the workplace environment.
5.	This is correct. This relates to provision #8, which states that the nurse will collaborate with other health professionals to meet health needs of the facility and community. An example is how the two patient-identifier practice was started by a group of concerned nurses.

PTS: 1

CON: Patient-Centered Care

18. ANS: 1, 2, 3, 5

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 3. List the 6 standards of practice and 11 standards of professional performance highlighted in *Pediatric Nursing: Scope and Standards of Practice* and discuss associated measurement criteria.

Page: 14

Heading: Box 2-2: Pediatric Nursing Standards of Practice and Professional Performance

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Safety; Ethics

Difficulty: Moderate

	Feedback
1.	This is correct. Teaching parents how to administer insulin relates to advocacy and empowerment for the patient and family.
2.	This is correct. Using these established tools to assess a patient's pain level is an example of effective communication.
3.	This is correct. This action ensures the privacy of the patient. Although the parents may be financially responsible for the adolescent, they are not entitled to her medical information without her express permission.
4.	This is incorrect. The criteria for analysis of this situation would involve the just and ethical distribution of resources. Would the facility approve of distribution of these resources, and would the nurse provide the same to all of the patients?
5	This is correct. Per the ANA and pediatric standards of care, the nurse must maintain competency in current standards of care.

PTS: 1 CON: Safety | Ethics

19. ANS: 1, 2

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 6. Identify ethics controversies commonly encountered in the practice of pediatric nursing, and discuss relevant principles, duties, rights, and virtues.

Page: 16

Heading: Box 2-3: Potential Contributing Factors to Challenging Family Behaviors

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Family Dynamics; Communication

Difficulty: Moderate

	Feedback
1.	This is correct. Family members who have a background in leadership may fear loss of control over their child and lash out at the staff.
2.	This is correct. If the family is in possession of inadequate information or misinformation, they may lash out or challenge the treatment protocol.
3.	This is incorrect. Maintaining therapeutic communication may facilitate a covenant between the patient/family and nurse/staff.
4.	This is incorrect. This would provide a therapeutic relationship between the nurse and patient/family.
5	This is incorrect. Providing a medically trained translator facilitates better communication between the staff and patient, which may promote better understanding and reliable information.

PTS: 1 CON: Family Dynamics | Communication

20. ANS: 2, 4, 5

Chapter: Chapter 2 Standards of Practice and Ethical Considerations

Chapter Learning Objective: 1. Identify and describe sources of standards of practice relevant to the day-to-day practice of pediatric nurses; 5. identify the nine provisions of the *Code of Ethics for Nurses* and relate them to practical situations encountered in the day-to-day practice of pediatric nursing.

Page: 16

Heading: Family-Centered Care > Overview of Family-Centered and Family-Focused Strategies

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1.	This is incorrect. Although this would facilitate rest for the patient, it is not family focused.
2.	This is correct. Meal storage and prep for meals at home will help decrease the discomfort for the family members of the patient. This is a culturally sensitive intervention as well.
3.	This is incorrect. Although this would be fair for all, it is not a family-focused set of rules.
4.	This is correct. This is family focused, because it involves the parents in the care of the child.
5	This is correct. Providing a dedicated family space for rooming-in parents will ease the discomfort incurred from the hospital stay. There are “Ronald McDonald Family Rooms” in many facilities.

PTS: 1

CON: Family Dynamics

Chapter 3 Family Dynamics and Communicating With Children and Families

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is assessing an infant during a well-baby visit in the family practice clinic. During this assessment, the nurse asks the mother several routine questions regarding the infant’s progress and notes that the mother nods her head but is looking at her cell phone. Which response by the nurse is most appropriate in this situation?
1. Continue the assessment and note the mother’s disinterest in the infant’s chart.
 2. Continue to ask the questions repeatedly until an acceptable answer is provided.
 3. Ask the mother if she could please put the phone down and discuss her infant’s progress.
 4. Contact the facility’s social worker for a referral to child protective services for an in-home visit.
- _____ 2. The nurse is assessing a pediatric patient in the emergency room for injuries sustained from a fall. The nurse asks the parents how the child was injured. Which response by the parent uses the most productive communication pattern?

1. "My wife forgot to check the baby gate like I had asked her to."
2. "I am the only one who checks the baby gate all the time."
3. "Typical man; he just doesn't follow directions."
4. "I forgot to check the baby gate when I came home."

_____ 3. The nurse assesses a toddler with an apparent ear infection in the emergency department. During the assessment, the nurse finds that the child is not up to date on immunizations or well-baby checkups. Upon further investigation, the nurse discovers that the family is uninsured and undocumented immigrants. What are the most likely barriers to effective communication at this time?

1. The insurance status affects many health-care practices such as immunizations.
2. The possible language issue may affect the family's comprehension of instructions.
3. The family may have barriers to transportation, employment, or knowledge.
4. The family may not have a primary care provider and relies upon emergency

_____ 4. The nurse is attempting to provide education to the family of a newborn in the home-care setting. Which nursing intervention will assess comprehension of the communication with the family?

1. The nurse identifies the primary caregiver of the infant for instructions and asks for privacy with the individual.
2. The nurse provides both written and verbal instructions at an age-appropriate level and asks for a return demonstration.
3. The nurse provides descriptions of signs and symptoms to report to the primary care provider.
4. The nurse requests to see their calendar with approximate dates for immunizations.

_____ 5. The emergency department nurse is providing care to a 3-year-old child who has suffered a major accident. The parents ask the nurse, "Is our child going to be okay?" What is the most effective communication response at this time?

1. "Your child has suffered a severe laceration to the spinal cord and may have terminal paralysis."
2. "Your child is being assessed at this time. I am confident the physician will resolve the problem."
3. "Your child has suffered a major injury; let me give you some privacy. May I call the chaplain just to be with you?"
4. "Your child has suffered a major injury. Please wait here while we finish assessing the injuries."

_____ 6. The nurse is assessing the psychosocial needs of a family that has a chronically ill child. During the interview process, the nurse notes that the grandmother answers for all the family members. What nursing intervention is most appropriate at this time?

1. Ask the grandmother if a medical translator is necessary.
2. Continue with the interview and document the findings.
3. Identify the grandmother as the primary source of information.
4. Ask the grandmother to leave the room for privacy.

_____ 7. The nurse is assessing a newborn who has been brought to the family clinic for her first well-child visit. The nurse accidentally drops a metal pan on the floor near the child. The child is startled, but the mother does not react. What is the most appropriate intervention at this time?

1. Speak loudly and clearly to mother to enhance comprehension.
2. Read the infant's chart to verify how to communicate.
3. Continue with the assessment and document the findings.
4. Ask the mother if she needs an interpreter to communicate.

_____ 8. In an acute care setting, the pediatric nurse is providing care to a 4-year-old patient with an exacerbation of cystic fibrosis. The nurse notes that there is little interaction between the patient and the older siblings when they visit. The siblings often seek to divert the staff's attention during their visits. What is the most appropriate nursing intervention at this time?

1. Ask the parents to leave the siblings at home to avoid disruptions.
2. Interact with the siblings and show them the playroom during visits.
3. Encourage the siblings to visit and bring familiar objects to the hospital.
4. Develop a plan to include the siblings in age-appropriate care of the patient.

_____ 9. The nurse is providing care to a 17-year-old patient in the family practice clinic. The patient reports concern for the parents, as she is the last one of the four children to leave the home. She is concerned that her parents may "fall apart" when she leaves for college in the fall. What is the best response from the nurse based on the family systems theory?

1. "Although changes that occur in one family member's life affects the entire family, keep in mind that your family is dynamic."
2. "Each member of the family has to differentiate themselves, whether good or bad, to influence family relationships."
3. "Although this may put a strain on their relationship, a healthy family recognizes the need love and growth."
4. "Although the family is growing apart, this is a time for your parents to refocus on extended family relationships."

_____ 10. The nurse is assessing preschool twins during a well-child visit in the family clinic. Which of the following statements by the parents would indicate a healthy transition for the family per Duvall's family development theory?

1. The parents state that they have maintained scheduled play dates to help the children socialize.
2. The parents have maintained the children's well-child examinations and immunizations.
3. The parents have compiled a list of goals and request suggested interventions.
4. The parents and children appear to interact well and display nurturing and affection.

_____ 11. The nurse is providing care to a pediatric patient in the mental health facility. The nurse asks the patient to draw a picture of her family. What is the function of this activity?

1. This drawing depicts the child's perspective of the family unit.
2. This drawing provides an outlet from the therapy sessions.
3. This activity is a tool to facilitate communication via play.
4. This activity develops problem-solving skills for the child.

- _____ 12. The nurse is providing care to a 4-year-old child who is recovering from an orthopedic injury in the hospital. The nurse notes there are two siblings who are ages 6 years and 1 year. The father is the primary caregiver, and the mother is the primary wage earner. During the assessment, the nurse notes that the 6-year-old seems to be the primary nurturer of the 1-year-old, while the father tends to the 4-year-old, and the mother talks on her cell phone. Which nursing diagnosis is appropriate for this family?
1. Compromised family coping r/t insufficient reciprocal support between the parents as evidenced by family behavior during hospital visits.
 2. Risk for delayed development r/t lack of parental involvement as evidenced by family behavior, mother and siblings not interacting.
 3. Ineffective family health management r/t family conflict as evidenced by mother's inability to interact with the child during hospitalization.
 4. Interrupted family processes r/t hospitalized child as evidenced by lack of full family interactions during hospitalization.
- _____ 13. The nurse is providing care to a 5-year-old child who is recovering from a tonsillectomy in the hospital. The nurse notes that the 9-year-old sibling is tending to the psychosocial needs of the patient, while the parents discuss their marital conflicts in front of the children. Which nursing diagnosis is appropriate for this family?
1. Dysfunctional family processes
 2. Risk for parent-child attachment
 3. Spiritual distress
 4. Risk for impaired parenting
- _____ 14. The nurse is assessing the family of a pediatric patient in the home-care setting. Which of the following family APGAR findings best correlates with the nursing diagnosis of caregiver role strain?
1. They have recently moved to the area and have not been able to form relationships within the community.
 2. They show affection to each other and are able to devote time to other family members.
 3. They have formed a partnership to carry forth the usual household responsibilities and support each other.
 4. The mother provides primary care of the ill child and household while the father works two jobs to provide financial support.
- _____ 15. The nurse is performing a functional family assessment of a chronically ill pediatric patient. In what way can the nurse enhance communication with the family?
1. Identify the leader of the family and provide options for respite care.
 2. Assess for abuse and report suspected abuse to the authorities.
 3. Complete a family APGAR questionnaire and share the results.
 4. Educate the family on resources in the community to help with care.
- _____ 16. The nurse is providing care to a 10-year-old patient hospitalized for multiple injuries from a motor vehicle crash. Which nursing intervention is appropriate for communication with this child?
1. Include the child in some decisions regarding care, such as choosing food from the hospital menu.
 2. Offer the child a chance to play with the medical instruments when possible.

3. Communicate primarily to the parents regarding treatments and the child's progress.
4. Recognize that behavior may fluctuate between adult and childlike during hospitalization.

- _____ 17. The nurse is providing discharge instructions to the family of a 14-year-old patient who was treated for an exacerbation of a chronic illness. The nurse recognizes that this will add stress to the family unit and has developed a plan of care based on the family-centered care model. Which intervention best addresses the crisis experienced by the family unit at this time?
1. Encourage the patient to contact friends from school for visits.
 2. Discuss treatment options and alternatives with the family.
 3. Provide privacy to discuss their options independently of staff.
 4. Request clergy to counsel the family prior to discharge.
- _____ 18. A nurse is providing care to an adolescent patient. Which statement by the nurse is most likely to enhance communication with the patient?
1. "Are you participating in any risky behaviors?"
 2. "What concerns do you have about your health?"
 3. "Are you taking your medication as prescribed?"
 4. "Is there a reason why you waited so long to come in?"
- _____ 19. The nurse is providing care to a 6-year-old child who is recovering from an infection after a round of chemotherapy in the hospital. The parents appear overwhelmed with the situation of wanting "quality time" and voice guilt over missing the healthy children's school functions when at the hospital. Which is an appropriate nursing goal for this family?
1. The family will demonstrate knowledge of the regimen and prevent further infections.
 2. The family will meet and seek counseling for each family member prior to the patient's discharge to home.
 3. The family will acknowledge the range of emotions during the child's hospitalization.
 4. The family will meet with the social worker to coordinate community resources to facilitate the transition to home.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 20. The nurse is providing care to an 11-month-old child who is hospitalized with pneumonia. Which nursing interventions are appropriate for this child? *Select all that apply.*
1. Communicate primarily with parents regarding treatment.
 2. Allow the child to tour the facility before treatments.
 3. Use simple terminology when talking to the child.
 4. Use a singsong wide-eyed approach to interact.
 5. Promote a sense of security with use of gentle touch.

Chapter 3 Family Dynamics and Communicating With Children and Families

Answer Section

MULTIPLE CHOICE

1. ANS: 3

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 8. Explain various influences in communication, including body language, tone, pitch, and environment.

Page: 35

Heading: Communication and Families > Principles of Communication in Families

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. The mother is the primary source of information for the infant.
2	This is incorrect. This would not be a favorable response, and it will take essential time away from the care of the infant.
3	This is correct. Attempting to interact with the mother is imperative, as she is the primary source of information.
4	This is incorrect. There is no cause to contact child protective services in this scenario.

PTS: 1

CON: Communication

2. ANS: 4

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 2. Identify age-specific approaches for communicating with parents, families, and children.

Page: 36

Heading: Communication and Families > Patterns of Family Communication

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. This is a masked and direct message aimed at the wife, and it does not answer the question.
2	This is incorrect. This is a masked and indirect message that is aimed at the household members, and it does not answer the question.

3	This is incorrect. This is a generalized statement and does not answer the question.
4	This is correct. This is a direct and clear message stating that the baby gate was not latched without displacing the blame.

PTS: 1 CON: Communication

3. ANS: 2

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 1. Describe the process of normal communication; 8. explain various influences in communicating with parents, families, toddlers, school-aged children, and adolescents.

Page: 37

Heading: Communication and Families > Communicating With Families: Nurse's Role

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. Although this may be an issue with access to adequate care, it is not a communication issue.
2	This is correct. There may be a language issue, which can affect comprehension of the instructions. A medical interpreter may be required.
3	This is incorrect. Although this may be an issue with access to adequate care, it is not a communication issue.
4	This is incorrect. Although this may be an issue with access to adequate care, it is not a communication issue.

PTS: 1 CON: Health Promotion

4. ANS: 2

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 7. Identify age-specific approaches for communicating with parents, families, and children.

Page: 37

Heading: Communication and Families > Communicating With Families: Nurse's Role

Integrated Processes: Communication and Documentation

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
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1	This is incorrect. Identifying the primary caregiver is important, as this person is the receiver of vital information. However, this alone does not ensure that information has been received or understood.
2	This is correct. If the caregivers are given both written and verbal instructions, it may enhance their understanding. If the caregiver is able to provide a return demonstration, it shows some comprehension of the task.
3	This is incorrect. This is only one aspect of the education, and it does not describe how the information is provided.
4	This is incorrect. A calendar may be a vital tool as a reminder for the immunizations, but it does not provide insight on whether the information has been understood.

PTS: 1 CON: Communication

5. ANS: 3

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 10. Describe communication with families during periods of emergency care.

Page: 37

Heading: Communication and Families > Communicating With Families: Nurse's Role

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. The use of medical jargon prohibits effective communication, especially with the stress of an emergency.
2	This is incorrect. This gives false hope to the family.
3	This is correct. Give the family time and provide support during this time, from either clergy or social workers.
4	This is incorrect. This does not allow therapeutic or effective communication. Effective communication techniques include providing a quiet environment for conversation and allowing time for questions.

PTS: 1 CON: Communication

6. ANS: 3

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 3. Describe family dynamics.

Page: 37

Heading: Communication and Families > Communicating With Families: Nurse's Role

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Culture

Difficulty: Moderate

	Feedback
1	This is incorrect. This is not an appropriate intervention, as there is no indication that there is a language barrier.
2	This is incorrect. It is important to identify the roles within the family, and the nurse should ask for clarification of the family roles if it is unclear. Maintaining communication with families and caregivers has become more challenging with the increased number of divorced parents, blended or reconstituted families, and grandparents as primary caregivers.
3	This is correct. It is important to identify the roles within the family to facilitate a healthy relationship and dynamics.
4	This is incorrect. This would prevent a healthy family relationship, as it appears that this is a matriarchal family unit, with the grandmother as the primary source of information. It is important to identify the roles within the family. Maintaining communications with families and caregivers has become more challenging with the increased number of divorced parents, blended or reconstituted families, and grandparents as primary caregivers.

PTS: 1 CON: Culture

7. ANS: 4

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 8. Explain various influences in communication, including body language, tone, pitch, and environment.

Page: 38

Heading: Safe and Effective Care: Clinical Pearl: Protection for Individuals With Hearing Disabilities

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. If the mother has a hearing deficit, speaking loudly may not help with comprehension or communication.
2	This is incorrect. This will not enhance communication with the mother regarding care of the infant.
3	This is incorrect. The mother plays an important part in the care of the infant and will need proper communication to verify information.
4	This is correct. According to the Americans with Disabilities Act (ADA), health-care providers have a duty to ensure that communication is effective.

PTS: 1 CON: Communication

8. ANS: 3

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 3. Describe family dynamics; 11. identify the role of family-centered care in caring for the hospitalized child.

Page: 42

Heading: Definition of a Family > Stressors on Families > Nursing Interventions to Assist Families

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is incorrect. This would disrupt the family-centered care and may increase the sibling rivalry between the ill and healthy children.
2	This is incorrect. This would distract the staff from performing their duties, and it would distract from the necessary family-centered care.
3	This is correct. Encouraging the siblings to bring a familiar object with them when they visit can increase their comfort level and encourage family-centered care.
4	This is incorrect. The sibling may not be comfortable with performing care of their sibling.

PTS: 1 CON: Family Dynamics

9. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 4. Describe family theories.

Page: 43

Heading: Family Theory > Family Systems Theory

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is correct. This is an example of how changes in one family member's life affect the entire family and that the family is dynamic.
2	This is incorrect. This response reflects Bowen's family system theory.
3	This is incorrect. This response does not exemplify any of the family theories, and it is a nontherapeutic answer to the patient's concerns.
4	This is incorrect. This response exemplifies Duvall's family development theory.

PTS: 1 CON: Family Dynamics

10. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 4. Describe family theories.

Page: 44

Heading: Family Theory > Duvall and Family Development Theory

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is correct. This is an example of Duvall's family development theory during the preschool stage. The family is preparing the children for entry into school by socializing them.
2	This is incorrect. This is an example of Neuman's system theory, where the family performs primary prevention to alleviate possible risk factors.
3	This is incorrect. This is an example of King's theory of goal attainment. The nurse and family work together via the nursing process to assist the child and family in achieving their goals through action, reaction, and interaction.
4	This is incorrect. This is an example of Roy adaptation model. This stage may be the interdependence mode, which is associated with relationships and interactions with others.

PTS: 1 CON: Family Dynamics

11. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families
Chapter Learning Objective: 5. Identify family function roles.

Page: 46

Heading: Family Assessment > Tools for Assessment > Kinetic Family Drawing

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Culture

Difficulty: Moderate

	Feedback
1	This is correct. This provides an opportunity for others to view the child's perspective of the family and can reflect the family's health and areas of distress.
2	This is incorrect. Although it may inadvertently provide a distraction, it is a tool to provide an opportunity to view the child's perspective.
3	This is incorrect. Although play is important for the child, it is not the purpose of this activity.
4	This is incorrect. Although it may help the child share their perspective of the family, it would need further discussion with the child and family regarding the functions and roles.

PTS: 1 CON: Culture

12. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 11. Identify the role of family-centered care in caring for the hospitalized child.

Page: 47

Heading: Box 3-1: Examples of Family Nursing Diagnoses

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is correct. The lack of interaction may be the family dynamics, or it can be the result of the crisis of hospitalization.
2	This is incorrect. This may be a reaction to the crisis to the child's hospitalization; the mother may be distracted due to work constraints.
3	This is incorrect. This may be a reaction to the crisis to the child's hospitalization; the mother may be distracted due to work constraints.
4	This is incorrect. This may be a reaction to the crisis to the child's hospitalization, where there are subsets of relationships with the older and younger sibling. The mother may be distracted due to work constraints.

PTS: 1 CON: Family Dynamics

13. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 3. Describe family dynamics.

Page: 47

Heading: Box 3-1: Examples of Family Nursing Diagnoses

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is correct. The marital conflicts can disrupt the family process.
2	This is incorrect. Although this may occur, if the dysfunctional family process were resolved, the risk may be minimal.
3	This is incorrect. Although this may occur, if the dysfunctional family process were resolved, the risk may be minimal.
4	This is incorrect. Although this may occur, if the dysfunctional family process were resolved, the risk may be minimal.

PTS: 1 CON: Family Dynamics

14. ANS: 4

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 5. Identify family function roles.

Page: 47

Heading: Box 3-1: Examples of Family Nursing Diagnoses

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. This may lead to caregiver role strain, but there is no indication of one member performing all of the care.
2	This is incorrect. This is a sign of a healthy relationship, where the family is able to show care toward each other.
3	This is incorrect. This is a sign of a healthy relationship, where the responsibilities are shared.
4	This is correct. It appears that both parents are strained with their responsibilities, the mother with primary care of the child, and the father's employment status.

PTS: 1 CON: Communication

15. ANS: 3

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 9. Describe strategies for incorporating communication into assessment.

Page: 47

Heading: Family Assessment > Tools for Assessment > Family APGAR Five-Item Questionnaire

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. Although it is important to locate the leader of the family, it is not necessary to locate respite care immediately.
2	This is incorrect. Although the nurse is legally obligated to report abuse, this is a stereotype of families with chronically ill children.
3	This is correct. Assessing the family regarding the family dynamics and ability to cope with crises enhances communication with the family.
4	This is incorrect. This will not enhance the communication with the family. This would be performed once communication is accomplished.

PTS: 1 CON: Communication

16. ANS: 1

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 7. Identify age-specific approaches for communicating with parents, families, and children.

Page: 51

Heading: Communicating With the Family > Communicating With Children > Communicating With School-Age Children > Nursing Interventions for Communicating With School-Age Children
Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. The school-aged child may lose control during hospitalization. Giving the child choices can facilitate empowerment and decrease fear and anxiety.
2	This is incorrect. This may be more appropriate for a toddler or preschool-aged child.
3	This is incorrect. This is appropriate for an infant, not a school-aged child. This may increase fear or anxiety and may cause the child to overreact during hospitalization.
4	This is incorrect. This is appropriate for an adolescent as they develop independence and maturity.

PTS: 1

CON: Patient-Centered Care

17. ANS: 2

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 11. Identify the role of family-centered care in caring for the hospitalized child.

Page: 52

Heading: Communicating With the Altered Family Unit > Chronic Physical or Mental Illness
Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is incorrect. This is a developmental intervention for the adolescent child to maintain peers during this life stage. It is not a family-centered intervention.
2	This is correct. By providing resources and options, the family may feel empowered.
3	This is incorrect. Although the family may request privacy, it is not described in this scenario that they will need it at this time.
4	This is incorrect. The family may have their own clergy or outside resources, and there is no specification noted in the scenario that this is necessary.

PTS: 1 CON: Family Dynamics

18. ANS: 2

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 7. Identify age-specific approaches for communicating with parents, families, toddlers, school-aged children, and adolescents; 9. describe strategies for incorporating communication into assessment.

Page: 52

Heading: Communicating With the Family > Communicating With Adolescent Children > Nursing Interventions for Communicating with Adolescents

Integrated Processes: Teaching and Learning

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. This closed-ended question will not incur trust of the nurse, and it may cause the patient to become defensive.
2	This is correct. Nurses should encourage adolescent patients to express their feelings and concerns, and they should use open-ended questions to do so.
3	This is incorrect. This is less likely to encourage honest, open communication because the patient may be reluctant to ask for clarification of how to take the medication. A more effective method is to ask the patient to verbalize or demonstrate proper medication administration.
4	This is incorrect. This question may put the patient on the defensive and prevent further communication as to why the patient waited for help with the issue.

PTS: 1 CON: Communication

19. ANS: 4

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 11. Identify the role of family-centered care in caring for the hospitalized child.

Page: 52

Heading: Communicating With the Altered Family Unit > Hospitalization

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Family Dynamics

Difficulty: Moderate

	Feedback
1	This is incorrect. Although the patient had an infection and the parents will need instructions on the prevention of any recurrence, the focus of this scenario is the parents' concerns.

2	This is incorrect. The family states they are overwhelmed with the situation; this may be the hospitalization or the disease process. Counseling may be an option, but at this time, the family appears to need more resources to help with the transition from hospital to home.
3	This is incorrect. Although there may be underlying emotions at this time, it is not a part of the scenario, which focuses on the family's concerns over home care.
4	This is correct. The family may not be aware of the available resources to their new community. They will need help in locating resources to facilitate the transition from the hospital to the home.

PTS: 1

CON: Family Dynamics

MULTIPLE RESPONSE

20. ANS: 1, 4, 5

Chapter: Chapter 3 Family Dynamics and Communicating With Children and Families

Chapter Learning Objective: 7. Identify age-specific approaches for communicating with parents, families, and children.

Page: 49

Heading: Communicating With the Family > Communicating With Children > Communicating With Infants > Nursing Interventions for Infants

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1.	This is correct. This would provide family-centered care and note the parent's role in the child's life and well-being.
2.	This is incorrect. This would be appropriate for a toddler-aged patient, not an infant.
3.	This is incorrect. This would be appropriate for a child with a grasp on verbal skills, such as a toddler or preschooler, not an infant.
4.	This is correct. This can gain the infant's attention.
5.	This is correct. This would promote a sense of comfort for the child.

PTS: 1

CON: Communication

Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is preparing to provide care to a child from a culture that is unfamiliar to the nurse. Which action by the nurse is most effective in providing culturally competent care?
1. Researching the culture on the Internet
 2. Recognizing that each person and family is unique
 3. Seeking advice from a coworker from the same culture
 4. Determining which family member is the decision maker
- _____ 2. A nurse is caring for an infant from a different culture than her own. The mother allows the infant to continue to cry without assessing for a cause of the distress. The nurse is disturbed about this parenting. Which is the best action for the nurse to take initially?
1. Ask the mother to share what she believes about infant crying.
 2. Teach the mother that infants cry for a reason that needs to be identified.
 3. Consult with the child-life specialist about the appropriateness of mother's behavior.
 4. Become a role model by demonstrating proper care of the infant to the mother.
- _____ 3. During orientation, the new nurse on a pediatric unit is informed that Leininger's cultural care theory is implemented because of a large multicultural population at the facility. Which aspect of Leininger's theory best defines the theory's impact on nursing care?
1. Transcultural nursing occurs when the nurse incorporates the patient's culture into the care provided.
 2. The theory considers the complexity and interrelatedness of an individual within an environment and the community to which he or she belongs.
 3. The theory helps explain how practices handed down from generations can impact an individual's health today.
 4. Leininger's theory has been able to capture and articulate how culture affects health.
- _____ 4. The nurse is providing care for a child of a family who are migrant workers. According to Dr. Madeleine Leininger's cultural care theory, which component of the theory is key for providing culturally competent nursing care?
1. Gaining the trust of patients/families of the culture
 2. Capturing and articulating how culture impacts health
 3. Understanding the environment created by migration
 4. Learning the culture practices embraced by the patient/family
- _____ 5. The nurse is considering taking classes to learn another language besides English. Which factor has the greatest impact on the nurse deciding which language to study?
1. The current cultural makeup of the area around the facility.
 2. The availability of foreign language classes within travel distance.
 3. The languages that are currently covered by professional interpreters.
 4. The 2020 projected ethnicity breakdown in the United States.
- _____ 6. The nurse is providing care for a pediatric patient whose family culture is Hispanic. Which action is best when the nurse notices the patient is eating poorly at meals and rejecting snacks?
1. Ask the family members to share the types of foods preferred by the patient.
 2. Encourage the family to prepare and bring favorite foods to the patient.
 3. Offer the patient a variety of healthy and nutritious foods every 2 hours.

4. Contact the health-care provider and ask that dietary supplements be ordered.
- _____ 7. The nurse is attempting to collect information from the parents of a critically ill child brought to the emergency department. The parents are from a different country, and English is their second language. The child's father suddenly shouts, "I cannot talk with you, care for my child now!" Which response by the nurse is correct?
1. "I will get an interpreter to help us understand each other."
 2. "This information is important for providing care for your child."
 3. "Getting angry will just complicate things; you need to calm down."
 4. "I know that you are frustrated, but getting this information is important."
- _____ 8. The nurse is admitting a 15-year-old girl of Asian descent for abdominal pain lasting 3 days. The accompanying parent reports that the patient is exhibiting intense pain, vomiting, and diarrhea. The nurse notes that the patient places her pain level at 3 on a scale of 0 to 10. Which conclusion does the nurse draw regarding the discrepancy in the level of pain?
1. The patient's response may be related to cultural expectations.
 2. The patient is angry that the parent insisted on seeking medical treatment.
 3. The parent's fear of serious illness is causing an exaggeration of symptoms.
 4. The parent wants to make sure the daughter gets immediate medical care.
- _____ 9. The nurse is completing the admission process for a pediatric patient by obtaining information related to the patient's culture. Which area of assessment is most important if the patient is an infant?
1. The religious beliefs of the patient's family
 2. The family's perception of the current health status
 3. The patient's food preferences
 4. The typical daily schedule followed by family and patient
- _____ 10. A child who is 10 years of age is brought to the clinic with symptoms of a serious lower respiratory infection. The attending adult self-identifies as a grandparent. The child states the grandparent is the only family of the patient. Which is the most important information for the nurse to acquire prior to treatment?
1. How to reach one of the patient's parents
 2. Which other family members live with the patient
 3. If the grandparent has legal custody of the patient
 4. What caused the grandparent to seek medical care
- _____ 11. The nurse works in a neighborhood pediatric clinic that serves a multicultural population. The nurse uses the Giger and Davidhizar transcultural assessment model. Which of the six aspects is most helpful when performing pediatric assessments?
1. Acceptance of social organizations of which the patient and family are members
 2. Awareness of personal space between the individuals who are communicating
 3. Recognition of communication and how thoughts and feelings are expressed
 4. Knowledge of biological variations, such as appropriate weight and development

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 12. The new nurse on the pediatric unit asks not to be assigned a child whose family is from the Middle East. Which personal questions does the nurse manager suggest the nurse contemplate as a method to develop self-awareness related to cultural differences? *Select all that apply.*
1. What have been my experiences with those of different cultures?
 2. Is my unacceptance related to political differences between countries?
 3. Am I attempting to not reject the culture of my family and my upbringing?
 4. Are there conflicting values between the patient/family and myself?
 5. Why do I feel so guilty and ashamed for not accepting a different culture?
- _____ 13. The nurse is making a home visit for a patient and family who are from a different culture than the nurse. Which assessment findings during the visit cause the nurse concern? *Select all that apply.*
1. The family home currently houses three generations of the family.
 2. The household income is supplemented by government programs.
 3. The interior of the home exhibits a lack of physical updates.
 4. The children are accompanied by an adult to a playground.
 5. The family lives 5 miles from a pediatric health-care facility.
- _____ 14. The nurse in a pediatric clinic is concerned about health-care disparities among the patient population. Which patient or family comment indicates the presence of health disparities related to culture? *Select all that apply.*
1. "I didn't bring my child sooner because I have a limited income."
 2. "I tried all the remedies my mother used, but he just got sicker."
 3. "My mother does not speak English and will need an interpreter."
 4. "I felt certain that you would see my child who is now an adolescent."
 5. "We took a bus and transferred twice to get our sick child here."
- _____ 15. As the director of nursing (DON), who is responsible for planning the needs of a pediatric unit for the next several years, the DON focuses on the changing demographics for the pediatric population. Based on Federal statistics, which needs does the DON prioritize for culturally sensitive care? *Select all that apply.*
1. Suggest for increased availability of educational literature in Chinese.
 2. Plan to acquire more interpreters to serve the French population.
 3. Determine whether local changes reflect national demographic changes.
 4. Identify the probable increase in Hispanic children requiring care.
 5. Obtain literature that is presented in both English and Spanish.
- _____ 16. The nurse provides care in a pediatric health-care facility with a culturally diverse patient population. In which manner will the nurse promote effective and culturally considerate communication with patients? *Select all that apply.*
1. Communicate in a language understood by the patient and family.
 2. Identify the person responsible for making the patient's medical decisions.
 3. Utilize opportunities to learn about interacting with diverse cultures.
 4. Identify patient's/family's perception about the cause of physical illness.
 5. Learn about and practice culturally correct nonverbal communication.
- _____ 17. The charge nurse on a pediatric unit is aware that the nurses do not demonstrate culturally competent care. Which actions does the charge nurse implement to help the nurses develop this competency? *Select all that apply.*

1. Arrange for classes to learn about culturally competent care.
2. Identify the cultural groups most frequently served by the facility.
3. Share the reasons why nurses need to be culturally competent.
4. Teach nurses that their own cultures and beliefs are to be eliminated.
5. Provide cultural assessment tools for the unit nurses to use.

_____ 18. The nurse is aware that pediatric patients and their families have cultural needs and influences during periods of hospitalization. Which cultural elements does the nurse specifically provide for the hospitalized patient? *Select all that apply.*

1. Follow the schedules for meals and sleep times observed at home.
2. Encourage visitations and play times with the patient's siblings.
3. Provide a private and suitable place for caregiver hygiene and meals.
4. Set scheduled times for parents to have uninterrupted access to the patient.
5. Promote staff acceptance to parenteral input to the patient's plan of care.

_____ 19. The nurse is conducting a health assessment on a 10-year-old boy from an Asian culture who has bruises on his arms and back. The bruises appear similar in size and shape. The nurse is familiar with Asian culture but should evaluate the patient by asking which questions? *Select all that apply.*

1. "Have you fallen or injured yourself recently?"
2. "How often do you feel hungry?"
3. "What does your family do to heal illness?"
4. "Are you playing any sports in school?"
5. "Has anyone hit you or otherwise hurt you?"

_____ 20. A 17-year-old pediatric patient is admitted for a fractured femur. The patient exhibits signs of being withdrawn and emotional throughout the day. During reassessment by the nurse, which approaches will reveal spiritual distress? *Select all that apply.*

1. Asking the patient about pain level using a scale of 0 to 10
2. Inquiring about the meaning of the fracture and hospitalization to the patient
3. Observing for and explaining the meaning of swelling and erythema of the leg
4. Performing an assessment of the patient's behaviors using the FICA scale
5. Discussing the patient's previous methods of handling distress with the parents

Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Answer Section

MULTIPLE CHOICE

1. ANS: 2

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 3. Identify effective cross-cultural communication, body language, nurse–patient relationships, and multidisciplinary relationships between the caregiver, family, and the pediatric patient.

Page: 57

Heading: Culturally Competent Care

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Culture

Difficulty: Difficult

	Feedback
1	This is incorrect. Although each culture has characteristics that are commonly shared among members, nurses must avoid stereotyping. The Internet is effective for acquiring an understanding of the culture, but it is not necessarily effective in providing culturally sensitive care for this specific patient.
2	This is correct. Each family and individual is unique, and family groups and/or family members of the same culture may apply and interpret their culture in different ways. The nurse needs to specifically assess the patient and family for cultural beliefs.
3	This is incorrect. Seeking information from a coworker who shares the same culture may provide a broad understanding, but it does not address the patient/family-specific cultural beliefs.
4	This is incorrect. At some point, the nurse needs to ascertain which family member will be responsible for making medical decisions. However, this is just a small part of cultural understanding.

PTS: 1

CON: Culture

2. ANS: 1

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 1. Identify individual cultural bias.

Page: 58

Heading: Culturally Competent Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Culture

Difficulty: Difficult

	Feedback
1	This is correct. To provide care that is culturally competent, nurses must be aware of and identify the belief systems of clients and caregivers. The initial action by the nurse is to ascertain the mother's belief related to infant crying. Understanding and respect for cultures is fundamental to cultural competence.
2	This is incorrect. After ascertaining the mother's belief related to infant crying, the nurse may or may not determine the need for teaching about how to respond to the infant and why. However, the nurse needs to approach this topic with cultural awareness.
3	This is incorrect. At some point, the nurse may gain greater understanding by consulting with a child-life specialist. However, initially the nurse needs to ascertain if the mother has a culturally impacted response to infant crying.
4	This is incorrect. Initially, the nurse needs to seek information about the mother's belief related to infant crying. The nurse must then determine if cultural bias may occur when the nurse places personal values before the values of a different culture.

PTS: 1 CON: Culture

3. ANS: 1

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 2. Discuss Leininger's theory and how it can be used to plan patient care.

Page: 58

Heading: Critical Component: Cultural Care Theory

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Evidence-Based Practice

Difficulty: Difficult

	Feedback
1	This is correct. Leininger's theory best defines the impact on nursing as when transcultural nursing occurs, because the nurse incorporates the patient's culture into the care provided
2	This is incorrect. The theory does address the complexity and interrelatedness of an individual within an environment and the community to which he or she belongs. However, this does not specifically define the theory's impact on nursing.
3	This is incorrect. The theory does help explain how practices handed down from generations can impact an individual's health today. However, this does not specifically define the theory's impact on nursing.
4	This is incorrect. Leininger's theory has been able to capture and articulate how culture affects health. However, the statement does not specifically define the theory's impact on nursing.

PTS: 1 CON: Evidence-Based Practice

4. ANS: 2

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 2. Discuss Leininger's theory and how it can be used to plan patient care.

Page: 58

Heading: Critical Component: Cultural Care Theory

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Application [Applying]

Concept: Evidence-Based Practice

Difficulty: Moderate

	Feedback
1	This is incorrect. Gaining the trust of patients/families of the culture is a desirable outcome from the use of Dr. Leininger's theory. However, it is not specifically a component of the theory.
2	This is correct. A key component of Dr. Leininger's theory is the ability to capture and articulate how culture affects health. The theory helps explain how practices handed down from generations can impact an individual's health today.
3	This is incorrect. Dr. Leininger's cultural care theory does consider the complexity and interrelatedness of an individual within an environment and within the community to which he or she belongs. However, the key component to understanding the impact of environment on health is gained through the ability to capture and articulate how culture, in general, affects health.
4	This is incorrect. It is important for the nurse to be aware of the specific cultural practices embraced by the patient/family. This is not considered to be a key component to Dr. Leininger's theory.

PTS: 1 CON: Evidence-Based Practice

5. ANS: 4

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 4. Identify strategies for overcoming language barriers and describe the importance of doing so.

Page: 59

Heading: Table 4-1: 2020 Projected Ethnicity Breakdown

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse needs to understand that the cultural makeup of the area surrounding the facility is subject to change.

2	This is incorrect. The availability of foreign language classes within traveling distance is not a strong factor in deciding which language to learn. The nurse may need to consider online classes.
3	This is incorrect. Evaluating the use of current professional interpreters is a factor that is subject to change as the culture changes.
4	This is correct. The nurse needs to consult valid resources when determining which language is best to learn. An appropriate resource is the projection for America's children in 2020. Data are from <i>America's Children in Brief: Key National Indicators of Well-Being</i> , 2016 (Federal Interagency Forum on Child and Family Statistics, 2016).

PTS: 1 CON: Communication

6. ANS: 2

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.

Page: 60

Heading: Culturally Competent Care > Health Disparities > Dietary Needs

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Culture

Difficulty: Moderate

	Feedback
1	This is incorrect. Asking the family about which foods are preferred by the patient may or may not be the most effective action by the nurse.
2	This is correct. Unless the patient has dietary restrictions, the most effective action by the nurse is to encourage the family to prepare and bring the patient's favorite foods to the hospital. Dietary preferences are part of an individual's culture and adequate nutrition is important.
3	This is incorrect. Offering the patient a variety of healthy and nutritious snacks every 2 hours may or may not improve the patient's nutritional status. It is more effective to ensure that culturally preferred foods are available to the patient.
4	This is incorrect. It is unlikely that the pediatric patient will be prone to drink/eat dietary supplements. The nurse needs to take actions to provide the patient with culturally preferred foods.

PTS: 1 CON: Culture

7. ANS: 1

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objectives: 4. Identify strategies for overcoming language barriers and describe the importance of doing so.

Page: 61

Heading: Cross-Cultural Communication > Language Barriers

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment: Management of Care
 Cognitive Level: Application [Applying]
 Concept: Communication
 Difficulty: Moderate

	Feedback
1	This is correct. When the nurse recognizes difficulty in communication with a patient's family member, the nurse needs to remain calm and contact an interpreter to aid in communication.
2	This is incorrect. The parents of a critically ill child are likely to be frustrated with an inability to provide important information to the nurse. The nurse's comment does not resolve the issue of ineffective communication.
3	This is incorrect. When the nurse points out the father's anger and tells him to calm down, the nurse is likely to further hinder meaningful communication.
4	This is incorrect. Acknowledging the parent's frustration without offering a solution to the communication issue is not helpful.

PTS: 1 CON: Communication

8. ANS: 1

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child
 Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.
 Page: 62
 Heading: Cross-Cultural Communication > Characteristics and Behaviors Determined by Culture
 Integrated Processes: Nursing Process
 Client Need: Physiological Integrity: Reduction of Risk Potential
 Cognitive Level: Analysis [Analyzing]
 Concept: Culture
 Difficulty: Moderate

	Feedback
1	This is correct. Individuals will act and respond as they have been taught to act and respond. Children may "pretend" by responding in an "acceptable" way rather than one that accurately communicates their feelings and/or condition.
2	This is incorrect. There is no indication that the patient is angry at the parent for seeking medical attention.
3	This is incorrect. There is no indication that the parent is experiencing fear or exaggerating the symptoms because of the possibility of a serious illness.
4	This is incorrect. There is no indication that the parent's report of symptoms is related to achieving immediate care.

PTS: 1 CON: Culture

9. ANS: 2

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.

Page: 63

Heading: Cultural Assessments

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Culture

Difficulty: Difficult

	Feedback
1	This is incorrect. The religious beliefs of the patient's family are not the most important cultural assessment due to the age of the patient. However, the nurse will ascertain if the parents have certain religion-based needs.
2	This is correct. Because of the age of the patient, the most important area of cultural assessment is the family's perception of the current health status. The nurse will need to know if the medical condition or treatments are impacted by cultural beliefs or practices.
3	This is incorrect. Food preferences are not the most important area of cultural assessment due to the age of the patient.
4	This is incorrect. Due to the age of the patient, the daily schedule followed by the patient and family is not the most important cultural assessment.

PTS: 1

CON: Culture

10. ANS: 3

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.

Page: 63

Heading: Safe and Effective Nursing Care: Clinical Pearl: What Defines a Family?

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Legal

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse may at some point need to ascertain how to contact one of the patient's parents; however, this is not the most important information for the nurse to determine prior to treatment.
2	This is incorrect. It is not important for the nurse to acquire information about the other family members who live with the patient prior to treatment.
3	This is correct. It is most important for the nurse to acquire information regarding whether the grandparent is the legal guardian of the patient. Only a legal guardian can give consent for treatment of a pediatric patient.

4	This is incorrect. At some point it is important for the nurse to ascertain why the grandparent is seeking medical care for the patient. However, determining legal guardianship is the most important information prior to treatment.
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PTS: 1 CON: Legal

11. ANS: 4

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child
Chapter Learning Objective: 6. Describe the components involved in a cultural assessment.
Page: 63
Heading: Cultural Assessments > Giger and Davidhizar Transcultural Assessment Model
Integrated Processes: Nursing Process
Client Need: Physiological Integrity: Basic Care and Comfort
Cognitive Level: Analysis [Analyzing]
Concept: Culture
Difficulty: Moderate

	Feedback
1	This is incorrect. Acceptance of social organizations of which the patient and family are members is not the most helpful aspect of this model. This aspect is most helpful when performing a cultural assessment.
2	This is incorrect. Awareness of personal space between the individuals who are communicating may be most important when communicating with adults.
3	This is incorrect. Recognition of communication and how thoughts and feelings are expressed may or may not be useful when performing pediatric assessments.
4	This is correct. Knowledge of biological variations, such as appropriate weight and development, is the most important aspect of this model during pediatric physical assessments because of the broad physical differences in children from different cultures.

PTS: 1 CON: Culture

MULTIPLE RESPONSE

12. ANS: 1, 4

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child
Chapter Learning Objective: 1. Identify individual cultural bias.
Page: 58
Heading: Safe and Effective Nursing Care: Cultural Competence: Self-Awareness
Integrated Processes: Caring
Client Need: Psychosocial Integrity
Cognitive Level: Analysis [Analyzing]
Concept: Culture
Difficulty: Difficult

	Feedback
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1.	This is correct. The nurse manager should suggest the nurse explore the nurse's experiences with those of different cultures. The nurse should explore if those experiences were either positive or negative.
2.	This is incorrect. The nurse manager is not likely to suggest that the nurse determine whether he or she is bothered by political differences between countries. Nursing care is provided to all patients equally without personal or political bias.
3.	This is incorrect. Nurses are not expected to reject their own cultural beliefs or values in order to provide culturally unbiased care to patients and their families.
4.	This is correct. In order to understand personal feelings, the nurse should ask if there are conflicting values between the patient/family and the nurse. The answer to this question will help the nurse recognize differences and work toward resolution.
5.	This is incorrect. There is not enough information in the question to ascertain that the nurse feels either guilt or shame for not accepting another culture.

PTS: 1 CON: Culture

13. ANS: 3, 5

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 6. Describe the components involved in a cultural assessment.

Page: 59

Heading: Culturally Competent Care > Environmental Considerations

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is incorrect. In some cultures it is common for multiple generations to occupy the same home. However, the question does not supply numbers or the size of the house; therefore, the presented factor alone does not cause the nurse concern.
2.	This is incorrect. The fact that the household income is supplemented by government programs is not a concern for the nurse. Supplements may provide for housing, food, and health care, which are important to the welfare of the family.
3.	This is correct. The fact that the home interior lacks evidence of physical updates is a cause for concern by the nurse. Of specific concern is the risk of lead-based paints, outdated electrical service, and/or deterioration leading to accidents.
4.	This is incorrect. The fact that the children are accompanied by an adult to a nearby playground is not a concern for the nurse.
5.	This is correct. The fact that the family lives 5 miles from a pediatric health-care facility is a concern to the nurse. The nurse needs to carefully explore the family's access to health care and available transportation.

PTS: 1 CON: Safety

14. ANS: 1, 2, 5

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.

Page: 59

Heading: Culturally Competent Care > Health Disparities

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Culture

Difficulty: Difficult

	Feedback
1.	This is correct. Socioeconomic status can be a cause of health-care disparity; parents may be reluctant to seek medical care if the family income is limited or there is a lack of health-care insurance.
2.	This is correct. Cultural background can contribute to health-care disparity. Some ethnic groups are likely to try “home” remedies before seeking professional health care.
3.	This is incorrect. When a patient is able to ask for an interpreter, it is an indication that health-care disparity is not present.
4.	This is incorrect. When a patient recognizes that age is not a barrier to health care, health-care disparity is not present.
5.	This is correct. This comment is related to geographic location, which is a contributor to health-care disparity.

PTS: 1 CON: Culture

15. ANS: 3, 4, 5

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 8. Identify environmental and societal considerations when caring for the pediatric patient and family.

Page: 59

Heading: Table 4-1: 2020 Projected Ethnicity Breakdown

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safe and Effective Care

Cognitive Level: Synthesis [Creating]

Concept: Culture

Difficulty: Difficult

	Feedback
1.	This is incorrect. It is predicted that by 2020 the Asian non-Hispanic population in the United States will be 5% of the total population.
2.	This is incorrect. It is predicted that by 2020 the white, non-Hispanic population in the United States will decrease to below 50% of the total population.
3.	This is correct. The data comes from <i>America's Children in Brief: Key National Indicators of Well-Being</i> (2016).
4.	This is correct. The Hispanic population in the United States is predicted to increase to 26% of the total population.

5.	This is correct. The White non-Hispanic and Hispanic populations in the United States are predicted to account for 76% of the total population.
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PTS: 1 CON: Culture

16. ANS: 1, 2, 3, 4, 5

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 3. Identify effective cross-cultural communication, body language, nurse–patient relationships, and multidisciplinary relationships between the caregiver, family, and the pediatric patient.

Page: 60

Heading: Cross-Cultural Communication > Best Practices for Effective Communication

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Communication

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse needs to communicate in a language understood by the patient and family, which may include the use of an interpreter and/or written materials in the appropriate language.
2.	This is correct. When caring for the culturally diverse pediatric patient, the nurse needs to determine who will make the patient’s medical decisions.
3.	This is correct. When working in a health-care facility that cares for culturally diverse patients, the nurse has a responsibility to learn about interacting with diverse cultures.
4.	This is correct. It is important for the nurse to identify the patient’s/family’s perception about the cause of physical illness. This information will reveal cultural reactions to medical treatment and illness prevention.
5.	This is correct. It is essential for the nurse to be aware that silence, eye contact, and personal space can mean different things in different cultures.

PTS: 1 CON: Communication

17. ANS: 1, 2, 3, 5

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 3. Identify effective cross-cultural communication, body language, nurse–patient relationships, and multidisciplinary relationships between the caregiver, family, and the pediatric patient.

Page: 60

Heading: Culturally Competent Care > Health Disparities > Staff Education

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Culture

Difficulty: Difficult

	Feedback
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1.	This is correct. Education to maintain current knowledge and understand new and evolving practices in culturally competent care will improve nursing care.
2.	This is correct. It is essential to identify cultural groups predominant in the organization's practice and to examine the beliefs and values of these cultures that may affect care.
3.	This is correct. Staff members are expected to strive for cultural competency and effectiveness in interactions with individuals of different cultures.
4.	This is incorrect. Nurses do not need to eliminate the influences of their own cultures in order to provide culturally competent care. Nurses do need to identify their own belief system and potential cultural bias in order to provide culturally competent care.
5.	This is correct. Provision of theory and tools are necessary to perform culturally competent care for all children and their caregivers.

PTS: 1 CON: Culture

18. ANS: 1, 2

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 3. Identify effective cross-cultural communication, body language, nurse-patient relationships, and multidisciplinary relationships between the caregiver, family, and the pediatric patient.

Page: 60

Heading: Culturally Competent Care > Health Disparities > Family-Centered Care Components

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
1.	This is correct. It is important for the hospitalized pediatric patient to maintain some level of normalcy, which can be provided when cultural patterns are maintained. Good examples are following home schedules for meals and sleep times.
2.	This is correct. The pediatric patient's cultural environment includes play time with siblings. The nurse can meet this cultural need by encouraging sibling visitations and play times.
3.	This is incorrect. The nurse does need to provide a private and suitable place for the patient's caregiver to maintain personal hygiene and meals. However, meeting this need is not specific to the cultural needs of the patient.
4.	This is incorrect. For the cultural benefit of both the parents/caregiver and patient, the parents/caregiver needs to have unlimited access to the patient and be involved when possible in the care of the patient. Unlimited parenteral access will help meet the cultural needs of the patient.
5.	This is incorrect. Parenteral input to the patient's plan of care is important, and acknowledgment that the staff is listening and interested is vital. However, this action is not specifically aimed at the cultural needs of the patient.

PTS: 1 CON: Family Dynamics

19. ANS: 1, 3, 5

Chapter: Chapter 4, Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 6. Describe the components involved in a cultural assessment.

Page: 63

Heading: Cross-Cultural Communication > Characteristics and Behaviors Determined by Culture:
Family Use of Alternative Medicine

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Culture

Difficulty: Difficult

	Feedback
1.	This is correct. Being familiar with Asian culture may cause the nurse to suspect the use of alternative medicine. However, this is an appropriate early question to rule out accidental injury.
2.	This is incorrect. Being hungry is not related to these bruises.
3.	This is correct. It is essential to know about specific cultural practices regarding health, as well as definitions of and beliefs about health and illness. The nurse is familiar with Asian culture and needs to validate the use of alternative medicine as a source of the bruising.
4.	This is incorrect. Playing sports in school is most likely unrelated to the bruises.
5.	This is correct. Life experiences such as immigration and refugee status may include a history of violence, oppression, and trauma. Even though the nurse is familiar with Asian culture and suspects bruising from alternative medicine, trauma still needs to be specifically ruled out.

PTS: 1 CON: Culture

20. ANS: 2, 4

Chapter: Chapter 4 Cultural, Spiritual, and Environmental Influences on the Child

Chapter Learning Objective: 7. Discuss how spiritual beliefs influence the care of a child and family.

Page: 64

Heading: Cultural Assessments > Spirituality > FICA Assessment Tool

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Spirituality

Difficulty: Difficult

	Feedback
1.	This is incorrect. Pain level will reveal physical distress, not spiritual distress.
2.	This is correct. The nurse needs to explore the patient's feelings about the fracture and the resulting hospitalization. It is important to identify manifestations of anger, guilt, or depression related to the patient's condition

3.	This is incorrect. Observing for swelling and erythema is a physiological concern. Including the patient in this assessment is not likely to reveal or improve spiritual distress.
4.	This is correct. The FICA scale stands for faith, importance, community, and address. The four components of the tool provide the nurse with baseline patient data that can be used to individualize care. Assessment with this tool reveals spirituality and spiritual concerns.
5.	This is incorrect. Talking directly to the patient is appropriate. The patient is old enough to recognize and evaluate previously used methods of handling distress. However, this approach will not necessarily reveal spiritual distress.

PTS: 1

CON: Spirituality

Chapter 5 End-of-Life Care

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is presenting information to pregnant couples about the “safe to sleep” campaign to prevent SIDS. One attendee states, “Babies sleep best on their bellies. What difference does position make?” Which answer by the nurse is best?
1. “Positioning on the back opens the airway fully.”
 2. “Sleeping face down increases the risk of aspiration.”
 3. “The most dangerous time is 2 to 4 months of age.”
 4. “Of greater importance is not using blankets.”
- _____ 2. The nurse is presenting information to a group of parents regarding pediatric safety and the most common causes of death. One attendee states, “I am so happy that my youngest child is now 16.” Which information does the nurse present in response to this comment?
1. The 16-year-old child is at high risk for death related to cancer.
 2. Life expectancy with congenital or genetic defects is 13 to 19 years.
 3. Adolescents between 5 and 19 years of age most commonly die from suicide.
 4. Between 15 and 19 years of age the death rate is 45.5 deaths/100,000.
- _____ 3. The nurse is providing care for a pediatric patient who is receiving end-of-life care at the age of 12 years. In addition to the patient, the nurse is caring for the family members from a Middle Eastern culture, including younger siblings of the patient. Which comment by the nurse exemplifies appropriate communication?
1. “Please let me know if you have pain so that I can make you more comfortable.”
 2. “I know your son is going to heaven and all of you will be together in the future.”
 3. “Your brother is suffering, and you need to be strong and brave for him now.”
 4. “I understand you will be ready to move away from this life and on to the next.”
- _____ 4. The nurse is providing end-of-life care to a pediatric patient and family. The nurse understands the need for communicating with the multidisciplinary care team regarding the patient and family needs. For which reason is multidisciplinary communication with the patient’s nurse so important?
1. The nurse has the most contact with the patient and family and acts as an advocate and voice for their care.

2. The multidisciplinary team relies on the nurse to provide appropriate education to the patient and family during end-of-life care.
3. As the sole communicator, the nurse can make sure that all members of the multidisciplinary team are equally informed.
4. The patient and family can communicate with the nurse with whom they have built a trusting relationship.

_____ 5. With endorsement from the family, the physician initiates hospice care for an adolescent patient who is terminally ill. For which family expectation about hospice care will the nurse provide additional information?

1. The patient will have a graceful, natural death.
2. The patient will die in the hospital.
3. Compassionate care will be focused on patient comfort.
4. The family can be involved in care as much as desired.

_____ 6. The parents of a toddler, diagnosed to be in the end stage of a terminal illness, are concerned about how to manage pain without the sedation effects of pain medications. Which suggestion by the nurse is likely to meet the needs of both the parents and the patient?

1. Encourage the parents to bring favorite toys and books.
2. Initiate playtime in the playroom with other patients.
3. Provide age-appropriate videos for patient distraction.
4. Suggest a parent hold the patient and perform gentle massage.

_____ 7. The nurse works in a pediatric hospice unit in an acute care facility. The nurse is currently providing care to an infant. Which assessment tool does the nurse use to identify the infant's level of pain?

1. Faces scale
2. FLACC
3. Visual analogy scale
4. NIPS

_____ 8. The nurse assesses the pain level of a school-age patient who is receiving end-of-life care and determines a need for pain medication. The physician has prescribed morphine sulfate to be administered either orally or rectally. For which reason will the nurse decide to administer the medication orally?

1. To prevent the patient from being embarrassed by rectal administration
2. To ensure the pain medication is absorbed as quickly as possible
3. To avoid stimulation of rectal spasms during insertion of the medication
4. To decrease the risk of respiratory suppression

_____ 9. The nurse is providing care for a pediatric patient and family during the time when death of the patient seems imminent. The family is of American Indian culture and has summoned tribal members to come and chant and pray at the bedside. Which behavior by the nurse is culturally correct?

1. Move the patient, family, and tribal members to an isolated location.
2. Ask the family to respect other patients by keeping the volume of chanting low.
3. Ask if the family has any additional needs, close the door, and provide privacy.
4. Call the nursing supervisor and ask for assistance in managing the situation.

- _____ 10. A 2-year-old has recently been diagnosed with a terminal type of cancer but is not experiencing any severe symptoms of the illness at this time. The mother of the patient is depressed and feels guilt over the illness. She admits to mentally planning the funeral and expecting to be devastated. The nurse identifies the mother as experiencing which stage of grief?
1. Anticipatory
 2. Anger
 3. Denial
 4. Bargaining
- _____ 11. A 16-year-old patient is approaching the terminal stage of a brain tumor. The nurse notices the patient is calling friends and making plans for a social gathering several months away. When friends are present the patient assures them of a full recovery because of a new doctor. The nurse recognizes the patient is experiencing which stage of grief?
1. Acceptance
 2. Denial
 3. Anger
 4. Bargaining
- _____ 12. The nurse is providing care for a pediatric patient who is 11 years of age. The patient is diagnosed with an aggressive form of cancer and is scheduled to begin chemotherapy. The patient tells the nurse, "I think I am going to die, but I also think I will get much sicker first." Which communication by the nurse is most appropriate for this patient?
1. Explain to the patient the importance of maintaining a hopeful outlook.
 2. Encourage the patient to ask the doctor to explain what is going to happen.
 3. Use basic terms to explain the disease progression and side effects of treatment.
 4. Provide information about the different options that can be considered for the patient.
- _____ 13. The nurses on a unit that cares for terminal pediatric patients wish to implement a change in facility policy to allow parents/guardians to remain during a patient code. Which is the most powerful reason that nurses can present to administration?
1. Family-centered care can be continued during a very stressful situation.
 2. The family knows everything possible was done to save the patient.
 3. The family experiences greater communication and decreased confusion.
 4. Family may desire to stay during specific interventions that seem inappropriate.
- _____ 14. The coroner is informed of the unexpected death of an infant at 3 months of age. The infant died during the night in the home of the parents. Which expectation does the nurse have regarding follow-up to the infant's death?
1. The hospital will pay for an autopsy if requested by the physician.
 2. The coroner's office will not charge the parents if they request an autopsy.
 3. The coroner can legally request an autopsy without the parent's consent.
 4. The parents must give consent before an autopsy is performed for any reason.
- _____ 15. The pediatric nurse is providing care for a terminally ill patient who is 17 years of age. The patient has been resistant to aggressive chemotherapy because of undesirable side effects. The patient states, "I have finally convinced my parents to sign a DNR order. It is my life, and I should be able to decide how I want to live." Which legal consideration causes the nurse greatest concern?

1. A DNR order can be reversed at any time by the legal guardians.
2. The primary physician can deny a DNR if the patient is a minor.
3. The patient does not understand all aspects of the DNR order.
4. A DNR can be written to provide partial life-sustaining interventions.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 16. A pediatric patient with a terminal disease is placed in hospice care. In addition to patient care, which other services provided by a multidisciplinary team will meet the needs of the patient and family? *Select all that apply.*
1. Social work
 2. Child-life specialist
 3. Community programs
 4. Physician
 5. Grief counselor
- _____ 17. The nurse is providing end-of-life education to parents of a child diagnosed with a terminal illness. Which topics of education are important for the nurse to provide? *Select all that apply.*
1. Explain how the progression of the disease will affect their child.
 2. Describe the physical changes that will occur during the dying process.
 3. Suggest several funeral homes to be contacted before death.
 4. Explain that pain management will interfere with communication.
 5. Describe what to expect after death including how the child will look.
- _____ 18. The parents of a dying child have decided to withdraw medical treatment and allow their child to die peacefully. Which nursing actions will occur after the withdrawal is implemented? *Select all that apply.*
1. Monitoring equipment is turned off.
 2. All invasive lines are disconnected.
 3. Periodic validation is sought from the family regarding the decision to end care.
 4. A comfortable, peaceful environment is created for the patient and family.
 5. The family is provided with undisturbed privacy.
- _____ 19. The nurse works on a pediatric unit that allows parents to remain in the room if the patient is coded. The nurse is aware that having persons in the room other than medical personnel can present safety issues. Which concerns are important to address? *Select all that apply.*
1. The possibility of an accidental shock during cardiac shock delivery
 2. The possibility of accidental contamination of sterile fields and procedures
 3. The possibility of medical staff being blocked from access to the patient
 4. The possibility of medical staff being interrupted by emotional verbalization
 5. The possibility of overcrowding interfering with needed medical equipment
- _____ 20. The nurse works on a pediatric unit where patients are on life support. Frequently, death is delayed for the benefit of organ donation and procurement. Which protocols for organ donation and procurement does the nurse always need to follow? *Select all that apply.*
1. The nurse approaches the family of a patient who is a viable candidate.

2. The best matched pediatric patients on the transplant list are notified.
3. The consent for organ donation can be given only by legal guardians.
4. The facility pediatric surgical team will procure the organs for transplant.
5. The parents only can accompany the patient to the operating room.

Chapter 5 End-of-Life Care

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 1. Understand causes of infant and pediatric death.

Page: 68

Heading: Figure 5-1: Always educate parents to put their infants on their backs to sleep to help prevent sudden infant death syndrome by keeping the airway fully open.

Integrated Processes: Teaching/Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Grief and Loss

Difficulty: Moderate

	Feedback
1	This is correct. The nurse should always educate parents to put their infants on their backs to sleep to help prevent sudden infant death syndrome by keeping the airway fully open.
2	This is incorrect. Placing the infant face down may increase the risk of aspiration if the infant vomits or spits up. However, the best answer is the one that defends how the parents can avoid the incidence of SIDS, which is the primary cause of unexpected infant death.
3	This is incorrect. The nurse can share that the most vulnerable age for SIDS is between 2 and 4 months of age. However, this does not reinforce the physiological reason why infants need to sleep on their back.
4	This is incorrect. Parents are instructed to keep blankets and other objects that can cause suffocation out of the infant's crib. However, this comment does not reinforce the physiological reason why infants need to sleep on their back.

PTS: 1

CON: Grief and Loss

2. ANS: 4

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 1. Understand causes of infant and pediatric death.

Page: 68

Heading: Box 5-1: Causes of Pediatric Death

Integrated Processes: Teaching/Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Violence

Difficulty: Moderate

	Feedback
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1	This is incorrect. Cancer is listed as a common cause of pediatric patients between the ages of 5 and 14 years.
2	This is incorrect. Death from congenital malformations, deformations, and chromosomal abnormalities most commonly occurs between 1 and 4 years of age.
3	This is incorrect. Between the ages of 5 and 19 years, suicide is listed as one of the leading causes of death. However, suicide is not singled out as the primary cause of death in this age group.
4	This is correct. Between 15 and 19 years of age the death rate is 45.5 deaths/100,000. This is the pediatric group with the highest incidence of death. The most common causes are accidents, suicide, and homicide.

PTS: 1 CON: Violence

3. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 2. Understand and describe appropriate communication with the patient and family in end-of-life care.

Page: 69

Heading: Communication in Care of the Dying Patient > Family-Centered Care in End of Life

Integrated Processes: Communication and Documentation

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is correct. End-of-life communication with the patient needs to be compassionate and developmentally appropriate. The nurse needs to communicate a caring attitude about the patient's pain and comfort status. This comment is appropriate.
2	This is incorrect. When the nurse expresses personal beliefs, the cultural beliefs of the patient and family is violated.
3	This is incorrect. Telling a sibling younger than 12 years of age that their brother is suffering is likely to cause anguish to the sibling. Encouraging the sibling to be strong and brave is suggesting denial of feelings.
4	This is incorrect. When the nurse expresses feelings related to the patient's impending death, the nurse is interjecting personal feelings into the patient's situation; the nurse needs to focus on the patient's and family's feelings.

PTS: 1 CON: Communication

4. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 3. Understand and describe appropriate communication within the multidisciplinary care team in end-of-life care.

Page: 69

Heading: Communication in the Care of the Dying Patient > Family-Centered Care in the End of Life

Integrated Processes: Communication and Documentation

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is correct. The nurse has the most contact with the patient and family and acts as an advocate and voice for their care. The nurse can determine effectively which member of the multidisciplinary team should be contacted in order to provide the best care.
2	This is incorrect. The nurse, during end-of-life care, is not expected to provide education to the patient and family; this task is more appropriate for hospice nurses.
3	This is incorrect. The nurse is not the sole communicator during end-of-life care; in addition, not all multidisciplinary team members need to receive the exact same communication.
4	This is incorrect. When the nurse is providing appropriate and effective end-of-life care, the patient and family will build a trusting relationship with the nurse. However, this role as advocate is different than the communication role with multidisciplinary team members.

PTS: 1 CON: Communication

5. ANS: 2

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 5. Define and describe hospice care, including the roles of the nurse and multidisciplinary team.

Page: 69

Heading: Hospice Care

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Grief and Loss

Difficulty: Moderate

	Feedback
1	This is incorrect. There is no need to provide additional information if the family understands that a hospice goal is for the patient to have a graceful, natural death.
2	This is correct. If the family anticipates that the patient will die in the hospital, the nurse can provide additional information that hospice care can also take place in the patient's home with visiting nurses and care aides. Some hospice groups have an independent facility.

3	This is incorrect. There is no need to provide additional information if the family understands that hospice will provide compassionate care focused on patient comfort.
4	This is incorrect. There is no need to provide additional information if the family understands they can be as involved as much as desired in the patient's care.

PTS: 1 CON: Grief and Loss

6. ANS: 4

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 6. Discuss assessment and treatment of pain in the hospice care of an infant or pediatric patient.

Page: 70

Heading: Hospice Care > Pediatric Pain Control

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Synthesis [Creating]

Concept: Grief and Loss

Difficulty: Difficult

	Feedback
1	This is incorrect. The toddler in the end stage of terminal illness may or may not be receptive to favorite toys or books. Holding and soothing are likely to be more effective.
2	This is incorrect. It is unlikely that a toddler in the end stage of terminal illness will be willing or able to play with other children. Holding and soothing are likely to be more effective.
3	This is incorrect. The toddler in the end stage of terminal illness may or may not be effectively distracted by age-appropriate videos. Holding and soothing are likely to be more effective.
4	This is correct. The parents have expressed a desire for pain management without the sedation effects of pain medication. The unspoken need of the parents is to have the ability to interact with their child and still achieve pain management. When the nurse suggests that a parent hold the patient and perform gentle massage, the needs of both the patient and parents are being met; the suggested actions will promote relaxation and potentially decrease pain.

PTS: 1 CON: Grief and Loss

7. ANS: 2

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 6. Discuss assessment and treatment of pain in the hospice care of an infant or pediatric patient.

Page: 70

Heading: Hospice Care > Pediatric Pain Control

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The faces scale is only for patients aged 3 years and older; the child must be developmentally able to read and recognize faces drawn with various levels of painful expressions. This pain scale asks the child to choose the face that best represents his or her pain level. The patient is too young for this scale.
2	This is correct. The FLACC scale (faces, legs, activity, cry, consolability scale) is used for newborn to 7 years. This assesses the patient's facial expression, leg positioning and flexion, activity level, crying level, and consolability. This is the appropriate scale for this patient.
3	This is incorrect. The VAS is for children aged 7 years and older who have the developmental ability to use the traditional pain scale based on numbers 0 to 10 for pain rating. The patient is too young for this scale.
4	This is incorrect. The neonatal and infant pain scale (NIPS) is for newborns. This assesses the newborn's cry, facial expression, respiratory pattern, position and flexion of the arms and legs, and level of alertness. The scale is better suited for newborns.

PTS: 1

CON: Patient-Centered Care

8. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 6. Discuss assessment and treatment of pain in the hospice care of an infant or pediatric patient.

Page: 71

Heading: Safe and Effective Nursing Care > Understanding Medication > Administration

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Comfort

Difficulty: Moderate

	Feedback
1	This is correct. The administration of medication rectally is likely to cause a school-age client to be embarrassed. When possible, the nurse will prevent psychosocial discomfort as well as physical discomfort.
2	This is incorrect. The oral form of morphine sulfate is effective; however, the rectal route is likely to be absorbed more quickly.
3	This is incorrect. The rectal insertion of morphine sulfate is not likely to stimulate rectal spasms.
4	This is incorrect. The side effects of morphine sulfate are the same regardless of the route of administration.

PTS: 1

CON: Comfort

9. ANS: 3

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 7. Understand and explain the role spiritual and cultural care have within end-of-life care.

Page: 71

Heading: Hospice Care > Cultural and Religious Care During End of Life

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Spirituality

Difficulty: Moderate

	Feedback
1	This is incorrect. Moving the family to an isolated location can be interpreted as disrespect for the patient's and family's culture.
2	This is incorrect. Asking the family to respect other patients by keeping the volume of chanting low can be interpreted as disrespect for the patient's and family's culture.
3	This is correct. Cultural considerations for pediatric end of life include respecting the cultural beliefs of the family and patient. Providing culturally competent care requires the nurse to incorporate the cultural beliefs of the family and patient into daily care. The spiritual needs of family and patient require the nurse to incorporate the spiritual beliefs of the family and patient into daily care.
4	This is incorrect. There is no reason to call the supervisor, and there is no situation to manage. The nurse can effectively provide the family cultural support by closing the patient's door and giving the family and tribal members' privacy.

PTS: 1

CON: Spirituality

10. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 8. Define and compare the stages of grief.

Page: 71

Heading: Grief

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Grief and loss

Difficulty: Difficult

	Feedback
1	This is correct. Anticipatory grief occurs before the stages of grief and is common in infant and pediatric death when the family of a patient with a terminal diagnosis prepares for death before the dying process.

2	This is incorrect. Anger concerning death results in feelings of wrath or indignation; it will often manifest as anger toward the disease, the cause of death of the infant or child, or even the medical staff and caregivers of the child.
3	This is incorrect. Following anticipatory grief is denial, which is a refusal to believe that an infant or child is dead or dying.
4	This is incorrect. Bargaining or negotiation is an attempt to create a change in the situation through an agreement for services exchanged.

PTS: 1 CON: Grief and Loss

11. ANS: 2

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 8. Define and compare the stages of grief.

Page: 72

Heading: Table 5-2: Kübler-Ross Stages of Grief

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Grief and Loss

Difficulty: Moderate

	Feedback
1	This is incorrect. Statements of understanding of the loss, positive outlook, and discussion of the future are indicative of acceptance.
2	This is correct. Unwillingness to accept the diagnosis and lack of trust in medical staff are indicative of denial.
3	This is incorrect. Anger or aggression toward staff or family members and verbal arguments and confrontations are common during the grief stage of anger.
4	This is incorrect. Reliance on a higher power to prevent death and believing that promises of future behavior will prevent death are indicative of bargaining.

PTS: 1 CON: Grief and Loss

12. ANS: 3

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 9. Understand and describe the role of nursing education in end-of-life care.

Page: 72

Heading: Nursing Education in End-of-Life Care

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Grief and Loss

Difficulty: Difficult

	Feedback
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1	This is incorrect. The patient is 11 years of age and has stated expectations related to the medical diagnosis. Encouraging the patient to maintain a hopeful outlook does not address the thoughts and concerns that already exist.
2	This is incorrect. Nurses have the capability to answer questions about the patient's condition and treatment; it is unnecessary to delay meaningful communication until the physician can answer. The nurse can encourage the patient to also talk with the physician and parents, but information does not need to be delayed.
3	This is correct. Nursing education to the pediatric patient in this scenario should include age-appropriate information about the disease and treatment.
4	This is incorrect. The patient is 11 years of age and unable to contemplate and decide about the medical treatment. The parents or legal guardian are considered to be the decision makers.

PTS: 1 CON: Grief and Loss

13. ANS: 2

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 9. Understand and describe the role of nursing education in end-of-life care.

Page: 73

Heading: Family Presence in Code Situations

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Synthesis [Creating]

Concept: Health Care System

Difficulty: Difficult

	Feedback
1	This is incorrect. Family involvement in pediatric code situations provides family-centered care during a very stressful situation. Without additional planning and discussion, this is not the most powerful reason for the nurses to present to administration.
2	This is correct. Having family stay during a code can be positive because the family knows everything possible was done to save the patient. This is the most powerful reason for the nurses to present to administration.
3	This is incorrect. Continuation of family-centered care in pediatric code situations is to create greater communication and decrease confusion for the family. However, this is not the most powerful reason for the nurses to present to administration unless there is strong support from other members of the multidisciplinary team members.
4	This is incorrect. The family may attempt to be at the bedside, which can be inappropriate with specific interventions; an example would be open-chest procedures. This point would require discussion and the development of guidelines.

PTS: 1 CON: Health-Care System

14. ANS: 3

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 13. Define and describe legal aspects of infant and pediatric end-of-life care.

Page: 74

Heading: Legal Aspects of Pediatric End of Life > Autopsies

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Legal

Difficulty: Moderate

	Feedback
1	This is incorrect. The physician may request an autopsy if the patient's cause of death is not known; however, the cost of the autopsy is covered by the coroner's office if the coroner agrees. The hospital is unlikely to be responsible for the cost.
2	This is incorrect. Families may request an autopsy if they wish to know the exact cause of the death of their child; if the coroner does not agree that an autopsy needs to be performed, the family may be responsible for the cost of the autopsy.
3	This is correct. The coroner has the legal right to request an autopsy in any suspicious or unexpected death and does not need consent of the family if it is considered legally necessary.
4	This is incorrect. The consent of the family is not needed if the autopsy is considered legally necessary, as in an unexpected or suspicious death.

PTS: 1

CON: Legal

15. ANS: 1

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 13. Define and describe legal aspects of infant and pediatric end-of-life care.

Page: 75

Heading: Legal Aspects of Pediatric End of Life > Do Not Resuscitate Orders

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Legal

Difficulty: Difficult

	Feedback
1	This is correct. The nurse is aware that the patient is a minor and only legal guardians can determine a minor's DNR status. The patient's sense of satisfaction is the nurse's greatest concern, because legal guardians can reverse a DNR decision at any time.

2	This is incorrect. Primary physicians cannot deny a DNR order just because the patient is a minor. The legal guardians have the right to request the order and the physician has the responsibility to make sure they understand all aspects of the decision.
3	This is incorrect. The minor patient may or may not understand all aspects of a DNR order; this is not likely to be the nurse's greatest legal concern.
4	This is incorrect. It is true that a DNR order can be written to provide partial or limited life-sustaining interventions. This is not the nurse's greatest legal concern.

PTS: 1

CON: Legal

MULTIPLE RESPONSE

16. ANS: 1, 2, 4

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 4. Define the roles of family-centered care and culturally competent care in end-of-life care.

Page: 69

Heading: Hospice Care > Multidisciplinary, Family-Centered Approach

Integrated Processes: Caring

Client Need: Safe and Effective Care Environment > Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Grief and Loss

Difficulty: Difficult

	Feedback
1.	This is correct. Social work ensures that the family's nonmedical needs, such as housing and employer notification, are met.
2.	This is correct. The child-life specialist provides age- and developmentally appropriate toys and environment for patient and siblings.
3.	This is incorrect. Community programs and support programs will most likely be recommended to the family after the death of the patient.
4.	This is correct. The physician leads medical care and orders medications and interventions while providing the patient and family with education regarding the diagnosis.
5.	This is incorrect. A grief counselor is most likely to assist the family after the death of the patient.

PTS: 1

CON: Grief and Loss

17. ANS: 1, 2, 5

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 9. Understand and describe the role of nursing education in end-of-life care.

Page: 72

Heading: Nursing Education in End-of-Life Care

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Grief and Loss

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse needs to cover the topic of how the progression of the disease will affect their child. This information will help the parents to distinguish between expected and unexpected changes.
2.	This is correct. The nurse needs to cover the topic of the physical changes that will occur during the dying process. Some topics will include skin alterations, breathing alterations, altered consciousness, and changes in verbalization.
3.	This is incorrect. End-of-life education does not involve providing a list of funeral homes to contact prior to the death. Families may have a funeral home in mind, and planning before the death may be offensive to some families.
4.	This is incorrect. Pain management may or may not interfere with communication. The parents need to be encouraged to talk to their child even when two-way communication is not possible.
5.	This is correct. The nurse needs to cover the topic of what to expect after death. Some families are concerned that the physical appearance is grossly altered. The parents also need to know about how the body will be cared for and about making funeral arrangements.

PTS: 1 CON: Grief and Loss

18. ANS: 1, 2, 4, 5

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 10. Define and understand withdrawal of care in infant and pediatric end-of-life care.

Page: 72

Heading: Withdrawal of Care

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Grief and Loss

Difficulty: Difficult

	Feedback
1.	This is correct. Monitoring equipment is turned off; when operating, the equipment can be a disturbing distraction.
2.	This is correct. Invasive lines and equipment are removed in order to present the patient with an appearance as normal as possible.
3.	This is incorrect. The family can decide to reinstate medical care if they desire; however, the nurse does not check periodically to see if the decision for withdrawal of care is changed.

4.	This is correct. The patient's linens can be changed or straightened, lights can be dimmed, and the patient placed in a comfortable position. Chairs and other desired accommodations are provided for the family.
5.	This is correct. In order to ensure uninterrupted privacy, the door should be closed and the family left undisturbed unless they seek assistance.

PTS: 1 CON: Grief and Loss

19. ANS: 1, 3, 5

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 9. Understand and describe the role of nursing education in end-of-life care.

Page: 73

Heading: Safe and Effective Nursing Care > Promoting Safety > Resuscitation

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. It is important for the nurse to be concerned about the safety of the family. The concern about family members not touching the bed or medical equipment during cardiac shock delivery will need to be addressed.
2.	This is incorrect. Most codes are not run in a sterile location. It is unlikely that the presence of a family member would cause more contamination than other members of the code team.
3.	This is correct. For the safety of all involved, it is important to ensure that family members are not in the way of medical staff or interfere with treatment.
4.	This is incorrect. It would be unexpected for emotional verbalization from a family member to interrupt the medical staff. A staff member should always be with the family to monitor behaviors and provide information.
5.	This is correct. For the safety of all involved, it is important to ensure that family members are not in the way of medical equipment.

PTS: 1 CON: Safety

20. ANS: 3

Chapter: Chapter 5 End-of-Life Care

Chapter Learning Objective: 12. Understand and describe organ donation and procurement.

Page: 74

Heading: Organ Donation

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Legal

Difficulty: Difficult

	Feedback
1.	This is incorrect. When a child is considered a viable candidate for organ donation, the family is approached by an organ donation approach team. This multidisciplinary team of health-care providers and ancillary staff is specially trained in approaching families to request that they donate the patient's organs.
2.	This is incorrect. Once consent is given, blood is drawn from the patient to determine the patients on the transplant list who are the best match for the organs available. However, pediatric organs can be transplanted into pediatric or adult patients.
3.	This is correct. Consent is needed from the legal guardians for organ donation.
4.	This is incorrect. Procurement of organs is commonly done by the transplanting surgical team. The team travels to the site of the organ donor, and the patient is taken to the operating room to remove the organs.
5.	This is incorrect. Parents and family members should say goodbye to the patient before transfer to the operating room; the patient will die in the operating room.

PTS: 1 CON: Legal

Chapter 6 Growth and Development of the Child

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse in a pediatric clinic is performing well-baby checks. The nurse is checking an infant who is 7 months old for developmental milestones. Which finding is of greatest concern to the nurse?
1. The infant remains flat when in a supine position.
 2. The infant exhibits a Babinski reflex.
 3. The infant opens and closes her hands to grasp objects.
 4. The infant exhibits a lack of startle reflex to sound.
- _____ 2. The nurse in a pediatric clinic is checking the developmental milestones for a 3-year-old patient. Which finding causes the nurse to perform additional assessments?
1. The patient's tee-shirt is on backward.
 2. The patient loses balance when kicking a ball.
 3. The patient draws a circle that is closed but oblong.
 4. The patient jumps with both feet about 2 inches high.
- _____ 3. A mother of a 9-month-old infant asks the nurse about what toys are age appropriate. Using Piaget's theory of development, which toy does the nurse recommend?
1. Building blocks
 2. Colorful mobiles
 3. Picture books
 4. Musical rattles

- _____ 4. During a well-baby checkup, the mother of an infant states, “Even if he is occupied with a toy, he cries as soon as he notices I have left the room.” Which explanation by the nurse is best?
1. “Your baby does not know you exist if he cannot see you.”
 2. “Babies learn very quickly how to get an adult’s attention.”
 3. “You should move the baby with you if you leave the room.”
 4. “Just ignore him; he will soon learn that you are still present.”
- _____ 5. The nurse is visiting the home of a new mother and a 2-month-old infant. The nurse notices the infant vigorously sucking on the fist and whining but not crying. The mother validates that the behavior is common. Which information does the nurse need to obtain from the mother?
1. If the mother is breast or bottle feeding
 2. How long the infant sleeps at night
 3. What type of feeding schedule is followed
 4. If the infant draws up the legs when crying
- _____ 6. The nurse works in an elementary school with students ranging from 6 to 11 years of age. The nurse uses knowledge related to Freud’s psychosocial theory to identify which behavior in this pediatric population?
1. Oedipal or Electra conflicts
 2. Energy focused on socialization
 3. Curiosity about anatomical differences
 4. Mild struggles with sexuality
- _____ 7. The nurse in a pediatric clinic is counseling a parent who expresses concern about a toddler who plays alone at daycare and does not interact with the other children who are present. Which information does the nurse provide to alleviate the parent’s concern?
1. Parallel play is being exhibited and is normal at this age.
 2. The toddler is likely to grow into a shy, introverted adult.
 3. It is important for the child to learn to be alone at this age.
 4. The toddler is exhibiting the normal behavior of solitary play.
- _____ 8. The school nurse in a high school setting expresses concern to school administration regarding the increase in student complaints about bullying, physical violence, and rejection. Which concern related to psychosocial development does the nurse share as being most important?
1. Students are preoccupied with how they are seen in the eyes of others.
 2. Students who are bullied will develop issues related to sexual orientation.
 3. Students may be unable to provide a meaningful definition of self.
 4. Students who are aggressive will develop a strong sense of guilt as adults.
- _____ 9. The nurse is counseling parents about management of their children who are 2, 4, and 6 years of age. One of the parents states, “We believe in Kohlberg’s theory of social-moral development.” The nurse is aware that the preconventional stage of this theory involves which characteristic?
1. Behavior is adjusted according to good/bad and right/wrong thinking.
 2. A personal and functional value system is constructed by the child.
 3. The focus of the child is on following rules and maintaining social order.
 4. Value systems are independent of authority figures and peers.

- _____ 10. The nurse is teaching a parenting class being held in a community clinic. The nurse is focusing on behaviors that will assist in increasing the number of children who score well in kindergarten readiness screening. Which comment by a parent indicates the need for additional information?
1. "I am not athletic, but the kids would love an outdoor play area."
 2. "Practicing counting with the kids while traveling is a good idea."
 3. "I like the suggestion to label basic items for word recognition."
 4. "In our family we watch TV; books are a waste of money."
- _____ 11. The nurse in a pediatric clinic is performing an assessment on an infant in the presence of both parents. The parents are short and moderately overweight. The father states, "We are going to do everything we can to raise a strong, tall, athletic child." How does the nurse respond?
1. Provides materials about healthy diets and lifestyles for families
 2. Shares the impact of genetics on environmental conditions
 3. Suggests to the parents how to alter their lifestyles
 4. Recognizes the parents for having positive attitudes and goals
- _____ 12. The pediatric home-care nurse is visiting a toddler born with a genetically related illness. Which comment by the parent is the greatest cause for the nurse to assess for additional information?
1. "I am having more difficulty keeping her confined."
 2. "A chronically ill child affects the entire family."
 3. "The other children seem so accepting of the illness."
 4. "Hospitalization would cause some severe stress."
- _____ 13. The nurse is performing a development assessment on a 3-month-old infant who was 6 weeks premature. The nurse states the infant's development is normal. The parent expresses that the baby seems behind what other babies the same age are doing. Which information does the nurse share to provide reassurance to the parent?
1. The infant will catch up developmentally by age 1 year.
 2. Developmental milestones vary from infant to infant.
 3. The infant's age is adjusted because of prematurity.
 4. Each infant is an individual with unique development.
- _____ 14. The pediatric nurse is providing care for a 14-year-old female patient. After the patient's parents leave the hospital, the patient begins to cry. The nurse explores the patient's feelings using therapeutic communication. Which information causes the nurse to report suspected sexual abuse?
1. The patient is frequently denied access to needed health care.
 2. The patient reports frequent episodes of genital irritation.
 3. The patient admits to multiple incidences of skipping school.
 4. The patient states that an older brother frequently "hurts" her.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. Erickson's psychosocial development theory proposes that the school-aged child between ages 6 and 12 years is in the stage of industry vs. inferiority. Based on this theory, how will the pediatric nurse design activities as part of a diversional program for children who are in a long-term medical facility? *Select all that apply.*

1. Identify adequate activities suited for solitary play.
2. Provide activities that involve more than one person.
3. Allow participation in simple tasks on the unit of care.
4. Designate methods of recognition for completed tasks.
5. Set guidelines and policies that are clear and enforceable.

_____ 16. The nurse is gathering information during a routine checkup for a preschool-age child who lives with grandparents. The grandmother expresses distress about “how loud and busy” the child is, and “how expensive it is to feed and clothe” the child. Which referrals does the nurse make to the grandmother? *Select all that apply.*

1. Community programs for the child aimed at playing and learning
2. Social service for determination of benefits available for the care of the child
3. Legal services to assist in obtaining financial support from the parents
4. Child protection services to survey the home and psychosocial environment
5. Caretaker programs and support groups for grandparents performing as parents

_____ 17. A widowed parent of two children informs the nurse of an upcoming marriage to a woman who has three children. The expressed intention is to adopt the three stepchildren. Which definition of family will the nurse apply? *Select all that apply.*

1. A nuclear family after the adoption of the stepchildren.
2. A nonnuclear family after the marriage has taken place.
3. A blended family after the marriage of the adults occurs.
4. A nuclear family before adoption if all children live in the home.
5. A blended family after the intention of marriage is expressed.

_____ 18. The nurse is performing a clinic assessment on a 1-month-old new patient. During the interview, the mother shares personal information. Which comments will cause the nurse concern about growth and development? *Select all that apply.*

1. “I was anemic during pregnancy and still take iron pills.”
2. “Fat people are gross; I only gained 16 pounds during pregnancy.”
3. “I don’t think I even had a single cold during my pregnancy.”
4. “During my pregnancy I never even took care of the cat.”
5. “I really decreased my smoking habit during my pregnancy.”

_____ 19. The nurses on a pediatric unit are concerned about developmental delays in patients who are hospitalized frequently and for extended periods of time. Which interventions do the nurses initiate to alleviate the concerns? *Select all that apply.*

1. Design a play/recreational area with age-appropriate sections.
2. Provide nurses with allotted time to play with confined children.
3. Extend the services of the child-life specialists to all patients.
4. Encourage family to bring favorite toys and books from home.
5. Have age-appropriate educational TV channels available.

_____ 20. The school nurse is asked to assess a student in the third grade who is failing to demonstrate academic success. Using Maslow’s hierarchy of needs, the nurse can create a needs list based on which comments by the student? *Select all that apply.*

1. “I have to go to bed at 10:00 every night.”
2. “I worry because my mom and dad fight all the time.”

3. “Game and movie nights are always fun at my house.”
4. “My grandma says I’m stupid just like my mother.”
5. “I taught my little brother to ride a bike in just one day.”

Chapter 6 Growth and Development of the Child

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 1. Describe general principles of growth and development.

Page: 80

Heading: General Principles of Growth and Development > Table 6-1 > Developmental Milestones

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is correct. An infant should be able at the age of 2 to 3 months to raise the head and chest and support the upper body with arms while in a prone position. The finding that the infant at 7 months lies flat when placed prone is a matter of concern to the nurse.
2	This is incorrect. It is not a matter of concern to the nurse if the 7-month-old infant still exhibits a Babinski sign. The Babinski reflex disappears by the age of 1 year.
3	This is incorrect. The ability to grasp objects by opening and closing the hands is normally present at the age of 2 to 3 months. The presence of this skill is expected to continue. This ability alone is not a reason for concern; however, assessment for progression is important.
4	This is incorrect. The startle reflex disappears around 4 to 6 months; the absence of this reflex in a 7-month-old infant is not cause for concern.

PTS: 1

CON: Growth and Development

2. ANS: 2

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 1. Describe general principles of growth and development.

Page: 80

Heading: General Principles of Growth and Development > Table 6-1 > Developmental Milestones

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. At the age of 3 years, the patient is beginning to self-dress; the backward shirt indicates the skill is developing but not refined. The backward shirt may also be indicative of toddler independence.
2	This is correct. Between the ages of 2 to 3 years, a toddler should be able to kick a ball. The fact that the patient loses balance when attempting this skill may require additional assessment.
3	This is incorrect. Toddlers correctly draw a circle when the curved line is closed; the shape is not the most important factor.
4	This is incorrect. No matter how high, jumping with both feet off the floor is an expected developmental skill for a toddler.

PTS: 1 CON: Growth and Development

3. ANS: 4

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 2. Discuss cognitive growth and development according to Jean Piaget.

Page: 81

Heading: Psychointellectual Development > Key Stages of Piaget's Theory

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse would not expect an infant of 8 months of age to play with building blocks
2	This is incorrect. An 8-month-old infant may or may not be interested in a colorful mobile. Tactile stimulation from this would be limited for safety reasons.
3	This is incorrect. An infant's interest in picture books is more likely to occur in Piaget's stage 6: inventions of new means/mental combinations, which occurs between 18 and 24 months.
4	This is correct. At 8 months, the infant should be in Piaget's stage 4: coordination of secondary schemata. To achieve a desired effect, the infant will repeat an action, such as repeatedly shaking a rattle to make sounds. The nurse will recommend a variety of rattles as appropriate toys for this patient.

PTS: 1 CON: Growth and Development

4. ANS: 1

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 2. Discuss cognitive growth and development according to Jean Piaget.

Page: 81

Heading: Psychointellectual Development > Critical Component > Object Permanence

Integrated Processes: Nursing Process
 Client Need: Health Promotion and Integrity
 Cognitive Level: Application [Applying]
 Concept: Growth and Development
 Difficulty: Moderate

	Feedback
1	This is correct. Object permanence is one of the most important developments in the sensorimotor stage. The child will learn that an object exists even when it cannot be seen or heard. Prior to this, the child does not understand that someone or something did not disappear. Playing peek-a-boo is a good way to help the development of object permanence.
2	This is incorrect. The nurse's comment is not the best answer; the baby is most likely to cry because of a lack of object permanence.
3	This is incorrect. Instructing the mother to move the baby when she leaves the room is not the best answer. The nurse needs to explain the development of object permanence and share ideas of how to assist the development.
4	This is incorrect. Ignoring the baby is not the best suggestion. The mother needs to understand what development is taking place and how to assist in the process.

PTS: 1 CON: Growth and Development

5. ANS: 3

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 4. Discuss psychosexual growth and development according to Sigmund Freud.

Page: 83

Heading: Psychosexual Development > Key Stages of Freud's Psychosexual Theory

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Freud's psychosexual theory states that from birth to 1 year, sexual gratification is achieved orally. However, it is not important if the mother breast or bottle feeds her infant.
2	This is incorrect. The infant may be attempting to gratify sexual urges with oral behaviors such as sucking, biting, chewing, and eating. It is not important to the nurse how long the infant sleeps at night.
3	This is correct. Normal development requires not depriving oral gratification, such as weaning too soon or a rigid feeding schedule. Because of the infant's vigorous fist sucking, the nurse needs to ascertain what type of feeding schedule is being followed.

4	This is incorrect. Asking if the infant draws up the legs when crying may be assessing for the presence of colic; the information is not related to the infant's fist-sucking behavior.
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PTS: 1 CON: Growth and Development

6. ANS: 2

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 4. Discuss psychosexual growth and development according to Sigmund Freud.

Page: 83

Heading: Psychosexual Development > Key Stages of Freud's Psychosexual Theory

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. Oedipal or Electra conflicts, which existed during the phallic stage, are resolved during the latency stage (6 to 11 years of age).
2	This is correct. The energy focus is on socialization and increasing problem-solving abilities.
3	This is incorrect. Curiosity about anatomical differences occurs in the phallic stage.
4	This is incorrect. The school-aged child's sexual drives are submerged.

PTS: 1 CON: Growth and Development

7. ANS: 1

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 3. Discuss psychosocial growth and development according to Eric Erikson.

Page: 84

Heading: Psychosocial Development

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is correct. Using knowledge about Erikson's theory of psychosocial growth and development, the nurse needs to reassure the parent that the toddler is expected to exhibit parallel play. Parallel play is part of the autonomy versus shame and doubt stage of Erikson's theory; autonomy and independence is being developed.

2	This is incorrect. The toddler's play behavior is normal and is not an indication of becoming a shy, introverted adult.
3	This is incorrect. The manner of play being exhibited by the toddler is more closely related to the development of autonomy and independence and not part of learning to be alone.
4	This is incorrect. Solitary play is expected during Erikson's stage of trust versus mistrust (birth to 1 year) and is not what the toddler is exhibiting.

PTS: 1 CON: Growth and Development

8. ANS: 3

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 3. Discuss psychosocial growth and development according to Eric Erikson.

Page: 85

Heading: Psychosocial Development > Key Stages of Erikson's Psychosocial Development Theory

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. During the identity versus role confusion stage of Erikson's theory, 12- to 18-year-olds are preoccupied with how they are seen in the eyes of others. However, this manifestation exists even in the absence of peer violence.
2	This is incorrect. Sexual orientation issues are not manifestations of bullying; however, sexual orientation can cause a person to be a target for bullying and other violence.
3	This is correct. The nurse's concern is focused on the possibility the students involved in any aspect of bullying, physical violence, and rejection will be unable to provide a meaningful definition of self, which places them at risk for role confusion in one or more roles throughout life.
4	This is incorrect. Students who are aggressive may develop a strong sense of guilt as adults, but this is just one aspect of the impact of bullying, physical violence, and rejection—not the most important one.

PTS: 1 CON: Growth and Development

9. ANS: 1

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 5. Discuss social-moral growth and development according to Lawrence Kohlberg.

Page: 87

Heading: Table 6-7: Kohlberg's Three Stages of Moral Development

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]
 Concept: Growth and Development
 Difficulty: Difficult

	Feedback
1	This is correct. The children in the family are all in the preconventional level of Kohlberg's theory. Stages include obedience and punishment orientation, and individualism and exchange. Characteristics include following rules set by those in authority and behavior adjusted according to good/bad and right/wrong thinking.
2	This is incorrect. A personal and functional value system is constructed by the child and is a characteristic of the postconventional autonomous level of Kohlberg's theory.
3	This is incorrect. When the focus of the child is on following rules and maintaining social order, the nurse recognizes the conventional level of Kohlberg's theory.
4	This is incorrect. When a child's value systems are independent of authority figures and peers, the child is exhibiting the characteristics of the postconventional autonomous level of Kohlberg's theory.

PTS: 1 CON: Growth and Development

10. ANS: 4

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 6. Discuss the theory of nature versus nurture.

Page: 87

Heading: Nature Versus Nurture

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse's information is adequate if a parent recognizes the importance of physical activity and development, even though the parent identifies as being nonathletic.
2	This is incorrect. The nurse's information is adequate if a parent recognizes the value of using "lost" time for learning.
3	This is incorrect. The nurse's information is adequate if a parent understands that labeling items will lead to word recognition.
4	This is correct. The nurse needs to provide additional information to the parent who thinks books are a waste of money. The parent needs to be aware of community agencies that will supply books to children and of programs that provide reading/story times.

PTS: 1 CON: Growth and Development

11. ANS: 2

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 6. Discuss the theory of nature versus nurture.

Page: 87

Heading: Nature Versus Nurture

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Materials for healthy diets and lifestyles may be helpful for the parents to alter their behaviors and implement good nurturing skills into their parenting; however, the topic of nature also needs to be addressed.
2	This is correct. The nurse is aware that nature involves the traits, capacities, and limitations that a person inherits from parents at conception. Genetically, the infant may not become tall due to any nurturing behaviors; however, the infant can be strong and athletic because of nurturing.
3	This is incorrect. At some point, the nurse can introduce information about how the parents can alter their lifestyles and positively impact the nurturing of their infant. At this time there is a more immediate need.
4	This is incorrect. The nurse needs to clarify the impact of both nature and nurturing during the infant's lifetime.

PTS: 1

CON: Growth and Development

12. ANS: 1

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 8. Analyze factors that affect growth and development.

Page: 88

Heading: Factors that Affect Growth and Development > Chronic Illness and Hospitalization

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is correct. When the parent states increasing difficulty keeping this toddler confined, the nurse needs to assess for additional information. The nurse needs to reinforce that play is what children do, and playing is important to learn the developmental skills needed to reach the most optimal functioning. Confinement may not be appropriate.
2	This is incorrect. The parent is expressing a truth; chronically ill children do affect the entire family. However, this is not the comment that gives the nurse the greatest cause for seeking additional information.

3	This is incorrect. Other children, especially siblings, often accept the chronically ill child without difficulty. This is not the comment that gives the nurse cause to seek additional information.
4	This is incorrect. It is true that hospitalization of a child does cause severe stress on the child and family members. However, the comment that causes the nurse to seek additional information is related to promoting behavior and development.

PTS: 1 CON: Growth and Development

13. ANS: 3

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 8. Analyze factors that affect growth and development.

Page: 88

Heading: Factors that Affect Growth and Development > Birth Events (Prematurity, Birth Trauma)

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. The premature infant is expected to catch up developmentally by 2 years of age.
2	This is incorrect. Developmental milestone have a narrow range for normal; variation is not extensive.
3	This is correct. Premature infants can experience delayed growth and development and are thus expected to reach developmental milestones at the same age they would have reached them if born at normal gestational age. Age is adjusted for assessments: subtract the weeks/months that the infant was born prematurely from the current chronological age.
4	This is correct. Infants are unique; however, the ranges set for developmental milestones are accurate enough for early detection of delays and other issues.

PTS: 1 CON: Growth and Development

14. ANS: 2

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 8. Analyze factors that affect growth and development.

Page: 89

Heading: Factors that Affect Growth and Development > Environmental Factors

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Violence

Difficulty: Difficult

	Feedback
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1	This is incorrect. Being denied needed medical attention is an indication of medical neglect and should be reported as such.
2	This is correct. Frequent episodes of genital irritation is indicative of possible sexual abuse and should be reported as such.
3	This is incorrect. The patient's admission of multiple incidences of skipping school may or may not indicate educational neglect; additional information is needed.
4	This is incorrect. When the patient reports that an older brother "hurts" her, the nurse recognizes possible physical abuse; however, sexual abuse may or may not be present. Additional information is needed before reporting sexual abuse, but physical abuse is reported.

PTS: 1 CON: Violence

MULTIPLE RESPONSE

15. ANS: 2, 3, 4, 5

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 9. Understand nursing applications of growth and development theories.

Page: 84

Heading: Psychosocial Development > Key Stages of Erikson's Psychosocial Development Theory

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is incorrect. Play during this stage is known as cooperative play and involves more than one person.
2.	This is correct. The school-aged child enjoys working in groups and forming social relationships.
3.	This is correct. Developing a sense of industry provides the child with purpose and confidence in being successful; participation in small tasks will fulfill this need.
4.	This is correct. If a child is unable to be successful, this can result in a sense of inferiority. Success needs to be recognized and rewarded.
5.	This is correct. The school-aged child in this stage follows the rules and likes order.

PTS: 1 CON: Growth and Development

16. ANS: 1, 2, 5

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 7. Apply principles of family-focused care in approaches toward the child.

Page: 87

Heading: Family

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is correct. The grandmother's statement indicates stress related to the expected behavior of a preschool child. A referral for community programs for the child aimed at playing and learning is appropriate and beneficial.
2.	This is correct. The nurse needs to make a referral to social services who can determine if the household is entitled to assistance for raising a grandchild. Services can include food, clothing, childcare, and medical services, to name a few.
3.	This is incorrect. The nurse does not have enough information to make a referral to legal services. The scenario does not clarify the reasons why the child is being raised by grandparents.
4.	This is incorrect. The scenario does not provide any information indicating the child is being neglected, abused, or living in an unfit environment.
5.	This is correct. The nurse needs to be sensitive that the grandmother may be expressing caregiver stress. The nurse needs to make referrals to programs that can benefit the grandparents physically, psychosocially, and spiritually.

PTS: 1

CON: Growth and Development

17. ANS: 1, 2, 3

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 7. Apply principles of family-focused care in approaches toward the child.

Page: 87

Heading: Family

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
1.	This is correct. The nuclear family is composed of a mother, a father, and a biological or adopted child or children.
2.	This is correct. The term nonnuclear family describes family forms other than traditional, such as single-parent homes, grandparents functioning in the role of parents, same-sex parents with a child or children, and blended families.

3.	This is correct. Blended families are those in which families from divorce are joined together by remarriage. This can also occur when a spouse has died and the remaining spouse remarries.
4.	This is incorrect. The blended family does not become a nuclear family just because all the children live in the same home.
5.	This is incorrect. The term blended family is not applied until after marriage occurs between two persons, each having biological children.

PTS: 1 CON: Family Dynamics

18. ANS: 2, 5

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 8. Analyze factors that affect growth and development.

Page: 88

Heading: Factors that Affect Growth and Development > Intrauterine Factors

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is incorrect. Anemia during pregnancy can cause anemia in the infant; however, the mother's comment indicates the condition was treated during pregnancy and continues. The comment does not cause the nurse concern.
2.	This is correct. The nurse is concerned by this comment on two levels. Poor nutrition in the mother can lead to low-birth-weight babies, as well as slow development, compromised neurological performance, and impaired immune status. The mother's attitude about "fat people" may carry over through the lifetime of the infant and cause insufficient nutrition for growth and development and/or psychosocial issues.
3.	This is incorrect. The comment by the mother that she was healthy during pregnancy does not cause the nurse concern. Certain maternal illnesses can harm the fetus, such as rubella.
4.	This is incorrect. The avoidance of cat feces during pregnancy does not cause the nurse concern.
5.	This is correct. Maternal smoking can result in infants with low birth weight and/or congenital anomalies such as cleft lip and cleft palate. The nurse needs to provide teaching about the effects of smoking during pregnancy, especially if another pregnancy is planned.

PTS: 1 CON: Growth and Development

19. ANS: 1, 3, 5

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 8. Analyze factors that affect growth and development.

Page: 88

Heading: Factors that Affect Growth and Development > Critical Component > Play as a Stress Reducer

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is correct. Play is what children do and should not be overlooked when a child is in the hospital. Play is important for younger children to build the skills needed for development. All ages of pediatric patients can use play as a stress reducer.
2.	This is incorrect. Confined children will benefit from designated play time. However, the pediatric nurse is focused on nursing care. A better plan is to have a program where volunteers will come and play with patients who are confined.
3.	This is correct. Many pediatric facilities have a child-life specialist on staff who can assist the child in fostering growth and developmental needs through play. An extension of services to meet the needs of all hospitalized children is appropriate.
4.	This is incorrect. Hospitalized pediatric patients feel more comfortable with their own favorite toys and books from home. However, this intervention does not necessarily address the nurses' concerns about developmental delays.
5.	This is correct. When hospitalized, patients will view TV as a distraction to the manifestations of illness and effects of treatment. However, with pediatric patients, TV provides an opportunity for skills development as well as entertainment.

PTS: 1 CON: Growth and Development

20. ANS: 1, 2, 4

Chapter: Chapter 6 Growth and Development of the Child

Chapter Learning Objective: 9. Understand nursing applications of growth and development theories.

Page: 89

Heading: Basic Needs

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
--	----------

1.	This is correct. If the student in question goes to bed every night at 10:00 the nurse recognizes a physiological need that is not being met. Children need more sleep than adults. Sleep deprivation can impact the growth and development of a child and cause delays.
2.	This is correct. If the student in question expresses worry about fighting between parents, the nurse recognizes the child has the need to be protected from harm and may not feel safe. Fear and worry can interfere with developmental achievements.
3.	This is incorrect. When the student in question expresses pleasure during family game and movie nights, the student feels loved and has a sense of belonging.
4.	This is correct. Negative feedback interferes with the development of esteem, which is related to the need to respect one's self and be respected by others.
5.	This incorrect. The student is expressing the feeling of self-esteem related to successfully teaching a sibling a physical skill.

PTS: 1

CON: Growth and Development

Chapter 7 Newborns and Infants

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is providing care for a neonate who is identified as being at risk for neonatal abstinence syndrome. Which assessment finding causes the nurse to expect pharmacological interventions for this neonate?
1. Central and autonomic nervous system irritability and dysfunction are present.
 2. The Finnegan neonatal abstinence score is 7 at 3 hours after birth.
 3. Symptoms are not alleviated with swaddling, comforting, and feeding.
 4. The Finnegan neonatal abstinence score is 10 at 3 hours after birth.
- _____ 2. The neonatal nurse is assessing a term neonate in the delivery room. Which respiratory assessment finding at 15 minutes after birth causes the greatest concern to the nurse?
1. A single episode of apnea occurs for 18 seconds in a 1-minute period.
 2. Respirations are irregular, sporadic, shallow, and diaphragmatic.
 3. Acrocyanosis is pronounced in all four extremities.
 4. Respiratory rate fluctuates between 30 and 60 breaths per minute.
- _____ 3. The nurse in the neonatal care unit notices that a term neonate has a respiratory rate of 66 breaths per minute and exhibits pallor and lethargy. Which action does the nurse take immediately?
1. Unwraps the neonate and assesses for the presence of hypotonia.
 2. Wraps in an additional blanket and puts a knitted cap on the neonate.
 3. Takes the neonate to the mother and assesses sucking reflex during a feeding.
 4. Contacts the neonate's physician and seeks permission to take a rectal temperature.

- _____ 4. A neonate is delivered 3 weeks before the due date. The nurse performs assessment of gestational age by using the Ballard maturational scoring tool. Which assessment result indicates a premature infant?
1. A rating of the highest scores for physical maturity and neuromuscular activity
 2. A rating of the lowest scores for physical maturity and neuromuscular activity
 3. A rating of the lowest scores based on weight, length, and head circumference
 4. A rating of the highest scores based on weight, length, and head circumference
- _____ 5. The nurse is assessing an infant at 1 month of age. At birth the infant weighed 7 pounds 10 ounces. Which is the minimum weight the nurse will expect during this assessment?
1. 8 pounds 2 ounces
 2. 8 pounds 10 ounces
 3. 8 pounds 14 ounces
 4. 9 pounds 6 ounces
- _____ 6. The nurse in a pediatric clinic is preparing to assess an infant at the age of 9 months. The nurse is aware that this well-baby checkup will involve assessment for developmental delays or disabilities. Which finding causes the nurse to suspect a developmental delay?
1. Displays sucking reflex when presented with bottle or pacifier.
 2. Cries and reaches for the parent when placed on the examination table.
 3. Loses interest in an object that is dropped out of sight by the nurse.
 4. Waves or shakes the head in response to verbal cues “bye-bye” and “no.”
- _____ 7. The RN in a pediatric office is preparing to administer oral medication to an infant. Before the actual administration of the medication, which initial action does the nurse take?
1. Obtain an accurate weight of the infant.
 2. Provide the caretaker with written information.
 3. Assist the caretaker in holding the infant supine.
 4. Acquire a calibrated syringe for administration.
- _____ 8. The nurse in a pediatric clinic is frequently required to administer medications to infants. Which approach will the nurse take when administering an immunization injection?
1. Use a 1-inch needle with a maximum of 1.5 mL of fluid.
 2. Keep the infant on the caregiver’s lap and use distraction.
 3. Explain the medication and the purpose in simple terms to the infant.
 4. Immediately comfort the infant with cuddling and patting.
- _____ 9. The nurse is teaching a class to parents about emergency care for newborns and infants. Which comment from a parent indicates a need for additional teaching?
1. “I will apply gentle tourniquet pressure for serious extremity bleeding.”
 2. “The most important behavior is to remain calm and get emergency help.”
 3. “A seriously injured child is moved only if they are in a dangerous situation.”
 4. “If a seizure occurs, the safest place for the child is on the floor with the head turned.”

- _____ 10. The nurse is providing care for a newborn who was delivered at 34 week's gestation. The nurse understands the newborn is a greater risk for death or chronic care needs. Which recommendation does the nurse make to the parent to prevent the newborn from developing a common complication?
1. Strictly limit the newborn's exposure to persons outside the family for 6 months.
 2. Start a series of palivizumab prophylaxis immunization as advised by the pediatrician.
 3. Strongly express the need for more frequent pediatric visits to prevent complications.
 4. Reinforce not allowing the premature newborn to cry for more than 10 minutes.
- _____ 11. During a parenting class, a father states, "My wife cannot hold the baby without singing, even in public. It's kind of embarrassing for me." Which opinion does the nurse share with the group after researching the topic?
1. Caregivers sometimes sing for their own attention and benefit.
 2. The father can walk away from public places if the mother sings.
 3. The mother needs to find another way of comforting her baby.
 4. Singing of lullabies by caregivers has a calming effect on infants.
- _____ 12. The nurse is interviewing a parent of a 9-month-old infant during a well-baby visit. Which statement by the parent causes the nurse concern about infant safety?
1. "He loves to get his toys out of a big plastic storage bag on the doorknob."
 2. "He thinks the TV remote is a toy, so it is kept on a shelf too high for him to reach."
 3. "When we eat cooked vegetables, I cut a few into bite size pieces so he can try them."
 4. "It really disturbed me to see my neighbor's infant with a pacifier on a cord around the neck."
- _____ 13. The nurse in a community pediatric clinic screens for conditions that cause concern for possible infant abuse or neglect. Which situation will prompt the nurse to recommend parenting education?
1. The stay-at-home mother of four children (ages 4 to 16 years) who is bringing home a newborn.
 2. The teenage couple with a newborn who live apart, but the father babysits during the day so the mother can attend school.
 3. The newly relocated couple with a young infant who are now 12 hours away from family and friends.
 4. The parents of an infant who live apart because the father of the infant is married and has a family with his wife.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 14. The nurse is preparing to teach to parents the importance of play in the newborn's and infant's life. Which information will the nurse plan to include in the class? *Select all that apply.*
1. Play is how infants learn about their environment and themselves.
 2. Infants may be startled by their own images in a reflective toy.

3. Older siblings are encouraged to share their toys with the infant.
4. Toys should provide a means of sensory stimulation for the infant.
5. Toys can help with physical and fine motor development.

_____ 15. The nurse is providing teaching to parents who are expecting their first child. For which reason does the nurse understand this teaching to be so important? *Select all that apply.*

1. It provides information that defines health and normalcy.
2. It instructs parents about general, safe infant care.
3. It helps reduce the appearance of regressive behavior.
4. It encourages development of parent-infant interactions.
5. It presents methods to help maintain infant/child health.

_____ 16. The nurse is providing counseling to the caregivers of a 4-year-old child who was born with a genetic condition that interferes with physical and psychosocial development. The child has had multiple hospitalizations for illnesses and infections related to the genetic condition. For which reasons will the nurse suggest the caregivers consider placement in a pediatric medical home? *Select all that apply.*

1. The caregivers will be relieved of care needed for 24 hours every day.
2. The home is set up to provide pediatric care from birth to adulthood.
3. The child's medical history indicates health risks related to family care.
4. The model is designed to reflect care standards suggested by pediatricians.
5. Care is coordinated by an interdisciplinary team that includes the caregivers.

_____ 17. The nurse is providing care for a hospitalized infant who is 4 months of age. While making a plan of care for the patient, which interventions specific to the patient's age will the nurse include? *Select any that apply.*

1. Encourage caregivers to go home to rest and sleep.
2. Suggest a favorite or comfort item be brought from home.
3. Plan for invasive procedures to be performed in a treatment room.
4. Monitor the infant for behaviors and cues indicating separation anxiety.
5. Educate caregivers to leave the side rails of the crib down during the day.

_____ 18. The nurse is providing information to parents of infants regarding home safety. Which information will the nurse present that is specific to this age group? *Select any that apply.*

1. Crib safety standards
2. Avoiding taking the infant out in public
3. Hazards of immunizations
4. Diaper rash prevention
5. Signs and symptoms of illness to report

_____ 19. The nurse in a pediatric emergency department is concerned when parents bring a 9-month-old infant in for possible injury. X-rays indicate the infant has a broken leg. Which information causes the nurse to report possible physical abuse to the nursing supervisor? *Select all that apply.*

1. The infant has been treated three times for injuries.
2. The parents insist on simple, noninvasive medical care.
3. The father states the infant climbed a stepladder and fell.
4. The infant buries his face in the mother's arm if the father talks.
5. The mother states she fell down the stairs with the baby.

- _____ 20. The parent of an infant reports to the nurse a suspicion that the babysitter is neglecting the infant. The parent states, "I saw some disturbing things on a hidden nanny-cam." The nurse will support the decision to replace the babysitter if which behavior is observed? *Select all that apply.*
1. The infant is in the crib and ignored until time for the parent to arrive home.
 2. The babysitter takes the infant out in the stroller for 1 hour on a cool day.
 3. The infant cries and is given a bottle, which is propped up in the crib.
 4. When the infant drops toys on the floor, the sitter tosses them back into the crib.
 5. Prescribed medication is not given to the infant according to written instructions.

Chapter 7 Newborns and Infants

Answer Section

MULTIPLE CHOICE

1. ANS: 4

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 1. Describe the perinatal factors that affect the newborn during the transition process.

Page: 94

Heading: Maternal History > Perinatal Substance Abuse

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Addiction and Behaviors

Difficulty: Difficult

	Feedback
1	This is incorrect. Central and autonomic nervous system irritability and dysfunction is an expected finding in the neonate with abstinence syndrome.
2	This is incorrect. A Finnegan neonatal abstinence score of 7 at 3 hours after birth is not an indication for pharmacological intervention.
3	This is incorrect. The American Academy of Pediatrics (AAP) recommends opioid replacement therapy for infants who do not respond to nonpharmacological management; however, there is a more accurate indicator in this scenario.
4	This is correct. The Finnegan neonatal abstinence score of 10 at 3 hours after birth is the strongest indicator of the need for pharmacological interventions for this neonate. Pharmacological interventions are with morphine sulfate or methadone adjusted based appropriately for scores greater than 8.

PTS: 1

CON: Addiction and Behaviors

2. ANS: 1

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 1. Describe the perinatal factors that affect the newborn during the transition process.

Page: 94

Heading: Transition to Extrauterine Life > Newborn Respiratory Assessment and Development

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
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1	This is correct. Apnea, a cessation of breathing longer than 15 to 20 seconds, is indicative of an alteration in respiratory transitioning. This assessment finding is of greatest concern to the nurse.
2	This is incorrect. Irregular, sporadic, shallow, and diaphragmatic respirations are considered normal for a neonate. This finding is not a cause of concern for the nurse.
3	This is incorrect. Acrocyanosis in hands and feet is normal in the term infant in the first 24 to 48 hours. After 24 to 48 hours of life, this may be an indication of cardiac disease. This finding is not a cause of concern for the nurse at this point in time.
4	This is incorrect. Average respiratory rate at birth is 30 to 60 breaths per minute. This finding is not a cause of concern for the nurse.

PTS: 1 CON: Patient-Centered Care

3. ANS: 2

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 1. Describe the perinatal factors that affect the newborn during the transition process.

Page: 96

Heading: Transition to Extrauterine Life > Thermoregulatory System Transition and Assessment

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The initial assessment findings of the neonate are indicative of cold stress. Hypotonia is also a manifestation of cold stress, and the condition can exacerbate cold stress. An immediate action by the nurse is not to unwrap the neonate.
2	This is correct. The initial assessment findings of the neonate are indicative of cold stress. Immediately, the nurse needs to take actions that will conserve body temperature. A knitted cap will cover the largest exposed area for heat loss; wrapping an additional blanket around the neonate will preserve heat.
3	This is incorrect. An additional sign of cold stress is a poor sucking reflex. The nurse can assess for this manifestation in the nursery, and care should not be delayed by transporting the neonate to the mother for sucking evaluation. The immediate action is to decrease heat loss.
4	This is incorrect. Rectal temperature is obtained after birth to ascertain anal patency and is repeated only if prescribed by the physician and then with great care. The nurse can obtain an axillary temperature. Immediate nursing actions are to decrease heat loss; additional assessments can be performed after efforts to preserve body heat.

PTS: 1 CON: Patient-Centered Care

4. ANS: 3

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 3. Describe the physical assessment approaches for the newborn and infant.

Page: 115

Heading: Physical Development: Table 7-8 Ballard Maturational Assessment Tool

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. The lowest score from all the assessments needs to be based on the physical size of the neonate and does not include neuromuscular assessment.
2	This is incorrect. The highest score from all the assessments needs to be based on the physical size of the neonate and does not include neuromuscular assessment.
3	This is correct. The scores for the assessments are plotted on a graph to provide a gestational age based on weight, length, and head circumference to determine if the neonate is appropriate for gestational age. Term neonates have higher scores than premature neonates. Premature neonates have lower scores; being born at 37 weeks' gestation does indicate prematurity.
4	This is incorrect. The scores for the assessments are plotted on a graph to provide a gestational age based on weight, length, and head circumference to determine if the neonate is appropriate for gestational age. Term neonates have higher scores than premature neonates.

PTS: 1

CON: Growth and Development

5. ANS: 3

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 2. Describe the growth and development that occur during the newborn and infancy period.

Page: 118

Heading: Developmental Milestones > Birth to 3 Months

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Birth to 1 month, the infant is expected to gain a minimum of 5 ounces weekly. The weight of 8 pounds 2 ounces is below the expected minimum weight.

2	This is incorrect. Birth to 1 month, the infant is expected to gain a minimum of 5 ounces weekly. The weight of 8 pounds 10 ounces is below the expected minimum weight.
3	This is correct. Birth to 1 month, the infant is expected to gain a minimum of 5 ounces weekly. The minimum expected weight for the infant weighing 7 pounds 10 ounces at birth is 8 pounds 14 ounces.
4	This is incorrect. Birth to 1 month, the infant is expected to gain a minimum of 5 ounces weekly. A weight of 9 pounds 6 ounces is the maximum expected weight for the infant weighing 7 pounds 10 ounces at birth. Normal weight gain during the first month is 5 to 7 ounces weekly.

PTS: 1 CON: Growth and Development

6. ANS: 3

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 3. Describe the physical assessment approaches for the newborn and infant.

Page: 119

Heading: Developmental Milestones > Six to Nine Months

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Evaluation [Evaluating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. This does not suggest a developmental delay. The sucking reflex does not disappear until 10 to 12 months of age.
2	This is incorrect. By the age of 9 months and 1 year, an infant is more comfortable with familiar surroundings and people and expresses dissatisfaction with strangers or strange surroundings (stranger anxiety). It is not concerning for the infant to display this behavior.
3	This is correct. When a 9-month-old infant loses interest in an object that is dropped out of sight by the nurse, the nurse is concerned by a possible developmental delay. At 9 months of age the infant is expected to know where to look for an object that has been dropped.
4	This is incorrect. Around 9 months the infant is expected to say “Dada” and “Mama” and understands bye-bye and no. The nurse is not concerned by the infant’s response.

PTS: 1 CON: Growth and Development

7. ANS: 1

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 5. Describe approaches to medication administration in the newborn and infant.

Page: 129

Heading: Nursing Procedures Specific to Newborns and Infants > Medication Administration

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is correct. Pediatric dosing must be precise to ensure adequate therapeutic levels; dosing is based on weight. The weight needs to be obtained first.
2	This is incorrect. The caretaker will be provided with written information, but this action can take place before or after the administration.
3	This is incorrect. The caretaker is asked to hold the infant during medication administration. The infant is held in a cradled position used for breast or bottle feeding. The infant is not laid in a supine position until he or she swallows all the medication.
4	This is incorrect. The nurse carefully calculates the correct dose and draws the medication up in a calibrated syringe to assure the exact dose is given. Caretakers are discouraged from using household spoons, because they vary in size and can result in over- or underdosing.

PTS: 1 CON: Safety

8. ANS: 2

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 4. Discuss the variations in nursing procedures related to the care of the newborn and infant.

Page: 130

Heading: Safe and Effective Nursing Care > Understanding Medication > Neonates and Medication Administration

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. For a small infant, a 5/8-inch needle with a maximum of 0.5 mL of fluid is used. For larger infants, a 1-inch needle with a maximum of 1.0 mL of fluid is administered into the vastus lateralis. For adolescents, the nurse should use a 1- to 1.5-inch needle (22- to 27-gauge) and up to 3 mL of fluid maximum.
2	This is correct. The nurse should approach the infant slowly and at eye level. Handle the infant gently, keep the infant on the caregiver's lap, and use distraction in order to decrease anxiety.
3	This is incorrect. The nurse may explain the medication and the purpose to the caregiver but not to the infant.

4	This is incorrect. The parent should be encouraged to comfort the infant with cuddling and patting. Additional interactions by the nurse at this time is likely to cause more stress.
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PTS: 1 CON: Safety

9. ANS: 1

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 7. Discuss the emergency care of the newborn and infant.

Page: 131

Heading: Emergency Care for the Infant and Newborn

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. The parent who describes applying a tourniquet in any manner for serious extremity bleeding needs additional teaching. When bleeding occurs with an infant, pressure with a clean cloth needs to be applied to the bleeding site.
2	This is incorrect. Additional teaching is not needed for the parent who understands the importance of remaining calm and getting help during an emergency situation.
3	This is incorrect. Additional teaching is not needed for the parent who understands that a seriously injured newborn/infant should only be moved if in a dangerous situation. Examples would be a burning building, in a car, or underwater.
4	This is incorrect. Additional teaching is not needed for the parent who understands that a seizing newborn/infant should be lowered to the floor and the head turned to the side. The nurse may remind the parents that nothing is placed in the mouth of a newborn/infant who is seizing.

PTS: 1 CON: Patient-Centered Care

10. ANS: 2

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 10. Discuss chronic care of the newborn and infant.

Page: 135

Heading: Chronic Care for the Infant and Newborn > Premature Infants

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
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1	This is incorrect. There is no protocol that supports the nurse recommending strict limitation of the newborn's exposure to persons outside the family for 6 months. However, the nurse should state that the presence of sick persons, and public exposure in general, increases the risk of illness in a premature newborn.
2	This is correct. The prevention for RSV (a common complication for premature neonates) is palivizumab (Synagis) prophylaxis immunization; there are significant benefits to premature infants and infants who are less than 35 weeks. Palivizumab is given in no more than five monthly doses during RSV season (late October to late January).
3	This is incorrect. There is no basis for the nurse to strongly express the need for more frequent pediatric visits to prevent complications.
4	This is incorrect. There is no basis for the nurse to reinforce not allowing the premature newborn to cry for more than 10 minutes.

PTS: 1 CON: Health Promotion

11. ANS: 4

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 13. Discuss alternative/complementary therapies for the family of the newborn and infant.

Page: 135

Heading: Evidence-Based Practice > Music Therapy and Infant

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Evidence-Based Practice

Difficulty: Moderate

	Feedback
1	This is incorrect. Research indicates the singing of lullabies by caregivers has a calming effect on infants, even if caregivers do not know the words to the songs. There is no mention that the parent is seeking attention or personal benefit.
2	This is incorrect. The father can walk away from the mother who sings to her baby in public; however, the action may create tension in the relationship. The nurse can suggest quiet humming or quiet recorded music when in public.
3	This is incorrect. Music is an effective way to calm an infant. Researchers and studies have shown that music therapy has the potential to improve cognitive development. Playing soft and soothing music improves psychological well-being. Quiet recorded music will still provide the positive benefits to the infant.
4	This is correct. Research indicates music has been shown to stimulate neurological growth and development, to calm, and to improve sleeping patterns in infants. This is an appropriate research finding to present to parents.

PTS: 1 CON: Evidence-Based Practice

12. ANS: 1

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 12. Describe the safety measures needed to care for a newborn and infant in the home.

Page: 136

Heading: Safety Measures for Infants and Newborns > Choking

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is correct. The nurse is concerned that the infant is getting toys out of a large plastic bag hanging on a doorknob. Plastic bags or wrappings are never kept where the infant can reach them because of the risk for suffocation or choking. If the bag is hung by a cord, there is an additional risk of choking.
2	This is incorrect. Swallowed batteries are a choking hazard and can cause serious damage to the GI tract if swallowed. The nurse is not concerned because the TV remote is kept out of the infant's reach.
3	This is incorrect. An infant who is 9 months of age can be safely given cooked vegetables cut into small bite-sized pieces. Pieces that are too large or too small can present a choking hazard.
4	This is incorrect. The parent's recognition that anything on a cord around an infant's neck is a choking hazard indicates an understanding about infant safety. The nurse can suggest that the information be shared with the neighbor.

PTS: 1 CON: Safety

13. ANS: 2

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 14. Describe child abuse considerations in the newborn and infant populations.

Page: 137

Heading: Safe and Effective Nursing Care > Clinical Pearl > Daddy Boot Camp

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Evaluation [Evaluating]

Concept: Violence

Difficulty: Difficult

	Feedback
1	This is incorrect. The stay-at-home mother of four children (ages 4 to 16 years) and a newborn does not cause the nurse concern about abuse or neglect.
2	This is correct. The teenage couple with a newborn will cause the nurse the greatest concern about infant abuse or neglect. Because the couple live apart, the father is not exposed to the infant except when he is babysitting; unfamiliarity with the infant's behaviors or needs can lead to neglect. Frustration related to a lack of knowledge and/or immaturity places the infant at risk for abuse. The nurse needs to strongly recommend a program such as daddy boot camp to this couple.

3	This is incorrect. The newly located couple with an infant will cause the nurse mild concern. Further assessment is needed regarding the mother's emotional and psychosocial needs. Technology allows close connections with family and friends who are located far away.
4	This is incorrect. The volatility of the situation surrounding the birth of this infant may or may not cause the nurse concern over the risk of infant abuse and/or neglect. Further assessment is needed.

PTS: 1

CON: Violence

MULTIPLE RESPONSE

14. ANS: 1, 4, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 11. Describe the importance and function of play in the newborn and infant's life.

Page: 121

Heading: Cognitive Development > Play

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse needs to explain that play is an important part of an infant's/child's development process. Play promotes learning about the environment and self.
2.	This is incorrect. Infants love to look at their image in a mirror or shiny surface; this activity promotes learning about self. It would be unusual and unexpected for an infant to be startled by their own image.
3.	This is incorrect. Infant toys are carefully selected for safety and age appropriateness. The toys of older siblings may have characteristics that present safety concerns.
4.	This is correct. Infants explore the world with their mouths and imitate those around them. It is important to provide toys to initiate and promote learning through these methods.
5.	This is correct. Physical development is promoted through play activities. Early on, infants will begin to develop gross and fine motor development; toys should be selected that will promote this type of learning.

PTS: 1

CON: Health Promotion

15. ANS: 1, 2, 4, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 6. Describe the health promotion functions necessary in an infant and newborn's family structure.

Page: 130

Heading: Anticipatory Guidance for the Family of a Newborn

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse includes developmental milestones, home infection control measures, the importance of immunizations, and the use of car seats and other safety equipment.
2.	This is correct. The nurse will discuss general health-care concerns, such as nutrition, oral health, the need for sleep, and appropriate hygiene care.
3.	This is incorrect. The nurse will help the parents with psychological preparation for the roles of caregiver, which is necessary to alleviate fears and anxiety in the care of the infant. Education for parents may help parents and family cope with sibling transition, including appearance of regressive behavior in siblings; however, this is the parents' first child.
4.	This is correct. The nurse will share the importance of parent–infant interactions, including playing, cuddling, the importance of talking to the child, and separation anxiety.
5.	This is correct. The nurse will share information about nutrition, oral health, prevention of illness and infections, prevention of injury, and childproofing the home. The nurse emphasizes the importance of learning infant/child CPR. The nurse will also cover the care for an infant/child that is ill.

PTS: 1 CON: Health Promotion

16. ANS: 2, 4, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 9. Describe the specific characteristics of a pediatric medical home during the newborn and infancy period.

Page: 130

Heading: Anticipatory Guidance for the Family of a Newborn > Pediatric Medical Home

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. The nurse may be concerned about caregiver stress and fatigue; however, the nurse focuses primarily on the needs of the patient.

2.	This is correct. The pediatrician-designed model of the pediatric medical home involves providing care to patients from birth to adulthood.
3.	This is incorrect. The nurse is not judgmental about family care and does not place blame on the caregiver's for the patient's unstable health. The state of the patient's health, and the necessary care, is not provided in detail.
4.	This is correct. The AAP developed the pediatric medical home model to deliver primary care to the child and family in a coordinated and comprehensive approach.
5.	This is correct. A medical home integrates and coordinates care through interdisciplinary coordination with the child, family, primary physicians, specialists, hospitals, health-care systems, public health, and the community.

PTS: 1 CON: Patient-Centered Care

17. ANS: 2, 3

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 8. Describe the specific characteristics of care of the hospitalized newborn and infant.

Page: 131

Heading: Hospitalized Infant and Newborn > Nursing Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is incorrect. The nurse should make arrangements in the plan of care for the caregiver(s) to room in with the hospitalized infant.
2.	This is correct. Even at 4 months, a familiar item from home provides comfort to the infant. The caregiver needs to bring a favorite toy or blanket.
3.	This is correct. The nurse will make arrangements in the plan of care to perform all painful or invasive procedures in a treatment room. Performing such procedures at the crib site will create anxiety for the infant whenever the nurse approaches.
4.	This is incorrect. At 4 months of age, the infant is not likely to experience separation anxiety, which most commonly occurs at 6 months of age.
5.	This is incorrect. The nurse will include safety teaching to the caregivers in the plan of care. Safety risks in the hospital, such as lowered crib rails, the infant's crawling on the floor, or the presence of items that may not be in the infant's home need to be addressed. Crib side rails need to be up, especially if the infant is alone in the room.

PTS: 1 CON: Safety

18. ANS: 1, 4, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 12. Describe the safety measures needed to care for a newborn and infant in the home.

Page: 135

Heading: Safety Measures for Infants and Newborns > Anticipatory Guidance for Injury Prevention

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. Infants spend most of their time in cribs, which must be safe—no bumper pads, slats no more than 2-3/8 inches apart.
2.	This is incorrect. Keep anyone with a cough, cold, or infectious disease away from the infant; otherwise, it is acceptable to take the infant out in public.
3.	This is incorrect. Immunizations need to be administered at specific times beginning as an infant.
4.	This is correct. Prevention is the best treatment for diaper rash, which can cause pain and possible infections because of a loss of skin integrity.
5.	This is correct. Parents need to understand the importance of calling a physician if the infant has a fever, refuses to eat, has vomiting and/or diarrhea, is more fussy or quieter than usual or looks jaundiced, and if they are worried or have questions about the infant's growth or development.

PTS: 1 CON: Safety

19. ANS: 1, 3, 4, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 14. Describe child abuse considerations in the newborn/infant population.

Page: 138

Heading: Child Abuse Considerations > Physical Abuse

Integrated Processes: Nursing Process

Client Need: Safe and Effective Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Violence

Difficulty: Difficult

	Feedback
1.	This is correct. Because of unexplained or repeated injuries such as welts, bruises, burns, fractured skull, broken bones, and especially spiral fractures, the nurse will report probable physical abuse to the nursing supervisor.
2.	This is incorrect. Neglect of the child (e.g., dirty, undernourished, inappropriate clothes for the weather, lack of medical or dental care) is indicated when the parents insist on simple, noninvasive medical care for a known diagnosis, such as a broken leg.

3.	This is correct. When the father states the infant climbed a stepladder and fell, the nurse will suspect physical abuse, because injury explanation is unlikely given the age or ability of the child. The nurse will report the situation to the nursing supervisor.
4.	This is correct. Fearful or detached behavior by the infant, especially when the father talks, is a probable sign of physical abuse. The nurse will report the behavior to the nursing supervisor.
5	This is correct. Disagreement or inconsistency in the parent/caregiver explanation of the injury can be an indication of physical abuse. The nurse can seek clarification about details of the accident and/or inquire about injury to the mother during the fall. The nurse is likely to report probable physical abuse of the infant.

PTS: 1 CON: Violence

20. ANS: 1, 5

Chapter: Chapter 7 Newborns and Infants

Chapter Learning Objective: 14. Describe child abuse considerations in the newborn/infant population.

Page: 138

Heading: Child Abuse Considerations > Neglect

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Evaluation [Evaluating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. Consistent failure to respond to the child's need for stimulation, nurturing, encouragement, and protection, or failure to acknowledge the child's presence, is indicative of neglect. This observation would warrant support from the nurse to replace the babysitter.
2.	This is incorrect. Taking the infant out in the stroller on a cool day is not considered neglect unless the infant is not dressed appropriately.
3.	This is incorrect. Propping a bottle is not alone considered to be neglect; the absence of nurturing is more closely related to neglect.
4.	This is incorrect. When the babysitter tosses toys back into the crib after the infant drops them on the floor there may or may not be an indication of neglect. Further evaluation of the attitude, manner, and verbal response is needed.
5.	This is correct. Failure to provide the infant with prescribed medication according to written instructions is considered neglect and warrants replacement of the babysitter.

PTS: 1 CON: Safety

Chapter 8 Toddlers and Preschoolers

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse in a pediatric clinic is assessing the motor development of a 3-year-old patient. The nurse reviews the toddler's last assessment results prior to determining changes. Which new development does the nurse expect to find during assessment?
1. Climbs on furniture unassisted
 2. Rides a bicycle with training wheels
 3. Independently builds a tower of seven blocks
 4. Climbs stairs while holding the railing
- _____ 2. The nurse is evaluating the language skills of a 2-year-old patient. Which assessment finding causes the nurse to suspect a developmental delay?
1. States, "Want mommy!"
 2. Points to objects named by the nurse
 3. Converses using two short sentences
 4. Repeats sounds but not words said by the nurse
- _____ 3. During a routine pediatric visit the nurse evaluates the cognitive skills of a toddler. The nurse draws a circle on paper and places the crayon in the toddler's right hand. The toddler shifts the crayon to the left hand and draws a circle. Which advice does the nurse provide to the parent?
1. Respect the toddler's preference of one hand over another.
 2. Watch the toddler's tendency to alternately use both hands.
 3. Reassure the parent that hand preference is not established until age 5 years.
 4. Gently insist drawing instruments be placed in the right hand.
- _____ 4. The nurse is evaluating the motor development of a preschooler at age 5 years. Which assessment finding is essential in order for the child to be considered ready for preschool?
1. Dress independently
 2. Use the toilet without assistance
 3. Draw stick figures with two or more body parts
 4. Throw overhand and catch a bounced ball
- _____ 5. The nurse in a pediatric clinic is performing a routine assessment on a preschool child. The nurse plans to evaluate some of the child's growth and development by interacting directly with the child. Which level of language does the nurse expect if the child is 4 years of age?
1. State full name and address without prompting
 2. Appropriately converse using two to three sentences
 3. Answer questions consistently with a "yes" or "no"
 4. Speak clearly enough to be understood
- _____ 6. The nurse is involved in a clinic screening for the kindergarten readiness of preschoolers. Primarily the children being screened are between the ages of 4 and 5 years. Which child does the nurse recognize as being ready to attend school?
1. The 4-year-old who points correctly to three different colors and voices reluctance to start school.

2. The 5-year-old who is unhappy about being told there is a 5-minute wait and then refuses to count.
3. The 4-year-old who counts to 10, recalls part of a story, and asks questions about the screening.
4. The 5-year-old who insists a parent stay with him, counts to 10, knows five colors, and recalls a story.

- _____ 7. The nurse is gathering assessment information from the parent of a 5-year-old child. The parent states, "I am very frustrated. She insists on doing things alone even if it is a struggle to do it right, and gets angry if I redo the task." Which information will the nurse share with the parent to promote greater understanding?
1. Assure the parent that the child will become more compliant as she matures.
 2. Suggest how the parent can critique the child's actions without hurting feelings.
 3. Encourage the parent to set aside time each week to teach the child the correct way to do things.
 4. Explain the child's interest in new things, the need to be independent, and pride in her abilities.
- _____ 8. A parent has brought a toddler to a new pediatric clinic for a routine visit. The nurse will obtain a health history from the parent. Which information is most important for the nurse to gather?
1. Chief complaint
 2. Family medical history
 3. Toddler medical history
 4. Social history
- _____ 9. The nurse in a pediatric office is performing physical assessments on multiple patients. Which patient will the nurse specifically report to the physician because of physical assessment findings?
1. The 4-year-old patient with a blood pressure of 110/75 mm Hg, pulse of 98 beats/minute.
 2. The 3-year-old patient with a history of prematurely closed fontanelles who has a headache.
 3. The 2-year-old patient with asthma who exhibits abdominal breathing at 26 breaths per minute.
 4. The 3-year-old patient with a soiled diaper, at the 70th percentile of weight and height.
- _____ 10. The nurse works in the pediatric unit of a hospital and is currently providing care for a 1-year-old patient. Which action by the nurse is most important for maintaining the safety of this patient?
1. Question about the presence of smoke and carbon monoxide detectors in the home.
 2. Check the temperatures of water, food, and drinks in order to prevent burns.
 3. Provide caregiver education on basic home, outdoor play, and car safety measures.
 4. Regularly check equipment in the crib environment for potential safety hazards.
- _____ 11. The nurse is providing care for a 5-year-old patient whose tonsils were removed this morning. The nurse identifies the patient is in pain but not willing to speak. The nurse uses the Wong-Baker FACES scale for pain evaluation. Which indicator does the nurse expect the patient to use to describe the level of pain?
1. The frowning face out of a series of faces

2. A number between 7 and 10 from a scale of 0 to 10
3. An intense red color on a range from pink to deep red
4. The word that identifies the degree of pain (i.e., ouch, hurts bad)

- _____ 12. A 4-year-old patient is on a regular regimen of medications for a chronic condition. The parent expresses frustration because of the difficulty in administering the medications. Which advice will the nurse provide to the parent for managing the process?
1. Ask if the child wants pills or a liquid form of medicine.
 2. Inquire if the child prefers the medication at a certain time.
 3. Make sure the medicine is sweet and refer to it as candy.
 4. Put the medication in juice or milk as preferred by the child.
- _____ 13. The nurse is providing care for a preschooler who is 5 years of age. The patient is hospitalized for treatment of a broken femur from a fall. Which behavior by the caregiver is of most concern to the nurse?
1. Requests updates on and changes in the patient
 2. Frequently asks questions and expresses concerns
 3. Remains adamant about staying at the bedside
 4. Insists in reviewing all changes in the plan of care
- _____ 14. The nurse is asking a parent of a toddler at age 18 months if there are any particular parenting challenges at this time. Which advice will the nurse offer if the parent shares issues with separation anxiety?
1. The parent needs to just leave quickly and ignore the toddler's protests.
 2. The parent needs to keep reassuring the toddler that the parent will return.
 3. The toddler is to be left only with family members until the fear subsides.
 4. The parent needs to plan leaving times to coincide with the toddler's naps.
- _____ 15. The nurse finishes a series of parenting classes on the topic of tantrums and discipline. Which comment by an attending parent causes the nurse concern?
1. "We have learned to ignore her and she stops."
 2. "I will give snack and a nap if he is that grumpy."
 3. "He plays and then suddenly screams for no reason."
 4. "She is learning that a tantrum means a time-out alone."

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 16. The nurse is caring for a 1-year-old patient after surgery for an intracranial shunt replacement. The nurse selects the FLACC scale for assessment because of the toddler's inability to participate in pain evaluation. The nurse will recognize which assessment finding as an indication of some level of pain? *Select all that apply.*
1. Constantly frowns, clenched jaw, quivering chin
 2. Squirms, shifting back and forth, tense
 3. Cries steadily and loudly, sometimes screams or sobs
 4. Legs are positioned normally and appear relaxed
 5. Answers to name, sucks thumb, and holds toy

- _____ 17. A community center is offering classes taught by pediatric nurses on summer safety for toddlers and preschoolers. Which topics will the nurses include in the teaching plan? *Select all that apply.*
1. Safety near swimming areas
 2. Dangers of toys being left in a pool
 3. Safety related to flotation devices
 4. Bicycle safety for riders or passengers
 5. Safety during trampoline play
- _____ 18. A parent tells the nurse a toddler is exhibiting signs of being ready to potty train. Which action by the parent will draw the nurse's approval? *Select all that apply.*
1. "I am initially teaching my son to urinate sitting down."
 2. "He has to sit on the potty 10 minutes each hour."
 3. "We are still using diapers in order to avoid messes."
 4. "Accidents result in the loss of a favorite toy for the day."
 5. "We are using treats, stickers, and new underwear as incentives."
- _____ 19. The parent of a toddler at age 2 years and a preschooler at age 4 years is sharing a concern about sibling rivalry. The parent states, "It is so upsetting to see them fighting with each other. I am afraid one of them will hurt the other." Which interventions will the parent and nurse design together in a plan for management? *Select all that apply.*
1. Set rules defining acceptable behavior.
 2. Separate them to opposite sides of the room.
 3. Teach children to be kind to each other.
 4. Recognize the toddler has increased risk for injury.
 5. Assist with appropriate expression of feelings.
- _____ 20. The community pediatric nurses are making home visits to families who have children either above or below the normal ranges in weight and/or height. One nurse visits a home with three qualifying children under the age of 5 years who are all below the standards for their ages. Which interventions will the nurse introduce to the caretakers? *Select all that apply.*
1. Prepare food for a toddler to eat seven times a day.
 2. Serve generous portions and insist on a clean plate.
 3. Encourage grazing throughout the day.
 4. Physically feed the children to assure better intake.
 5. Serve a variety of foods to provide varied nutrients.

Chapter 8 Toddlers and Preschoolers

Answer Section

MULTIPLE CHOICE

1. ANS: 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 143

Heading: Toddler Growth and Development (Age 1 to 3 Years) > Motor Development

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse recognizes that a toddler can climb on furniture by age 2 years.
2	This is incorrect. The nurse expects a toddler to ride a tricycle (3 wheels by 3 years); however, riding a bicycle with training wheels is not expected at this time.
3	This is correct. A toddler by age 3 is expected to build a tower with six blocks or more; a 2-year-old will build a tower of four blocks or more.
4	This is incorrect. The nurse expects a toddler to be able to climb steps while holding on by age 21 months.

PTS: 1

CON: Development

2. ANS: 4

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 144

Heading: Toddler Growth and Development (Age 1 to 3 Years) > Language Skills of a Toddler

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Evaluation [Evaluating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. The toddler at age 2 years is expected to speak in sentences consisting of two to three words; however, words such as "I," "me," and "you" are not expected to be used until age 3 years.

2	This is incorrect. A toddler is expected to point at objects named by another person; this is an expected skill by age 2 years.
3	This is incorrect. A toddler who is 3 years of age can converse using two to three sentences.
4	This is correct. Toddlers like to repeat words that are overheard. The expectation is for the toddler at 2 years of age to repeat a single word spoken by the nurse. The inability to perform this action may cause the nurse to suspect a developmental delay or a hearing defect.

PTS: 1 CON: Growth and Development

3. ANS: 1

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 144

Heading: Toddler Growth and Development (Age 1 to 3 Years) > Cognitive Skills of a Toddler

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is correct. The nurse advises the parent to respect the toddler's preference of using one hand over the other.
2	This is incorrect. There is no reason for the nurse to advise the parent to watch for the toddler having a tendency to alternate the use of both hands.
3	This is incorrect. The preference of using one hand over the other is established as a toddler; well before the age of 5 years.
4	This is incorrect. The nurse does not advise the parent to insist, in any manner, that writing instruments be placed in the right hand. The toddler is expected to demonstrate a preference for using one hand over the other; ignoring the preference is a source of frustration and/or confusion for the toddler.

PTS: 1 CON: Growth and Development

4. ANS: 2

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 144

Heading: Preschooler Growth and Development (Age 3 to 5 Years) > Motor Development of a Preschooler

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. A preschooler is expected to dress and undress independently by age 5 years. Some assistance may be required for tying shoe laces; however, this is not an essential skill for entering preschool.
2	This is correct. Before entering preschool, the preschooler needs to be able to use the toilet without assistance.
3	This is incorrect. A preschooler at age 4 years is expected to draw stick figures with more than two body parts. However, this is not an essential skill in order for a 5-year-old to enter preschool.
4	This is incorrect. By the age of 4 years, most preschoolers can throw overhanded and catch a bounced ball. However, this is not an essential skill in order for a 5-year-old to enter preschool.

PTS: 1 CON: Growth and Development

5. ANS: 2

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 145

Heading: Preschooler Growth and Development (Age 3 to 5 Years) > Language Skills of a Preschooler

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Evaluation [Evaluating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Preschoolers may not be able to state their full name and address without prompting until closer to the age of 5 years.
2	This is incorrect. A preschooler 4 years of age is expected to speak in sentences consisting of 4 to 5 words. A toddler is expected to converse using two to three sentences; the expectation is the same for a preschooler 4 years of age.
3	This is incorrect. A 4-year-old preschooler may or may not consistently answer questions with a “yes” or “no.” The format of the question will influence the length of the answers.
4	This is correct. The nurse cannot always expect the 4-year-old preschooler to speak clearly enough to be understood by strangers. This ability should be evident by age 5 years.

PTS: 1 CON: Growth and Development

6. ANS: 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 145

Heading: Preschooler Growth and Development (Age 3 to 5 Years) > Cognitive Skills of a Preschooler

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. This 4-year-old does not know at least 4 colors, but most importantly, voices reluctance to start school. It is best if the preschooler feels positively about school.
2	This is incorrect. The 5-year-old appears to have difficulty understanding the concept of time. The refusal to count may be indicative of immaturity related to a refusal to follow directions.
3	This is correct. A 4-year-old who can count to 10, recall part of a story, and asks questions about the screening exhibits a readiness for school.
4	This is incorrect. This 5-year-old is passing the screening requirements; however, there should be concern about maturity. A preschooler who is reluctant to be away from a parent may not be ready to attend school.

PTS: 1

CON: Patient-Centered Care

7. ANS: 4

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 1. Identify normal growth and development for toddlers and preschoolers.

Page: 145

Heading: Preschooler Growth and Development (Age 3 to 5 Years) > Social and Emotional Milestones of a Preschooler

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. The preschooler is demonstrating appropriate milestones related to social and emotional development. The nurse does not tell the parent that the child will become more compliant with age.
2	This is incorrect. Both parent and child will benefit from knowledge that the preschooler is demonstrating expected growth and development. The parent needs to encourage and not critique the child's efforts.

3	This is incorrect. Social and emotional development is based on the child's interest in demonstrating independence and ability. The parent can best teach by example.
4	This is correct. The behavior of this preschooler reflects the milestones related to social and emotional development. The nurse should explain behaviors in a positive way and encourage the parent to support expected growth and development.

PTS: 1 CON: Growth and Development

8. ANS: 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 2. Discuss the nursing assessment of toddlers and preschoolers.

Page: 145

Heading: Nursing Assessment > Health History

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The information provided in this scenario indicates the visit is routine at a new pediatric clinic; there is not likely to be a chief complaint.
2	This is incorrect. Family history (genetic disorders, chronic diseases, childhood cancers) is important; however, there is another choice that is most important.
3	This is correct. The most important health history information is the toddler's medical history, which will include childhood illnesses, hospitalizations, surgeries, immunizations, and results of vision/hearing/developmental screens.
4	This is incorrect. While the information will be included, the social history (living arrangements, day care/preschool, and behavior) is likely to be considered the least important during an initial visit.

PTS: 1 CON: Patient-Centered Care

9. ANS: 2

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 2. Discuss the nursing assessment of toddlers and preschoolers.

Page: 145

Heading: Nursing Assessment > Physical Assessment

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
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1	This is incorrect. The blood pressure and pulse rates for the 4-year-old patient are both within normal limits. The nurse does not need to specifically report these findings.
2	This is correct. The 3-year-old patient with a history of prematurely closed fontanelles has a condition that will affect the growth of the head. The presence of a headache is an indicator of possible increased intracranial pressure. Because there is a potential for brain damage, the nurse will report this finding specifically (and immediately) to the physician.
3	This is incorrect. Pediatric patients are more normally expected to be abdominal breathers, and a respiratory rate of 26 breaths/minute is normal for a 2-year-old patient. There is no reason for the nurse to specifically report these findings to the pediatrician. Symptoms related to asthma are absent.
4	This is incorrect. The 3-year-old patient who is in a soiled diaper and at the 70th percentile for weight and height may be exhibiting manifestations of neglect. The nurse may or may not specifically report these findings to the physician. Another patient is at greater risk at this time.

PTS: 1 CON: Growth and Development

10. ANS: 4

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 3. Identify safety risks for toddlers and preschoolers.

Page: 148

Heading: Safety > Nursing Interventions

Integrated Processes: Nursing Process

Objective: 4. Discuss safety interventions to minimize risks.

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse may inquire about the presence of smoke and carbon monoxide detectors in the home; however, the nurse in the acute care setting is most concerned with safety in the immediate environment.
2	This is incorrect. Checking the temperatures of water, food, and drinks is important, but the safety of the immediate environment is most important given the risk potential.
3	This is incorrect. Teaching the caregivers about safety measures outside of the hospital is important; however, the nurse is most concerned about the risks present in the acute care setting.
4	This is correct. The nurse in an acute care setting is most focused on safety during the hospitalization of a pediatric patient. The nurse needs to check equipment regularly, with special attention to wire and cord placement to minimize entanglement, suction availability at crib side, and minimal equipment and crib attachments to decrease choking and suffocation hazards.

PTS: 1 CON: Safety

11. ANS: 1

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 5. Identify and discuss pain scales used with toddlers and preschoolers.

Page: 149

Heading: Pain

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Comfort

Difficulty: Moderate

	Feedback
1	This is correct. The Wong-Baker FACES pain scale is a self-reporting rating scale that assigns a number value to a facial expression that is chosen by a child.
2	This is incorrect. Preschoolers have not developed the concept of numbers as values and cannot use a numbering scale. In addition, the patient is not willing to speak due to pain.
3	This is incorrect. There is no pain scale that uses colors.
4	This is incorrect. Preschoolers do not have the vocabulary to identify concepts such as pain.

PTS: 1 CON: Comfort

12. ANS: 1

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 6. Identify appropriate nursing interventions and education regarding pain and safety for toddlers and preschoolers and their caregivers.

Page: 149

Heading: Pain > Critical Component > Provide Appropriate Choices When Giving Medications

Integrated Processes: Teaching/Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is correct. Allowing a 4-year-old to have some choices regarding medication is likely to foster some cooperation. When possible, the child can decide if the medication is desired in a pill or liquid form.
2	This is incorrect. Medication is given at the prescribed time. The parent is advised to never ask the child if or when they want to take their medications.
3	This is incorrect. Regardless of the taste, medicine should never be referred to as candy. A child at 4 years of age is capable of understanding a simple, honest explanation such as, "This medicine is to help you feel better."

4	This is incorrect. Putting medication in any liquid or food becomes an issue if the child refuses to consume all of whatever is mixed with the medication. Offering a liquid or food of choice afterward is acceptable. However, the nurse needs to inform the parent if certain liquids or foods are to be avoided for prescribed medications.
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PTS: 1 CON: Safety

13. ANS: 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 6. Identify appropriate nursing interventions and education regarding pain and safety for toddlers and preschoolers and their caregivers.

Page: 154

Heading: Caregiver Support > In Hospital (Acute and Chronic Care)

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse encourages the caretakers of a hospitalized child to request updates on and changes in the patient. This behavior is not a concern to the nurse.
2	This is incorrect. The nurse encourages and supports the caretakers of a hospitalized child frequently asking questions and expressing concerns.
3	This is correct. The nurse needs to remind caregivers to care for themselves—reassure them that it is fine to go home for a while, whether to relieve stress, take a shower, go to sleep, and/or take care of other responsibilities. The caretaker's insistence on staying at the bedside is of greatest concern to the nurse.
4	This is incorrect. The nurse supports the caregiver's request that the plan of care and all changes be shared.

PTS: 1 CON: Patient-Centered Care

14. ANS: 2

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 7. Identify and discuss different challenges faced by toddlers and preschoolers.

Page: 152

Heading: Age Group Challenges > Separation Anxiety

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse will not advise the parent to ignore the toddler's protests; this action is likely to increase anxiety.
2	This is correct. The nurse will advise the parent to repeatedly reassure the toddler that the parent will be back. After the toddler is reassured, the parent needs to leave quickly.
3	This is incorrect. The nurse will not advise the parent to leave the toddler only with family members; the fear and anxiety will eventually stop. Some parents do not have family members available to fill this need.
4	This is incorrect. Coordinating times to be away with the toddler's nap time will only cause a different type of anxiety. The toddler may resist napping because of fear the parent will leave while the toddler sleeps.

PTS: 1 CON: Growth and Development

15. ANS: 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 7. Identify and discuss different challenges faced by toddlers and preschoolers.

Page: 152

Heading: Age Group Challenges > Tantrum and Discipline

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Evaluation [Evaluating]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse is likely to have shared that tantrums are often an attempt to get attention. If the parent finds that ignoring the behavior works, the nurse is not concerned.
2	This is incorrect. The nurse is likely to have shared that tantrum triggers can be hunger or tiredness, and so the parent sharing that grumpiness is treated with a snack and a nap does not cause the nurse concern.
3	This is correct. The nurse is aware that some tantrum triggers may indicate a problem related to mental, physical, or emotional issues. The child that is playing and suddenly screams for no reason will cause the nurse concern.
4	This is incorrect. The nurse is likely to have shared that appropriate discipline for a tantrum is a brief period of time-out in another room. The mother who states the use of this suggestion does not cause the nurse concern.

PTS: 1 CON: Growth and Development

MULTIPLE RESPONSE

16. ANS: 1, 2, 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 5. Identify and discuss pain scales used with toddlers and preschoolers.

Page: 149

Heading: Pain > FLACC Pain Scale

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Comfort

Difficulty: Difficult

	Feedback
1.	This is correct. Constantly frowning, with clenched jaw and quivering chin are in the face category on the FLACC pain scale, and the score is 2.
2.	This is correct. Squirming, shifting back and forth, and appearing tense are in the activity category on the FLACC pain scale, and the score is 1.
3.	This is correct. Crying steadily and loudly, sometimes screaming or sobbing, are in the cry category on the FLACC pain scale, and the score is 2.
4.	This is incorrect. FLACC pain scale assesses legs; if the legs are in a normal position and/or relaxed, the score is 0.
5.	This is incorrect. The FLACC pain scale assesses consolability; if the patient answers to their name and is sucking a thumb and holding a toy, the score is 0.

PTS: 1

CON: Comfort

17. ANS: 1, 2, 3, 4

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 6. Identify appropriate nursing interventions and education regarding pain and safety for toddlers and preschoolers and their caregivers.

Page: 149

Heading: Safety > Safety Education for Caregivers > Outdoor Safety

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. Child-proof all swimming areas, including access to pools, ponds, and lakes. Never leave children unattended near swimming areas, even if they can swim.
2.	This is correct. Toys left in a pool are a dangerous temptation, because children may be tempted to retrieve them.
3.	This is correct. Use flotation devices specifically designed for child safety. Floating toys, rafts, and rings do not provide adequate safety.
4.	This is correct. Children should always wear a bicycle helmet whether they are riding a bike, or they are the passenger on a bicycle. Children need to know the safety rules for riding on bicycle paths, sidewalks, or on the street.

5.	This is correct. Trampolines are a safety risk for children of all ages, but toddlers and preschoolers should not be permitted to play on trampolines.
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PTS: 1 CON: Safety

18. ANS: 1, 5

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 7. Identify and discuss different challenges faced by toddlers and preschoolers.

Page: 151

Heading: Age Group Challenges > Potty Training

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Evaluation [Evaluating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will approve of the parent initially teaching a boy to sit to urinate. Once mastered, then move on to standing. The parent or caregiver may use flushable toilet targets for teaching purposes.
2.	This is incorrect. The parent needs to have the child practice sitting on the potty for a few minutes at a time. To a toddler, sitting for a period of 10 minutes is frustrating and may begin to feel like a punishment. The nurse will not give approval of this action by the parent.
3.	This is incorrect. When the toddler exhibits readiness and potty training begins, the child is placed in disposable pullup underwear. The use of a diaper does not support a change in behavior. The nurse will not give approval of this action by the parent.
4.	This is incorrect. Taking away a favorite toy for the day after an accident is a form of punishment; the nurse will not give approval of this action by the parent.
5.	This is correct. Incentives are appropriate, and the nurse will approve of providing encouragement in the form of praise and celebration, along with rewards and incentives such as treats, stickers, and new underwear.

PTS: 1 CON: Growth and Development

19. ANS: 1, 3, 5

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 7. Identify and discuss different challenges faced by toddlers and preschoolers.

Page: 152

Heading: Age Group Challenges > Sibling Rivalry

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
1.	This is correct. Together the nurse and parent develop a plan that will set the rules for acceptable behavior. Rules will cover such behaviors such as no name calling, no pushing, and no slamming things.
2.	This is incorrect. The nurse and parent will opt for an intervention that places each child in their own space when a confrontational interaction occurs. Opposite sides of the same room still keeps the children within visual and auditory range of each other.
3.	This is correct. The nurse and parent will identify ways to teach the children to be kind to each other by encouraging apologizing, sharing, and comforting each other when hurt. This intervention will foster positive feelings and behaviors.
4.	This is incorrect. The toddler may or may not be at greater risk for injury; the parent needs to remain neutral and not pick sides. When there is a favored child, that child's negative behavior can escalate.
5.	This is correct. Children may be unable to express the proper feelings of anger and frustration. The nurse and parent will identify therapeutic methods designed for self-expression.

PTS: 1 CON: Family Dynamics

20. ANS: 1, 3

Chapter: Chapter 8 Toddlers and Preschoolers

Chapter Learning Objective: 7. Identify and discuss different challenges faced by toddlers and preschoolers.

Page: 152

Heading: Age Group Challenges > Nutrition

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Synthesis [Creating]

Concept: Nutrition

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will suggest the caregivers make food available for toddlers to eat approximately seven times a day, consuming more meals than snacks.
2.	This is incorrect. Serving larger portions than recommended and/or pressuring a child to clean his or her plate can lead to overeating and increased risk for childhood obesity. Allow children to self-regulate intake while providing healthy, nutrient-filled choices. In addition, underweight children do not need to be overfed.
3.	This is correct. Children should be allowed to graze throughout the day, as toddlers may not sit for three meals.
4.	This is incorrect. Toddlers and preschoolers are not likely to cooperate with being fed. Children are encouraged to self-regulate their intake in order to promote a healthy intake of nutrient-filled choices.
5.	This is incorrect. Introducing a variety of foods to children helps to broaden food choices. However, toddlers are also known to have "food jags," preferring to eat only one food over and over. This is a normal and self-limiting behavior.

PTS: 1

CON: Nutrition

Chapter 9 School-Age Children

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse in a pediatric clinic is gathering physical assessment information during a yearly visit on a 6-year-old child. Which finding does the nurse expect during the assessment?
1. A weight gain of 6-1/2 pounds
 2. A height increase of 5 inches
 3. A blood pressure of 108/72 mm Hg
 4. A pulse rate of 85 beats per minute
- _____ 2. The nurse is examining a 10-year-old child brought to the clinic because of episodes of shortness of breath, headaches, and stomach upset. The nurse notices bruises in various stages of resolution on the upper arms and upper legs. Which additional information is most important for the nurse to obtain?
1. Ask about the duration of the presenting symptoms.
 2. Ascertain if there is a change in school performance.
 3. Notify the physician of physical or sexual abuse.
 4. Seek information about the cause of the bruises.
- _____ 3. The nurses in a local health department are scheduled to perform screenings at a public elementary school. Which printed information will the nurses most likely send home with the students after the screening?
1. Importance of keeping immunizations up to date
 2. Procedure for handwashing to prevent illnesses
 3. Methods of treating and avoiding lice infestation
 4. Proper way to cover up when coughing or sneezing
- _____ 4. A school-aged child is being treated for an overdose of cough medicine administered by the parent. The child is 12 years of age and weighs 98 pounds. The label on the bottle states not to give the medication to children younger than 6 years, and provides a dosing chart that indicates an adult dose for children over 50 kg. For which reason does the nurse suspect the overdose occurred?
1. The cough continued after being medicated.
 2. The parent misunderstood weight parameters.
 3. The parent measured with a household spoon.
 4. The parent attempted to induce sleep in the child.
- _____ 5. The nurse is preparing to administer medications to school-aged patients. The nurse is aware the pediatric patient doses are different than medication doses for adults. Which factor does the nurse apply to administering pediatric medications?
1. Children's bodies are smaller and need half of the adult dose.
 2. The metabolic rate of a child is slower and can cause overdosing.
 3. Doses of medications are ordered according to the child's weight.
 4. The first dosage consideration is based on the age of the child.

- _____ 6. A new nurse on a pediatric unit is learning to use EMLA cream in preparation for painful procedures. A school-age patient is scheduled to receive IM medication. Which behavior by the nurse indicates an understanding about the use of EMLA?
1. Medication is applied 2 hours before the IM.
 2. The cream is covered with a section of gauze.
 3. A thin layer of medication is applied to the area.
 4. Cream remaining on the skin is gently rubbed in.
- _____ 7. The nurse in a pediatric office is preparing to remove stitches from an 8-year-old child's arm. Which approach by the nurse before the procedure is most effective?
1. Provide information in advance of how the procedure is performed.
 2. Tell the patient, "I will be back in fifteen minutes to take out your stitches."
 3. Have a coworker in the room to assist if the patient becomes uncooperative.
 4. Bring the instruments to the room and announce, "Let's get those stitches out."
- _____ 8. The nurse is invited to present a program on child safety to parents of school-age children in the fourth and fifth grades. Considering the age, which information does the nurse include in the teaching plan?
1. Immunization schedule
 2. Head injury prevention
 3. Age-appropriate toys
 4. Symptoms of illnesses
- _____ 9. The school nurse is attending a meeting with the teachers, school counselors, and parents of a student who has recently refused to attend school. The parents share that the student is either pretending to be ill or being untruthful about going to classes. Which initial intervention by the nurse is best?
1. Inquire about the student's feelings regarding school.
 2. Ask the parents if they have noticed physical injuries.
 3. Explain the impact of missing school to the student.
 4. Suggest homeschooling until the problem is resolved.
- _____ 10. The school nurse is presenting information on the importance of exercise to children in the fifth and sixth grades of school. Which reason does the nurse cite as being the best reason for being physically active?
1. Sports provide an opportunity to bond with peers.
 2. Normal childhood activities make exercise fun.
 3. Physical activity needs to replace technology hours.
 4. Early exercise will carry over as good habits for adults.
- _____ 11. The nurse is visiting the home of a family who is providing care for a school-age child with profound disabilities. The nurse notes that the primary caregiver looks tired and thin. The caregiver admits to feeling overwhelmed and then guilty about negative feelings. Which intervention by the nurse is most helpful?
1. Arrange for a counselor to meet with the family and assess for well-being.
 2. Suggest a means of child management so the parents can plan a short getaway.
 3. Provide literature about extended-care facilities where the child can be cared for.
 4. Recommend the caregiver to a support group where feelings can be shared.

- _____ 12. The nurse is gathering assessment data on a child who is 8 years of age and newly diagnosed with type 1 diabetes mellitus. The caregiver shares a reluctance to allow the child to return to school because the child's glucose levels need to be checked several times daily. Which information is most important for the nurse to share?
1. The caregiver may consider personally attending school with the child.
 2. The child's medical condition is manageable in school and other places.
 3. The school nurse can care for the special needs of children in school.
 4. The caregiver is interrupting psychosocial development in the child.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 13. The nurse in a pediatric clinic is preparing to assess an adolescent who is 12 years of age. Which behavior by the adolescent prompts the nurse to ask if the parent should leave the room? *Select all that apply.*
1. Acknowledges that the parent is aware of adolescent's sexual activity
 2. Refers some of the nurse's questions about health history to the parent
 3. Makes attempts to keep their body covered during physical assessment
 4. Looks tense and anxious when asked about body functions and changes
 5. Does not respond to questions relating to thoughts, feelings, and opinions
- _____ 14. The nurse is performing a routine physical assessment on a 7-year-old child. Which specific approaches does the nurse use for this child? *Select all that apply.*
1. Listen to information shared by the child.
 2. Start the exam with obtaining vital signs.
 3. Expect the child to be undressed and in a gown.
 4. Visually inspect the child's general appearance.
 5. Share which immunizations will be given today.
- _____ 15. The emergency department nurse manager receives a call that a school-age patient will be arriving shortly. The nurse is instructed to have a chaplain and social worker available for the family. The nurse manager is likely to anticipate which possible conditions of the arriving patient? *Select all that apply.*
1. Pneumonia
 2. Terminal cancer
 3. Car accident
 4. Child abuse
 5. Sports injury
- _____ 16. The school nurse attends a workshop focusing on the identification and prevention of bullying among school-age children. The nurse will be better able to identify both bullies and victims due to knowledge of which factors? *Select all that apply.*
1. Poor academic achievement
 2. Jealousy over sexual attention
 3. Lower education level of caregivers
 4. Leader or follower personality types

5. Poor health status or increased health needs

- _____ 17. The nurses at a community pediatric clinic are preparing a presentation about nutrition for the school-age child. Which information is important for the nurses to include? *Select all that apply.*
1. Over 35% of school-age children are considered to be obese or overweight.
 2. Overweight children have an increased likelihood of being overweight adults.
 3. Children with high BMIs have increased levels of lipids, insulin, and blood pressure.
 4. Notably higher health risks exist for adults who were obese during childhood.
 5. School-age children need comparatively more calories than infants or adolescents.
- _____ 18. The nurse is making a home visit for a child who is 9 years of age and currently unable to attend school because of an illness causing immunosuppression. The child seems depressed and tells the nurse, “I miss my friends and all the fun we had.” Which interventions will the nurse add to the plan of care to meet the child’s psychosocial needs? *Select all that apply.*
1. Have the parents take videos of friends for the child to view.
 2. Allow friends to visit from the doorway of the child’s room.
 3. Provide instructions on how to visit friends on a computer.
 4. Suggest that the child and friends send letters to each other.
 5. Ask parents to arrange daily time for telephone visiting with friends.
- _____ 19. The school nurse is discussing a student’s reasons for leaving school to be home schooled. The student has a chronic condition that causes mobility and strength deficits. The student states, “I want to stay in school, but I am always late for class. I can’t manage getting around fast enough with all my books.” Which suggestion by the nurse will best meet the needs of the student? *Select all that apply.*
1. Place the designated textbook in each class the student attends.
 2. Plan for the student to leave class early to get to the next class.
 3. Arrange for a set of textbooks to be left in the student’s home.
 4. Set up closed-circuit TV so the student “attends” from a set location.
 5. Inquire if the parents are able to purchase a motorized wheelchair.
- _____ 20. A 13-year-old student has cerebral palsy but is able to communicate well with others. The parents support their child being mainstreamed into a school setting even though self-care is limited. The school nurse likely needs to arrange for which type of care for this student? *Select all that apply.*
1. Tracheostomy suctioning
 2. Straight catheterizations
 3. Colostomy management
 4. Oral or enteral feedings
 5. Medication administration

Chapter 9 School-Age Children

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 1. Describe growth and development during the school-age years.

Page: 158

Heading: Growth and Development: Table 9-1: Vital Signs Typical for Age

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is correct. Children in this age group should gain 3 kg/year in weight, which is comparable to 6.6 pounds.
2	This is incorrect. Children in this age group should gain 5 cm/year in height (approximately 2 inches). The stated growth is too high.
3	This is incorrect. The expected blood pressure for a 6-year-old child is 95/55–70 mm Hg. The listed blood pressure is too high.
4	This is incorrect. The expected pulse rate for a 6-year-old child is 95 beats per minute; a rate of 85 beats per minute is too low.

PTS: 1

CON: Growth and Development

2. ANS: 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for school-age patients.

Page: 159

Heading: Growth and Development > General Survey

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Violence

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse does ask about the duration of the presenting symptoms; however, this is not the most important information for the nurse to obtain given other physical findings.

2	This is incorrect. The nurse does ask about changes in school performance because the physical symptoms may be an indication of school or home avoidance/problems, anxiety and stress, or depression. However, there is other more important information for the nurse to obtain.
3	This is incorrect. At some point, the nurse may identify the need to report suspected physical or sexual abuse to the physician. However, other information is needed first.
4	This is correct. The most important information for the nurse to obtain is the source of the bruising. The child may have a valid explanation, or the explanation may increase the nurse's concern. The nurse will evaluate the child's response and act accordingly.

PTS: 1 CON: Violence

3. ANS: 3

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for school-age patients.

Page: 159

Heading: Growth and Development > General Survey

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1	This is incorrect. Printed information regarding the importance of keeping immunizations up to date is appropriate and important. However, this need does not require the health department nurses to provide on-site screenings.
2	This is incorrect. Printed information about the proper procedure for handwashing to prevent illnesses is appropriate to send home. However, this need does not require the health department nurses to provide on-site screenings.
3	This is correct. Infestations of head lice are prevalent among school-age children. The health department nurses will perform on-site screenings for the presence of the problem. Infected students will receive notifications about positive identification. All students will receive printed material about the treatment and prevention of lice infestation.
4	This is incorrect. Teaching about how to cover the nose and mouth when coughing or sneezing can be shared with both the children and parents through printed information. The health department nurses do not need to perform on-site screenings.

PTS: 1 CON: Health Promotion

4. ANS: 2

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 3. Explain medication approaches for school-age patients.

Page: 161

Heading: Safe and Effective Nursing Care > Understanding Medication > Medication Dosing

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. There is no information in the scenario that indicates the cough medicine was ineffective.
2	This is correct. The nurse suspects that the parent misunderstood the weight parameters for the medication. The parent may have thought 50 kg was another way of indicating 50 pounds and given the child an adult dose. Multiple doses caused an overdose.
3	This is incorrect. Using a household spoon to administer liquid medication can cause either overdosing or underdosing with medication. There is no reference to the manner of measurement in the scenario.
4	This is incorrect. Some medications are capable of inducing sleep. There is no information in the scenario regarding the parent's intention other than the control of coughing.

PTS: 1 CON: Safety

5. ANS: 3

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 3. Explain medication approaches for school-age patients.

Page: 161

Heading: Safe and Effective Nursing Care > Understanding Medication > Medication Dosing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity > Pharmacological and Parenteral Therapies

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. Typically, children older than age 14 and over 50 kg should receive a standard adult dose. This is just a guide to ensure that the obese child does not receive a medication dose higher than the recommended adult dose.
2	This is incorrect. A child has a faster metabolic rate than an adult.
3	This is correct. Basing medication dosage on weight gives an accurate and safe dose for each patient.
4	This is incorrect. Medication dosing should not be based on age since patients' sizes may vary.

PTS: 1 CON: Safety

6. ANS: 1

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 3. Explain medication approaches for school-age patients.

Page: 161

Heading: Safe and Effective Nursing Care: Understanding Medication > Eutectic Mixture of Lidocaine and Prilocaine (EMLA) Cream

Integrated Processes: Nursing Process

Client Need: Physiological Integrity > Pharmacological and Parenteral Therapies

Cognitive Level: Evaluation [Evaluating]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is correct. It is most effective to apply EMLA to the site for an IM at least two hours before the procedure. The longer the medication is in place, the deeper it will penetrate.
2	This is incorrect. EMLA is covered with an occlusive dressing; a section of gauze will cause the medication to dry out or be removed.
3	This is incorrect. The application of EMLA includes placing a large “glob” on the skin and covering with an occlusive dressing.
4	This is incorrect. When preparing to perform the procedure, the remaining EMLA cream is removed, and then the skin is prepped for the procedure. EMLA cream is not rubbed or massaged into the skin.

PTS: 1

CON: Safety

7. ANS: 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 4. Explain variations in nursing procedures for school-age patients.

Page: 162

Heading: Physical Assessment Approaches > Variations on Nursing Procedures

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Stress

Difficulty: Moderate

	Feedback
1	This is incorrect. The longer a child is aware that a possibly painful or uncomfortable procedure is coming, the greater the stress that can occur.
2	This is incorrect. Anticipation of an uncomfortable or painful procedure will cause the patient to become more anxious and stressed.
3	This is incorrect. If the nurse brings a coworker into the room, the patient is likely to anticipate the nurse needing help during a painful procedure. A coworker can be close by if assistance is needed, but not in the room.

4	This is correct. If a procedure must be performed, have the instruments ready and inform the child immediately before the procedure. The action will keep the patient from becoming more anxious. The nurse needs to use a friendly and calm manner to help reduce patient stress.
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PTS: 1 CON: Stress

8. ANS: 2

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among school-age patients.

Page: 162

Heading: Health Promotion > Safety

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is incorrect. Most states require that children receive specific immunizations prior to beginning school. The topic of immunization is not appropriate for a presentation on child safety.
2	This is correct. Unintentional injury, including head injury, is the leading cause of death in this age group. Studies show that parents' knowledge of head injuries is limited. This information and the ages of the children makes this an important topic for the nurse to cover.
3	This is incorrect. Concern about age-appropriate toys is most likely to be of interest to parents of children who are preschoolers and younger. However, the nurse plans to cover bicycle, scooter, skateboard, and sports safety in the presentation.
4	This is incorrect. Presenting the symptoms of illnesses may be important in a different presentation. The nurse needs to present information that is age appropriate and safety focused.

PTS: 1 CON: Safety

9. ANS: 1

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 11. Recognize child abuse considerations relevant to school-age patients.

Page: 163

Heading: Health Promotion > School

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. Initially, the reasons for the student's behavior needs to be identified and addressed, and a good approach is to ask about the student's feelings. The nurse is particularly interested in psychological issues.
2	This is incorrect. The nurse may be concerned about the student being bullied at school; however, when the issue should be first approached, it should be with the student and not the parents.
3	This is incorrect. At some point it may be appropriate to discuss the impact of skipping school; however, initially the nurse needs to focus on the cause.
4	This is incorrect. Homeschool may be an option at some point; however, the initial focus is on the cause of the student's behavior.

PTS: 1 CON: Patient-Centered Care

10. ANS: 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among school-age patients.

Page: 163

Heading: Health Promotion > Exercise

Integrated Processes: Nursing Process.

Client Need: Health Promotion

Cognitive Level: Application [Applying]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. Encouraging participation in school exercise programs and available sports provides opportunity for physical activity; however, it is not necessarily the most important action for bonding with peers, which is a psychosocial need. The nurse needs to emphasize the physical purposes and benefits of exercise.
2	This is incorrect. Encouraging normal school-age activities, such as ball, jumping rope, bike riding, skating, and using playground equipment does make it fun to be physically active; however, the best reason for exercise and activity is the long-term impact.
3	This is incorrect. Today's children are seriously immersed in the technological world, which makes promotion of physical activity more difficult. Technology is likely to always be a part of the younger generation's lives. Replacement is not reasonable; balance needs to be promoted.
4	This is correct. As with nutrition, early education and experience with exercise can help to form good habits that can last a lifetime. This is the best reason for the nurse to promote physical activity, along with proven health benefits.

PTS: 1 CON: Health Promotion

11. ANS: 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 9. Integrate home-care concepts and considerations specific to the school-age population.

Page: 165

Heading: Care When Chronically Ill > Role of the Primary Caregiver

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
1	This is incorrect. The home care for a disabled child is frequently as stressful as it can be rewarding. The process can have a significant negative effect on the family; however, the nurse is evaluating the physical and psychological effects on the primary caregiver. The scenario does not present a need for family counseling.
2	This is incorrect. Sometimes respite or relief is needed. Marital stress and divorce may be more common with a family who has a child with special needs; however, the scenario does not indicate marital problems.
3	This is incorrect. The family of a profoundly disabled child may at some point be ready to consider placement in an extended-care facility; however, the immediate situation requires the nurse to attend to the needs of the caregiver.
4	This is correct. To combat the effects of caregiver fatigue, caregivers must have a good support network and ability to participate in activities that promote stress relief. The nurse's recommendation for a support group is the best intervention to address the caregiver's immediate needs.

PTS: 1

CON: Family Dynamics

12. ANS: 3

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 9. Integrate chronic care concepts and considerations specific to the school-age population.

Page: 165

Heading: Care When Chronically Ill > Home Care Considerations

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. Personal caregivers can attend school with children who have multiple needs. The caregiver will benefit most from information that addresses the management of the child's condition within the school setting.

2	This is incorrect. The nurse can provide information to the caregiver about methods to manage the child's medical condition in a variety of settings; however, management in school is the current concern.
3	This is correct. Most children can be cared for at school by a school nurse. Medications for asthma and diabetes are commonly administered by the school nurse. This information will be most beneficial to both the child and the caregiver.
4	This is incorrect. Informing the caregiver of interrupting the child's psychosocial development is untrue and places potential blame on the caregiver's concerns.

PTS: 1

CON: Health Promotion

MULTIPLE RESPONSE

13. ANS: 3, 4, 5

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 7. Identify strategies to support school-aged adolescents.

Page: 160

Heading: Physical Assessment Approaches

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. The nurse views the adolescent's openness with a parent as not being a reason to ask the parent to leave the examination room. The nurse will be cognizant of behaviors that indicate a change of feelings.
2.	This is incorrect. When the adolescent refers questions to the parent regarding the patient's health history, the nurse does not at that time feel a need to ask the parent to leave the examination room.
3.	This is correct. When the nurse notices the adolescent's attempts to keep their body covered, the nurse should inquire if the patient wants the parent to leave the room.
4.	This is correct. When the adolescent looks tense and anxious at any time during the examination, the nurse should inquire if the patient wants the parent to leave the room.
5.	This is correct. When the nurse notices the adolescent's lack of response to questions related to thoughts, feelings, and opinions, the nurse should ask if the patient wants the parent to leave the room.

PTS: 1

CON: Patient-Centered Care

14. ANS: 1, 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 4. Explain variations in nursing procedures for school-age patients.

Page: 162

Heading: Variations on Nursing Procedures

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse needs to listen to any information the child provides, especially if it pertains to any recent history or problems.
2.	This is incorrect. The nurse does not begin the physical exam with procedures; the nurse will look and listen first.
3.	This is incorrect. There is no reason for the school-age child to be undressed and in a gown for a routine examination. The child may be partially dressed or in their underwear.
4.	This is correct. The nurse can gather initial assessment data by visually examining the child for general appearance, muscle tone, and skin condition.
5.	This is incorrect. If the child needs an immunization, the nurse will explain it later after the physical examination is completed. Informing the child too early will cause stress and anxiety that may interfere with the physical examination.

PTS: 1 CON: Patient-Centered Care

15. ANS: 3, 5

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 6. Describe emergency care considerations for school-age patients.

Page: 163

Heading: Emergency Care

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Collaboration

Difficulty: Difficulty

	Feedback
1.	This is incorrect. Pneumonia is an illness that can require emergency care; however, the nurse is most likely to expect an unintentional accident because of the age group and the request for support staff.
2.	This is incorrect. Unintentional injuries and cancer are the leading causes of death for children between the ages of 5 to 14 years. It is unlikely for the nurse manager to expect the arrival of a child with terminal cancer.
3.	This is correct. The nurse is aware that unintentional injuries and cancer are the leading causes of death for children between the ages of 5 to 14 years. A car accident can be a source of serious body and head injuries, prompting the need for a chaplain and the services of a social worker.

4.	This is incorrect. Child abuse is an intentional act; the victims that are seen in the emergency department are most often younger children.
5.	This is correct. School-age children are at risk for unintentional injuries; sports are activities that can result in injuries requiring emergency medical attention. The request for a chaplain and social worker may be indicative of serious injury.

PTS: 1 CON: Collaboration

16. ANS: 1, 3, 5

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among school-age patients.

Page: 163

Heading: Health Promotion > Bullying

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will be able to associate poor academic achievement as a cause for either the bully or the victim.
2.	This is incorrect. Jealousy over sexual attention is not commonly recognized as a factor that contributes to the behavior of the bully; however, victims of bullying may be sexually abused.
3.	This is correct. The nurse will recognize that bullies and victims frequently have parents and caregivers with a high school or lower educational level.
4.	This is incorrect. Leader or follower personality traits can impact multiple areas of development in the school-age child. However, these traits are not specifically associated with bullies and their victims.
5.	This is correct. The nurse will recognize that poor health status, increased health needs, and mental health issues can be a factor in a child becoming either a bully or a victim.

PTS: 1 CON: Safety

17. ANS: 1, 2, 3, 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among school-age patients.

Page: 163

Heading: Health Promotion > Nutrition

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Nutrition

Difficulty: Difficult

	Feedback
1.	This is correct. Approximately 17.4% of school children are obese, with a BMI defined as greater than the 95th percentile by the CDC, and another 18% are overweight.
2.	This is correct. It is a fact that overweight children have a greater risk of being overweight adults. The health consequences of obesity in children will have a negative effect on their morbidity and mortality as adults.
3.	This is correct. A high BMI in children is linked with increased lipid levels, insulin levels, and blood pressure; these can lead to higher risks for atherosclerosis and obesity in adulthood.
4.	This is correct. The health consequences of obesity in children will have a negative effect on their morbidity and mortality as adults.
5.	This is incorrect. School-age children have a decreased caloric requirement compared with previous stages of development. During the next stage, adolescence, the child's nutrition requirements will increase again.

PTS: 1 CON: Nutrition

18. ANS: 3, 5

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 9. Integrate chronic care concepts and considerations specific to the school-age population.

Page: 165

Heading: Care When Chronically Ill > Home Care Considerations

Integrated Processes: Nursing Process

Client Need: Psychosocial integrity

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1.	This is incorrect. Watching videos of friends is a one-way method of communicating. The videos of the child's friends may actually cause additional depression. The child needs to be involved.
2.	This is incorrect. Immunosuppressed children are carefully kept away from any possible exposure from other persons. Visiting from the doorway is not the safest intervention.
3.	This is correct. School-age children are very technology savvy and most of them have access to computers. Providing instructions about how the child and friends can visit safely in current time will help meet the child's psychosocial needs.
4.	This is incorrect. Nine-year-old children may not be interested in written communication. Current technology can provide more personal and satisfying methods for meeting the child's psychosocial needs.
5.	This is correct. Verbal communication is a good way to help the isolated child keep in touch with friends. Daily telephone conversations will help meet the child's psychosocial needs.

PTS: 1 CON: Health Promotion

19. ANS: 1, 3

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 8. Integrate chronic care concepts and considerations specific to the school-age population.

Page: 166

Heading: Care When Chronically Ill

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The problem the nurse is attempting to solve is the student's inability to get to class because of difficulty carrying heavy textbooks. The student will benefit if a textbook is left in each assigned class.
2.	This is incorrect. Arranging for the student to leave class early in order to get to the next class does not address the cause of the problem, which is managing heavy textbooks.
3.	This is correct. Arranging for a set of textbooks to remain in the student's home will eliminate the need to carry heavy books back and forth from school to home.
4.	This is incorrect. Setting up a closed circuit TV so the student can attend class remotely does not fulfill the psychosocial needs of the student.
5.	This is incorrect. The nurse does not want to stifle the student's independence or diminish the student's physical abilities. A motorized wheelchair may be considered if the student becomes less mobile.

PTS: 1 CON: Patient-Centered Care

20. ANS: 2, 4

Chapter: Chapter 9 School-Age Children

Chapter Learning Objective: 8. Integrate chronic care concepts and considerations specific to the school-age population.

Page: 166

Heading: Care for Chronically Ill

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. Not all patients with cerebral palsy have a tracheostomy. The scenario does not indicate this is a need for this student.

2.	This is correct. Patients with cerebral palsy may have an indwelling catheter or need straight catheterization because of a lack of bladder control and limited self-care.
3.	This is incorrect. Patients with cerebral palsy do not routinely have colostomies. There is not information in the scenario to indicate this is a need for this student.
4.	This is correct. Due to limited self-care, the student is likely to need assistance with either oral or enteral feedings.
5.	This is incorrect. The student may or may not need medications during the school day.

PTS: 1

CON: Patient-Centered Care

Chapter 10 Adolescents

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is aware that the prefrontal cortex of the adolescent brain is still developing. The processes of critical thinking and decision making are in a stage of development. Which change does the nurse expect in the adolescent's thinking?
1. Some awareness of personal limitations
 2. Disinterested in politics and social issues
 3. Capable of identifying and setting short-term goals
 4. Exhibits confidence by not comparing self to peers
- _____ 2. The parents of an adolescent are distressed about the psychological changes in their child. Which example does the nurse validate as being a source of concern?
1. Constantly compares their body with others
 2. Comfortable with doing what the crowd does
 3. Regularly tests family limits and rules
 4. Presents self in a constantly changing personae
- _____ 3. The nurse is gathering health data on an adolescent who is 16 years of age. Which comment by the adolescent will cause the nurse to seek additional information?
1. "I have to be the clumsiest kid in the world. Always tripping over my own feet."
 2. "Some days I just hate school. I want to get out and on to a job or college."
 3. "I try to keep my distance from a kid in my class who coughs all day long."
 4. "I know that I have always been a skinny kid, but wish I could gain weight."
- _____ 4. The nurse is preparing to perform a physical examination of an adolescent who is 13 years of age. Which action by the nurse will decrease the adolescent's anxiety during the examination?
1. Teaching the anatomical names of body parts
 2. Verbalizing findings of physical abnormalities
 3. Referring patient concerns to the physician
 4. Telling the patient what to expect and why
- _____ 5. The nurse is preparing to perform a physical examination on a female who is 15 years of age. The patient states, "I am anxious about having my pelvic organs examined." Which statement does the nurse make?
1. "Slow breaths and relaxation will manage the pain."

2. "I will only briefly examine your external genitalia."
3. "Pelvic exams are recommended after your periods begin."
4. "The instruments are small and the examination is brief."

- _____ 6. The nurse is performing a physical examination on a male who is 15 years of age. The nurse notices the presence of gynecomastia. The patient states, "I hate these breasts and won't even take my shirt off in front of my friends." Which information does the nurse provide for this patient?
1. The condition is self-limiting.
 2. Surgical removal is recommended.
 3. It indicates the patient is overweight.
 4. The male hormone testosterone is deficient.

- _____ 7. The nurse is preparing information for an adolescent patient regarding a prescribed medication regimen. Which information does the nurse present to the patient?
1. The nurse directly verbalizes medication warnings to the patient.
 2. The patient will receive administration clarification by the pharmacist.
 3. Most medications are metabolized faster by adolescent patients.
 4. Symbols or phrases are sufficient to warn adolescents of medicine-related risks.

- _____ 8. The nurse is collecting health data from a 16-year-old male patient. The patient states, "I need to tell you, I had sex with another boy, and I am very confused." Which comment by the nurse is most therapeutic?
1. "Share your thoughts and feelings with me."
 2. "Maybe you should speak with your parents."
 3. "Tell me what was appealing about this person."
 4. "Let's explore your feelings toward the opposite sex."

- _____ 9. The nurse is screening adolescents for substance abuse. Which comment by an adolescent will require additional assessment by the nurse?
1. "I don't smoke cigarettes."
 2. "My grandpa died of lung cancer,"
 3. "I can't afford any kind of smoking."
 4. "If I smoke, I pay my own way to college."

- _____ 10. The nurse in a pediatric clinic is assessing a female adolescent who is 15 years of age. When the nurse performs a sexuality assessment, the patient states, "I have never had anything but safe sex." Which approach does the nurse take next?
1. Provide birth-control options.
 2. Ascertain the number of sex partners.
 3. Ask the patient to define "safe sex."
 4. Inquire about treatment of an STI.

- _____ 11. The nurse working in the emergency department of a pediatric care facility is receiving an adolescent patient with a gunshot wound to the head. The patient is unconscious but exhibiting signs of life-threatening deterioration. Which action does the nurse take?
1. Keep attempting to reach the parents.
 2. Determine if the patient is emancipated.
 3. Prepare the patient for surgery.

4. Call the facility's legal advisor.

- _____ 12. The nurse in an acute pediatric care setting is providing care for a 15-year-old patient. The patient is recovering from abdominal surgery. Which nursing intervention is appropriate for this patient?
1. Use FACES pain scale to show the patient a sense of humor.
 2. Ask if the patient wants to learn how to care for the incision.
 3. Tell the patient that IM injections will feel like a small pinch.
 4. Discourage long visits by peers by reinforcing the need for rest.
- _____ 13. The nurse is providing care for an adolescent patient who is hospitalized following a grand mal seizure. The condition has existed for 5 years and resulted in multiple hospitalizations. Which nursing intervention is appropriate for this client?
1. Ask the patient and family to consider homeschooling.
 2. Arrange for home care and regular nursing visitations.
 3. Explain methods to minimize the chronic medical events.
 4. Refer patient and family to applicable internet resources.
- _____ 14. During admission of an adolescent to the hospital for an acute illness, the nurse asks about the use of complementary and alternative medicine (CAM). In which manner does the nurse approach the topic to obtain an accurate answer?
1. Tells the adolescent that unreported CAM is a major cause of complications
 2. Asks the adolescent what kind of CAM is used in the home and among friends
 3. Informs the adolescent that CAM must be completely avoided when hospitalized
 4. States that CAM is actually a primitive type of modern medication therapy

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. The health department informs the school nurse that a high school student has been identified with active TB, and students are to be given a TB skin test. Which consideration does the nurse make in regards to student testing? *Select all that apply.*
1. Permission for testing is obtained from parents or guardians.
 2. Students and parents/guardians are assured of confidentiality.
 3. Only students who were in physical contact with the infected student are tested.
 4. Students are informed the test administration will cause minor pain.
 5. The rationale for the testing is explained to students and parents/guardians.
- _____ 16. An adolescent who is 12 years of age is a year into treatment for a malignant brain tumor. For which psychosocial and spiritual care will the nurse plan? *Select all that apply.*
1. Remind the patient that achieving a vocation/career is not likely.
 2. Encourage parents to remain hopeful about the adolescent's future.
 3. Promote self-esteem and confidence with praise for accomplishments.
 4. Assess the patient for indications of fear from facing a premature death.
 5. Use developmental age because there may be some developmental delays.
- _____ 17. The nurse is advocating for a transition to home care from a medical inpatient facility for an adolescent patient with a complex medical condition. Which assessment findings/information supports a transition to home for this patient? *Select all that apply.*

1. Home care is covered by the adolescent's primary and secondary health-care plan.
2. During home care a goal for mainstreaming the adolescent into school is set.
3. The adolescent's parents are older and will benefit from home-care assistance.
4. Home care allows for collaboration and management of care by a medical team.
5. A family member is concerned about the adolescent's complex medication regimen.

- _____ 18. The nurse works in a pediatric emergency department (ED) and frequently sees adolescents who are victims of sexual abuse. Which assessment findings does the nurse recognize as placing an adolescent at greater risk for sexual assault? *Select all that apply.*
1. Sexual promiscuity and alcohol/substance use
 2. An adolescent with developmental disabilities
 3. Adolescents with a history of depression and anxiety
 4. Frequent visits to the ED to seek attention
 5. A female adolescent who dates older men or boys
- _____ 19. Nurses in pediatric emergency departments attend an educational program about identifying victims of human trafficking. Which characteristics have the nurses learned to identify? *Select all that apply.*
1. Women
 2. Age less than 18 years
 3. Frequent UTIs and STIs
 4. Minors who are pregnant
 5. Appearing relaxed and calm
- _____ 20. The nurse is preparing to teach a class of adolescents about the increasing numbers of young people being sexually assaulted. Which advice by the nurse will promote adolescent safety from sex crimes? *Select all that apply.*
1. Go out with groups of friends.
 2. Avoid alcohol and substance use.
 3. Remain in public places.
 4. Bring a friend along on a date.
 5. Research a date's background.

Chapter 10 Adolescents

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 1. Describe the growth and development that occur during adolescence.

Page: 170

Heading: Growth and Development > Cognitive

Integrated Process: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is correct. During adolescence, the nurse is aware there is beginning to be some awareness of personal limitations.
2	This is incorrect. Adolescents show concern for politics and social issues.
3	This is incorrect. Adolescents become capable of thinking long term and setting goals.
4	This is incorrect. Adolescents are still inclined to compare themselves with peers.

PTS: 1

CON: Growth and Development

2. ANS: 2

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 1. Describe the growth and development that occur during adolescence.

Page: 170

Heading: Growth and Development > Psychological

Integrated Process: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. It is characteristic of an adolescent to compare their body with the bodies of others. This is not a source of concern.
2	This is correct. Because of impulsiveness and a sense of invincibility are common characteristics of the adolescent, the nurse will validate the parents' concern when the adolescent is comfortable doing what the crowd does.

3	This is incorrect. It is common for the adolescent to test family rules and limits. This is not always a source of concern.
4	This is incorrect. "Trying on" different styles of dress, communication, and personae are common when the adolescent becomes interested in sexuality and gender roles. The nurse will encourage the parents to be supportive and patient.

PTS: 1 CON: Growth and Development

3. ANS: 3

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 1. Describe the growth and development that occur during adolescence.

Page: 170

Heading: Biopsychosocial Assessment > General Survey

Integrated Process: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse is aware that a sense of physical awkwardness is normal for the adolescent.
2	This is incorrect. The nurse is aware that adolescents will often express dislike for school; however, the comment about a job, and especially college, keeps the nurse from seeking additional information.
3	This is correct. When the adolescent expresses trying to stay away from someone who is constantly coughing, the nurse seeks additional information and may recommend TB testing for the patient.
4	This is incorrect. When the adolescent states a desire to gain weight, the nurse will provide nutrition information and encourage a healthy diet. The nurse will also remind the adolescent that physical changes are not yet complete.

PTS: 1 CON: Health Promotion

4. ANS: 4

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for adolescents.

Page: 171

Heading: Biopsychosocial Assessment > Physical

Integrated Process: Caring

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
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1	This is incorrect. The nurse should refer to body parts by the correct anatomical names; however, teaching the information to the adolescent is likely to cause embarrassment or anxiety.
2	This is incorrect. The nurse will focus on the positive aspects of the individual. The nurse needs to be cautious about pointing out physical abnormalities, which may cause the adolescent anxiety.
3	This is incorrect. The nurse is capable of addressing the adolescent's concerns directly; recommending the adolescent address a concern to the physician can cause anxiety.
4	This is correct. The most effective way to decrease an adolescent's anxiety about a physical examination is to inform the adolescent of the nurse's actions and explain the rationales.

PTS: 1 CON: Patient-Centered Care

5. ANS: 2

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for adolescents.

Page: 172

Heading: Biopsychosocial Assessment > Physical > Reproductive

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Instruction about how to manage pain during a pelvic examination is not necessary; pelvic examinations are recommended at the age of 21 years in the absence of medical concerns.
2	This is correct. The first gynecological examination should occur between ages 13 and 15 years for external examination only; pelvic examinations are performed on an adolescent when problems arise such as pain or abnormal bleeding.
3	This is incorrect. Pelvic examinations and PAP tests are currently delayed until the age of 21 years.
4	This is incorrect. There is no reason for the nurse to describe or explain anything about a pelvic examination; the external genitalia only will be examined.

PTS: 1 CON: Patient-Centered Care

6. ANS: 1

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for adolescents.

Page: 172

Heading: Biopsychosocial Assessment > Physical > Reproductive

Integrative Processes: Nursing Process
 Client Need: Psychosocial Integrity
 Cognitive Level: Application [Applying]
 Concept: Patient-Centered Care
 Difficulty: Moderate

	Feedback
1	This is correct. Gynecomastia refers to abnormal breast development in boys. This is a self-limiting condition.
2	This is incorrect. The condition is self-limiting and is not routinely treated by surgery.
3	This is incorrect. Gynecomastia is more common in overweight boys. However, any developing male can exhibit the condition.
4	This is incorrect. Gynecomastia may be related to the increase in sex hormones; however, it is not indicative of a testosterone deficiency.

PTS: 1 CON: Patient-Centered Care

7. ANS: 1

Chapter: Chapter 10 Adolescents
 Chapter Learning Objective: 3. Explain medication approaches for adolescent patients.
 Page: 177
 Heading: Approaches to Medication Administration > Pharmaceutical Warnings
 Integrated Processes: Nursing Process
 Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies
 Cognitive Level: Synthesis [Creating]
 Concept: Safety
 Difficulty: Difficult

	Feedback
1	This is correct. Adolescents have the best understanding of medication warnings when directly informed verbally by health-care personnel.
2	This is incorrect. The nurse clarifies administration information with the client; pharmacists are not responsible for this information.
3	This is incorrect. The nurse and physician needs to know most medications are metabolized faster by adolescent patient; there is no need for the patient to receive this information.
4	This is incorrect. Nurses cannot rely on symbols or phrases to warn adolescents of medicine-related risks. Verbal information is best.

PTS: 1 CON: Safety

8. ANS: 1

Chapter: Chapter 10 Adolescents
 Chapter Learning Objective: 2. Identify age-specific physical assessment approaches for adolescents.
 Page: 177

Heading: Critical Component: Sexual Identity
Integrated Processes: Communication and Documentation
Client Need: Health Promotion and Maintenance
Cognitive Level: Application [Applying]
Concept: Patient-Centered Care
Difficulty: Moderate

	Feedback
1	This is correct. The goal is to create an environment that makes the adolescent feel comfortable to discuss their concerns with the nurse. Therapeutic communication avoids opinions and judgments and attempts to explore the patient's thoughts and feelings.
2	This is incorrect. Adolescents may not feel able to discuss personal matters with parents; the nurse's comment is likely to stop communication on the topic between the nurse and the patient.
3	This is incorrect. Asking the patient what was appealing about the other person is nontherapeutic and unnecessary.
4	This is incorrect. Some adolescents identify themselves as gay, lesbian, or bisexual. When communicating with adolescents, do not assume heterosexuality. The nurse should not focus on the adolescent's orientation, because some adolescents may have had a same-sex experience and not be gay, and teens who identify as gay may have never had a same-sex experience.

PTS: 1 CON: Patient-Centered Care

9. ANS: 1

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among adolescents.

Page: 178

Heading: Health Promotion and Education > Drugs/Diet Assessment

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is correct. The nurse needs to perform additional assessment on the adolescent who specifically states not smoking cigarettes. The NAPNAP paper specifically examines the serious health hazards associated with smoking, smokeless (electronic) cigarettes, and secondhand smoke.
2	This is incorrect. The adolescent who lost a family member to lung cancer is not likely to need additional assessment. Smoking is the leading cause of lung cancer.

3	This is incorrect. The adolescent who states the inability to afford any kind of smoking may or may not need additional assessment. The nurse needs to inquire specifically if the adolescent is furnished smoking materials by another person.
4	This is incorrect. This adolescent is obviously facing serious consequences from parents if smoking occurs. The nurse may or may not decide to assess a little further.

PTS: 1 CON: Health Promotion

10. ANS: 3

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 5. Describe strategies for assessing health promotion practices among adolescents.

Page: 179

Heading: Health Promotion and Education > Sexuality Assessment

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse may decide to present birth-control options, but first the nurse needs to determine in which type of sexual activity the patient is involved.
2	This is incorrect. The nurse may decide to ascertain the number of the patient's sexual partners, especially if she has been treated for an STI.
3	This is correct. Given the patient's comment about having only safe sex, the nurse needs to determine the patient's definition of the term. Engagement in oral/anal sex is often considered "safe" by adolescents. If this defines the patient's sexual practice, the nurse needs to educate the patient about risks such as acquiring STIs via oral and anal routes.
4	This is incorrect. It is appropriate for the nurse to ask a sexually active adolescent about treatment for STIs; however, the definition of "safe sex" is more important.

PTS: 1 CON: Health Promotion

11. ANS: 3

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 6. Describe emergency care considerations for adolescent patients.

Page: 180

Heading: Emergency Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Legal

Difficulty: Difficult

	Feedback
1	This is incorrect. If reasonable attempts have been made to reach the patient's parents, the medical team will make a decision about treatment.
2	This is incorrect. In this situation, it does not matter if the patient is emancipated or not.
3	This is correct. Under the Emergency Medical Treatment and Active Labor Act of 1986, adolescents who require emergency care may be treated regardless of whether caregiver consent has been obtained. The nurse will prepare the patient for surgery.
4	This is incorrect. There is no need to contact the facility's legal advisor; the law is clear and defensible.

PTS: 1 CON: Legal

12. ANS: 2

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 7. Identify strategies to support hospitalized adolescents.

Page: 180

Heading: Acute-Care Hospitalization > Environment of Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. Using a FACES pain scale for assessment of a 15-year-old patient is inappropriate for any reason. The nurse is not expected to entertain the patient.
2	This is correct. It is appropriate for the nurse to encourage the adolescent's active participation in meeting health-care needs. Because the patient may be involved in wound care after discharge, this is a prime time for the nurse to provide appropriate patient education.
3	This is incorrect. The nurse is always to be honest about pain and side effects associated with patient care.
4	This is incorrect. The nurse needs to cluster activities to leave periods of free time for relaxation, homework, or visiting with family and friends. Interaction with family and peers is important to adolescents.

PTS: 1 CON: Patient-Centered Care

13. ANS: 4

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 8. Integrate chronic-care concepts and adolescent-specific considerations

Page: 181

Heading: Care When Chronically Ill > Physical Care

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. It is most appropriate for the nurse to promote self-care and functional independence. The nurse can also coordinate services between health-care providers, school, and home.
2	This is incorrect. The patient does have a chronic condition, but the manifestations are unpredictable. Home care and/or nursing visitations need to be planned as, or if, needed.
3	This is incorrect. There may not be methods by which the patient and family can minimize the chronic medical events.
4	This is correct. The adolescent patient is likely to have an interest and the ability to do research on the internet. In order to promote understanding through valid and applicable resources, the nurse needs to refer the patient and family to reliable websites.

PTS: 1 CON: Patient-Centered Care

14. ANS: 2

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 10. Recognize complementary and alternative therapies used by adolescents.

Page: 182

Heading: Safe and Effective Nursing Care: Clinical Pearl: Use of Complementary and Alternative Medicine

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. If the nurse tells the adolescent that unreported CAM is a major cause of complications, the nurse is attempting to elicit information with the use of fear. This approach is inappropriate.
2	This is correct. Adolescents are often reluctant to discuss their use of CAM. Start out by asking the adolescent about family and peer use of CAM. If the adolescent reports use by a close family member or peer, then the adolescent is most likely also using CAM. The nurse can use therapeutic communication skills to acquire additional information.

3	This is incorrect. If the nurse informs the adolescent that CAM must be completely avoided when hospitalized, the nurse is using a poorly disguised threat to gain information. This approach is inappropriate.
4	This is incorrect. If the nurse states that CAM is actually a primitive type of modern medication therapy, the nurse is projecting an insulting attitude toward the adolescent. This approach will increase the adolescent's unwillingness to share information about the use of CAM.

PTS: 1

CON: Safety

MULTIPLE RESPONSE

15. ANS: 1, 2, 4, 5

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 4. Explain variations in nursing procedures for adolescent patients.

Page: 177

Heading: Variations in Nursing Procedures

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Legal

Difficulty: Difficult

	Feedback
1.	This is correct. In a high school setting, a majority of students may be minors, and it is necessary for the nurse to obtain parent/guardian permission to administer a TB test.
2.	This is correct. As in all situations of medical care, confidentiality is assured.
3.	This is incorrect. Because TB is an airborne infection, discerning the students who have or have not been exposed is nearly impossible. All students will be tested.
4.	This is correct. The nurse must be honest about any pain during a medical procedure; the nurse will inform the students of expected minor pain.
5.	This is correct. Before seeking parent/guardian permission, the nurse must explain the reasons the testing is being performed. Adolescents also need to know the rationales so they will be compliant and assent to the process.

PTS: 1

CON: Legal

16. ANS: 3, 4, 5

Chapter: Chapter 10 Adolescents

Chapter Objective: 8. Integrate chronic-care concepts and adolescent-specific considerations

Page: 181

Heading: Care When Chronically Ill > Psychosocial/Spiritual Care

Integrated Processes: Nursing Process

Client Need: Psychosocial integrity

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. When caring for a chronically ill adolescent, the nurse needs to support an adolescent's desire for a vocation or career. The nurse needs to understand that treatment may be successful and outcomes are unknown.
2.	This is incorrect. Fostering realistic expectations in parents regarding the adolescent's future personal, academic, and career potential will prevent the parents from developing false hopes and expectations.
3.	This is correct. The nurse needs to allow for the adolescent's completion of tasks as able. Then, the nurse needs to promote self-esteem and confidence with sincere praise of accomplishments.
4.	This is correct. The possibility of a premature death can be especially difficult for an adolescent patient. The nurse needs to assess for fear of facing their own premature death.
5.	This is correct. The nurse needs to use developmental rather than chronological age when caring for chronically ill adolescents, because there may be some developmental delays.

PTS: 1 CON: Patient-Centered Care

17. ANS: 1, 4

Chapter: Chapter 10 Adolescents

Chapter Objective: 9. Integrate home-care concepts and adolescent-specific considerations.

Page: 181

Heading: Home-Care Considerations

Integrative Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Evaluation [Evaluating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. Home care decreases financial costs and travel costs. When the adolescent has primary and secondary health-care plans that cover home care, the nurse is assured that the transition will not be a financial burden.
2.	This is incorrect. With a complex medical condition it is unlikely the adolescent will be mainstreamed into school. A more realistic goal for the adolescent is for increased opportunities for socializing with peers and participating in school/extracurricular activities.
3.	This is incorrect. Having older parents who would also benefit from home assistance is not a reason to transition the adolescent to home care. The adolescent's home care is not likely to include persons other than the adolescent.
4.	This is correct. Home care allows collaboration with the adolescent's medical team and increases family satisfaction. The adolescent will be followed by a medical team that is set up to meet the adolescent's medical and psychosocial needs.

5.	This is incorrect. If a family member is concerned about the adolescent's complex medication regimen, the nurse understands that a transition to home care will depend on the ability to get assistance with medication administration.
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PTS: 1 CON: Safety

18. ANS: 1, 2, 5

Chapter: Chapter 10 Adolescents

Chapter Objective: 11. Recognize child abuse considerations relevant to adolescents.

Page: 182

Heading: Child Abuse Considerations

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Violence

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse recognizes that adolescents with a history of sexual abuse are more likely to engage in risky behaviors such as sexual promiscuity and alcohol/substance use. The behaviors place the adolescent at risk for additional sexual abuse.
2.	This is correct. The nurse recognizes that adolescents with developmental disabilities are more likely to become victims of sexual abuse.
3.	This is incorrect. Sexually abused adolescents have increased rates of mental health problems, such as depression, anxiety, and post-traumatic stress disorder. But, adolescents are not at greater risk for sexual assault because of the conditions.
4.	This is incorrect. The nurse recognizes that nonspecific physical complaints by an adolescent could be an indication of sexual abuse. However, frequent visits to the ED to seek attention are not a risk.
5.	This is correct. There is an increasing ability to obtain date-rape drugs, such as flunitrazepam (Rohypnol), hydroxybutyrate (GHB), and ketamine. As a result, the use of these substances in adolescent acquaintance-rape assaults is on the rise. An adolescent dating older men or boys is at increased risk.

PTS: 1 CON: Violence

19. ANS: 2, 3, 4

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 11. Recognize child abuse considerations relevant to adolescents.

Page: 183

Heading: Child Abuse Considerations > Human Trafficking

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Violence

Difficulty: Difficult

	Feedback
1.	This is incorrect. Victims can include children, women, and men.
2	This is correct; 33% of human trafficking victims are minors. The average age is 12 to 15 years.
3.	This is correct. Poor health conditions among this population include STIs; urinary tract infections; multiple pregnancies, abortions, and/or miscarriages; and mental health issues with depression, anxiety, and suicide attempts.
4.	This is correct. Many young women will have had multiple pregnancies, abortions, and miscarriages.
5.	This is incorrect. Mental health issues with depression, anxiety, and suicide attempts are common for trafficking victims.

PTS: 1 CON: Violence

20. ANS: 1, 2, 3

Chapter: Chapter 10 Adolescents

Chapter Learning Objective: 11. Recognize child abuse considerations relevant to adolescents.

Page: 183

Heading: Safe and Effective Nursing Care > Promoting Safety > Sexual Assault Prevention

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Violence

Difficulty: Difficult

	Feedback
1.	This is correct. Nurses should reinforce safety strategies for avoiding sexual assault, such as going out with a group of friends. The group needs to stay together and should not let a member go off with someone alone.
2.	This is correct. Nurses should reinforce safety strategies for avoiding sexual assault, such as avoiding alcohol and substance use, which decreases the ability to think clearly or defend oneself.
3.	This is correct. Nurses should reinforce safety strategies for avoiding sexual assault, such as remaining in public places. In addition, the public places should be well lit, well used, and in a safe area.
4.	This is incorrect. Bringing a friend along on a date does not necessarily assure protection from sexual assault.
5.	This is incorrect. Researching a date's background is not a fail-proof action to avoid sexual assault. Each perpetrator has a first victim.

PTS: 1 CON: Violence

Chapter 11 Respiratory Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The pediatric nurse is reviewing anatomy and physiology in order to have a better understanding of the pediatric respiratory system. The nurse is aware that fluid in the chest cavity can be normal. Which application of this knowledge is correct?
1. Pleural fluid is abundant at birth and decreases over the lifetime.
 2. Only enough fluid is present to promote painless movement.
 3. Fluid will accumulate in the plural cavity from immobility.
 4. Infections such as pneumonia cause fluid in the plural cavity.
- _____ 2. The nurse in the newborn unit of a pediatric hospital is providing care for a neonate born at 34 weeks' gestation. The nurse is aware that the immediate risk to the neonate is which condition?
1. A lack of a phospholipid in the alveoli
 2. Inability to maintain body temperature
 3. Delay in closure of cardiac foramen
 4. A decrease in renal function
- _____ 3. The nurse is providing care for an infant who is 2 months old. Which assessment finding will cause the nurse to suspect an upper respiratory infection?
1. A raspy cry and occasional cough
 2. Adventitious lung sounds bilaterally
 3. A stuffy nose and reddened eardrums
 4. A fever, lethargy, and skin pallor
- _____ 4. The nurse is providing care for a school-age patient who received a head injury while playing sports. Which initial assessment finding causes the nurse greatest concern?
1. Confusion and disorientation
 2. Headache with periods of nausea
 3. Immediate loss of consciousness
 4. Changes in breathing and heart rates
- _____ 5. The pediatric nurse is preparing a teaching plan for new mothers with small infants. Which is a key point for the nurse to include in the teaching plan?
1. Infants are obligatory mouth breathers for the first month.
 2. All sinuses are formed and aerating within 2 months of birth.
 3. Infants are abdominal breathers until they are 12 months old.
 4. Infant airways get blocked more easily than those in older children.
- _____ 6. A toddler who is 2 years old is playing in the playroom at the hospital and suddenly begins to choke and cough. The nurse attending the toddler places the child in which position to dislodge a possibly inhaled object?
1. Head down and on the left side
 2. Head down and on the right side
 3. Head horizontal to the floor and supine
 4. Head in a neutral position and prone
- _____ 7. The nurse in a pediatric clinic is assessing an infant who is 3 months old during a well-baby visit. Which assessment finding will be of greatest concern to the nurse?
1. The infant mouth breathes when crying.

2. The infant's eardrums are pink in color.
3. The infant exhibits 15-second periods of apnea.
4. The infant's respiratory rate is fast and irregular.

- _____ 8. The nurse is providing care for an infant with an inner ear infection. The nurse is aware that the condition has occurred multiple times in a 3-month period. Which comment by the parent indicates to the nurse that specific teaching is needed regarding the incidences of infection?
1. "I now put her to bed with a bottle."
 2. "I clean her ears with cotton swabs."
 3. "She likes her ears submerged while bathing."
 4. "Her older brother brings colds home from school."
- _____ 9. The nurse in a pediatric clinic is performing assessments on multiple infants. Which infant does the nurse recognize as being at greatest risk for a respiratory disorder?
1. The infant born at 36 weeks who exhibited respiratory problems at birth
 2. The infant who was born at term and recently adopted from another country
 3. The infant who sleeps all night, exhibits eczema, and has a family history of asthma
 4. The infant with recurrent sore throats and both pets and smokers in the house
- _____ 10. The nurse in an acute care pediatric facility is preparing to assume care of multiple patients at the change of shift. Which patient will the nurse plan to assess first?
1. The toddler who exhibits clubbing of the fingertips
 2. The preschooler with pneumonia who has poor skin turgor
 3. The infant who can sleep only with the head of the bed elevated
 4. The infant who prefers a tripod position instead of lying down
- _____ 11. Parents bring a toddler who is 2-1/2 years old to the hospital because of observed difficulty with breathing. In addition, they share that at bedtime the toddler has a barking cough. The toddler is diagnosed with laryngotracheobronchitis, commonly referred to as croup. Which assessment finding does the nurse expect related to the diagnosis?
1. Fever accompanied by a congested cough
 2. Inspiratory stridor heard in the upper airway
 3. Elevated temperature and diaphoresis
 4. Snoring sounds throughout respirations
- _____ 12. The nurse on a pediatric acute care unit is providing care for an infant who is 11 months of age. The infant is diagnosed with a lower respiratory infection that produces large amounts of thick secretions the infant cannot cough up. When planning to suction the infant, which factor is important to remember?
1. The parents are likely to become angry about the procedure.
 2. The infant will respond negatively to a temporary loss of breath.
 3. The nurse should apologize after the procedure for stress related to the procedure.
 4. The nurse should have assistance to immobilize the infant during the procedure.
- _____ 13. A 6-year-old patient is being assessed by the pediatrician for breathing difficulties. The pediatrician expresses a need for diagnostic tests to identify or rule out asthma. Which tests does the nurse anticipate ordering?

1. Throat culture
2. Pulmonary function tests
3. Electrocardiogram
4. Peak flow meter

_____ 14. The parents of three children aged 4, 6, and 9 years are preparing to travel abroad as missionaries. The children are in good health and up to date for immunizations; however, the parents are concerned about the high level of TB in the area to which they are assigned. Which recommendation does the nurse make for the protection of the children?

1. Protect the children with good nutrition.
2. Have the children receive the BCG vaccine.
3. Arrange for monthly TB testing for the family.
4. Start the children on preventive medication.

_____ 15. The nurse is interviewing an adolescent patient 17 years of age who was diagnosed with cystic fibrosis (CF) as an infant. The patient shares feelings of frustration about needing to always live with parents. Which information provided by the nurse is likely to be most important to the patient?

1. How chest physiotherapy (CPT) can be performed independently
2. The availability of home meal delivery to those needing a therapeutic diet
3. Organizations that will provide transportation for persons with chronic illness
4. A list of social organizations available for young persons who have special needs

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

_____ 16. The nurse is providing teaching to the parent of a toddler 2 years of age diagnosed with otitis media. The toddler presented with a fever of 100.9°F (38.3°C) and does not indicate symptoms related to pain. Which information does the nurse give the parent when the physician orders 48 to 72 hours of supportive care? *Select all that apply.*

1. Provide age-appropriate analgesics as needed.
2. Administer all of the prescribed antibiotic.
3. Support hydration with fluid increases.
4. Monitor temperature and report increases.
5. Apply topical steroid preparations as instructed.

_____ 17. The nurse is providing postoperative teaching to the parents of a preschool child after a tonsillectomy. For which events does the nurse prompt the parents to contact the physician? *Select all that apply.*

1. The child keeps an emesis basin close by.
2. The child is frequently swallowing without food or fluids.
3. Bright red blood is noticed in the child's mouth.
4. The child is asking for ice chips and popsicles.
5. The child refuses pain pills because it hurts to swallow.

_____ 18. The nurse is caring for a newborn diagnosed with esophageal atresia and tracheoesophageal fistula. Which information does the nurse provide to the parents? *Select all that apply.*

1. Prenatal conditions that contribute to the problem
2. The manifestation supporting the diagnosis
3. Diagnostic tests performed since the birth
4. Methods of treating the condition
5. Actions for promoting recovery

- _____ 19. The nurse is conducting a class for parents of children with asthma. After covering the topic of asthma triggers, the nurse asks for feedback on the information. Which statements indicate the parents understand actions to reduce asthma triggers? *Select all that apply.*
1. "I think that we will need to stop using the fireplace."
 2. "We will be rethinking the possibility of a family pet."
 3. "Now may be the time for relocating to a warmer climate."
 4. "No more going outside without a mask from now on."
 5. "Reorganizing our schedules will definitely reduce stress."
- _____ 20. The nurse in a pediatric clinic is assessing a toddler brought in by a parent who states, "I have noticed some increasing incidents of coughing and wheezing over the last few weeks." Auscultation by the nurse reveals some adventitious breath sounds in the upper right lobe. Which questions does the nurse ask the parent? *Select all that apply.*
1. "Have you noticed any missing small toys?"
 2. "How often is the child allowed to self-feed?"
 3. "Is there an older child who gives this child food?"
 4. "Can you recall a specific time of gagging or cyanosis?"
 5. "Have you noticed any foreign objects in the child's stool?"

Chapter 11 Respiratory Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 1. Identify normal assessment of anatomy and physiology of the pediatric respiratory system.

Page: 187

Heading: Anatomy and Physiology

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Fluid is not abundant in the plural cavity at birth; fluid is in the lungs at birth. It is suctioned in order to promote normal respirations. Normal fluid in the pleural cavity does not decrease over the lifetime.
2	This is correct. There are two pleural membranes: one around the lungs and one covering the inside of the pleural cavity. The two pleural membranes are normally separated by only enough fluid to lubricate the surfaces for painless movement.
3	This is incorrect. Fluid can accumulate in the lungs as a result of immobility.
4	This is incorrect. Pneumonia is an infection that causes fluid to build up in the lungs.

PTS: 1

CON: Patient-Centered Care

2. ANS: 1

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 1. Identify normal assessment of anatomy and physiology of the pediatric respiratory system.

Page: 187

Heading: Anatomy and Physiology

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Oxygenation

Difficulty: Moderate

	Feedback
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1	This is correct. The nurse's immediate concern is related to respiratory function. A premature neonate is likely to have a low level of surfactant, which is a phospholipid in the alveoli that keeps alveoli pliable, preventing them from collapsing completely at the end of each expiration.
2	This is incorrect. All neonates are at risk for a loss of body heat that can result in complications from cold stress. The premature neonate is at greater risk due to low body weight; however, environment management keeps this from being an immediate risk.
3	This is incorrect. Neonates at any level of maturity can have delays in the closure of cardiac foramen.
4	This is incorrect. The immediate risk for a premature neonate is the ability to provide adequate oxygenation. A decrease in renal function in a premature neonate can be related to poor oxygenation because of compromised respiratory function.

PTS: 1 CON: Oxygenation

3. ANS: 3

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 1. Identify normal assessment of anatomy and physiology of the pediatric respiratory system.

Page: 187

Heading: Anatomy and Physiology

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. A raspy cry results from inflammation of the larynx; however, an occasional cough is more indicative of trachea irritation. The manifestations do not necessarily indicate an upper respiratory infection because structures of both the upper and lower respiratory tract are involved.
2	This is incorrect. The lungs are part of the lower respiratory system due to the presence of the terminal bronchioles, which end in sacs called alveoli. This finding is indicative of a lower respiratory infection.
3	This is correct. The upper respiratory tract is a passageway that includes the nasopharynx and oropharynx and is connected to the ears by the eustachian tubes. Because of the stuffy nose and reddened eardrums, the nurse suspects an upper respiratory infection.
4	This is incorrect. Fever, lethargy, and pallor can be seen in either an upper or lower respiratory infection.

PTS: 1 CON: Patient-Centered Care

4. ANS: 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 1. Identify normal assessment of anatomy and physiology of the pediatric respiratory system.

Page: 187

Heading: Anatomy and Physiology

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. Confusion and disorientation are common manifestations of a head injury. This finding does not cause the greatest concern for the nurse.
2	This is incorrect. Headache and periods of nausea are not uncommon after a head injury. While initially this finding does not cause the nurse greatest concern, frequent reassessment is necessary to identify manifestations of increasing intracranial pressure.
3	This is incorrect. Immediate loss of consciousness at the time of a head injury is not uncommon. However, the nurse will continue to monitor for manifestations of increasing intracranial pressure.
4	This is correct. Normal breathing is involuntary; the central nervous system controls rate and volume of respiration. Adjustments are made in respiration rate, heart rate, and cardiac output to maintain adequate gas exchange. The finding will alert the nurse to either hypoxia in the brain or injury to the part of the brain that controls respiratory function. The scenario does not specify an increase or decrease in the rates.

PTS: 1

CON: Patient-Centered Care

5. ANS: 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 2. Identify differences in the anatomy and physiology of the pediatric and adult patient.

Page: 188

Heading: Developmental Differences in the Respiratory System

Integrated Process: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is incorrect. Newborns are obligatory nose breathers until 4 weeks of age, which is the reason newborns with upper respiratory infections have difficulty with feeding from the breast or bottle.

2	This is incorrect. Only ethmoid and maxillary sinuses are present at birth and are not aerated until 4 months. Sphenoid and frontal sinuses develop later in childhood and continue to mature into adolescence.
3	This is incorrect. The infant's intercostal muscles are not fully developed, and pronounced abdominal wall movement with respiration is normal until 6 years of age.
4	This is correct. Newborn airways are approximately 4 mm in diameter compared with 20 mm for the average adult's airway. Inflammation 1 mm in circumference would decrease a child's airway diameter 50% but only 20% for an adult.

PTS: 1 CON: Health Promotion

6. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 2. Identify differences in the anatomy and physiology of the pediatric and adult patient.

Page: 188

Heading: Developmental Differences in the Respiratory System

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. The right bronchus is more likely to be blocked because of its steep angle into the lungs; turning the child to the left will impede the removal of the object.
2	This is correct. In children, the bifurcation of the right and left bronchi occurs higher in the airway, and the right bronchus enters the lung at a steeper angle than does the bronchi of an adult. Placing the child head down and on the right side will help to dislodge the object.
3	This is incorrect. The object needs to move up and out of the bronchus; it cannot do that lying flat or horizontal.
4	This is incorrect. Lying prone and in a neutral position will not enable the object to be coughed up and out.

PTS: 1 CON: Safety

7. ANS: 3

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 2. Identify differences in the anatomy and physiology of the pediatric and adult patient.

Page: 188

Heading: Developmental Differences in the Respiratory System

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]
 Concept: Growth and Development
 Difficulty: Difficult

	Feedback
1	This is incorrect. Newborns are obligatory nose breathers until 4 weeks of age. It is not unexpected or of concern for a 3-month-old infant to take mouth breaths when crying.
2	This is incorrect. The nurse will perform further assessment to determine if the infant is exhibiting signs of a mild ear infection. This finding is of concern, but periods of apnea are of greatest concern.
3	This is correct. Periods of apnea (the absence of respiration) that last up to 15 seconds are typical of newborns; however, at 3 months of age this patient is considered an infant. The finding is not expected and causes the nurse concern.
4	This is incorrect. A child's metabolic rate is higher than that of an adult's, creating a higher oxygen demand. An infant's respiratory rate is expected to be faster, with an irregular pattern.

PTS: 1 CON: Growth and Development

8. ANS: 1

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 2. Identify differences in the anatomy and physiology of the pediatric and adult patient.

Page: 188

Heading: Developmental Differences in the Respiratory System

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is correct. Eustachian tubes are shorter and more horizontal in children than adults and are more prone for migration of substances in the mouth to the inner ear, causing an inner ear infection. The migration of milk is enhanced by putting an infant to bed with a bottle. The nurse will present teaching on this topic.
2	This is incorrect. The nurse will reinforce that cotton swabs should not be placed in a child's ear. However, the practice is less likely to be the cause of an inner infection.
3	This is incorrect. Some infants do not mind having their ears submerged when in a bath. This practice alone is not likely to be the cause of an inner infection.
4	This is incorrect. An infant may be susceptible to an upper respiratory infection from an older sibling who attends school. However, the scenario does not present other symptoms of a cold, such as a stuffy nose.

PTS: 1 CON: Patient-Centered Care

9. ANS: 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 3. Identify the clinical presentation and nursing care of the pediatric patient with respiratory disorders.

Page: 188

Heading: Respiratory Disorders > General History

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Without additional manifestations or information, the premature infant who exhibited respiratory problems at birth is not the patient with the greatest risk for respiratory disorders. The only risk factor is gestational age.
2	This is incorrect. The infant who was recently adopted from another country is at risk for respiratory problems because of unknown environmental, physiologic, and/or genetic influences. The only risk factor is being born in another country.
3	This is incorrect. The infant with eczema and a family history of asthma has two risks for the development of a respiratory disorder. Sleeping all night is not a risk factor.
4	This is correct. The infant with recurrent sore throats and exposure to environmental irritants such as pets and smokers in the household is at greatest risk for developing a respiratory disorder. This patient is recognized by the nurse as having three risk factors.

PTS: 1

CON: Patient-Centered Care

10. ANS: 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 3. Identify the clinical presentation and nursing care of the pediatric patient with respiratory disorders.

Page: 189

Heading: Respiratory Disorders > Physical Assessment

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
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1	This is incorrect. A toddler may exhibit clubbing of the fingertips if diagnosed with chronic hypoxia from a chronic respiratory disorder. The manifestation occurs over a period of time and does not alone indicate a need for being assessed first.
2	This is incorrect. The preschooler with pneumonia and poor skin turgor may be dehydrated from mouth breathing, tachypnea, fever, and/or anorexia. Without additional symptoms, this is not the patient the nurse will assess first.
3	This is incorrect. The infant who can sleep only with the head of the bed elevated is not presently in distress. This patient does not need to be assessed first.
4	This is correct. When an infant prefers to sit in a tripod position, exhibits a jaw thrust, or is insistent on sitting upright, the indications are relevant to air hunger and oxygen deficiency. This is the patient the nurse will assess first.

PTS: 1 CON: Patient-Centered Care

11. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 3. Identify the clinical presentation and nursing care of the pediatric patient with respiratory disorders.

Page: 189

Heading: Respiratory Disorders > Auscultation

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Although fever accompanies croup, the cough is not congested but dry and barking.
2	This is correct. Inspiratory stridor, hoarseness, and air hunger are attributed to croup.
3	This is incorrect. Although an elevated temperature may accompany croup, diaphoresis or sweating does not.
4	This is incorrect. Snoring (rhonchi) is low-pitched sounds heard throughout the respiration as air passes through thick secretions. Snoring does not occur with croup.

PTS: 1 CON: Patient-Centered Care

12. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 3. Identify the clinical presentation and nursing care of the pediatric patient with respiratory disorders.

Page: 189

Heading: Safe and Effective Nursing Care > Clinical Pearl > The Skill of Suctioning

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential
 Cognitive Level: Application [Applying]
 Concept: Patient-Centered Care
 Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse needs to remember the parents and infant may both be startled by the process of suctioning; the nurse needs to be prepared for tears from the parents and/or the infant. The nurse does not expect anger from the parents.
2	This is correct. During the suctioning, the nurse will hold the infant tightly and the infant will temporarily experience a loss of breath.
3	This is incorrect. Providing emotional support and verbal explanation of the procedure to both child and parent is crucial for establishing a good rapport; however, the explanation and support needs to occur before the procedure.
4	This is incorrect. The nurse should be able to manage an infant during the process of suctioning. Additional assistance may cause additional stress and should only be acquired if necessary.

PTS: 1 CON: Patient-Centered Care

13. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 4. Describe common tests used in diagnosing and treating pediatric respiratory disorders.

Page: 190

Heading: Respiratory Disorders > Diagnostic Tests

Integrated Process: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. A throat culture is performed to identify any pathogens causing sore throat.
2	This is correct. Pulmonary function tests measure the volumes of inhalation and exhalation, normal and forced. The test evaluates the effectiveness of the lungs.
3	This is incorrect. Electrocardiograms record the electrical activity of the heart, not the function of the lungs.
4	This is incorrect. Peak flow meters are portable, handheld devices that measure the ability to push air out of the lungs. It is not diagnostic but is used for monitoring therapy. The parameters range from below “personal best” to the personal best level measured when in optimal health. The need for interventions is based on color zones.

PTS: 1 CON: Patient-Centered Care

14. ANS: 2

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for caregiver of a child with a respiratory disorder.

Page: 203

Heading: Lower Airway Infectious Disorders

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. Good nutrition and providing overall good health to prevent infections is beneficial for the entire family regardless of their location.
2	This is correct. Before traveling abroad, the children should get a BCG vaccine, which is a live attenuated strain of <i>Mycobacterium bovis</i> . The vaccine is not widely used in the United States because there is less risk for infection in the United States. It provides incomplete protection, and other precautions are warranted. The BCG vaccine does provide better protection for children than adults.
3	This is incorrect. The diagnosis of TB in children is challenging, because symptoms are nonspecific and vary and are dependent on the age of the child. Children 5 to 10 years of age may not have clinical symptoms, but an x-ray will confirm if TB symptoms exist. Laboratory tests are less likely to be positive in children, and diagnosis is often made without laboratory confirmation.
4	This is incorrect. The children can be protected with nutrition, hand hygiene, avoidance of persons identified with TB, and the BCG vaccine. Preventive medications can have undesirable side effects.

PTS: 1

CON: Patient-Centered Care

15. ANS: 1

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for the caregiver of a child with a respiratory disorder.

Page: 204

Heading: Lower Airway Noninfectious Disorders > Cystic Fibrosis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
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1	This is correct. Chest physiotherapy is necessary three to four times daily for the patient with CF. The nurse can inform the patient about equipment and techniques. Some suggestions will include handheld massager and an oscillating vest. The nurse will use a multidisciplinary approach to promote independence for the patient.
2	This is incorrect. The patient will benefit from teaching about how to independently meet dietary needs and does not necessarily need home-delivered meals.
3	This is incorrect. Persons with CF are not excluded from driving.
4	This is incorrect. The patient is likely to have a social circle of friends. Referring the patient to organizations that are designed for persons with special needs is not necessary/appropriate.

PTS: 1

CON: Patient-Centered Care

MULTIPLE RESPONSE

16. ANS: 1, 3, 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for caregiver of a child with a respiratory disorder.

Page: 193

Heading: Upper Airway Infectious Disorders > Otitis Media > Nursing Interventions

Integrated Processes: Teaching/Learning

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Infection

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will instruct the parent to administer age-appropriate analgesics as needed. This supportive care is recommended if a child older than 23 months is not in severe discomfort and fever is lower than 102.2°F. Observation for 48 to 72 hours is an important option.
2.	This is incorrect. Antibiotics will not be prescribed immediately and then will be prescribed conservatively. Supportive care is cost-effective and decreases overuse of antibiotics that lead to resistant strains of pathogens.
3.	This is correct. The nurse will instruct the parent to avoid dehydration in the child by frequently offering fluids.
4.	This is correct. The nurse will instruct the parent to monitor the child's temperature and report increases, especially if it is 102.2°F or higher.
5.	This is incorrect. Topical antibiotics, an acidifying agent, and steroid preparation are prescribed for otitis externa.

PTS: 1

CON: Infection

17. ANS: 2, 3, 5

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for caregiver of a child with a respiratory disorder.

Page: 195

Heading: Upper Airway Infectious Disorders > Tonsillitis > Family Education

Integrated Processes: Teaching/Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. It is not unusual for a child to experience nausea from drainage after a tonsillectomy. There is no need to contact the physician unless the nausea is severe and there is a risk for vomiting.
2.	This is correct. Frequent swallowing without the presence of food or fluid is an indication of bleeding; the parents are instructed to call the physician if this occurs.
3.	This is correct. Bright red blood in the child's nose or mouth is indicative of bleeding, and the physician needs to be called.
4.	This is incorrect. The child's willingness to eat ice chips and popsicles will soothe the throat, help to stop bleeding, and promote hydration.
5.	This is correct. If the child refuses to take pain pills because it hurts to swallow, the doctor needs to be contacted. It is likely the prescription can be changed to a liquid.

PTS: 1

CON: Patient-Centered Care

18. ANS: 2, 3, 4, 5

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for caregiver of a child with a respiratory disorder.

Page: 198

Heading: Upper Airway Noninfectious Disorders > Esophageal Atresia and Tracheoesophageal Fistula

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. Even if known, the nurse will not present information about any prenatal conditions that contributed to the newborn's condition. The physician may need to discuss the topic after the initial treatment is performed.

2.	This is correct. The parents need to know the manifestations that support the diagnosis. The newborn is likely to experience respiratory distress within minutes, days, or weeks of birth; excessive oral secretions; cyanosis; coughing spells; abdominal distention; and upper airway instability.
3.	This is correct. The nurse will inform the parents that x-rays, tracheoscopy, echocardiography, and ultrasound was performed after the birth and with the onset of manifestations.
4.	This is correct. The nurse will inform the parents that the newborn will require surgery for the repair of the defect. After the physician acquires an informed consent, the nurse will ask about questions and concerns.
5.	This is correct. Parent teaching aimed at recovery of the newborn will include information about preventing aspiration, feeding, positioning, and the importance of adherence to frequent follow-up appointments.

PTS: 1 CON: Patient-Centered Care

19. ANS: 1, 2, 5

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for the caregiver of a child with a respiratory disorder.

Page: 206

Heading: Lower Airway Noninfectious Disorders > Asthma

Integrated Processes: Teaching/Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. Smoke from any source is a commonly recognized trigger for asthma; understanding the need to stop using the fireplace is indicative of an appropriate action.
2.	This is correct. Pet dander is a commonly recognized trigger for asthma; voicing the need to reconsider getting a family pet is indicative of an appropriate action.
3.	This is incorrect. Cold air is commonly recognized as a trigger for asthma; however, moving to a warmer climate is not necessary. The patient needs to place a scarf over the mouth and nose before being exposed to cold air.
4.	This is incorrect. Viral infections are commonly recognized as a trigger for asthma; however, wearing a mask whenever going outside the home is not a necessary action. Staying away from people who are ill, good hand hygiene, and promoting overall good health is important. Masks may be considered during flu season.
5.	This is correct. Stress is a commonly recognized trigger for asthma; reorganizing schedules may be an effective way to reduce stress.

PTS: 1 CON: Patient-Centered Care

20. ANS: 1, 3, 4

Chapter: Chapter 11 Respiratory Disorders

Chapter Learning Objective: 5. Identify areas of education for the caregiver of a child with a respiratory disorder.

Page: 210

Heading: Other Noninfectious Disorders > Foreign Body Aspiration

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse needs to ascertain if and what type of foreign object the toddler may have aspirated. This is an appropriate question if it is suspected that the child aspirated a foreign object.
2.	This is incorrect. Asking how often the child is allowed to self-feed is not an important question if the nurse suspects aspiration of a foreign object.
3.	This is correct. The nurse is appropriate in asking if there is another child who may have given the toddler food that could be aspirated. The parent may not be aware but needs to consider the possibility. Of greatest concern is peanuts, tree nuts, hard candies, etc.
4.	This is correct. At the time, if the parent noticed the toddler gagging or appearing cyanotic, the nurse may have a better timeline, and the parent's memory may be jogged about a cause.
5.	This is incorrect. Questioning the parent about noticing a foreign object in the toddler's stool is not applicable to the possibility of aspirating a foreign body. Swallowing an object may cause passage in the stool.

PTS: 1

CON: Patient-Centered Care

Chapter 12 Cardiovascular Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is providing information to the parents of a toddler who is scheduled for surgery for the replacement of the pulmonic valve. The parents have many questions about the function of the valve. Which information from the nurse is correct?
1. The valve must work correctly to get oxygen from the lungs to the body.
 2. If the valve does not work correctly, blood is kept from entering the heart.
 3. When the valve is defective, the blood leaving the heart is decreased.
 4. A defect in the valve causes less blood to get to the lungs for oxygenation.
- _____ 2. The nurse is teaching a pediatric electrocardiogram (EKG) class to nurses in a pediatric cardiac unit. Which anatomical structure does the nurse use to describe the initiation of cardiac electrical conduction?
1. The Purkinje fibers in the ventricles

2. The bundle branch in the left atrium
3. The sinoatrial node in the right atrium
4. The bundle of His in the ventricle walls

_____ 3. The nurse is aware the neonate's blood circulation is different before birth than after birth. Which circulation pattern does the nurse recognize as occurring prior to birth?

1. Oxygenated blood flows from the right atrium to the left atrium through the foramen ovale.
2. Oxygenated blood flows from the right ventricle to the lungs and then to the left ventricle.
3. For a short time after birth, the neonate continues to depend on the mother for oxygen supply.
4. Once the neonate takes a first breath, the ductus venosus closes and blood goes to the lungs.

_____ 4. The nurse is providing care for a neonate diagnosed with a cardiovascular disorder immediately after birth. When gathering assessment information from the mother, which comment will the nurse recognize as the most likely contributing factor for the defect?

1. "We live in the country, and we get all our water from a well."
2. "I quit my preschool job when a child was diagnosed with measles."
3. "The baby was born a week early; I hope that is not the cause."
4. "We were in a European country before pregnancy was confirmed."

_____ 5. The nurse is assisting with high school sports physicals. The nurse performs a physical assessment on a male student who is tall and thin, with disproportionately long arms. Which additional finding will prompt the nurse to recommend a cardiac evaluation?

1. Notable laxity of joints
2. Sparsity of body hair
3. Deep tone to the voice
4. Slow, rhythmic gait

_____ 6. The nurse is providing care for a 12-year-old patient who is hospitalized with generalized weakness and muscle wasting, which began in the hips, pelvic area, thighs, and shoulders. The physician suspects Duchenne muscular dystrophy. Which action by the physician does the nurse expect?

1. Prescribe physical therapy to improve muscle strength.
2. Suggest homeschooling until the acute stage ends.
3. Perform an echocardiogram to evaluate cardiac functioning.
4. Perform muscle biopsies to identify the stage of the disease.

_____ 7. The nurse is performing a physical assessment on a 7-year-old child as a requirement for playing a sport at school. The nurse reports which assessment finding as abnormal and requests a follow-up from a primary care physician?

1. Systolic blood pressure is 84 mm Hg.
2. Systolic blood pressure is 90 mm Hg and diastolic is 20 mm Hg.
3. Pulse oximeter reading is 95% on room air.
4. PMI is at 4th or 5th intercostal space at the midclavicular line.

- _____ 8. The nurse is preparing an 8-year-old patient for a cardiac catheterization. Which intervention will the nurse initiate immediately postprocedure?
1. Observe for signs and symptoms of infection.
 2. Hold food and fluids until gag reflex returns.
 3. Keep the involved extremity straight for 4 to 6 hours.
 4. Notify physician if green or yellow drainage is noted.
- _____ 9. A 3-month-old infant is diagnosed with pulmonary stenosis. Which parent teaching does the nurse provide?
1. Options for treatment include a repair of the artery or the valve.
 2. Balloon angioplasty is performed as an outpatient procedure.
 3. Pulmonary stenosis repair can be delayed until 1 year of age.
 4. After repair, the child is no longer at risk for cardiac problems.
- _____ 10. The parents are preparing to take their newborn, who was diagnosed with tetralogy of Fallot with pulmonary atresia, home. The nurse is developing a teaching sheet regarding care of the newborn for the parents. Which information does the nurse need to include in the teaching plan?
1. There is no need to limit activities.
 2. It is important to maintain caloric intake.
 3. No secondary complications are expected.
 4. The neonate has natural immunity to infections.
- _____ 11. The nurse is reviewing medications for the treatment of a heart rhythm disorder in a patient who is 8 years of age. The parent of the patient states that the physician recently prescribed medication to treat the patient's attention deficit-hyperactivity disorder. Using knowledge of recent professional recommendations, which statement by the nurse is correct?
1. "We need to remind the physician there is a heart condition."
 2. "Do not start the medication until I can check for safety warnings."
 3. "Children with heart disorders have a higher incidence of ADHD."
 4. "Giving the medication can cause death if there is a cardiac issue."
- _____ 12. The nurse is providing care for a neonate diagnosed with tetralogy of Fallot. Prostaglandin E1 therapy is prescribed to keep the foramen ovale and the ductus arteriosus open. Which is the most important intervention for the nurse to include in the neonate's plan of care?
1. Maintain a separate IV access for continuous administration of the medication.
 2. Watch for respiratory distress or apnea after adding medication to the breathing tube.
 3. Monitor for and document evidence of flushing, bradycardia, and irritability as expected.
 4. Monitor weight and adjust the dosage using a scale of 0.05 to 0.1 mcg/kg/min IV infusion.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 13. The nurse in a pediatric office is aware that certain factors may be indicators of heart disease in children. Which children does the nurse recognize with manifestations related to heart disease? *Select all that apply.*
1. The newborn with dysmorphic facial features
 2. The school-age patient with slow capillary refill
 3. Identification of scoliosis in a new adolescent patient
 4. An infant who is unable to meet developmental milestones
 5. A toddler with clubbing and erythema of the fingers and toes
- _____ 14. A new mother brings her 2-week-old neonate to the pediatrician's office, stating, "I think something is wrong with my baby." When the infant is undressed, the nurse notices signs of possible cardiac problems. Which assessment findings support the nurse's suspicions? *Select all that apply.*
1. Prolonged capillary refill time
 2. Bluish tinge to oral structures
 3. Peripheral cyanosis of left leg
 4. Amount of urinary output
 5. Mottled appearance of skin
- _____ 15. A neonate became dusky and developed respiratory distress at the age of 4 days and is diagnosed with a hypoplastic left heart. The surgeon obtains an informed consent from the parents to perform emergency surgery. Which information will the nurse provide to promote parental understanding? *Select all that apply.*
1. The left side of the neonate's heart did not develop correctly.
 2. The function of left side of the heart is to pump blood to the body.
 3. Provide reassurance to the parents that surgery will fix the problem.
 4. Share that medical management of the condition is needed for one year.
 5. A normally existing hole in the wall of the heart at birth will be enlarged.
- _____ 16. The nurse is providing care for a 9-year-old patient who was recently diagnosed with cardiomyopathy after a viral infection. Which teaching does the nurse provide to the patient's parents about the diagnosed condition? *Select all that apply.*
1. Need for intensive care of the patient
 2. Preparation for anticipatory grieving
 3. The necessity for physical activity
 4. Allowing patient to discuss feelings
 5. Reasons for frequent medical visits
- _____ 17. The nurse on a pediatric unit is providing care for a 5-year-old child diagnosed with congestive heart failure. The physician prescribes digoxin therapy. Which medication-focused interventions does the nurse include when creating a plan of care for the patient? *Select all that apply.*
1. Hold medication if an antibiotic is prescribed.
 2. Evaluate parent's ability to obtain radial pulse.
 3. Administer medication at the same time every day.
 4. Administer 1 hour before or 2 hours after meals.
 5. Replace medication if a dose is vomited within 1 hour.

- _____ 18. The pediatric nurse is providing care for a school-age patient diagnosed with rheumatic heart disease. When developing a plan of care for the patient's hospital stay, which interventions does the nurse include? *Select all that apply.*
1. Administer inflammatory and antibiotic medications as prescribed.
 2. Assess for the presence of strep throat or other throat infections.
 3. Include chest pain and heart palpation assessment with vital signs.
 4. Begin patient/family teaching about the possibility of surgery.
 5. Organize daily care and treatments to provide for joint pain relief.
- _____ 19. The nurse works in a pediatric clinic with patients who have heart disease or who have undergone treatment for heart disease. The nurse is frequently asked about the need for prophylactic antibiotics for these patients during invasive dental care. Which patients does the nurse identify as being at risk for endocarditis and being in need of antibiotic therapy? *Select all that apply.*
1. A child with a prosthetic heart valve
 2. A child with a congenital defect scheduled for surgical repair
 3. A child who was previously diagnosed with endocarditis
 4. A child with a confirmed diagnosis of rheumatic heart disease
 5. A child who had a congenital heart defect repaired 9 months ago
- _____ 20. A parent brings a 2-year-old child with a fever and a rash to the pediatric clinic. The health-care provider suggests the child may have one of several conditions that present with similar symptoms, but wants to rule out Kawasaki disease. Which tests does the nurse expect to be performed? *Select all that apply.*
1. Chest x-ray
 2. White blood cell count
 3. Allergy testing
 4. Baseline echocardiograms
 5. MRI of the chest

Chapter 12 Cardiovascular Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 4

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 1. Identify the normal assessment of the anatomy and physiology of the pediatric cardiovascular system.

Page: 215

Heading: Heart Anatomy and Physiology > Heart Valves

Integrated Process: Teaching and Learning

Client Need: Physiological Integrity > Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. The aortic valve must work correctly for oxygenated blood to be carried from the left side of the heart to the rest of the body.
2	This is incorrect. Blood must enter the right side of the heart from the body in order to be reoxygenated. The unoxygenated blood enters the right atrium through the tricuspid valve into the right ventricle.
3	This is incorrect. A defect in the aortic valve would cause problems with oxygenated blood leaving the left ventricle.
4	This is correct. When there is a defect in the right pulmonic valve, the blood has difficulty leaving the right ventricle and getting to the lungs for reoxygenation. This explanation by the nurse correctly describes the function of the valve and the purpose of the surgery.

PTS: 1

CON: Patient-Centered Care

2. ANS: 3

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 1. Identify the normal assessment of the anatomy and physiology of the pediatric cardiovascular system.

Page: 216

Heading: Heart Anatomy and Physiology > Electrical Conduction

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
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1	This is incorrect. The Purkinje fibers are located in the ventricles; however, the process of electrical conduction does not start here. The fibers initiate contractions of the ventricles.
2	This is incorrect. The bundle branches are divided into either left or right bundles; however, the process of electrical conduction does not start here.
3	This is correct. When teaching the electrical conduction of the heart, the nurse starts with the sinoatrial (SA) node in the right ventricle, which is known as the pacemaker of the heart.
4	This is incorrect. From the SA, the electrical impulse is conducted to the fibers called the bundle of His located in the walls of the ventricles. The process of electrical conduction does not start here.

PTS: 1 CON: Health Promotion

3. ANS: 1

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 1. Identify the normal assessment of the anatomy and physiology of the pediatric cardiovascular system.

Page: 217

Heading: Heart Anatomy and Physiology > Fetal Circulation

Integrated Processes: Nursing Process

Client Need: Physiological Integrity > Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. Prior to birth, oxygenated blood crosses from the right atrium to the left atrium via the patent foramen ovale (PFO) and is pumped by the left ventricle.
2	This is incorrect. Prior to birth, oxygenated blood does not flow from the right ventricle to the lungs and then to the left ventricle.
3	This is incorrect. Before birth, 90% of blood bypasses the lungs; the placenta is the organ of respiration. After the cord is cut and the placenta is delivered, the infant is expected to independently breathe.
4	This is incorrect. Upon birth and first breath, the foramen ovale and ductus arteriosus close.

PTS: 1 CON: Patient-Centered Care

4. ANS: 2

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 2. Identify the physical assessment components of pediatric clients with cardiovascular disease.

Page: 220

Heading: Assessment > General History

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]
 Concept: Growth and Development
 Difficulty: Moderate

	Feedback
1	This is incorrect. Using a well as a water source does not necessarily place a neonate at risk for developing a heart defect.
2	This is correct. The nurse is most likely to contribute exposure to a child with measles as a causative factor for the neonate's heart defect. The nurse will assess further for timelines and manifestations.
3	This is incorrect. The mother's concern about the neonate being born a week early is not recognized as a contributing factor to the neonate's heart defect.
4	This is incorrect. Traveling to a foreign country during the first trimester may or may not effect fetal development. The nurse needs additional information before making a connection between the travel and heart defect.

PTS: 1 CON: Growth and Development

5. ANS: 1

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 3. Identify the anatomic features, clinical presentation, stabilization, emergent, and long-term care of the pediatric client with cardiovascular disease.

Page: 222

Heading: Table 12-2: Syndromes Associated with Cardiac Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is correct. Tall and thin with arms disproportionately long and with laxity of joints are physical manifestations of Marfan syndrome. The nurse may also notice dislocation of lenses, spinal problems, stretch marks, hernia, pectus abnormalities, and/or restrictive lung disease. Marfan syndrome is also associated with aortic aneurism as well as aortic and/or mitral regurgitation.
2	This is incorrect. A sparsity of body hair is not an indication of cardiac issues; male high school students may not exhibit heavy body hair.
3	This is incorrect. A male high school student is expected to have a deep tone to his voice.
4	This is incorrect. A slow, rhythmic gait may be unique to this student; however, it does not support the presence of Marfan syndrome.

PTS: 1 CON: Growth and Development

6. ANS: 3

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 3. Identify the anatomic features, clinical presentation, stabilization, emergent, and long-term care of the pediatric client with cardiovascular disease.

Page: 222

Heading: Table 12-2: Syndromes Associated with Cardiac Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Duchenne muscular dystrophy is a progressive genetic disease. Physical therapy is not prescribed to improve muscle strength; it is used to maintain body function for as long as possible.
2	This is incorrect. Duchenne muscular dystrophy is a progressive genetic disease. The physician may recommend homeschooling when the patient is no longer able to attend classes. There is no acute stage to this disease that will improve.
3	This is correct. The physician is likely to order cardiac testing, especially to rule out cardiomyopathy, a condition frequently associated with Duchenne muscular dystrophy.
4	This is incorrect. The physician is more likely to prescribe neuromuscular tests to determine the extent of the disease.

PTS: 1 CON: Growth and Development

7. ANS: 2

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 2. Identify the physical assessment components of pediatric clients with cardiovascular disease.

Page: 224

Heading: Assessment: Physical Examination > Cardiovascular System

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Perfusion

Difficulty: Difficult

	Feedback
1	This is incorrect. Average systolic blood pressures are considered 70+ (2× age in years). This would be 84.
2	This is correct. Wide pulse pressures—diastolic pressures are low, with a wide gap between diastolic and systolic pressures—are indicative of such processes as patent ductus arteriosus.
3	This is incorrect. Pulse oximeter readings of 95% of room air are considered normal.
4	This is incorrect. <i>The point of maximum impulse (PMI) is heard at the fourth or fifth intercostal space at the midclavicular line (MCL).</i>

PTS: 1 CON: Perfusion

8. ANS: 3

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 5. Describe the common diagnostic tests used in diagnosing and treating cardiovascular diseases in the pediatric population.

Page: 227

Heading: Safe and Effective Nursing Care > Promoting Safety > Nursing Care During Cardiac Catheterization

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. It is not likely for the patient to develop an infection immediately following a cardiac catheterization.
2	This is incorrect. The patient will receive a sedative before the procedure to reduce anxiety; a local anesthetic is used at the puncture site. Neither intervention is likely to cause an absent or depressed gag reflex.
3	This is correct. Immediately after the procedure the nurse will assure that the limb used for cardiac catheterization is kept straight with no movement for 4 to 6 hours. The child should be positioned flat on the back; a sandbag may be used on the extremity. All precautions are to prevent bleeding from the puncture site.
4	This is incorrect. Immediately after a cardiac catheterization, the nurse will not expect to see yellow or green drainage, which is indicative of an infection. The nurse will include this intervention in parent teaching.

PTS: 1 CON: Patient-Centered Care

9. ANS: 1

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 4. Identify the nursing interventions necessary to educate the caregiver to care for a child with cardiovascular disease.

Page: 235

Heading: Congenital Heart Disease With Obstructive Disorders > Pulmonary Stenosis > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is correct. Options are to repair the pulmonary artery and/or pulmonic valve as soon as possible to avoid worsening side effects of the condition.

2	This is incorrect. Side effects of pulmonary stenosis include increased workload of the right ventricle, CHF, hepatomegaly, development of murmurs, shortness of breath, and cyanosis. Delaying corrective surgery will worsen effects of the condition.
3	This is incorrect. The infant with pulmonary stenosis will be hospitalized for assessment and treatment.
4	This is incorrect. The child must return to the cardiologist frequently for follow-up, and caregivers should be provided information on the recurrence of symptoms that may occur due to restenosis.

PTS: 1 CON: Patient-Centered Care

10. ANS: 2

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 4. Identify the nursing interventions necessary to educate the caregiver to care for a child with cardiovascular disease.

Page: 236

Heading: Congenital Heart Disease With Obstructive Disorders > Tetralogy of Fallot With Pulmonary Atresia > Caregiver Education

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. The parents will be taught to calm the infant by holding the infant over the caregiver's shoulders with the infant's knees drawn up toward the chest. This will increase the blood flow to the lungs. The parents will limit cardiac stress by keeping the infant calm.
2	This is correct. Due to a clinical finding of failure to gain weight, the parents are instructed on the importance of maintaining caloric intake. Frequent small feedings are necessary to meet this need and not increase cardiac stress.
3	This is incorrect. Parents need to monitor for signs and symptoms of heart failure.
4	This is incorrect. Educate parents on the increased risk of bacterial endocarditis and the prescribed medication regimen.

PTS: 1 CON: Patient-Centered Care

11. ANS: 3

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 4. Identify the nursing interventions necessary to educate the caregiver to care for a child with cardiovascular disease.

Page: 241

Heading: Safe and Effective Nursing Care: Understanding Medication: Medications for Attention Deficit-Hyperactivity Disorder

Integrated Processes: Communication and Documentation

Client Need: Pharmacological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Application [Applying]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is incorrect. A black box warning about giving medications for ADHD to children with heart problems was issued in 2006. The AAP and the AHA state that medications used to treat ADHD have not been shown in most cases to cause heart disease or result in sudden cardiac death (CDC, 2016; Magellan Health, 2016a). There is no reason to contact the prescribing physician.
2	This is incorrect. If the nurse is knowledgeable about current medication recommendations, the nurse does not tell the parent to hold the medication until the safety can be checked by the nurse.
3	This is correct. Joint statements by the AAP and the AHA show that children with heart conditions have a higher incidence of ADHD, but that medications used to treat ADHD have not been shown in most cases to cause heart disease or result in sudden cardiac death.
4	This is incorrect. It is inaccurate and inappropriate for the nurse to tell the parent that the prescribed medication can cause death in children with cardiac issues.

PTS: 1

CON: Safety

12. ANS: 1

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 6. Develop a nursing care plan for a child with a cardiac condition.

Page: 245

Heading: Safe and Effective Nursing Care > Understanding Medication: Prostaglandin E1

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is correct. During the administration of prostaglandin E1, the nurse starts and maintains a separate IV access for continuous administration of the medication.
2	This is incorrect. During the administration of prostaglandin E1, the nurse monitors for respiratory distress or apnea. However, the medication is not administered via the neonate's breathing tube.
3	This is incorrect. During the administration of prostaglandin E1, the nurse will watch the neonate for flushing, bradycardia, irritability, and diarrhea, and monitor for bleeding. The nurse always documents assessment findings; however, the manifestations need to be reported to the primary health-care provider.

4	This is incorrect. The dosage for prostaglandin is usually 0.05 to 0.1 mcg/kg/min IV infusion, as ordered by the primary health-care provider. The nurse does not adjust the dose using any parameter.
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PTS: 1

CON: Safety

MULTIPLE RESPONSE

13. ANS: 1, 3, 4, 5

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 2. Identify the physical assessment components of pediatric clients with cardiovascular disease

Page: 223

Heading: Assessment > Physical Examination > Possible Indicators of Heart Disease in Children

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. Dysmorphic facial features can be identified at any age and can be indicative of heart disease.
2.	This is incorrect. Slow capillary refill can be from multiple causes, such as poor circulation, low oxygenation, anemia, exposure to cold, and/or stress. The finding alone does not cause the nurse to recognize a manifestation of heart disease.
3.	This is correct. When assessing a new patient, the nurse recognizes that scoliosis is common in adolescents with congenital heart disease. The nurse will perform additional assessment.
4.	This is correct. When an infant is unable to meet developmental milestones, the nurse recognizes that children with congenital heart defects are more likely to have developmental disabilities.
5.	This is correct. Clubbing and erythema in fingers and toes may result from longstanding cyanosis due to increased formation and enlargement of the capillaries in the periphery to improve circulation. This is a manifestation of heart disease in a toddler.

PTS: 1

CON: Patient-Centered Care

14. ANS: 1, 2, 5

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 3. Identify the anatomic features, clinical presentation, stabilization, emergent, and long-term care of the pediatric client with cardiovascular disease.

Page: 225

Heading: Assessment > Physical Examination > Possible Indicators of Heart Disease in Children > Color

Integrative Processes: Nursing Process
 Client Need: Physiological Integrity: Physiological Adaptation
 Cognitive Level: Analysis [Analyzing]
 Concept: Perfusion
 Difficulty: Difficult

	Feedback
1.	This is correct. Prolonged capillary refill time is a sign of poor perfusion; it can be indicative of possible cardiac or lung issues. The assessment is made by pressing on the sternum or forehead.
2.	This is correct. Central cyanosis is the presence of bluish discoloration of mucous membranes, tongue, circumoral, or core body and is due to problems with the heart or lungs.
3.	This is incorrect. Peripheral cyanosis (acrocyanosis) is often due to interruption in blood flow to the extremity.
4.	This is incorrect. Urine output is indicative of perfusion to the kidneys; however, the nurse is unable to determine the amount of urinary output through observation.
5.	This is correct. Skin that is pale, mottled, or gray in appearance indicates poor perfusion.

- PTS: 1 CON: Perfusion
15. ANS: 1, 2, 5
- Chapter: Chapter 12 Cardiovascular Disorders
 Chapter Learning Objectives: 4. Identify the nursing interventions necessary to educate the caregiver to care for a child with cardiovascular disease.
 Page: 238
 Heading: Congenital Heart Disease With Mixed Disorders > Hypoplastic Left Heart
 Integrated Processes: Nursing Process
 Client Need: Physiological Integrity: Reduction of Risk Potential
 Cognitive Level: Application [Applying]
 Concept: Perfusion
 Difficulty: Difficult

	Feedback
1.	This is correct. The hypoplastic left heart is the second most common congenital heart defect, caused by underdevelopment of the left side of the heart, aorta, aortic valve, left ventricle, and mitral valve.
2.	This is correct. The nurse is correct in providing information about the normal functioning of the left side of the heart to clarify the existing condition.
3.	This is incorrect. The nurse needs to provide the parents with information about the severity of the condition. The nurse anticipates possible transport out of the facility for heart transplant.
4.	This is incorrect. The neonate will require clinical and medication management for life.

5.	This is correct. Symptoms appear when the PDA closes; the condition is usually fatal within the first days or months of life unless treated. It is appropriate for the nurse to describe the surgery in terms the parents can understand.
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PTS: 1 CON: Perfusion

16. ANS: 1, 2, 4, 5

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 4. Identify the nursing interventions necessary to educate the caregiver.

Page: 239

Heading: Acquired Congenital Heart Disease > Cardiomyopathy > Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will provide teaching about the intensive care necessary for a child with this life-threatening condition.
2.	This is correct. The patient with cardiomyopathy has a possible terminal status. The nurse will prepare the parents for expected anticipatory grieving, which is expected with this life-threatening condition.
3.	This is incorrect. The nurse will provide teaching about the activity restrictions to prevent overstimulation of the heart.
4.	This is correct. The nurse will instruct the parents to allow the child to discuss feelings concerning the restriction of activity in the previously active child.
5.	This is correct. The parents need to understand that frequent echocardiograms will be required to monitor the size and function of the heart.

PTS: 1 CON: Patient-Centered Care

17. ANS: 3, 4

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objective: 6. Develop a nursing care plan for a child with a cardiac condition.

Page: 241

Heading: Safe and Effective Care > Understanding Medication > Digoxin (Lanoxin)

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
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1.	This is incorrect. The child on digoxin therapy needs to be closely monitored for digoxin toxicity with antibiotic therapy, which is caused by changes in intestinal flora. The nurse will not include any interventions for holding digoxin on the plan of care.
2.	This is incorrect. Educating parents on how to assess an infant's apical pulse rate and to notify the health-care practitioner if the heart rate is out of the range set by the health-care provider is included in the plan of care. However, the nurse will not include teaching parents how to take radial pulses.
3.	This is correct. The nurse will include interventions on the plan of care regarding medication administration. The medication is administered at the same time every day and at the correct frequency.
4.	This is correct. In order to promote absorption of digoxin, the nurse will administer the medication 1 hour before or 2 hours after meals. This intervention can be included on the plan of care.
5.	This is incorrect. The nurse will not include an intervention on the plan of care addressing when to replace vomited or missed doses; the medication is not to be replaced. There is no way to determine how medication was already absorbed.

PTS: 1 CON: Patient-Centered Care

18. ANS: 1, 3, 5

Chapter: Chapter 12 Cardiovascular Disease

Chapter Learning Objective: 6. Develop a nursing care plan for a child with a cardiac condition.

Page: 245

Heading: Rheumatic Heart Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse will address interventions for the administration of inflammatory and antibiotic medications as prescribed.
2.	This is incorrect. The nurse does not need to add assessment interventions for strep throat or other throat infections in a patient diagnosed with rheumatic heart disease.
3.	This is correct. The nurse will include the intervention of including assessment for chest pain or heart palpitations with vital signs, because rheumatic heart disease causes heart valve damage.
4.	This is incorrect. The scenario does not specify the reason for the patient's hospitalization. Patient/family teaching is not initiated until surgery is planned.
5.	This is correct. Rheumatic heart disease can cause polyarthritis; it is necessary for the nurse to include interventions that will promote rest and relieve joint pain.

PTS: 1 CON: Patient-Centered Care

19. ANS: 1, 2, 3, 4

Chapter: Chapter 12 Cardiovascular disorders

Chapter Learning Objective: 6. Develop a nursing care plan for a child with a cardiac condition.

Page: 246

Heading: Evidence-Based Practice > Dental Procedures for Children With Heart Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The American Heart Association recommends prophylactic antibiotics for the child with a valve replacement.
2.	This is correct. The American Heart Association recommends prophylactic antibiotics for the child with a congenital defect who is scheduled for surgical repair.
3.	This is correct. The American Heart Association recommends prophylactic antibiotics for the child who was previously diagnosed with endocarditis.
4.	This is correct. The American Heart Association recommends prophylactic antibiotics for the child who is diagnosed with rheumatic heart disease, which affects the heart valves.
5.	This is incorrect. Prophylactic antibiotics are recommended for the child with a congenital heart defect that has been corrected for the first 6 months after the repair. The child who is 9 months past the repair does not need prophylactic antibiotics.

PTS: 1 CON: Patient-Centered Care

20. ANS: 2, 4

Chapter: Chapter 12 Cardiovascular Disorders

Chapter Learning Objectives: 5. Describe the common diagnostic tests used in diagnosing and treating cardiovascular diseases in the pediatric population.

Page: 246

Heading: Kawasaki Disease > Diagnostic Tests

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. A chest x-ray will not reveal Kawasaki's, as it is a blood infection.
2.	This is correct. The WBC will reveal lymphocytosis and thrombosis, which are present with Kawasaki disease.
3.	This is incorrect. Kawasaki is not an allergy reaction, and allergy testing is inappropriate.

4.	This is correct. A life-threatening complication of Kawasaki disease is the development of coronary artery aneurysms. Baseline echocardiogram 6 to 8 weeks after the onset of symptoms is used to rule out this complication.
5.	This is incorrect. Magnetic resonance imaging will not reveal Kawasaki disease.

PTS: 1 CON: Patient-Centered Care

Chapter 13 Neurological and Sensory Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is assessing a 7-year-old child at a pediatric clinic. The nurse notices that several developmental milestones have been missed or are late during previous visits. The parent states, "I know she is a little slow, but she will catch up quickly." Which action by the nurse is warranted?
1. Explain to the parent that rapid development takes place in infancy and early childhood.
 2. Suggest activities in the home that may improve mental and physical development.
 3. Recommend that the child be placed in special classes aimed at promoting development.
 4. Ask the parent detailed questions about the pregnancy, birth, and early childhood health.
- _____ 2. The nurse is assessing a 4-month-old infant during a routine well-baby visit. During the neurological assessment, which finding is a reason for concern?
1. When the cheek is brushed, the head is turned toward the stimuli.
 2. Toes fan out when the sole of the foot is stroked upward.
 3. Placing a small object in the palm inconsistently elicits a grasp.
 4. A light puff of air in the face causes the eyes to close.
- _____ 3. The nurse is performing a developmental assessment on a toddler at age 3 years. The nurse notices a variety of mixed developmental milestones that have been missed during the visit. Which delay does the nurse expect to be of greatest concern to the parent?
1. Difficulty putting small objects into a bottle
 2. An inability to kick a ball back to the nurse
 3. Difficulty with and reluctance to self-dress
 4. An inability to express needs with language
- _____ 4. The nurse is presenting a class to high school females about decreasing the developmental risks related to pregnancy. Which information does the nurse consider to be most important?
1. Young women should begin taking 600 mg of calcium twice a day.
 2. All females of child-bearing age should take 0.4 mg of folic acid daily.
 3. Early prenatal care is essential for a healthy pregnancy and baby.
 4. Important fetal development occurs before pregnancy is suspected.
- _____ 5. The nurse is performing well-baby checks in a pediatric clinic. During physical examination of a 1-month-old infant, the nurse notices a dimple with a tuft of hair in the lumbar sacral area indicative of spina bifida. Which developmental delays does the nurse expect for this infant?

1. There may be issues related to bowel and bladder control.
2. Some degree of paralysis of the lower limbs is expected.
3. The infant is not expected to experience physical delays.
4. Muscles of the legs will be flaccid with some sensory loss.

_____ 6. The nurse in the newborn nursery is providing care for a neonate with an open spinal cord defect. The neonate will be transported to a pediatric surgery hospital as soon as possible. Which description of the nurse's care of the neonate is correct?

1. Using aseptic technique, place a sterile plastic bag around the defect and loosely tie it closed.
2. Place the newborn prone on a loose diaper and cover the defect with a second saline-moistened diaper.
3. Position the newborn on the side with a moistened dressing on the defect; wrap the defect and newborn in a blanket.
4. Cover the defect with a sterile dressing moistened with warm sterile normal saline, using aseptic technique.

_____ 7. The nurse is gathering health information on a child who is 8 years of age. The parent reports the child is extremely difficult to wake in the morning. Which other information will prompt the nurse to recommend screening for a sleep disorder?

1. The bedroom is shared with a sibling.
2. The nurse validates the child is obese.
3. There is a TV in the child's bedroom.
4. It is difficult to get the child to bed.

_____ 8. A third-grade teacher discusses behavioral problems with a student. The teacher states, "He walks around class making horrible sucking noises. He does not respond to me." Which information does the nurse seek from the student's parents?

1. Ask if the student has been tested by a physician for seizure disorder.
2. Inquire if the student is either diagnosed or medicated for ADHD.
3. Ascertain if the student has experienced recent illness or a fever.
4. Suggest the student be screened for possible developmental delays.

_____ 9. The school nurse is present at a school assembly when a student falls to the floor with a seizure. Which intervention does the nurse initiate when providing care to the student during the seizure?

1. Protect the student from injury related to seizure movement.
2. Remove or loosen any tight clothing around the neck or waist.
3. Provide comfort and promote resting in a quiet environment.
4. If incontinent, cover the student with a blanket or sheet.

_____ 10. The nurse is providing care for an infant at 3 months of age. The parent reports sudden flexor or extensor movements of the neck, trunk, and extremities occurring multiple times a day. The infant is diagnosed with infant spasms and is prescribed corticotropin (Acthar jell) therapy. Which instruction is most important for the nurse to provide for the parent?

1. Reason for weekly laboratory visits
2. Expected medication side effects
3. Signs and symptoms of infections
4. How to administer IM medication

- _____ 11. The nurse is collecting information about a school-age patient brought to a pediatric clinic by a parent. The parent reports several incidences of syncope. Which assessment question helps the nurse to identify a possible diagnosis of vasovagal syncope?
1. "Has your daughter been diagnosed with diabetes mellitus?"
 2. "Did your child feel strange and faint after standing up?"
 3. "Was your child in a stressful situation before fainting?"
 4. "Does your daughter have any cardiac conditions?"
- _____ 12. The nurse is providing care for a pediatric patient who received a concussion while playing football. The patient had brief loss of consciousness and now reports a headache with a pain level of 6 on a 0 to 10 scale. The patient states, "My team plays again in five days and I should be better." Which information is vital for the patient and parents to understand?
1. A realistic timeframe regarding complete recovery
 2. Type of equipment to prevent a second head injury
 3. The risk of acquiring second impact syndrome
 4. The potential for long-term headaches
- _____ 13. The pediatric nurse is examining the skin of a young child and notices eight café-au-lait spots between 1.5 and 3 inches in diameter on the body, along with axillary freckling. Which recommendation does the nurse make to the parent?
1. Refrain from having additional children without counseling.
 2. Make an appointment with a physician for testing and evaluation.
 3. Agree to blood testing of the child to identify a defect in the NF1 gene.
 4. Arrange for psychological therapy to address self-esteem problems.
- _____ 14. A parent brings a child who is 8 years of age to the pediatric clinic and tells the nurse, "I think he has Tourette's syndrome. He recently began some eye-blinking and grimacing actions." Which information does the nurse provide to help the parent distinguish between transient tic of childhood and Tourette's syndrome?
1. Vocal tics frequently become chronic in children with transient tic of childhood diagnosis.
 2. Transient tic of childhood begins with a high level of tic activity and usually disappears completely by age 12.
 3. Tourette's syndrome is a disorder of complex motor and vocal tics that have been present for more than 1 year.
 4. Tourette's syndrome is a disorder of complex motor and vocal tics that develop between the ages of 3 to 8 years.
- _____ 15. The pediatric nurse in an acute care facility is providing care for a patient who is 12 years of age with a history of sickle cell anemia. During this hospitalization, it is determined that the patient has experienced a stroke. Which teaching is most important for the nurse to provide to the patient and parents?
1. A need for intensive physical and speech therapies
 2. Reasons to have a designated social worker
 3. The necessity for an individualized education plan
 4. Manifestations of increased intracranial pressure

- _____ 16. The nurse in the emergency department of a pediatric hospital is providing care for a toddler with a sudden high fever. The parent states, “She has been grumpy all day and I thought she just needed a nap.” Which finding does the nurse recognize as an indication of an immediate medical emergency?
1. The toddler keeps eyes closed or covered at all times.
 2. The nurse elicits a positive Brudzinski’s sign.
 3. A rash of scattered red bumps is found on the skin.
 4. The toddler cries when head and neck are moved.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 17. The nurse in a pediatric unit is providing care for a 2-month-old infant just diagnosed with spinal muscle atrophy. Which characteristics of the condition does the nurse expect to find during physical assessment? *Select all that apply.*
1. Hyperreflexia in deep tendons
 2. Few spontaneous movements
 3. Deep, rapid respirations
 4. Fasciculations of the tongue
 5. Proximal muscle atrophy
- _____ 18. The NICU nurse is providing care for a neonate exhibiting manifestations of congenital Zika syndrome. Which distinct features does the nurse associate with the syndrome? *Select all that apply.*
1. Partially collapsed skull
 2. Decreased brain tissue
 3. Damage to the back of the eyes
 4. Multiple joint contractures
 5. Agitated body movement
- _____ 19. A parent brings an infant to the pediatric clinic and expresses concern about irritability and poor feeding, along with recent symptoms of flu lasting a few days. The nurse notices multiple raised mosquito bites on the infant. Which additional knowledge causes the nurse to suspect encephalitis? *Select all that apply.*
1. A recent local outbreak of West Nile fever
 2. Bulging fontanel when in a quiet state
 3. Signs of facial and eyelid weakness
 4. Loss of deep tendon reflexes
 5. Drooling instead of swallowing saliva
- _____ 20. A parent brings an adolescent who is 16 years of age to the pediatric clinic, because the patient is experiencing unusual sensations in the feet. The nurse learns the patient was diagnosed with type 1 diabetes mellitus as a toddler; glucose levels have always been erratic and difficult to control. Which assessment findings does the nurse expect based on the health history? *Select all that apply.*
1. Inability to identify a sharp or blunt sensation on the sole of the foot
 2. Feet warm to the touch and capillary refill within normal limits
 3. Problems with balance when standing without support

4. Toe nails smooth in appearance and nail beds pink in color
5. Signs of weakness during neuromuscular checks to the lower legs

Chapter 13 Neurological and Sensory Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 4

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 2. Describe normal growth and development of the brain and spinal cord.

Page: 255

Heading: Neurological Development

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. Explaining the expected periods and timeframes when development rapidly occurs is not beneficial to the parent.
2	This is incorrect. Suggesting home activities that improve mental and physical development may or may not be effective. Further assessment is needed to better define the findings.
3	This is incorrect. It is premature for the nurse to recommend special classes for the child.
4	This is correct. The nurse needs to further assess for possible contributing factors for the child's developmental delays. Along with information about the pregnancy, birth, and early childhood health, the nurse will explore the family health history and home environment.

PTS: 1

CON: Growth and Development

2. ANS: 1

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 1. Identify the normal assessment of the pediatric nervous system.

Page: 256

Heading: Box 13-3: Primitive Reflexes

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
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1	This is correct. The nurse is assessing for the presence of the rooting reflex, which disappears by the age of 3 to 4 months. The presence of this reflex at 4 months is a reason for concern.
2	This is incorrect. The nurse is checking for the presence of the Babinski sign, which is elicited by stroking the sole of the foot upward, causing the toes to fan out. The sign should disappear between 9 and 12 months of age.
3	This is incorrect. The nurse is checking the presence of a palmar grasp reflex, which should weaken by 3 months and completely disappear by 6 months of age. The ability to elicit an inconsistent grasp is not a reason for concern.
4	This is incorrect. At birth, one permanent reflex is the blinking (glabellar) reflex. The reflex is elicited when a light puff of air hits the face.

PTS: 1 CON: Growth and Development

3. ANS: 4

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 3. Identify disorders that can result in intellectual and developmental delays in children.

Page: 258

Heading: Intellectual and Developmental Delays

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. At the age of 3 years, it is expected that the child will have the fine motor control needed to place small objects in a bottle. The lack of this ability may or may not be the parent's greatest concern.
2	This is incorrect. At the age of 3 years, the inability to kick a ball back to the nurse is an indication of gross motor delay. This may or may not be the parent's greatest concern.
3	This is incorrect. Difficulty with and reluctance to self-dress indicates a delay in adaptive skills. This may or may not be the parent's greatest concern.
4	This is correct. Language is an important developmental milestone, and the inability to verbally express needs by the age of 3 years is a real concern. The most common parental concern is delayed development of expressive language.

PTS: 1 CON: Growth and Development

4. ANS: 2

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Outcomes: 4. Identify common neurological congenital abnormalities of the nervous system.

Page: 261

Heading: Congenital Abnormalities of the Nervous System (Neural Tube Defects)

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1	This is incorrect. Women at a young age should begin taking calcium supplements to promote lifelong bone health. However, calcium is not the most important information to decrease developmental risks during pregnancy.
2	This is correct. Because neural tube closure occurs before most women even know they are pregnant, it is important to teach adolescent girls to begin taking folic acid supplements before pregnancy occurs. Teen pregnancies are usually unplanned, which makes folic acid intake very important.
3	This is incorrect. It is true that early prenatal care is important for a healthy pregnancy and baby; however, the most important information is about folic acid and preventing neural tube defects.
4	This is incorrect. High school females need to be aware that all essential fetal development occurs before pregnancy is even suspected. Neural tube closure normally occurs around the 28th day after fertilization.

PTS: 1 CON: Health Promotion

5. ANS: 3

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Outcomes: 4. Identify common neurological congenital abnormalities of the nervous system.

Page: 261

Heading: Congenital Abnormalities of the Nervous System (Neural Tube Defects) > Spina Bifida

Integrated Processes: Nursing Process

Client Need: Physical Integrity: Physical Adaptation

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. The infant is exhibiting the characteristics of spina bifida occulta. Issues related to bowel and bladder control are seen in spina bifida cystica or with a meningocele.
2	This is incorrect. Some degree of paralysis is common with spina bifida cystica or a meningocele.
3	This is correct. The infant is exhibiting the characteristics of spina bifida occulta, which occurs from a section of the spinal vertebrae being malformed, but the spinal cord and nerves are normal. No developmental delays are expected with this condition.
4	This is incorrect. Flaccid leg muscles and sensory loss are associated with spina bifida cystica or a meningocele.

PTS: 1 CON: Growth and Development

6. ANS: 4

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Outcome: 4. Identify common neurological congenital abnormalities of the nervous system.

Page: 262

Heading: Congenital Abnormalities of the Nervous System (Neural Tube Defects) > Spina Bifida

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. A sterile bag is not placed around the defect and loosely tied closed.
2	This is incorrect. The nurse will use aseptic technique; the defect is not covered by a second saline moistened diaper, which can be clean but is not likely to be sterile.
3	This is incorrect. The defect must remain as sterile as possible and the neonate is positioned to prevent any pressure on the defect. A side-lying position is not used, and the defect and neonate are not wrapped in a blanket. Body heat will be preserved by placing the newborn in a warmer.
4	This is correct. The nurse must exercise caution to keep the defect covered and protected until surgical correction can occur. Using aseptic technique, the nurse should cover the defect with a sterile dressing moistened with warm sterile normal saline. The neonate will be positioned prone and lying on an open diaper.

PTS: 1 CON: Patient-Centered Care

7. ANS: 2

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 11. Identify common behavioral concerns in children.

Page: 267

Heading: Sleep Disorders

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Sleep and Rest

Difficulty: Difficult

	Feedback
1	This is incorrect. The fact that the patient's bedroom is shared with a sibling is an environment factor that can interrupt sleep.

2	This is correct. Obesity in a child can cause sleep apnea, which can result in heavy snoring or choking sounds during sleep, as well as daytime fatigue, irritability, or learning problems in school. This finding will prompt the nurse to recommend screening for sleep disorder.
3	This is incorrect. TV in the bedroom is a distraction that delays sleep; this is an environmental factor.
4	This is incorrect. Children who are difficult to get to bed are likely to have sleep deprivation because of resisting sleep or falling asleep late at night. This is a behavioral factor.

PTS: 1 CON: Sleep and Rest

8. ANS: 1

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 5. Recognize seizure disorders and treatment for seizure types.

Page: 269

Heading: Table 13-5: Seizure Types

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Neurologic Regulation

Difficulty: Moderate

	Feedback
1	This is correct. The student's behavior is commonly seen with complex partial seizures; the nurse needs to ask if the student has been tested for seizure disorder. Manifestations include automatisms such as lip smacking, chewing, sucking, repetitive and involuntary movements, walking, and restlessness. Consciousness is altered, but the person remains awake.
2	This is incorrect. The student's altered consciousness while remaining awake does not fit a diagnosis of ADHD.
3	This is incorrect. Complex partial seizures do not occur as a result of illness or fever; there is no identification of the cause.
4	This is incorrect. Certain conditions that cause seizure activity can also be the cause of developmental delay; however, not all seizure activity is caused by or results in developmental delays.

PTS: 1 CON: Neurological Regulation

9. ANS: 1

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 5. Recognize seizure disorders and treatment for seizure types.

Page: 269

Heading: Seizure Disorders > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Neurologic Regulation

Difficulty: Moderate

	Feedback
1	This is correct. During the seizure, the nurse needs to protect the student from injury caused by seizure movement.
2	This is incorrect. Tight clothing around the neck should be loosened to keep the airway unimpaired. The clothing around the waist is not loosened, and no clothing is removed.
3	This is incorrect. After the seizure, the student should be comforted, reoriented, and allowed to rest.
4	This is incorrect. The student may experience bladder or bowel incontinence during a seizure; however, the student should not be covered with any item that can become entangled from seizure activity. Once the seizure is over, the student can be assisted with cleaning up.

PTS: 1 CON: Neurological Regulation

10. ANS: 4

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 5. Recognize seizure disorders and treatment for seizure types.

Page: 270

Heading: Table 13-6: Antiepileptic Medications

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is incorrect. Corticotropin therapy requires weekly monitoring of glucose and electrolytes during therapy. The nurse will share the importance of keeping these appointments; however, another option is most important.
2	This is incorrect. The nurse will inform the parent of expected side effects, such as increased appetite, weight gain, irritability, edema, hypertension, and risk for infections. However, another option is most important.
3	This is incorrect. Because the infant on corticotropin therapy is at greater risk for infections, the nurse will instruct the parent about signs and symptoms and when to contact the physician. However, another option is most important.
4	This is correct. Corticotropin (Acthar jell) is administered IM, so the most important information for the nurse to provide to the parent is how to administer the medication.

PTS: 1 CON: Safety

11. ANS: 2

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 6. Recognize disorders resulting in temporary loss of consciousness.

Page: 278

Heading: Syncope

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. This question rules out metabolic syncope, which is when a person faints in response to metabolic conditions such as hypoglycemia or hyperventilation.
2	This is correct. This question is appropriate for identifying vasovagal syncope, which is the most common type of syncope. Blood pressure drops quickly, reducing the blood flow to the brain. Standing results in a flow of blood in the lower extremities, and the autonomic nervous system needs to act in conjunction with the heart to normalize blood pressure.
3	This is incorrect. This question rules out psychogenic syncope, fainting in response to anxiety or panic.
4	This is incorrect. This question rules out cardiac syncope, a loss of consciousness because of a heart condition that interferes with blood flow to the brain.

PTS: 1

CON: Patient-Centered Care

12. ANS: 3

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 7. Identify headache disorders in children.

Page: 278

Heading: Headaches and Migraines > Types of Headaches

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse needs to inform the patient and parents that full recovery from a concussion may take days, weeks, or months. This information is important, but it is not considered vital.
2	This is incorrect. Information about the type of equipment is needed to avoid a second head injury is important; however, it is not vital.
3	This is correct. There is risk for a lethal condition known as second impact syndrome, in which the brain swells rapidly and the person succumbs quickly if a second concussion occurs before the first concussion has resolved. The patient's remark makes this information vitally important.
4	This is incorrect. Head injuries may or may not cause long-term headaches.

PTS: 1 CON: Patient-Centered Care

13. ANS: 2

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 8. Identify neurocutaneous disorders in children.

Page: 280

Heading: Neurocutaneous Disorders > Neurofibromatosis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. Without a valid diagnosis, it is not appropriate for the nurse to recommend the parents refrain from having additional children.
2	This is correct. The nurse may recognize that the child's skin manifestations are related to a strong possibility of neurofibromatosis. The nurse needs to recommend seeing a physician for testing and evaluation.
3	This is incorrect. Part of the routine testing on the child is likely to involve a blood test to evaluate for a defect in the NF1 gene.
4	This is incorrect. Once a valid diagnosis is obtained, the nurse will recommend that psychological therapy may become necessary for self-esteem issues from the presence of visible skin tumors.

PTS: 1 CON: Patient-Centered Care

14. ANS: 3

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 9. Identify common movement disorders in children.

Page: 281

Heading: Movement Disorders in Children

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiologic Adaptation

Cognitive Level: Application [Applying]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. Transient tic of childhood is typically a disorder of motor or vocal tics that may last for several months but not greater than 1 year.
2	This is incorrect. Transient tic of childhood occurs before the age of 10 years, may worsen around age 12 years, and usually disappears completely by age 18.
3	This is correct. Tourette's syndrome is a disorder of complex motor and vocal tics that have been present for more than 1 year and began before the child's 18th birthday.

4	This is incorrect. Transient tic of childhood is usually a simple tic that appears between the ages of 3 and 9 years and lasts for less than 1 year.
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PTS: 1 CON: Patient-Centered Care

15. ANS: 4

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 10. Identify common causes of pediatric stroke.

Page: 282

Heading: Pediatric Stroke

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. The patient and family need teaching regarding physical, occupational, and speech therapies, which may be required for the patient's recovery. However, another need is most important.
2	This is incorrect. Caring for a child after a stroke is emotionally, physically, and financially exhausting, and the nurse needs to provide information about the benefit of having a designated social worker. However, another need is most important.
3	This is incorrect. After a pediatric patient experiences a stroke, an individualized education plan may be needed to assist the patient with academic achievement. However, another need is most important.
4	This is correct. Screening for the development of hydrocephalus is indicated, because it is a common complication of pediatric stroke. Important teaching for the patient and parents is related to the manifestations of increased intracranial pressure, which occurs with hydrocephalus.

PTS: 1 CON: Patient-Centered Care

16. ANS: 3

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 13. Identify types of infections that affect the neurological system.

Page: 283

Heading: Infections of the Nervous System

Integrated Processes: Nursing Process

Client need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
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1	This is incorrect. Meningitis should always be suspected in acutely ill infants and children with fever and lethargy until proven otherwise; photophobia is a positive sign for meningitis. Meningitis can be a life-threatening infection and must be treated as a medical emergency; however, there is another finding that is more serious.
2	This is incorrect. Brudzinski's sign is positive for meningitis when flexion of the neck causes involuntary flexion of the knee and hip; however, there is another finding that is more serious.
3	This is correct. The scenario describes the existence of a specific bacterial infection such as Neisseria meningitidis. This infection results in a purpuric rash, often combined with sepsis. Death can occur in hours after the rash appears. This finding is an immediate medical emergency.
4	This is incorrect. Meningitis is commonly associated with a stiff neck; crying when the head and neck is moved is indicative of pain.

PTS: 1

CON: Patient-Centered Care

MULTIPLE RESPONSE

17. ANS: 2, 4

Chapter: Chapter 13 Neurological and Sensory

Chapter Learning Objective: 11. Identify neuromuscular disorders in children.

Page: 285

Heading: Neuromuscular Disorders > Spinal Muscle Atrophy

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. SMA type 1 begins in utero or early infancy and is the most serious type. The infant will exhibit a loss of deep tendon reflexes, not hyperreflexia.
2.	This is correct. The infant with SMA type 1 will exhibit few spontaneous movements and will be unable to lift their head.
3.	This is incorrect. The respiratory muscles are weak, and death occurs by age 3 years because of respiratory compromise. Infants who display symptoms at birth usually have a shorter life span and die before they are 12 months old.
4.	This is correct. Fasciculations, or constant, wormlike movements of the tongue, are noted in the infant diagnosed with SMA type 1.
5.	This is incorrect. Proximal muscle atrophy is seen in SMA type 3, a juvenile form of the disease. This manifestation is not seen with SMA type 1.

PTS: 1

CON: Patient-Centered Care

18. ANS: 1, 2, 3, 4, 5

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 13. Identify types of infections that affect the neurological system.

Page: 285

Heading: Infections of the Nervous System > Zika Virus

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. A neonate with Zika virus syndrome will exhibit severe microcephaly in which the skull has partially collapsed.
2.	This is correct. A neonate with Zika virus syndrome will have decreased brain tissue with a specific pattern of brain damage, including subcortical calcifications.
3.	This is correct. A neonate with Zika syndrome will exhibit damage to the back of the eye, including macular scarring and focal pigmentary retinal mottling.
4.	This is correct. A neonate with Zika virus syndrome will have congenital contractures such as clubfoot or arthrogryposis, a condition in which the infant is born with multiple joint contractures throughout the body.
5.	This is incorrect. A neonate with Zika virus syndrome will exhibit hypertonia, which restricts body movement soon after birth.

PTS: 1

CON: Patient-Centered Care

19. ANS: 1, 2

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 13. Identify types of infections that affect the neurological system.

Page: 285

Heading: Infections of the Nervous System

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. Encephalitis is an acute infection of the brain related to viral infections such as the herpes simplex virus and West Nile virus.
2.	This is correct. Symptoms of encephalitis in infants are irritability, poor feeding, bulging fontanel, vomiting, and body stiffness.
3.	This is incorrect. Acute flaccid myelitis symptoms include weakness in the face or eyelids.
4.	This is incorrect. Loss of deep tendon reflexes is associated with acute flaccid myelitis.
5.	This is incorrect. Difficulty swallowing is associated with acute flaccid myelitis, which can be exhibited in an infant with drooling instead of swallowing their saliva.

PTS: 1 CON: Patient-Centered Care

20. ANS: 1, 3, 5

Chapter: Chapter 13 Neurological and Sensory Disorders

Chapter Learning Objective: 12. Identify types of neuropathy that occur in children.

Page: 288

Heading: Neuropathy Syndromes

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. The nurse is likely to suspect neuropathy, which is a condition of peripheral nerve damage resulting from a metabolic or endocrine disorder such as diabetes. The inability to distinguish between a sharp or blunt sensation on the sole of the foot supports the nurse's suspicion.
2.	This is incorrect. The nurse does not expect to find feet that are warm to the touch, and capillary refill to be normal with suspected diabetic neuropathy. Compromised circulation is most likely.
3.	This is correct. When neuropathy of the feet is present, balance can be adversely affected by pain, tingling, and loss of sensation. This finding supports the nurse's suspicions.
4.	This is incorrect. Smooth toe nails and nail beds that are pink in color indicate good circulation.
5.	This is correct. The presence of neuropathy in the lower extremities can be manifested by weakness.

PTS: 1 CON: Patient-Centered Care

Chapter 14 Mental Health Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is performing a well-baby check on an infant at 6 months of age. The mother shares that the infant sometimes seems unhappy. Which question is most important for the nurse to ask the mother?
1. "Is it easy to make the baby laugh if he seems unhappy?"
 2. "Can you cheer him up by playing with his favorite toys?"
 3. "Do you ever remember feelings of being depressed or sad?"
 4. "Are you noticing any problems with him eating or sleeping?"
- _____ 2. The school nurse in a middle school is aware of a student who takes lithium for a bipolar disorder type 1. Which observation by the nurse will indicate a need for a laboratory test?
1. The student is exhibiting multiple signs of mania.

2. The student gets a bathroom pass during every class.
 3. The student shoved other students at lunch and in the hall.
 4. The student is exhibiting signs of a respiratory infection.
- _____ 3. The nurse is attending a high school sports event when a student suddenly stands and shouts, “I need to get out of here—get me out!” Which intervention by the nurse is most appropriate at this time?
1. Attempt to calm the student with quiet breathing and relaxation.
 2. Identify the events that led to the student’s behaviors.
 3. Look for the student’s parents and ask about the behavior.
 4. Assist the student to a quiet place and remain with the student.
- _____ 4. The nurse at a pediatric clinic is gathering assessment information on a school age patient who is 9 years of age. The mother expresses concern about a recent habit of handwashing to “get rid of germs.” Which recommendation by the nurse is appropriate?
1. Allow the child to complete each session of handwashing.
 2. Assign tasks to the child that involves putting hands in water.
 3. Interrupt the handwashing by moving the child away from the sink.
 4. During the handwashing, ask the child about worries and concerns.
- _____ 5. The nurse is performing a routine pediatric assessment on an 11-year-old student who is being medicated for ADHD. The parent reports disruptive behavior and acting out both at school and at home. The parent asks about a possible medication increase. Which information is the most important for the nurse to acquire?
1. Whether the student is having problems sleeping
 2. How often the student is getting medication
 3. The student’s weight and level of appetite
 4. The student’s perception of medication effects
- _____ 6. The nurse is providing care for a student who was involved in a school violence incidence. The student becomes agitated and angry on the anniversary of the event. Which action by the nurse is most helpful to the student?
1. Administer the physician-prescribed dose of propranolol (Inderal).
 2. Gently and quietly try to verbally and physically soothe the student.
 3. Call for a psychotherapist to come and assist the student with PTSD.
 4. Offer verbal support and encourage the student to express feelings.
- _____ 7. The school nurse is called to attend to a child who is 10 years of age. The teacher reports the child panicked when asked to present a verbal book report to the class. Which intervention will the nurse initiate with the child?
1. Take the child to the clinic and report the event to the parents.
 2. Place the child on a clinic bed and allow some alone time.
 3. Walk with the child in the hallway and provide reassurance.
 4. Join the teacher in attempting to find the source of panic.

- _____ 8. A parent brings a toddler to a pediatric clinic for advice about dealing with a fear of water. The parent shares that the toddler screams and throws a tantrum if anyone attempts to get him into a pool. The nurse also learns of an incident when the toddler was pushed into a pool. Which recommendation will the nurse make to help the toddler overcome this phobia?
1. Make sure the toddler has a safe flotation device.
 2. Talk calmly as the toddler is taken slowly into the pool.
 3. Plan recreation activities that do not involve water.
 4. Allow the toddler to decide his own approach to the pool.
- _____ 9. A parent of an adolescent tells the nurse, "He had some bad habits as a child, but now he is in trouble with the law for destruction of property, stealing, and hurting animals. I think his ODD is getting worse." Which comment by the nurse is accurate?
1. "Your son has developed conduct disorder."
 2. "Increasing his ADHD medication may help."
 3. "Right now he needs your feedback and support."
 4. "There are lawyers that specifically help troubled teens."
- _____ 10. The nurse is counseling a parent of a child diagnosed with ADHD. The parent states, "He is now also diagnosed with oppositional defiant disorder (ODD). I don't know what to do." Which information does the nurse provide for the parent?
1. The fact that 40% to 60% of children with ADHD also have ODD.
 2. The importance of not showing emotional reactions to the behaviors.
 3. How to remain consistent with consequences related to ODD behaviors.
 4. The need and availability of parent training for behavior management.
- _____ 11. The nurse in a pediatric emergency department is providing care for a school-age child with first- and second-degree burns to the hands and arms. The parent states, "She is so fascinated with the color and movement of the flames; she just got too close." For which reason does the nurse recommend psychotherapy for this child?
1. The child is exhibiting an inability to recognize danger.
 2. The child does not obey instructions to stay away from matches.
 3. The child may have excessive interest in or attraction to fire.
 4. The child is likely to repeat the behavior and cause worse injuries.
- _____ 12. The nurse is discussing a child's diagnosis of autism (ASD) with the child's parents. The parents tell the nurse the child is completely resistant to any type of stimulation. The nurse suspects sensory processing disorder and will recommend which intervention?
1. Make a rocking horse or trampoline available to the child.
 2. Place colored lights and automated toys in the child's room.
 3. Set specific times of day when the child is held and cuddled.
 4. Child-appropriate music is played throughout the day.
- _____ 13. The nurse is providing care for a female adolescent at an in-patient facility for persons with eating disorders. The adolescent's current weight is less than 85% of ideal body weight. The adolescent appears to be unexpectedly agreeable with the interventions being implemented for weight gain. Which is an important intervention for the nurse to perform?
1. Eat with the client in order to demonstrate adequate intake.
 2. Closely monitor the adolescent's bathroom behavior.

- 3. Allow the adolescent to select the flavor of nasogastric (NG) tube feedings.
 - 4. Provide information about the effects of malnutrition on the body.
- _____ 14. A middle-school teacher notifies the school nurse of a student who sleeps in class, smells of alcohol, and exhibits behavior impairment. The student tells the nurse, "I drink too much and want to quit, but I keep failing." Which recommendation does the nurse make to the student?
- 1. "You may be the perfect candidate for attending AA meetings."
 - 2. "Many young people benefit from individual and group therapy."
 - 3. "Maybe you need a few days at home to see if you can quit on your own."
 - 4. "I am suggesting you and your parents see a doctor who can help."
- _____ 15. The nurse works in a facility where treatment of adolescents with addiction issues is the focus. A 14-year-old patient is being admitted for treatment of cocaine dependence. Which information does the nurse provide relative to the patient's withdrawal?
- 1. Acute symptoms of withdrawal last for 2 to 3 weeks.
 - 2. The patient will be sedated throughout most of the process.
 - 3. After acute withdrawal some symptoms may become chronic.
 - 4. Close monitoring is important due to life-threatening symptoms.
- _____ 16. The nurse is presenting a program to the parents of school-age children about prescription drug abuse among adolescents. Which information does the nurse provide to parents about preventing their children from abusing prescription drugs?
- 1. Most drugs are obtained cost free from friends or family members.
 - 2. The abused drugs are not commonly found in the normal household.
 - 3. Adolescents are at great risk for life-threatening effects from these drugs.
 - 4. Withdrawal from this type of drug can be managed by the adolescent's family.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 17. The nurse is providing care for a preschool child who is 4 years of age. The child is being treated for horrific abuse that occurred in the home since the child was an infant. Which comments by the child indicate to the nurse a possible dissociative disorder? *Select all that apply.*
- 1. "Someday I will live with grandma."
 - 2. "If it hurts, I will just go away."
 - 3. "I have a friend who always stays with me."
 - 4. "I want a really big lunch and dinner."
 - 5. "I can be invisible if they get mad at me."
- _____ 18. Parents of an adolescent female are concerned about the adolescent's recent, rapid weight loss. Nursing assessment reveals the adolescent to be below the ideal weight for her height and age. Which questions will the nurse ask to help identify an eating disorder? *Select all that apply.*
- 1. "How much weight have you lost in the past 3 months?"
 - 2. "What words would you use to describe your body right now?"
 - 3. "Do you have a sports activity causing you to exercise excessively?"
 - 4. "Can you tell me some of your daily thoughts about food?"
 - 5. "Would you consider yourself to be a good student at school?"

- _____ 19. The nurse at a pediatric clinic notices a female high school student has had extensive dental work and is currently exhibiting additional dental caries. The nurse also identifies the bilateral existence of Russell's sign. Based on these findings, for which comorbid manifestation will the nurse assess the student? *Select all that apply.*
1. Frequent absenteeism from school
 2. Issues with overspending
 3. Thoughts of suicide
 4. Presence of cutting activity
 5. Casual sexual encounters
- _____ 20. A teacher in an elementary school voices concerns to the school nurse about a student in her second-grade class. The student has recently become withdrawn from adults but constantly tries to please the teacher. Today the teacher saw bruises around his neck. Which plan does the school nurse develop and implement? *Select all that apply.*
1. Talk to child alone in the school clinic about any pain or concerns.
 2. Inspect the back, chest, and legs in the presence of the principal.
 3. Report possible child abuse with assessment findings to proper authorities.
 4. Call the parents and report that authorities have been notified of abuse.
 5. Develop a trusting rapport with the child.

Chapter 14 Mental Health Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 3

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 5. Explain two mood disorders diagnosed in children and their recommended treatments.

Page: 303

Heading: Mood Disorders > Depression

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Mood

Difficulty: Moderate

	Feedback
1	This is incorrect. A manifestation of depression in an infant is a sad expression or lack of expression. Also, depressed infants will look away when spoken to. The question helps identify a characteristic of depression; however, another question is the most important.
2	This is incorrect. A depressed infant may lose interest in play; the nurse can ascertain if the infant is receptive to playing and if it changes the infant's mood. However, another question is the most important.
3	This is correct. The most important question for the nurse to ask is related to the moods of the mother. There is an increased risk of depression in infants of depressed mothers.
4	This is incorrect. Depression in infants can sometime be exhibited as problems with eating or sleeping, especially if the behavior is new or intermittent. This question helps the nurse identify a symptom of depression, but it is not the most important question for the nurse to ask.

PTS: 1

CON: Mood

2. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 5. Explain two mood disorders diagnosed in children and their recommended treatments.

Page: 306

Heading: Mood Disorders > Bipolar Disorder

Integrated Process: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Mood

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse expects the student with bipolar disorder type 1 to exhibit multiple manifestations of mania, even when taking lithium.
2	This is incorrect. When a student is taking lithium for bipolar disorder, a common side effect is polyuria, along with polydipsia, weight gain, and gastrointestinal upset. This observation does not indicate a need for a laboratory test.
3	This is incorrect. During a manic phase of bipolar disorder type 1, the nurse is aware that the student may exhibit boundary violations. The nurse will address the behavior but will not necessarily recommend a lab test.
4	This is correct. The greatest concern for a patient taking lithium is the possibility of toxicity. The student appears to have signs of a respiratory infection but may actually be exhibiting signs of lithium toxicity. Signs of toxicity include runny nose, coughing, chest congestion, and fever. The nurse will seek a prescription for a laboratory test.

PTS: 1 CON: Mood

3. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 6. Explain five anxiety disorders diagnosed in children and their recommended treatments.

Page: 307

Heading: Anxiety Disorders > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analyze [Analyzing]

Concept: Stress

Difficulty: Difficult

	Feedback
1	This is incorrect. At some point, the nurse will attempt to calm the student; however, the student needs to be removed from the scene first.
2	This is incorrect. The student may or may not be able to express the cause of anxiety. This information is not necessary at this time.
3	This is incorrect. The student's parents may or may not be present. The nurse's action at this time involves getting the student to a quiet environment.
4	This is correct. The nurse's best intervention at this time is to move the student to a quiet environment to reduce stimuli and to remain with the student until the anxiety passes and the student is calm.

PTS: 1 CON: Stress

4. ANS: 1

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 6. Explain five anxiety disorders diagnosed in children and their recommended treatments.

Page: 307

Heading: Anxiety Disorders > Obsessive-Compulsive Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Stress

Difficulty: Difficult

	Feedback
1	This is correct. Obsessive-compulsive disorder (OCD) is characterized by severe obsessions (unwanted, reoccurring thoughts) and/or compulsions (repetitive behaviors) that interfere with quality of life. Obsessions create anxiety, and compulsions are performed to reduce anxiety. The child is allowed to complete the compulsive behavior; interrupting the behavior will increase anxiety.
2	This is incorrect. Assigning the child tasks that involves putting the hands in water will not fulfill the compulsive ritual the child uses to reduce anxiety. The nurse will not make this recommendation.
3	This is incorrect. Interrupting the handwashing and moving the child away from the sink will stop the behavior the child uses to reduce anxiety, but it will increase the level of anxiety. The nurse will not make this recommendation.
4	This is incorrect. The child may not be able to discuss the behavior or the cause of the behavior. The nurse needs to recommend professional assistance for the child.

PTS: 1 CON: Stress

5. ANS: 2

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 7. Identify three disruptive disorders diagnosed in children and their recommended treatments.

Page: 308

Heading: Disruptive Disorders in Children > Attention Deficit Hyperactivity Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Safety

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse will want to assess the student for possible side effects to the medication, especially with a parenteral request for an increase. However, other information is the most important to acquire.
2	This is correct. The nurse needs to ascertain how often the student is receiving the medication, which should be every day. In addition, ADHD medication has street value and may be sold by the parent instead of given to the student.

3	This is incorrect. The nurse will want to assess the student for possible side effects of the medication related to appetite and weight; however, there is other information that is the most important to acquire.
4	This is incorrect. The parent's description of the student's behavior is taken into consideration. The student's perception of how the medication is working is also important. However, other information is the most important to acquire.

PTS: 1 CON: Safety

6. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 6. Explain five anxiety disorders diagnosed in children and their recommended treatments.

Page: 308

Heading: Anxiety Disorders > Post-Traumatic Stress Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Stress

Difficulty Level: Difficult

	Feedback
1	This is incorrect. Research shows that administering a dose of propranolol (Inderal) right after a trauma decreases the risk for PTSD. At the time of the event's anniversary, the time has passed for this intervention to be effective.
2	This is incorrect. The student is exhibiting signs of PTSD and may be having a flashback. The nurse should not attempt to interact with a patient who is experiencing a flashback. If the nurse touches or talks to the patient, the nurse can become a part of the flashback and could be in physical danger.
3	This is incorrect. It is likely the student is under the care of a psychotherapist for PTSD caused by a violent event. Calling for the psychotherapist to come and assist the student does not address the student's immediate needs.
4	This is correct. If the possibility of a flashback passes or resolves, the nurse needs to offer the student verbal support and ask the student to talk about feelings.

PTS: 1 CON: Stress

7. ANS: 3

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 6. Explain five anxiety disorders diagnosed in children and their recommended treatments.

Page: 308

Heading: Anxiety Disorders > Panic Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Stress

Difficulty: Moderate

	Feedback
1	This is incorrect. Taking the child to the clinic and informing the parents about the event is not an intervention that will assist the child with current feelings of panic.
2	This is incorrect. When a panic attack occurs, the person affected does not need to be alone. The nurse should stay with the child and offer reassurance that the nurse will not leave.
3	This is correct. The nurse's initial intervention is to assist the patient to an environment with minimal stimulation, such as a hallway. Walking with the patient will provide a physical outlet for panic.
4	This is incorrect. When experiencing a panic attack, the person affected is frequently unable to describe feeling or identify the cause. The nurse needs to stay with the child, offer assurance, and remain in a quiet environment with the child.

PTS: 1 CON: Stress

8. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 6. Explain five anxiety disorders diagnosed in children and their recommended treatments.

Page: 308

Heading: Anxiety Disorders > Phobia

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Stress

Difficulty: Difficult

	Feedback
1	This is incorrect. Making sure the toddler has a safe flotation device is appropriate for water safety; however, it will not help the toddler overcome a phobia about water.
2	This is incorrect. Talking calmly while taking the toddler into the pool may be an attempt at exposure therapy; however, this action may compound and intensify the anxiety related to the phobia.
3	This is incorrect. The toddler is already exhibiting avoidance behavior by screaming and throwing a tantrum. The complete avoidance of recreational water may reinforce the toddler's phobia.
4	This is correct. The best advice by the nurse is to allow the toddler to decide his own approach to the pool. Initial behavior may involve playing in the kiddie pool, placing his feet in the shallow end of the pool, or sitting on the pool steps. The parents should encourage any positive behavior and protect the toddler from additional negative experiences.

PTS: 1 CON: Stress

9. ANS: 1

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 7. Identify three disruptive disorders diagnosed in children and their recommended treatments.

Page: 309

Heading: Disruptive Disorders in Children > Conduct Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Self

Difficulty: Difficult

	Feedback
1	This is correct. Children with conduct disorder (CD) generally also present with a history of developmental delays, ADHD, and ODD. Adolescents are often hostile, sarcastic, defensive, and provocative toward others. This comment by the nurse is most accurate.
2	This is incorrect. Increasing the adolescent's ADHD medication is not likely to be effective in the treatment of CD. Medications such as antipsychotics (e.g., haloperidol and low-dose risperidone), mood stabilizers (e.g., lithium carbonate), and Depakote are more likely to be prescribed.
3	This is incorrect. The nurse will recommend positive, specific feedback for desirable behavior, and allow for natural consequences of the adolescent's actions. CD is characterized by <i>serious</i> violations of social norms, including aggressive behaviors, destruction of property, and cruelty to animals, which cannot happen without consequences.
4	This is incorrect. There may be lawyers that specifically help troubled teens. However, the needs of an adolescent with CD go beyond assistance from a lawyer.

PTS: 1 CON: Self

10. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 7. Identify three disruptive disorders diagnosed in children and their recommended treatments.

Page: 309

Heading: Disruptive Disorders in Children > Oppositional Defiant Disorder

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
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1	This is incorrect. Providing statistical information to the parent about the incidence of ADHD and ODD is not going to specifically address the parent's concern about behavioral management.
2	This is incorrect. Learning how to not show emotional reaction to ODD behavior is important but is only one point of learning presented in a parent training program.
3	This is incorrect. Remaining consistent about consequences related to ODD behavior is important but is only one point of learning presented in a parent training program.
4	This is correct. The nurse needs to provide information to the parent about how to learn how to manage ODD behavior. The parent needs to know about and how to access a parent training program.

PTS: 1 CON: Family Dynamics

11. ANS: 3

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 8. Discuss impulse-control disorder and its recommended treatments.

Page: 310

Heading: Impulse-Control Disorders > Pyromania

Integrated Process: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Addiction and Behaviors

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse does not recommend psychotherapy, because the child is exhibiting an inability to recognize danger. There is more concern about the fascination with fire.
2	This is incorrect. There is no information in the scenario that indicates that the child was warned to stay away from matches.
3	This is correct. The nurse may suspect the child is exhibiting manifestations of pyromania because of fascination and interest in a fire that led to burns. The nurse recognizes the child may benefit from psychotherapy or psychoanalysis.
4	This is incorrect. The nurse would discuss safety and obedience if the concern was of the child repeating the behavior. The nurse is most likely concerned about the behavior escalating.

PTS: 1 CON: Addiction and Behaviors

12. ANS: 1

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 10. Describe autism spectrum disorders.

Page: 312

Heading: Autism Spectrum Disorder > Sensory Processing Disorder > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Cognition

Difficulty: Moderate

	Feedback
1	This is correct. Children with ASD often have a sensory processing disorder, which makes them highly sensitive to sensory stimuli. The nurse recommends short periods of exposure to increasing stimuli to build tolerance. The child is most receptive to items that cause motion, such as a rocking horse or trampoline.
2	This is incorrect. These children are highly sensitive to sensory stimuli. Lights seem extra bright, noises seem extra loud, and even mild tactile stimulation seems like a big push or shove.
3	This is incorrect. These children are highly sensitive to sensory stimuli. Lights seem extra bright, noises seem extra loud, and even mild tactile stimulation seems like a big push or shove. Holding the child and cuddling at specific times may not be effective. The stimulus is introduced slowly in order to build tolerance.
4	This is incorrect. These children are highly sensitive to sensory stimuli. Lights seem extra bright, noises seem extra loud, and even mild tactile stimulation seems like a big push or shove. Playing music throughout the day is likely to be stressful to this child. The stimulus is introduced slowly in order to build tolerance.

PTS: 1 CON: Cognition

13. ANS: 2

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 11. Identify three eating disorders diagnosed in children and their recommended treatments.

Page: 313

Heading: Eating Disorders > Anorexia Nervosa > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Addiction and Behaviors

Difficulty: Moderate

	Feedback
1	This is incorrect. Eating with the adolescent in order to demonstrate adequate intake is unlikely to be helpful. The adolescent's concern is about her body and her food intake.
2	This is correct. Because the adolescent seems to be unexpectedly agreeable with the interventions implemented for weight gain, the nurse needs to carefully monitor the adolescent's bathroom behavior. Patients with anorexia nervosa are likely to attempt to dispose of food in the toilet. Vomiting and pocketing of food is common.

3	This is incorrect. If NG tube feedings are prescribed for the patient with anorexia nervosa, the flavor is not of concern to the patient.
4	This is incorrect. When an adolescent weight is less than 85% of ideal body weight, information about the effects of malnutrition of the body is not likely to change the adolescent's perception of her body.

PTS: 1 CON: Addiction and Behaviors

14. ANS: 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 12. Describe three current treatment modalities in pediatric substance abuse treatment.

Page: 314

Heading: Substance-Related Disorders > Marijuana

Integrative Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Addiction and Behaviors

Difficulty: Moderate

	Feedback
1	This is incorrect. At some point, recommending AA meetings may be appropriate. However, the student is admitting to unsuccessful attempts at stopping.
2	This is incorrect. Individual and group therapy is often successful after an alcoholic stops drinking; however, the student needs some assistance with ceasing to drink.
3	This is incorrect. The student may be at risk for serious withdrawal manifestations, which place the student at a high level of risk. The student is not encouraged to stop drinking when physically alone.
4	This is correct. Because the student shares unsuccessful attempts to stop drinking independently, the nurse needs to recommend the assistance of a physician or medical facility that can help the student quit. Because the student is most likely a minor, the parents will need to participate and be financially responsible.

PTS: 1 CON: Addiction and Behaviors

15. ANS: 3

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 12. Describe three current treatment modalities in pediatric substance abuse treatment.

Page: 317

Heading: Substance-Related Disorders > Cocaine

Integrative Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Addiction and Behaviors

Difficulty: Difficult

	Feedback
1	This is incorrect. The symptoms related to acute withdrawal from cocaine will last for 4 to 10 days and include intense craving, agitation, depression, poor appetite, and insomnia.
2	This is incorrect. During cocaine withdrawal, the patient is not sedated throughout most of the process. The patient will receive anxiolytics, antidepressants, antipsychotics, and dopamine agonists (amantadine, bromocriptine) as ordered.
3	This is correct. After withdrawal from cocaine, some withdrawal symptoms may become chronic and may include fatigue, depression, anhedonia (lack of pleasure), mood disturbance, and cravings.
4	This is incorrect. Withdrawal from cocaine is rarely life-threatening; however, the nurse will monitor vital signs, especially heart rate and blood pressure.

PTS: 1 CON: Addiction and Behaviors

16. ANS: 1

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 12. Describe three current treatment modalities in pediatric substance abuse treatment.

Page: 317

Heading: Substance-Related Disorders > Prescription Drug Abuse

Integrated Process: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Application [Applying]

Concept: Addiction and Behaviors

Difficulty: Difficult

	Feedback
1	This is correct. It is true that most prescription drugs are available at no cost from friends or family members. When this source is no longer available, the abuser will turn to opiates from street sources.
2	This is incorrect. Commonly abused prescription drugs include opioid pain relievers, stimulants, and benzodiazepines, which are frequently in the household medicine cabinets or bedside tables.
3	This is incorrect. It is possible to overdose on prescription drugs; however, the greatest life-threatening risk is from street opiates, because the content and strength is unregulated.
4	This is incorrect. The nurse will refer children who are prescription drug abusers to inpatient or outpatient withdrawal management. Withdrawal will require medical management and monitoring.

PTS: 1 CON: Addiction and Behaviors

MULTIPLE RESPONSE

17. ANS: 2, 3, 5

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 14. Discuss child abuse assessment, diagnosis, and recommended treatments.

Page: 308

Heading: Dissociative Disorders

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Self

Difficulty: Difficult

	Feedback
1.	This is incorrect. When the child voices “Someday I will live with grandma,” it is not an example of dissociative disorder.
2.	This is correct. The nurse will recognize that voicing the ability of “going away” if something hurts is a possible indication of dissociative disorder and is probably related to magical thinking.
3.	This is correct. The nurse is aware that the child’s belief that there is a friend who always stays with the child alerts the nurse to an imaginary friend and magical thinking, which can indicate dissociative thinking in an abused child.
4.	This is incorrect. Expressing the desire for a big lunch and dinners is most likely an indication of hunger and not a dissociative disorder.
5.	This is correct. The magical thinking of a child prompts the child to mentally become invisible if “they” are mad. This is indicative of dissociative thinking.

PTS: 1

CON: Self

18. ANS: 2, 3, 4

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 11. Identify three eating disorders diagnosed in children and their recommended treatments.

Page: 313

Heading: Safe and Effective Nursing Care > Clinical Pearl > Screening for Eating Disorders

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Addiction and Behaviors

Difficulty: Difficult

	Feedback
1.	This is incorrect. It is unlikely the adolescent will be honest about her weight loss over a 3-month period. A 3-month loss of 12 pounds or more is of concern.
2.	This is correct. Asking the adolescent to describe her body will allow the nurse to assess the adolescent’s physical perception and relate to her body image.

3.	This is correct. Inquiring about the amount of exercise related to a sports activity will allow the nurse to determine whether the adolescent's current exercise regimen is excessive.
4.	This is correct. Adolescents with eating disorders, whether anorexia, bulimia, or overeating, may find themselves thinking obsessively about food: when they will eat; what they will eat; the amount they will eat; and how they will deal with the effects eating will have on their body.
5.	This is incorrect. Achievement in school does not necessarily relate to the problem of an eating disorder.

PTS: 1 CON: Addiction and Behaviors

19. ANS: 2, 3, 4, 5

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 11. Identify three eating disorders diagnosed in children and their recommended treatments.

Page: 314

Heading: Eating Disorders > Bulimia Nervosa > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Addiction and Behaviors

Difficulty: Difficult

	Feedback
1.	This is incorrect. The nurse recognizes some physiological manifestations related to bulimia nervosa. Frequent absenteeism is not a comorbid manifestation of bulimia; it is a possible sign of substance use.
2.	This is correct. The nurse recognizes some physiological manifestations related to bulimia nervosa. It is important for the nurse to assess for comorbid conditions and behaviors that exist with the condition, such as issues with overspending or shoplifting.
3.	This is correct. The nurse recognizes some physiological manifestations related to bulimia nervosa. It is important for the nurse to assess for comorbid conditions and behaviors that exist with the condition, such as suicidal thoughts.
4.	This is correct. The nurse recognizes some physiological manifestations related to bulimia nervosa. It is important for the nurse to assess for comorbid conditions and behaviors that exist with the condition, such as cutting, an example of deliberate self-harm.
5.	This is correct. The nurse recognizes some physiological manifestations related to bulimia nervosa. It is important for the nurse to assess for comorbid conditions and behaviors that exist with the condition, such as casual sexual encounters.

PTS: 1 CON: Addiction and Behaviors

20. ANS: 1, 2, 3, 5

Chapter: Chapter 14 Mental Health Disorders

Chapter Learning Objective: 14. Discuss child abuse assessment, diagnosis, and recommended treatments.

Page: 318

Heading: Childhood Abuse

Integrated Processes: Nursing Process

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Violence

Difficulty: Difficult

	Feedback
1.	This is correct. Evaluation of the child should be done in a safe, supportive environment.
2.	This is correct. Signs of physical abuse include bruises or lacerations, especially on areas that are not exposed by clothing; marks from objects such as belts, ropes, hands, or cords; bite marks from adults; and bald spots on hair. Involving a witness to the assessment is professionally and legally appropriate.
3.	This is correct. Nurses are required by law to report any suspected abuse or neglect to child protective agencies.
4.	This is incorrect. It is not the role of the nurse to inform the parents that possible abuse has been reported to the authorities. This is the role of social worker or police.
5.	This is correct. Developing a trusting relationship will provide support to the child.

PTS: 1

CON: Violence

Chapter 15 Gastrointestinal Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The pediatric nurse in a clinic is mentoring a newly hired nurse who has no experience in pediatrics. The new nurse is performing a physical assessment on an infant who is 1 month of age. Which observation will prompt the nurse to discuss assessment skills with the new nurse?
1. The new nurse states, "How can I hear bowel sounds when he cries?"
 2. The new nurse keeps the sleeping infant covered for parts of the assessment.
 3. The new nurse performs all observations before physical assessment.
 4. The new nurse informs the attending parent about the assessment actions.
- _____ 2. The NICU nurse is providing care for a neonate who presents with an overabundance of secretions that the neonate cannot manage. The nurse also identifies the neonate is anorectal and exhibits some limb deformity. Which assessment process will the nurse perform first?
1. Check whether there are deformities of the palate.
 2. Check for choking after a feeding tube is passed.
 3. Observe if cyanosis occurs during bottle feeding.
 4. Determine the extent to which a feeding tube can be passed.

- _____ 3. The nurse in a pediatric clinic is obtaining information about a 7-month-old infant with GI symptoms. The parent informs the nurse that bloating, flatulence, and foul-smelling stools occurred with the introduction of wheat cereal. Which additional information will cause the nurse to initiate emergency care?
1. Dental enamel defects of the teeth
 2. Presence of dermatitis herpetiformis
 3. Severe vomiting and diarrhea
 4. Weight loss indicated by thinness of extremities
- _____ 4. The nurse is providing care for an adolescent diagnosed with Crohn's disease. The nurse provides patient teaching regarding which manifestation of the condition?
1. Urgency to defecate
 2. Possibility of oral aphthous ulcers
 3. Episodic epigastric pain
 4. Nocturnal awakening events
- _____ 5. A parent brings an 18-month-old toddler to the pediatric emergency department for abdominal pain and stool mixed with blood and mucus. The pain is recurring three to four times an hour. Which intervention will the nurse initiate first?
1. Assess laboratory results.
 2. Initiate intravenous access.
 3. Maintain strict intake and output.
 4. Prepare for ultrasound studies.
- _____ 6. The nurse in a pediatric clinic is obtaining a health history on a child who is 9 years of age. The nurse learns the child exhibits a chronic cough, midsternal discomfort, and frequent sore throats without infection. Physical assessment indicates the child is on the 50th percentile on the height chart and on the 85th percentile for weight. Which recommendation does the nurse make?
1. Serve citrus juices instead of carbonated beverages.
 2. Begin an age-appropriate weight loss program.
 3. Initiate a practice of no eating or drinking after dinner.
 4. Encourage lying on the left side after eating a meal.
- _____ 7. The nurse in a pediatric clinic is assessing an infant 2 months of age. The mother states, "He always spits up, but it has become so much worse. Vomit goes everywhere." Which additional assessment will help the nurse identify a possible diagnosis for the infant?
1. A hard mass is palpated in the mid-epigastrium.
 2. Vomiting occurs both before and after eating.
 3. Weight is normal even with frequent vomiting.
 4. Normal skin turgor is noted over the sternum.
- _____ 8. The nurse in a pediatric clinic is working with a preschool patient and a parent about managing the child's functional constipation. Which is the most important information for the nurse to share?
1. The child is allowed to select a reward for having a bowel movement.
 2. The child is informed of the treatments for constipation and/or impaction.
 3. Parental action is required for the onset of vomiting or severe abdominal pain.
 4. The parents expect the child to sit on the toilet for a period of time each day.

- _____ 9. The nurse is informing a new mother of the concern about her newborn who is 36 hours old and has not passed any meconium. The nurse shares a suspicion of Hirschsprung's disease. The mother asks the nurse multiple questions about the condition. Which information will the nurse provide?
1. Retained meconium is a source of severe infection in newborns.
 2. A positive diagnosis indicates the newborn is terminally ill.
 3. The absence of nerves in the colon also indicates mobility issues.
 4. The condition is congenital and causes blockage of the intestines.
- _____ 10. A neonate is born with gastroschisis. Which action will the nurse perform immediately?
1. Prepare the mother for a serious birth defect in the neonate.
 2. Promote nonnutritive sucking to fulfill the neonate's needs.
 3. Protect the defect with a nonadherent sterile saline dressing.
 4. Place an orogastric tube to decompress the neonate's intestines.
- _____ 11. The nurse is admitting an infant who is 3 months of age. The parents sought medical attention when the infant began passing pale-colored stools that are nearly white. The infant had been diagnosed with biliary atresia at birth and underwent corrective surgery. For which treatment will the nurse prepare the parents?
1. A liver transplant
 2. A second corrective surgery
 3. Initiating comfort care
 4. Focusing on diet therapy
- _____ 12. The nurse is preparing teaching materials for an adolescent patient recently diagnosed with nonalcoholic fatty liver disease (NAFLD). The adolescent initially presented with right upper quadrant pain, obesity, and hepatomegaly. Which teaching will the nurse initially present?
1. Review lifestyle changes and diet modification with the adolescent.
 2. Explain the care that is provided in the event acute liver failure occurs.
 3. Discuss feelings the adolescent has related to the disease diagnosis.
 4. Begin to introduce the probability for a liver transplant later in life.
- _____ 13. The nurse is providing care for a 2-month-old infant admitted to the hospital for testing because of a persistent low-grade fever. Laboratory tests and ultrasound of the abdomen confirm the presence of gallstones. Which procedure does the nurse expect to be prescribed for this infant?
1. Immediate preparation for abdominal surgery
 2. Monitoring without surgical interventions
 3. Endoscopic removal of stones and gallbladder
 4. Placing the infant on low-fat, soy-based formula
- _____ 14. An adult female arrives in the emergency department following a spontaneous birth at home. The female indicates that no prenatal care has been received. Which assessment finding about the female causes the nurse greatest concern for the newborn?
1. A laboratory result reveals a positive hepatitis A anti-HAV-total.
 2. The mother is emaciated and has indications of drug abuse.
 3. The mother has no permanent address and denies having family.
 4. A laboratory result reveals a positive hepatitis B e antigen (HBeAg).

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. The nurse is collecting assessment information on a pediatric patient who is 13 years of age. The patient is at the clinic for recurrent gastrointestinal distress. Which questions are appropriate for the nurse to ask the patient? *Select all that apply.*
1. "Can you describe the pain you are having?"
 2. "Do you ever have cramping or bloating?"
 3. "Is there a family history of GI problems?"
 4. "Do you have a history of previous illnesses?"
 5. "Are there any changes at home or school?"
- _____ 16. A neonate is born with a 6-cm omphalocele, in which the stomach and intestines are contained within a sac of amnio, peritoneum, and Wharton's jelly outside of the abdomen. For which additional anomalies will the nurse assess? *Select all that apply.*
1. Neural tube defects
 2. Cardiac defects
 3. Rupture of the sac
 4. Herniation of the brainstem
 5. Exstrophy of the urinary bladder
- _____ 17. The nurse in a neonatal nursery is mentoring a newly hired nurse. The new nurse expresses uncertainty about the facts of physiological and pathological jaundice. Which information does the nurse provide? *Select all that apply.*
1. In newborns, a low level of jaundice is normal.
 2. Normal jaundice usually appears within a week of birth.
 3. Immaturity of the liver prevents effective metabolism of bilirubin.
 4. Greatest concern is when jaundice develops before the first 24 hours.
 5. High levels of bilirubin cause hyperactivity and insatiable hunger.
- _____ 18. The nurse in a pediatric clinic is performing a physical examination of a patient who is 8 years of age. The patient's weight is over the 95th percentile on the growth chart. The patient also expresses the presence of knee and abdominal pain. The patient's parent states, "He will outgrow it; all my boys start off like this." Which information does the nurse present to the parent? *Select all that apply.*
1. Obesity is related to the development of diabetes mellitus.
 2. Being a social outcast can cause feelings of poor self-esteem.
 3. Children with obesity are more likely to drop out of school.
 4. There is a high risk for cardiac disease and hypertension.
 5. Obesity adversely affects joint health and function.
- _____ 19. A patient who is 17 years old comes to his health-care provider for a sports physical. The nurse's visual assessment places the patient in high percentiles for both weight and height. Which additional assessments does the nurse expect to be conducted for a complete health evaluation? *Select all that apply.*
1. Body mass index
 2. Bedtime cortisol levels
 3. Glucose levels after meals
 4. Lipid profile

5. Thyroid-stimulating hormone level

- _____ 20. A grandmother brings a toddler to a pediatric clinic and states, “I am worried that my grandchild is not getting adequate care.” The nurse is able to verify the child is underweight for height and age. Which findings will cause the nurse to initiate additional assessment? *Select all that apply.*
1. The grandmother cannot provide an adequate feeding history.
 2. The toddler’s weight for height is less than the 20th percentile.
 3. The toddler repeatedly asks if the nurse will get some food.
 4. The toddler’s evaluation at birth indicates prematurity.
 5. The mother is a single parent and lives alone with the toddler.

Chapter 15 Gastrointestinal Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 1. Identify the components of an abdominal examination.

Page: 323

Heading: Assessment > Abdominal Examination

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Evaluation [Evaluating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is correct. The new nurse needs to know that auscultation of the abdomen of an infant is performed prior to percussion and palpation in order to keep the infant quiet for auscultation.
2	This is incorrect. When the infant is sleeping, the new nurse is correct in keeping the infant partially covered for parts of the assessment. Auscultation of the abdomen can be performed while the infant is partially covered.
3	This is incorrect. The new nurse is correct to perform all inspection assessment before any physical assessment.
4	This is incorrect. The new nurse is correct in informing the parent about the assessment actions.

PTS: 1

CON: Patient-Centered Care

2. ANS: 4

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 3. Identify and describe GI disease that is associated with the esophagus: Tracheoesophageal atresia.

Page: 324

Heading: Tracheoesophageal Atresia > Clinical Presentation

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Nutrition

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse is likely to recognize the symptoms of tracheoesophageal atresia, which does not commonly include palate deformities.

2	This is incorrect. The nurse will want to check if choking is initiated and/or continues after a feeding tube is passed, which can indicate the feeding tube is in the lungs. This is not the first assessment for tracheoesophageal atresia.
3	This is incorrect. With the lack of indications of tracheoesophageal atresia, feeding of the neonate may be attempted. The presence of cyanosis during feeding is an indication of the condition. Feeding is not performed first if tracheoesophageal atresia is suspected; the goal is to keep liquids out of the respiratory system.
4	This is correct. Because of the presence of anatomical and physiological manifestations, the nurse will attempt to assess for tracheoesophageal atresia. The first assessment is to determine the extent to which a feeding tube can be passed. Performing this assessment first will help prevent liquids from entering the respiratory system.

PTS: 1 CON: Nutrition

3. ANS: 3

Chapter: Chapter 15 Gastrointestinal Disorders

Objective: 4. Identify and describe GI diseases that present with abdominal pain: Celiac disease.

Page: 326

Heading: Gastrointestinal Problems Manifested by Abdominal Pain > Celiac Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Nutrition

Difficulty: Difficult

	Feedback
1	This is incorrect. The infant is exhibiting signs of celiac disease, which can manifest with dental enamel defects in the teeth. However, this would not be identifiable in a 7-month-old infant.
2	This is incorrect. Dermatitis herpetiformis is a blistering, pruritic skin rash on elbows, buttocks, or knees. The finding is more common in adults and does not occur in all cases. If the condition is present, it does not cause the nurse to initiate emergency care.
3	This is correct. If additional health information includes severe vomiting and diarrhea, the nurse will suspect dehydration; the manifestations together will cause the nurse to initiate emergency care.
4	This is incorrect. Weight loss, in which the infant may appear very skinny in the extremities but normal weight in the face, is not unexpected in an infant with celiac disease. This finding will be of concern but does not warrant initiating emergency care.

PTS: 1 CON: Nutrition

4. ANS: 2

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 4. Identify and describe GI diseases that present with abdominal pain: Irritable bowel syndrome (IBS).

Page: 330

Heading: Inflammatory Bowel Disease > Crohn's Disease > Assessment

Integrated Process: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Elimination

Difficulty: Moderate

	Feedback
1	This is incorrect. Urgency to defecate is a manifestation of ulcerative colitis and not of Crohn's disease.
2	This is correct. Oral aphthous ulcers are canker sores, which are small, shallow lesions that develop on the soft tissues in the mouth or at the base of the gums. Unlike cold sores, canker sores do not occur on the surface of the lips and are not contagious. The condition is present in patients who have Crohn's disease, and information of the condition will be provided in patient teaching.
3	This is incorrect. Episodic epigastric pain is a manifestation of peptic ulcers and not of Crohn's disease.
4	This is incorrect. Nocturnal awakening events are a manifestation of peptic ulcers and not of Crohn's disease.

PTS: 1 CON: Elimination

5. ANS: 2

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 5. Identify and describe GI disorders associated with regurgitation or vomiting: Intestinal obstruction, including volvulus and intussusception.

Page: 341

Heading: Intestinal Obstruction > Intussusception

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse strongly suspects intussusception because of the passage of stool mixed with blood and mucus. The nurse does assess laboratory results; however, this is not the nurse's first intervention.
2	This is correct. The nurse recognizes the existence of an emergency based on the toddler's presenting symptoms. The nurse will first initiate intravenous access in order to have a route established for medications and/or emergency interventions.
3	This is incorrect. The initial action by the nurse does not involve maintaining a strict intake and output.

4	This is incorrect. Once the toddler has an established IV, the nurse will initiate other interventions, such as preparations for ultrasound studies.
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PTS: 1 CON: Elimination

6. ANS: 2

Chapter: Chapter 15, Gastrointestinal Disorders

Chapter Learning Objective: 5. Identify and describe GI disorders associated with regurgitation or vomiting: Gastroesophageal reflux.

Page: 337

Heading: Gastrointestinal Problems Manifested by Vomiting > Gastroesophageal Reflux

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Nutrition

Difficulty: Moderate

	Feedback
1	This is incorrect. The child is exhibiting the symptoms of gastroesophageal reflux (GERD). The nurse will recommend avoidance of fatty foods, acidic foods (citrus juices, carbonated beverages, tomato products), and caffeine.
2	This is correct. When a child is on the 50th percentile in height and the 85th percentile for weight, the child is overweight. The nurse needs to recommend an age-appropriate weight loss program.
3	This is incorrect. The child with GERD needs to avoid food and drinks for 2 hours before bedtime.
4	This is incorrect. Lying on the left side after eating is likely to exacerbate the symptoms of GERD; the patient needs to remain upright after eating.

PTS: 1 CON: Nutrition

7. ANS: 1

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 5. Identify and describe GI disorders associated with regurgitation or vomiting: Pyloric stenosis.

Page: 339

Heading: Gastrointestinal Problems Manifested by Vomiting > Pyloric Stenosis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is correct. Vomiting after eating that grows worse and evolves into projectile vomiting are signs of pyloric stenosis. If the nurse palpates the infant's mid-epigastrium and finds a pyloric mass, it is likely indicative of pyloric stenosis. This finding is called the olive sign.

2	This is incorrect. With pyloric stenosis, vomiting always occurs after eating.
3	This is incorrect. When an infant has pyloric stenosis, there is poor weight gain.
4	This is incorrect. A serious manifestation of pyloric stenosis is the development of dehydration and a decrease in serum chloride.

PTS: 1 CON: Growth and Development

8. ANS: 3

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 6. Identify and describe GI disorders associated with constipation: Functional constipation.

Page: 345

Heading: Gastrointestinal Problems Manifested by Constipation > Functional Constipation

Integrated Processes: Nursing Process

Client Need: Physiological Adaptation: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. Rewards are great motivators for young children; however, the parent sets the award. Reward should be a privilege such as extra TV or video game time; no food awards should be given. This is important information but not the most important.
2	This is incorrect. Informing the child of the treatments for constipation and/or impaction will instill fear in the child and may compound the problem.
3	This is correct. The nurse needs to inform the parent of what actions to take if the child starts to vomit or has severe abdominal pain. Because the symptoms are indicative of a medical emergency, the caregiver should take the child to a medical facility for immediate evaluation.
4	This is incorrect. The nurse needs to provide information applicable to behavior modification. The child's condition is likely to improve with regular times for stooling, usually after meals, and sitting on the toilet for 5 to 10 minutes. However, this is not the most important information for the nurse to provide.

PTS: 1 CON: Elimination

9. ANS: 4

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 6. Identify and describe GI disorders associated with constipation: Hirschsprung's disease.

Page: 346

Heading: Gastrointestinal Problems Manifested by Constipation > Hirschsprung's Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Elimination

Difficulty: Moderate

	Feedback
1	This is incorrect. Retained meconium is not a source of severe infections in newborns. The inability to pass the meconium is the concern.
2	This is incorrect. Hirschsprung's disease, in itself, is not considered a terminal disease.
3	This is incorrect. The physical manifestation of Hirschsprung's disease is an absence of nerves in the bottom segment of the colon, which causes an intestinal blockage.
4	This is correct. Hirschsprung's disease is a congenital condition that causes blockage of the intestine because of a lack of nerves in the bottom segment of the colon. These nerves normally allow the muscles in the wall of the bowel to contract and move digested material toward the anus to be eliminated.

PTS: 1 CON: Elimination

10. ANS: 3

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 7. Identify and describe gastrointestinal problems manifested by an anterior abdominal wall defect: Gastroschisis.

Page: 347

Heading: Gastrointestinal Problems Manifested by Anterior Abdominal Wall Defect > Omphalocele and Gastroschisis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is incorrect. The mother is likely to be aware of the defect before the neonate is born. The defect is initially picked up because of high AFP levels; the defect is then verified with ultrasound studies. Often, delivery is by cesarean.
2	This is incorrect. The neonate will be offered a pacifier to promote nonnutritive sucking. However, this intervention is not initiated immediately.
3	This is correct. Immediately after birth, the nurse will support and wrap the defect to prevent fluid loss and hypothermia. The nurse uses sterile normal saline with a nonadherent dressing. The neonate is placed under warmer to prevent heat loss through the opening/exposed contents.
4	This is incorrect. Immediately after birth, the nurse will not place an orogastric tube to decompress the intestine.

PTS: 1 CON: Patient-Centered Care

11. ANS: 1

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 8. Identify and describe GI disorders associated with the liver, pancreas, or gallbladder: Biliary atresia.

Page: 349

Heading: Gastrointestinal Problems Manifested by Liver, Pancreatic, or Gallbladder Involvement
> Biliary Atresia

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is correct. If initial surgery for biliary atresia is not successful, then a liver transplant may be indicated. The occurrence of pale, gray, or white stools is an indication the condition is still present.
2	This is incorrect. A second surgery for biliary atresia is not planned if the first surgery is not successful.
3	This is incorrect. The infant will receive active and corrective medical care. Comfort care will be initiated if the condition worsens and/or a liver is not available for transplant.
4	This is incorrect. At this point, the infant's condition is considered seriously ill; nutrition needs are likely to be met by a means other than oral intake.

PTS: 1 CON: Metabolism

12. ANS: 1

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 8. Identify and describe gastrointestinal disorders associated with the liver, pancreas, or gallbladder: Fatty liver disease.

Page: 350

Heading: Gastrointestinal Problems Manifested by Liver, Pancreatic, or Gallbladder Involvement
> Nonalcoholic Fatty Liver Disease

Integrated Process: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is correct. Initially, the nurse will present information about the dietary and lifestyle changes necessary to prevent worsening of the condition. The nurse will emphasize the danger of rapid weight loss and the benefits of regular exercise.
2	This is incorrect. At this point in time, there is no reason to explain care for a condition that does not exist and may not occur.

3	This is incorrect. It is always appropriate to discuss a patient's feelings related to a diagnosis; however, this intervention will be interwoven through all patient teachings.
4	This is incorrect. At this point in time, there is no reason to explain care for a condition that does not exist and may not occur.

PTS: 1 CON: Patient-Centered Care

13. ANS: 2

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 8. Gastrointestinal problems manifested by liver, pancreatic, or gallbladder involvement: Gall bladder disease.

Page: 353

Heading: Gastrointestinal Problems Manifested by Liver, Pancreatic, or Gallbladder Involvement
> Cholecystitis and Gallstones

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis

Concept: Inflammation

Difficulty: Difficult

	Feedback
1	This is incorrect. Gallbladder surgery, if necessary, is performed as an endoscopic procedure.
2	This is correct. Gallstones in infancy do not need removal unless symptomatic, because gallstones usually resolve spontaneously. The presence of a low-grade fever is indicative of inflammation. A symptomatic presentation would include jaundice and possibly vomiting.
3	This is incorrect. The expectation is that the gallstones resolve spontaneously; no surgical intervention is planned unless symptoms evolve or worsen.
4	This is incorrect. There is no indication that the infant's formula needs to be changed.

PTS: 1 CON: Inflammation

14. ANS: 4

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 8. Identify and describe GI disorders associated with the liver, pancreas, or gallbladder: Hepatitis.

Page: 354

Heading: Gastrointestinal Problems Manifested by Liver, Pancreatic, or Gallbladder Involvement
> Hepatitis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Difficult

	Feedback
1	This is incorrect. A positive hepatitis A anti-HAV-total, predominantly IgG, confirms previous exposure to hepatitis A, recovery, and immunity; it does not distinguish recent from past infection. Another finding causes greater concern for the newborn.
2	This is incorrect. The physical condition of the female coupled with evidence of drug abuse is of concern; however, there is one finding that will cause greater concern.
3	This is incorrect. Not having a permanent address and the denial of having family is a concern; however, another finding is of greater concern.
4	This is correct. A positive laboratory test for hepatitis B e antigen (HBeAg) causes the nurse greatest concern. The test is positive in blood only when the virus is present, and it can be passed to others. An HBeAg-positive mother indicates a 90% chance of the newborn acquiring the infection.

PTS: 1

CON: Infection

MULTIPLE RESPONSE

15. ANS: 1, 2, 5

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 2. Identify the important questions to ask when taking a history about gastrointestinal (GI) problems.

Page: 323

Heading: Assessment > History

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. At the age of 13 years, the patient is able to provide the nurse with an appropriate and accurate description of pain.
2.	This is correct. At the age of 13 years, the patient is able to understand and provide the nurse with accurate information about cramping and bloating.
3.	This is incorrect. The nurse should direct the question about family history to the parent. A complete genogram with at least three generations to show patterns of illness is helpful if possible. The patient is not likely to be a good source for this information.
4.	This is incorrect. Most patients at the age of 13 years are not good historians regarding their medical history beyond the last year or two. The nurse needs to address this question to a parent.
5.	This is correct. At the age of 13 years, the patient is the best person to consult about recent changes at home or school. The patient is likely to focus on personal concerns that are unknown to the parent.

PTS: 1 CON: Patient-Centered Care

16. ANS: 1, 2, 5

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 7. Identify and describe GI problems manifested by an anterior abdominal wall defect: Omphalocele.

Page: 347

Heading: Gastrointestinal Problems Manifested by Anterior Abdominal Wall Defect > Omphalocele and Gastroschisis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is correct. Infants born with an omphalocele will commonly exhibit abnormalities such as neural tube defects.
2.	This is correct. Infants born with an omphalocele will commonly exhibit abnormalities such as cardiac defects.
3.	This is incorrect. With an omphalocele, the danger of a sac rupture is highest when the fetus is in utero.
4.	This is incorrect. Herniation of the brain stem is not an abnormality associated with the presence of an omphalocele defect.
5.	This is correct. Exstrophy of the urinary bladder (location outside the abdominal wall but not in the omphalocele) is a common abnormality associated with an omphalocele. If the omphalocele is large, the liver, spleen, gonads, and bladder may also be contained in the sac.

PTS: 1 CON: Patient-Centered Care

17. ANS: 1, 3, 4

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 8. Identify and describe GI disorders associated with the liver, pancreas, or gallbladder: Neonatal jaundice.

Page: 348

Heading: Gastrointestinal Problems Manifested by Liver, Pancreatic, or Gallbladder Involvement > Neonatal Jaundice

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is correct. In newborns, a low level of jaundice is normal.

2.	This is incorrect. Normal neonatal jaundice typically appears between the second and fifth days of life and clears with time.
3.	This is correct. Neonatal jaundice is caused by the breakdown of red blood cells (which releases bilirubin into the blood) and the immaturity of the newborn's liver, which cannot effectively metabolize the bilirubin and prepare it for excretion into the urine.
4.	This is correct. Concern of pathological jaundice exists if jaundice develops before the first 24 hours of life.
5.	This is incorrect. A high level of bilirubin or hyperbilirubinemia may cause the infant to be sleepy and eat poorly, causing the jaundice level to rise.

PTS: 1 CON: Metabolism

18. ANS: 1, 4, 5

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 9. Identify and describe gastrointestinal disorders associated with nutrition problems: Obesity

Page: 355

Heading: Gastrointestinal Problems Manifested by Nutritional Changes > Obesity

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Nutrition

Difficulty: Difficult

	Feedback
1.	This is correct. The child who is obese is at risk for developing diabetes mellitus, which is an associated condition.
2.	This is incorrect. There is no specific information to support the statement that obese children will become a social outcast; however, obesity can negatively affect self-esteem.
3.	This is incorrect. There is no specific information to support that obesity alone is a reason why a child will drop out of school.
4.	This is correct. The obese child is at high risk for developing cardiac disease and/or hypertension, which can both cause serious and lifelong health problems.
5.	This is correct. The patient is already experiencing knee pain. In addition, the patient is at risk for slipped capital femoral epiphysis, which can effect growth and function.

PTS: 1 CON: Nutrition

19. ANS: 1, 4, 5

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Learning Objective: 9. Identify and describe gastrointestinal disorders associated with nutritional problems: Obesity.

Page: 355

Heading: Gastrointestinal Problems Manifested by Nutritional Changes > Obesity

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Nutrition
Difficulty: Difficult

	Feedback
1.	This is correct. For a 17-year-old, the 50th percentiles for body mass index are 21.2. The patient's body mass index will be obtained for a comparison.
2.	This is incorrect. Morning cortisol provides an indicator of obesity. Bedtime evaluation is performed at bedtime and is not an appropriate measurement.
3.	This is incorrect. Fasting glucose levels are included in indicators of obesity; levels after eating are affected by the nutritional intake.
4.	This is correct. A lipid profile provides information about circulating fats.
5.	This is correct. The TSH provides an indicator of thyroid function, which affects growth.

PTS: 1 CON: Nutrition

20. ANS: 1, 3, 4, 5

Chapter: Chapter 15 Gastrointestinal Disorders

Chapter Objective: 9. Identify and describe gastrointestinal disorders associated with nutritional problems: Failure to thrive.

Page: 356

Heading: Gastrointestinal Problems Manifested by Nutritional Changes > Failure to Thrive

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Nutrition

Difficulty: Difficult

	Feedback
1.	This is correct. A feeding assessment is performed when a child exhibits a low body weight. The nurse needs a feeding history including calorie intake, feeding behaviors, frequency, and intake. If the grandmother cannot supply the information, the nurse will initiate additional assessment by seeking information from the mother.
2.	This is incorrect. When a child's weight for height is less than the 10th percentile, the nurse will initiate additional assessment to rule out possible causes.
3.	This is correct. When a toddler repeatedly asks the nurse for food, the nurse recognizes a behavior indicative of hunger. The nurse will further assess for information about when and what the toddler ate last.
4.	This is correct. Prematurity may or may not be a cause of the toddler's current low weight. However, the nurse will need to further assess the prenatal, perinatal, neonatal, and postnatal health history.
5.	This is correct. The nurse will perform additional assessment to ascertain the family and psychosocial environment of the toddler. Financial, educational, and mental health can all contribute to a failure to thrive condition.

PTS: 1 CON: Nutrition

Chapter 16 Renal Disorder

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse is admitting an adolescent who is 19 years of age with a diagnosis of acute renal infection. While obtaining medical and health history information, which finding does the nurse identify as supporting the diagnosis?
1. Hospitalization for removal of tonsils at 10 years of age
 2. Prolonged use of acetaminophen for frequent headaches
 3. A family history that is positive for renal calculi formation
 4. Acknowledgment of being sexually active since 14 years of age
- _____ 2. A 9-year-old male patient arrives at the emergency department with suprapubic tenderness, nausea, vomiting, and painful urination. Which laboratory result does the nurse expect from a urinalysis?
1. White blood cells: 15,000 cells/ μ L
 2. Positive for glucose and protein
 3. Potassium: 3.5–5.0 mEq/L
 4. Hematocrit: 37%
- _____ 3. A 6-year-old patient is brought to the pediatrician's office with symptoms of feeling ill, periorbital edema, weight gain, and anorexia. The nurse suspects nephrotic syndrome. Which laboratory value confirms the nurse's suspicion?
1. Serum sodium of 138 mEq/L
 2. Serum potassium of 4.5 mEq/L
 3. A high level of protein in the urine
 4. Low serum levels of HDL and LDLs
- _____ 4. The nurse in a pediatric clinic is collecting information for the reason a parent has brought a toddler to the clinic. The parent states the toddler cries with urination and is sometimes incontinent. The nurse obtains an axillary temperature of 101.2°F (38.4°C). For which additional reason does the nurse suspect a urinary tract infection (UTI)?
1. The toddler is 3 years of age.
 2. The toddler attends a preschool.
 3. The toddler is a circumcised male.
 4. The symptoms are comparable to an adult UTI.
- _____ 5. A parent brings a 12-month-old toddler to the pediatrician because the toddler cries a lot and then stops on her own. The parent has noticed a little blood in the diaper every time it is wet. Assessment reveals a temperature of 101.3°F (38.5°C). Which condition does the nurse anticipate after a urinalysis?
1. Hypospadias
 2. Henoch-Schönlein purpura
 3. Acute kidney injury
 4. Urinary tract infection

- _____ 6. The pediatric nurse receives a medical prescription to obtain a urine sample for culture from an infant 6 months of age diagnosed with a UTI. By which method will the nurse collect the sample?
1. Applying clean catch techniques
 2. Attaching an external urine bag
 3. Catching urine in a sterile diaper
 4. Performing urinary catheterization
- _____ 7. The nurse is admitting a 6-month-old infant for testing because of a second UTI and suprapubic pain with palpation. Diagnostic tests reveal a grade II vesicoureteral reflux (VUR). Which information does the nurse provide to the family?
1. Preparation necessary for surgery
 2. Information about medication therapy
 3. Importance of genetic counseling
 4. Necessity of establishing dialysis
- _____ 8. The nurse is providing care for an adolescent patient admitted with a diagnosis of nephrolithiasis. The patient's symptoms include flank pain, hematuria, and vomiting. The nurse notices an hourly output of 20 mL/hour. Patient's medical history includes UTIs, type 1 diabetes mellitus, and one kidney at birth. Which medical prescription does the nurse expect immediately from the physician?
1. Increase IV fluids to 125 mL/hour.
 2. Cover blood glucose on a sliding scale.
 3. Establish NPO status and prepare patient for surgery.
 4. Administer IV morphine 5 mg every 2 hours for pain.
- _____ 9. The nurse is providing care for a 7-year-old child whose admitting diagnosis is poststreptococcal glomerulonephritis. The nurse expects which care to be prescribed for the child?
1. Hemodialysis
 2. Nifedipine orally
 3. Increase fluids
 4. Antibiotic therapy
- _____ 10. The nurse in a pediatric clinic is gathering information from the parent of a toddler who has anorexia, generalized edema, and joint pain following a bout with strep throat. Which question(s) will most likely give the nurse information for a specific condition?
1. "What behavior did you see to indicate joint pain?"
 2. "When and where did you first notice swelling?"
 3. "Is the child urinating, and what color is the urine?"
 4. "How were you managing the symptoms at home?"
- _____ 11. The nurse is providing care for a 9-year-old patient diagnosed with postinfectious glomerulonephritis. The nurse is aware of hypertension and a prescribed dose of nifedipine 0.5 mg/kg/dose every 4 hours. The patient weighs 63 pounds. Which dose does the nurse give every 4 hours?
1. 14 mg
 2. 18 mg
 3. 22 mg
 4. 30 mg

- _____ 12. The nurse is providing care to a school-age child admitted because of the presence of colicky abdominal pain, palpable purpura on the lower extremities, edema of the face and lips, and anorexia. The suspected diagnosis is Henoch-Schönlein purpura. Which diagnostic test result does the nurse expect to validate the diagnosis?
1. Elevated serum creatinine
 2. Positive for proteinuria
 3. Stool positive for occult blood
 4. Renal biopsy shows IgA deposition
- _____ 13. The nurse is providing teaching for the parents of a child diagnosed with hemolytic uremic syndrome (HUS) 10 months ago. Which statement by a parent indicates the teaching is understood?
1. "The diet will be low calorie, low carbohydrate, no added salt, and low potassium."
 2. "Nonsteroidal medications are used only if acetaminophen is not effective."
 3. "Careful skin inspection and care is given because of swelling and poor circulation."
 4. "We can initially treat diarrhea with over-the-counter antidiarrheal medications."
- _____ 14. A high school male adolescent arrives at the emergency department following a fall sustained while rock climbing. The physician prescribes diagnostic tests to rule out acute kidney injury (AKI). Which diagnostic finding does the nurse report immediately to the health-care provider?
1. Serum creatinine level of 0.6
 2. Hematocrit level of 38%
 3. Serum BUN of 20 mg/dL
 4. Serum potassium of 5.7 mEq/L
- _____ 15. An adolescent who is 15 years of age is brought to the pediatric clinic because of bloody urine. Which additional finding during assessment will cause the nurse to consider acute kidney injury (AKI)?
1. Bruising in the flank area
 2. Tenderness in the lower back
 3. Hesitation and pain with urination
 4. Suprapubic swelling and pain
- _____ 16. Shortly after the birth of a male neonate, the parents are informed about the diagnosis of hypospadias. The physician explains that the neonate's urethral opening is located midpenile, and surgery will occur between the ages of 6 and 12 months. Which additional explanation does the nurse provide to the parents?
1. The neonate should be circumcised immediately.
 2. The diagnosis is usually an isolated anomaly.
 3. A ventral curvature of the penis is likely.
 4. A pediatric surgeon will perform the surgery.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 17. The nurse is providing care for a 10-year-old patient admitted for chronic kidney disease (CKD). The patient is diagnosed with CKD stage 3. Which nursing actions are most important for the nurse to include on the patient's plan of care? *Select all that apply.*
1. Arrange for hemodialysis.
 2. Correct electrolyte imbalances.
 3. Monitor blood pressure.
 4. Prepare for renal replacement.
 5. Obtain accurate daily I&O and weight.
- _____ 18. An adolescent who is 16 years of age is being discharged home after treatment for kidney stones. The nurse provides the patient and parents with written instructions for reference at home. Which information will the nurse include? *Select all that apply.*
1. The patient is not on a restricted diet or fluid intake at this time.
 2. All urine is to be strained and sediment kept for analysis.
 3. The thiazide diuretic may be discontinued on discharge.
 4. The patient and parents need to understand any medication regimen.
 5. A metabolic workup is no longer necessary.
- _____ 19. The nurse is providing care for a 3-year-old toddler admitted with a diagnosis of nephrotic syndrome related to a recent upper respiratory infection. In preparation for discharge, which teaching does the nurse provide for the parents? *Select all that apply.*
1. Administration of prophylactic antibiotic medication
 2. Reason for checking feet and lower legs for edema
 3. Method to use when strictly monitoring daily weight
 4. Suggestions about maintaining a low activity level
 5. Provision of written material about diet and fluid restriction
- _____ 20. The nurse is providing teaching to the parents of a preschool-age toddler diagnosed with chronic kidney disease (CKD). Which information does the nurse cover regarding vaccinations for the toddler? *Select all that apply.*
1. Routine immunizations given to healthy children are administered.
 2. Annual pneumococcal conjugate vaccinations are encouraged.
 3. Live viral vaccinations such as varicella and MMR are appropriate.
 4. Intranasal influenza vaccine are preferred for children with CKD.
 5. Annual attenuated influenza vaccinations are recommended.

Chapter 16 Renal Disorder

Answer Section

MULTIPLE CHOICE

1. ANS: 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 1. Review, comprehend, and apply the basic pathophysiology of selected renal disorders.

Page: 364

Heading: Assessment

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Difficult

	Feedback
1	This is incorrect. Hospitalization for the removal of tonsils at the age of 10 years does not support the current diagnosis of an acute renal infection.
2	This is incorrect. Some medications are known to be nephrotoxic, such as aminoglycosides, amphotericin B, vancomycin, nonsteroidal anti-inflammatories, heavy metals, and angiotensin-converting enzyme inhibitors. However, acetaminophen is hepatotoxic.
3	This is incorrect. Sometimes a family history of renal calculi puts an individual at risk for the same disorder; however, this history does not specifically support the diagnosis of an acute renal infection.
4	This is correct. Sexual activity is a risk factor, although the mechanism remains unclear. This the only finding that supports the patient's diagnosis.

PTS: 1

CON: Infection

2. ANS: 1

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 2. Understand and recognize normal and abnormal values of common laboratory findings in selected renal disorders.

Page: 366

Heading: Box 16-2: Selected Normal Laboratory Values

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Levels: Analysis [Analyzing]

Concept: Infection

Difficulty: Difficult

	Feedback
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1	This is correct. The patient's symptoms are indicative of a urinary tract infection (UTI). An elevation in the white blood cell count supports the presence of an infection.
2	This is incorrect. There is no reason for the patient to be positive for glucose and protein in the urine. This finding is expected in kidney conditions.
3	This is incorrect. The potassium level is within normal limits. Electrolyte imbalances are expected with nausea and vomiting.
4	This is incorrect. With nausea and vomiting, dehydration is possible, and that increases the hematocrit. The value given is lower than the normal range.

PTS: 1 CON: Infection

3. ANS: 3

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 2. Understand and recognize normal and abnormal values of common laboratory findings in selected renal disorders.

Page: 366

Heading: Box 16-2: Selected Normal Laboratory Values

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. The patient with nephrotic syndrome occasionally exhibits hyponatremia; the presented value is within normal limits.
2	This is incorrect. Serum potassium is not usually affected by nephrotic syndrome; the presented value is within normal limits.
3	This is correct. Nephrotic syndrome manifests with massive proteinuria (greater than 50 mg/kg/day).
4	This is incorrect. Serum cholesterol and low-density lipoproteins are grossly increased with nephrotic syndrome; the values should not be low.

PTS: 1 CON: Elimination

4. ANS: 1

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 1. Review, comprehend, and apply the basic pathophysiology of selected renal disorders.

Page: 367

Heading: Disorders of the Renal System > Urinary Tract Infection

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is correct. The nurse is aware that UTIs are more commonly seen in children between 2 and 6 years of age.
2	This is incorrect. The fact that the toddler attends a preschool does not cause the nurse to suspect a UTI.
3	This is incorrect. Circumcised male children are less likely to get a UTI.
4	This is incorrect. Symptoms of a UTI in infants and young children do not mimic the typical urinary symptoms in adults and may be as subtle as fever alone.

PTS: 1 CON: Infection

5. ANS: 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 3. Accurately distinguish between selected renal disorders based on clinical presentation.

Page: 367

Heading: Disorders of the Urinary System > Urinary Tract Infection

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is incorrect. Hypospadias is a urethral opening on the ventral surface of the penis.
2	This is incorrect. Henoch-Schönlein purpura (HSP) is classified by a tetrad of symptoms: palpable rash, arthralgias, abdominal pain, and hematuria. The most common age for HSP is from 2 to 6 years of age.
3	This is incorrect. Acute kidney injury is manifested by a history of fluid loss, a history of taking nephrotoxic medications, oliguria/anuria, hypertension, changes in the level of consciousness, and edema.
4	This is correct. Symptoms of a UTI in infants and young children do not mimic the typical urinary symptoms in adults and may be as subtle as fever alone. Intermittent crying can be a sign of painful urination.

PTS: 1 CON: Infection

6. ANS: 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 4. Safely perform nursing interventions on selected renal disorders while in the acute hospital setting.

Page: 367

Heading: Safe and Effective Nursing Care > Clinical Pearl > Prevention of Contamination in Urine Cultures

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential
 Cognitive Level: Application [Applying]
 Concept: Elimination
 Difficulty: Moderate

	Feedback
1	This is incorrect. In pediatrics, using clean catch techniques to obtain a sterile urine sample for culture is likely to work only if the patient is toilet trained.
2	This is incorrect. A bag urine specimen is not an appropriate collection method for culture because of multiple opportunity for contamination of the specimen.
3	This is incorrect. The diaper may be sterile, but contamination of the infant's skin is still possible. The method is not supported or suggested.
4	This is correct. Urine cultures should be obtained by catheterization or suprapubic tap to decrease secondary contamination of the urine specimen.

PTS: 1 CON: Elimination

7. ANS: 2

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 1. Review, comprehend, and apply the basic pathophysiology of selected renal disorders.

Page: 369

Heading: Disorders of the Renal System > Vesicoureteral Reflux

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Infection

Difficulty: Difficult

	Feedback
1	This is incorrect. Surgical intervention is not necessary for grade II VUR, in which there is not dilation noted in the ureter, pelvis, and calyces, and there are normal calyceal fornices.
2	This is correct. The nurse will provide information about the medication prescribed for the existing UTI. The reason, route, length of therapy, and side effects will be covered.
3	This is incorrect. VUR is a congenital anatomical anomaly and is the most common congenital anomaly affecting the urinary tract in children. Siblings should be checked for the same defect; however, it is not a genetic disorder.
4	This is incorrect. Dialysis is established only in the presence of renal failure. Grade II VUR is not expected to cause damage to the kidneys that would cause renal failure.

PTS: 1 CON: Infection

8. ANS: 3

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 4. Safely perform nursing interventions on selected renal disorders while in the acute hospital setting.

Page: 371

Heading: Safe and Effective Nursing Care > Promoting Safety > Children With Renal Calculi and One Kidney

Integrated Processes: Nursing Process

Cognitive Levels: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse can expect an increase in fluids by IV and orally, if oral fluids are appropriate.
2	This is incorrect. The physician may or may not prescribe blood glucose coverage using a sliding scale. There is no specific information given about the patient's type 1 diabetes mellitus.
3	This is correct. Children with a single kidney who have symptoms consistent with calculi (colicky flank pain, hematuria, vomiting, and abdominal pain) and have signs of acute renal failure (oliguria, azotemia, hypertension, hyperkalemia) are to be treated as a surgical emergency.
4	This is incorrect. The nurse can expect a prescription for narcotic pain management; however, the medication is likely to be prescribed to be given as a pediatric dose (0.1 to 0.2 mg/kg/dose every 2 to 4 hours IV). There is not enough information in the scenario to determine the specific age or weight of the patient.

PTS: 1

CON: Elimination

9. ANS: 4

Chapter: Chapter 16 Disorders of Renal System

Chapter Learning Objective: 1. Review, comprehend, and apply the basic pathophysiology of selected renal disorders.

Page: 375

Heading: Disorders of the Renal System > Acute Postinfectious Glomerulonephritis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is incorrect. Hemodialysis is emergency care in the presence of renal failure. There is no information to support this procedure.
2	This is incorrect. Nifedipine is prescribed as part of the emergency care for hypertension related to the diagnosis. There is no information to support this action.

3	This is incorrect. When the child is admitted for poststreptococcal glomerulonephritis, the nurse expects an order for strict intake and output monitoring. Fluid restrictions are most commonly prescribed.
4	This is correct. The nurse will expect that antibiotic therapy will be prescribed to eradicate the offending organism if present. A common cause of poststreptococcal glomerulonephritis is either a missed strep diagnosis or uncompleted antibiotic therapy.

PTS: 1 CON: Infection

10. ANS: 3

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 3. Accurately distinguish between selected renal disorders based on clinical presentation.

Page: 375

Heading: Disorders of the Renal System > Acute Postinfectious Glomerulonephritis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. The question about the parent-recognized joint pain does not provide information specific to a condition.
2	This is incorrect. Asking specifically about the timeline for the existing edema does not provide information specific to a condition.
3	This is correct. The nurse may suspect a kidney disorder from the presenting symptoms and recent health history. The question about urination and the appearance of the urine will give the nurse information to validate a specific condition, which is acute postinfectious glomerulonephritis. The urine will appear tea-colored, cola-colored, or bright red.
4	This is incorrect. It is not incorrect to ask how symptoms were being managed at home; however, this question does not provide information about a specific condition.

PTS: 1 CON: Elimination

11. ANS: 1

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 4. Safely perform nursing interventions on selected renal disorders while in the acute hospital setting.

Page: 375

Heading: Disorders of the Renal System > Acute Postinfectious Glomerulonephritis > Nursing Interventions > Emergency Care

Integrated Processes: Nursing Process

Client: Pharmacological and Parenteral Therapies

Cognitive Level: Application [Applying]

Concept: Safety
Difficulty: Difficult

	Feedback
1	This is correct; $63 \text{ pounds}/2.2 \text{ kg} = 28.6 \text{ kg} \times 0.5 \text{ mg} = 14.3 \text{ mg}$ (round down).
2	This is incorrect; 18 mg is above the recommended dose.
3	This is incorrect; 22 mg is above the recommended dose.
4	This is incorrect; 30 mg is above the recommended dose.

PTS: 1 CON: Safety

12. ANS: 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 2. Understand and recognize normal and abnormal values of common laboratory findings in selected renal disorders.

Page: 377

Heading: Disorders of the Renal System > Henoch-Schönlein Purpura

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Urinary Infection

Difficulty: Difficult

	Feedback
1	This is incorrect. Elevated serum creatinine is expected in multiple kidney conditions and is not specific to Henoch-Schönlein purpura.
2	This is incorrect. Positive proteinuria is expected in multiple kidney conditions. In Henoch-Schönlein purpura the level of protein in the urine may be increased or decreased.
3	This is incorrect. With Henoch-Schönlein purpura, an expected finding is the presence of occult blood in the stool. However, this finding can occur in multiple conditions.
4	This is correct. Increased IgA from either a skin or kidney biopsy is a histological confirmation of Henoch-Schönlein purpura.

PTS: 1 CON: Urinary Infection

13. ANS: 3

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 5. Educate family and patient regarding home therapies for selected renal disorders.

Page: 378

Heading: Disorders of the Renal System > Hemolytic Uremic Syndrome > Nursing Interventions > Chronic Home Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. The patient with HUS needs a high-calorie, high-carbohydrate, no-added-salt, low-potassium diet.
2	This is incorrect. The family is instructed to administer analgesics if necessary, avoiding nonsteroidal anti-inflammatory medications.
3	This is correct. The parent should provide skin care for peritoneal or hemodialysis catheter site and for possible breakdown related to edema and decreased perfusion.
4	This is incorrect. The nurse should educate the family and patient to avoid the use of antidiarrheal medications and antibiotics with gastroenteritis-type illnesses.

PTS: 1

CON: Elimination

14. ANS: 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 2. Understand and recognize normal and abnormal values of common laboratory findings in selected renal disorders.

Page: 379

Heading: Disorders of the Renal System > Acute Kidney Injury

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. Normal creatinine level is between 0.6 and 1.2; the patient's level is within normal limits.
2	This is incorrect. Normal hematocrit in a male is 39% to 49%. At 38%, the patient's hematocrit is slightly below normal, which may be indicative of some blood loss from the injury. This laboratory result does not require the nurse to immediately contact the physician.
3	This is incorrect. Serum BUN normally is between 8 and 20 mg/dL; the patient's BUN is at the high level of normal and does not need to be reported to the physician.
4	This is correct. A normal serum potassium level is between 3.5 and 5.0 mEq/L; the patient's potassium level is elevated. The nurse will report this finding immediately to the physician as elevated potassium levels can interfere with normal cardiac functioning.

PTS: 1

CON: Elimination

15. ANS: 1

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 3. Accurately distinguish between selected renal disorders based on clinical presentation.

Page: 379

Heading: Disorders of the Renal System > Acute Kidney Injury

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is correct. AKI is the result of poor renal perfusion or injury. In children, this is caused by ischemia, toxicity, nephropathy, or sepsis. Bruising in the flank area can be an indication of physical injury, which can cause bloody urine.
2	This is incorrect. Tenderness in the lower back can result from multiple different causes. This finding alone does not support AKI.
3	This is incorrect. Hesitation and pain with urination is most frequently associated with a bladder infection. Blood may or may not appear in the urine.
4	This is incorrect. Suprapubic swelling and pain is associated with bladder issues and may be the result of infection, inability to urinate, or urinary retention.

PTS: 1

CON: Elimination

16. ANS: 2

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 3. Accurately distinguish between selected renal disorders based on clinical presentation.

Page: 383

Heading: Genitourinary Anomalies > Hypospadias

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Elimination

Difficulty: Difficult

	Feedback
1	This is incorrect. Newborn males with hypospadias should not undergo a circumcision until they have been evaluated by a pediatric urologist. Often the foreskin is used for the surgical repair of the hypospadias.
2	This is correct. Hypospadias is usually an isolated anomaly; however, the neonate will be examined for other genitourinary anomalies, including undescended testicles or an inguinal hernia.
3	This is incorrect. A ventral curve of the penis may or may not occur in the neonate diagnosed with hypospadias.
4	This is incorrect. The surgical repair for hypospadias will require the expertise of a pediatric urologist.

PTS: 1

CON: Elimination

MULTIPLE RESPONSE

17. ANS: 1, 2, 3, 5

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 4. Safely perform nursing interventions on selected renal disorders while in the acute hospital setting.

Page: 381

Heading: Disorders of the Renal System > Chronic Kidney Disease > Nursing Interventions > Acute Hospital Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Elimination

Difficulty: Difficult

	Feedback
1.	This is correct. The patient admitted to the hospital with CKD will need to have hemodialysis.
2.	This is correct. The nurse will recognize and correct life-threatening electrolyte imbalances. The nurse will also obtain an electrocardiography when electrolyte imbalances are present.
3.	This is correct. The nurse will obtain vital signs as prescribed, but emphasis is on the blood pressure because of the likelihood of hypertension.
4.	This is incorrect. The patient's CKD is a stage 3, which indicates a GFR of 40% to 59% plus moderate kidney disease. Preparation for renal replacement is not an important intervention at this time.
5.	This is correct. Accurate monitoring of intake, output, and weight daily is very important when caring for a patient with CKD. Accurate fluid replacement of insensible losses (400 mL/m ² /24 hr) is maintained. Daily weight is an important indicator of fluid retention.

PTS: 1

CON: Elimination

18. ANS: 2, 4

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 5. Educate family and patient regarding home therapies for selected renal disorders.

Page: 371

Heading: Disorders of the Renal System > Nephrolithiasis (Renal Calculi)

Integrated Processes: Nursing Process

Cognitive Level: Synthesis [Creating]

Concept: Elimination

Difficulty: Difficult

	Feeding
1.	This is incorrect. Educate family and patient on appropriate nutritional requirements based on age. Educate family and patient on appropriate nutritional restrictions related to renal calculi formation.
2.	This is correct. Educate family and patient on how and why to strain urine. Sediment should be retained for analysis.
3.	This is incorrect. Educate family and patient on home medications if ordered. Thiazide diuretics may be beneficial in the treatment of certain types of kidney stones.
4.	This is correct. The patient will need to complete the course of antimicrobial therapy if urinary tract infection is present. The patient and parents need to have a solid understanding of any medication regimen.
5.	This is incorrect. Educate family and patient on the importance of metabolic testing.

PTS: 1 CON: Elimination

19. ANS: 2, 3, 5

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 5. Educate family and patient regarding home therapies for selected renal disorders.

Page: 373

Heading: Disorders of the Renal System > Nephrotic Syndrome

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Elimination

Difficulty: Difficult

	Feedback
1.	This is incorrect. Infection is the most common complication, followed by thromboembolism. Broad-spectrum antibiotics should be initiated when an infection is suspected. Antibiotics are not given prophylactically.
2.	This is correct. The parents need to know that edema is gravity dependent and occurs gradually; therefore, the parents need to check the feet and lower legs for swelling.
3.	This is correct. Because increase in weight is a sign of fluid retention, the parents are taught weight should be taken at the same time each day, on the same scale, unclothed.
4.	This is incorrect. The nurse should teach the importance of encouraging ambulation and physical activity to prevent stasis complications.
5.	This is correct. Compliance with nutrition requirements and fluid restriction is important with a toddler with nephrotic syndrome. The nurse provides written materials to make sure that the correct dietary and fluid intake is maintained.

PTS: 1 CON: Elimination

20. ANS: 1, 2, 5

Chapter: Chapter 16 Renal Disorders

Chapter Learning Objective: 5. Educate family and patient regarding home therapies for selected renal disorders.

Page: 382

Heading: Safe and Effective Nursing Care > Medication Safety > Vaccinations for Children With Chronic Kidney Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Elimination

Difficulty: Difficult

	Feedback
1.	This is correct. Children with CKD are at risk for development of illnesses that are preventable by vaccinations. Immunizations routinely administered to healthy children are provided to children with CKD.
2.	This is correct. Annual attenuated influenza vaccinations are recommended.
3.	This is incorrect. Live viral vaccinations such as varicella and MMR are not recommended for children with CKD.
4.	This is incorrect. Intranasal influenza vaccine is avoided in children with CKD.
5.	This is correct. Annual attenuated influenza vaccinations are recommended for children with CKD.

PTS: 1

CON: Elimination

Chapter 17 Endocrine Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The school nurse is teaching a middle-school class about the hormones that regulate body functions. Which information provided by the nurse is accurate?
1. Hormones are chemicals secreted by endocrine glands.
 2. Hormones act specifically on the glands that produce them.
 3. Hormones react with negative feedback as levels decline.
 4. Hormones are made from proteins, fats, and carbohydrates.
- _____ 2. The nurse at a pediatric clinic is assessing a 12-year-old female. The patient ask the nurse, “I am scared about what’s happening to my body. How does it happen?” Which information from the nurse is most appropriate?
1. The ovaries are located on each side of the uterus.
 2. The ovaries secrete hormones that regulate the menstrual cycle.
 3. The ovaries play a role in the regulation of puberty and fertility.
 4. The body changes will indicate the patient is a woman and not a child.
- _____ 3. A 15-year-old female asks the pediatric nurse how tall she may be as an adult. The adolescent’s father is 6 feet 0 inches tall; her mother is 5 feet 4 inches tall. Which calculation will the nurse use to provide a probable answer?

1. Add the parents' heights in inches together; divide by 2; add 2.5 inches.
2. Add the parents' heights in inches together; divide by 2; subtract 2.5 inches.
3. Add the parents' heights in inches together and divide the total by 4.
4. Add the parents' heights in inches together; divide by 4; add 2.5 inches.

_____ 4. The nurse in a pediatrician's office is assessing a 9-year-old male patient who is being monitored for the possible diagnosis of hypopituitarism. Which assessment finding does the nurse recognize specifically as an indication of growth hormone deficiency?

1. High weight-to-height ratio
2. Large hands and feet for body size
3. Severe aching in knees and ankles
4. Height increase of 1.75 inches in 12 months

_____ 5. The nurse on a pediatric unit is admitting a 6-week-old infant. Symptoms include a wet daily diaper count of 10 to 12 a day, irritability, constipation, and dehydration. For which medical prescription does the nurse contact the physician?

1. Limit oral intake of water to 200 mL per shift.
2. Weigh diapers to measure 24-hour urine output.
3. Check urine-specific gravity every 8 hours.
4. Allow the mother to continue breastfeeding.

_____ 6. A 13-year-old male patient is diagnosed with hypopituitarism and is prescribed to begin growth hormone replacement therapy. Which patient teaching information will best optimize the replacement therapy outcomes for the patient?

1. Clear communication about side effects of therapy and how they are managed
2. The chemical makeup and differences between the brands of somatotropin
3. How much and how quickly the patient will see the effects of the therapy
4. Psychotherapy for the family to deal with emotional problems of the condition

_____ 7. The nurse is providing care for a child who is diagnosed with syndrome of inappropriate antidiuretic hormone (SIADH). Which laboratory result does the nurse expect with this condition?

1. Low urine-specific gravity
2. High urine and low serum osmolarity
3. High serum sodium level
4. Increase in the hematocrit level

_____ 8. The nurse is providing care for a 12-month-old patient who is experiencing poor weight gain. Physical assessment reveals an open anterior fontanel and open cranial sutures. To differentiate between a decrease in growth hormone and a congenital thyroid problem, which laboratory test does the nurse expect to be ordered?

1. Serum calcium
2. CBC
3. TSH
4. FSH

_____ 9. The nurse in a NICU nursery is providing care for a newborn diagnosed with congenital hypothyroidism. During hospitalization, which home-care concept will the nurse include in the newborn's care?

1. Mix thyroid replacement hormone medication in a bottle of milk.
 2. Increase dietary fiber with a soy-based formula to prevent constipation.
 3. Ask the breastfeeding mother to bring breastmilk to the hospital.
 4. Administer hormone replacement medication using a medicine dropper.
- _____ 10. A 12-year-old patient has a diagnosis of hyperthyroidism and is hospitalized for the manifestations of a thyroid storm. Which home-care concept will the nurse include in the care of the patient during hospitalization?
1. Provide a low-stress, low-pressure environment.
 2. Ensure medications are given on the home schedule.
 3. Limit intake of caffeine and carbonated fluids.
 4. Increase intake of foods high in calcium and vitamin K.
- _____ 11. The nurse is assessing a 4-month-old infant who has a diagnosis of hypoparathyroidism. In which manner will the nurse assess the infant for pain related to the diagnosis?
1. Tap on a facial nerve and note the response.
 2. Monitor closely for signs of seizure activity.
 3. Assess for hyperreflexia of the muscles.
 4. Carefully monitor cardiovascular status.
- _____ 12. The nurse in a pediatric emergency department is providing care for a 1-year-old patient with a history of congenital adrenal hyperplasia (CAH). The patient is exhibiting the manifestations of a febrile illness. Which medical intervention does the nurse expect to be prescribed?
1. Laboratory testing for elevated serum 17-OHP level
 2. Cultures and testing for the cause of the febrile illness
 3. A quiet, cool environment for the patient
 4. Administration of corticosteroids by injection
- _____ 13. A 16-year-old adolescent has Addison's disease. The adolescent's current medication involves corticosteroid and mineralocorticoid replacement therapy. During sports practice, the adolescent collapses and loses consciousness with sudden, penetrating pain in the lower back and legs. Which action is taken by the school nurse?
1. Administer the glucagon kept for the adolescent in the clinic.
 2. Place the adolescent in side-lying position in case vomiting occurs.
 3. Notify the parents of the incident and request permission to transport to the hospital.
 4. Give IM Solu-Cortef and call the paramedics for emergency IV infusion.
- _____ 14. The nurse is planning a teaching session for a 10-year-old patient and the patient's parents. The patient is newly diagnosed with type 1 diabetes mellitus. Which is the most important topic for the nurse to cover?
1. Methods for preventing hypoglycemia during exercise
 2. The purpose of setting up a dietary consult for the patient
 3. All procedures involved in insulin administration
 4. Instructions for blood glucose and urine ketone testing
- _____ 15. A 10-year-old patient is diagnosed with type 2 diabetes mellitus. Which medical history finding will help the nurse identify alternative interventions for managing the patient's condition?

1. The patient's ethnicity group is African American.
2. The patient's BMI is greater than 85th percentile for age and weight.
3. The patient's mother had gestational diabetes during her pregnancy.
4. The patient's extended family exhibits a high incidence of diabetes.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 16. The pediatric nurse is preparing a community education program for parents and children who have endocrine disorders. With which normal regulatory functions does the nurse begin the presentation before covering endocrine disorders? *Select all that apply.*
1. Growth and development
 2. Sexual development
 3. Energy use and storage
 4. An individual's response to stress
 5. Levels of glucose, fluid, and sodium in the blood
- _____ 17. The nurse is preparing a teaching plan for a patient and family. The patient is diagnosed with hyperpituitarism. Which teaching information will optimize therapy outcomes for the patient? *Select all that apply.*
1. Education about home administration of medications
 2. Education about the disorder and treatment options
 3. Explanations of long-term complications for noncompliance
 4. Signs of excess bone growth and other features of gigantism
 5. The impact of a tumor on or near the hypothalamus or pituitary gland
- _____ 18. The nurse on a pediatric unit is providing care for a preschool child with syndrome of inappropriate antidiuretic hormone (SIADH). The parents brought the child to the hospital to receive IV therapy. Which statements by the parents indicate to the nurse that the child is receiving appropriate care? *Select all that apply.*
1. "We were getting concerned about her loving salt."
 2. "Popsicles have become a favorite daytime snack."
 3. "We recognized the symptoms of sodium depletion."
 4. "The confusion, headache, and irritability are unusual."
 5. "She loves her new little bracelet and shows it to everyone."
- _____ 19. A 19-year-old patient has a history of hyperthyroidism that is managed with medication. The patient recently moved into an apartment and is living independently. Which behavior indicates to the nurse the patient is continuing appropriate health management? *Select all that apply.*
1. The patient called for refills of antithyroid medications and beta-blocking agents.
 2. The patient went to an urgent care facility over the weekend for a sore throat and fever.
 3. The patient reports experiencing tachycardia, restlessness, and tremors for a week.
 4. The patient's last laboratory results indicates a high level of T4.
 5. The patient stops the medication for 1 month once a year to promote hair regrowth.

- _____ 20. The nurse is providing family teaching for a child diagnosed with hypoparathyroidism. Which additional teaching will the nurse include related to alternative dietary management? *Select all that apply.*
1. Avoiding caffeine and limiting the intake of carbonated beverages
 2. Encouraging foods high in calcium and vitamin K
 3. Including dietary supplements such as magnesium and boron
 4. Giving calcium and vitamin D with acidic substances
 5. Providing green leafy vegetables as the primary source of calcium

Chapter 17 Endocrine Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 1. Identify the anatomy and physiology of the endocrine system.

Page: 387

Heading: Anatomy and Physiology

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Health Promotion

Difficulty: Moderate

	Feedback
1	This is correct. It is true that hormones are chemicals produced by the endocrine glands and circulated in the bloodstream to another part of the body.
2	This is incorrect. Hormones do not act specifically on the glands that produce them; they activate or inhibit cells in the target organs and act as messengers to those organs.
3	This is incorrect. Positive feedback occurs when hormone levels start to decline, causing the hypothalamus to secrete hormones that stimulate the pituitary gland to release hormones that then affect the target organ.
4	This is incorrect. There are two types of hormones: protein hormones from amino acids and steroid hormones from fat. Carbohydrates are not involved in the formation of hormones.

PTS: 1

CON: Health Promotion

2. ANS: 3

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 2. Identify areas of focus for performing an age-appropriate endocrine assessment.

Page: 389

Heading: Anatomy and Physiology > Glands of the Endocrine System > Sex Glands (Gonads)

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analyzing [Analysis]

Concept: Sexuality

Difficulty: Difficult

	Feedback
1	This is incorrect. The patient wants to know “how” her body is changing; providing the location of the ovaries does not answer the patient’s question.

2	This is incorrect. Telling the patient a function of the hormones secreted by the ovaries does not specifically answer the patient's question of "how."
3	This is correct. The most appropriate answer to the patient's "how" is to explain how the sex glands play a role in the regulation of puberty and fertility. The nurse will use language and examples that are within the realm of the patient's understanding.
4	This is incorrect. Telling the patient that the changes she seems afraid of will indicate she is a woman and not a child is not the appropriate answer. This information may cause more doubt and fear; the patient asked specifically "how."

PTS: 1 CON: Sexuality

3. ANS: 2

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 2. Identify areas of focus for performing an age-appropriate endocrine assessment.

Page: 390

Heading: Assessment > Physical Examination > Height and Weight

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1	This is incorrect. The height of both parents is 136 inches; divided by 2 = 68, plus 2.5 inches for a male. A son of these parents will be 5 feet 9 inches.
2	This is correct. The height of both parents is 136 inches; divided by 2 = 68 minus 2.5 inches for a female. The patient will be 5 feet 4.5 inches tall.
3	This is incorrect. This calculation is wrong and would predict a height of 34 inches.
4	This is incorrect. This calculation is wrong and would predict a height of 36.5 inches.

PTS: 1 CON: Growth and Development

4. ANS: 4

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 3. Identify clinical manifestations of various endocrine disorders.

Page: 391

Heading: Hypopituitarism/Panhypopituitarism > Growth Hormone Deficiency

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Moderate

	Feedback
1	This is incorrect. Hypopituitarism can result in a high weight-to-height ratio; however, the finding alone can be caused by other conditions as well.
2	This is incorrect. Large hands and feet when compared to the body size is most likely associated with hyperpituitarism after closure of the epiphyseal disks.
3	This is incorrect. A side effect of growth hormone replacement therapy can be joint aches primarily in the knees, ankles, and wrists.
4	This is correct. A cardinal sign of hypopituitarism is delayed growth of less than 2 inches (3 to 4 cm) per year.

PTS: 1 CON: Growth and Development

5. ANS: 1

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 3. Identify clinical manifestations of various endocrine disorders.

Page: 392

Heading: Diabetes Insipidus

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is correct. The patient is exhibiting the symptoms of diabetes insipidus (DI). Dehydration is a critical effect of DI in children. Severe dehydration can occur very quickly in infants and smaller children, so fluids must be increased as soon as possible during exacerbation. Water deprivation testing is the only time water is restricted with this diagnosis. The nurse will contact the physician about this prescription.
2	This is incorrect. A 24-hour output will help confirm the diagnosis of DI.
3	This is incorrect. It is not unusual for the urine-specific gravity of a patient suspected of DI to be checked every 8 hours.
4	This is incorrect. The method of feeding is not an issue; however, irritability in infants can only be relieved by giving water instead of formula or breast milk.

PTS: 1 CON: Metabolism

6. ANS: 1

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 9. Develop a family teaching plan that will optimize therapy outcomes for patients with endocrine disorders.

Page: 392

Heading: Hypopituitarism/Panhypopituitarism > Growth Hormone Deficiency

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is correct. Some of the side effects include headache and achiness in the joints and muscles, particularly the knees, ankles, and wrists. If side effects occur, the physician may decrease the starting dose and then increase the dose slowly until the side effects resolve. This information will best optimize the patient's outcomes.
2	This is incorrect. GH replacement therapy consists of daily subcutaneous injections of manufactured GH called somatropin. Several manufacturers have developed GH with different trade names. These derivatives of GH lack one amino acid but are otherwise identical to human GH.
3	This is incorrect. It is recommended to start children on GH therapy as early as possible to achieve the maximum growth benefit; however, the specific outcome is not predictable.
4	This is incorrect. Psychotherapy for the family to deal with emotional problems of the condition may or may not be necessary. If psychotherapy is recommended, it does not influence the physical outcomes of the hormone replacement therapy.

PTS: 1 CON: Metabolism

7. ANS: 2

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 4. Recognize diagnostic and laboratory findings of patients with endocrine disorders.

Page: 394

Heading: Syndrome of Inappropriate Antidiuretic Hormone > Diagnostic Testing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is incorrect. Because fluid is retained with the diagnosis of SIADH, the specific gravity of urine is expected to be high (less fluid to the ratio of solutes).
2	This is correct. With a diagnosis of SIADH, the nurse will expect to see high urine osmolarity but low serum osmolarity on laboratory tests. The kidneys are unable to conserve appropriate amounts of water, so the body retains water, leading to water intoxication, hyponatremia, and cellular edema.
3	This is incorrect. The expected laboratory finding with SIADH is a low serum sodium level. Sodium will follow water back into the cells.
4	This is incorrect. The expected laboratory finding with SIADH is a decreased hematocrit.

PTS: 1 CON: Metabolism

8. ANS: 3

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 4. Recognize diagnostic and laboratory findings of patients with endocrine disorders.

Page: 396

Heading: Hypothyroidism > Congenital Hypothyroidism > Assessment > Diagnostic Testing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Metabolism

Difficulty: Moderate

	Feedback
1	This is incorrect. Serum calcium will indicate parathyroid activity.
2	This is incorrect. A complete blood count (CBC) will provide information regarding blood cells, which is not affected by either condition.
3	This is correct. A laboratory test for thyroid-stimulating hormone (TSH) will indicate the functioning of the pituitary gland and the effects it has on the thyroid gland.
4	This is incorrect. Follicle-stimulating hormone (FSH) will indicate functioning of the pituitary as it affects the ovary follicles.

PTS: 1 CON: Metabolism

9. ANS: 3

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 7. Integrate home-care concepts into nursing interventions for the patient with an endocrine disorder.

Page: 397

Heading: Hypothyroidism > Congenital Hypothyroidism > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Metabolism

Difficulty: Moderate

	Feedback
1	This is incorrect. Instruct parents to administer medication via needleless syringe and not to put medication in a bottle, which may or may not be completely consumed.
2	This is incorrect. Instruct parents that formula should be milk-based and not soy-based, because soy-based formulas can break down the effects of the medication.
3	This is correct. Because the mother is breastfeeding, the nurse can use breastmilk for the administration of thyroid hormone replacement medications. The medication will be crushed and mixed with a small amount of breastmilk.

4	This is incorrect. Medications are administered to newborns and infants using a needleless syringe to assure accurate dosage.
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PTS: 1 CON: Metabolism

10. ANS: 1

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 7. Integrate home-care concepts into nursing interventions for the patient with an endocrine disorder.

Page: 398

Heading: Hyperthyroidism > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is correct. A low-stress, low-pressure environment is needed after a thyroid storm both during and after hospitalization and until the child's symptoms of hyperthyroidism are decreasing. The nurse needs to integrate this home-care concept.
2	This is incorrect. Medications for hyperthyroidism should be given as prescribed by the physician, regardless of whether the patient is in the hospital or at home.
3	This is incorrect. Limiting the intake of caffeine and carbonated beverages is appropriate for the care of the patient with hypoparathyroidism.
4	This is incorrect. Increasing the intake of foods high in calcium and vitamin K is appropriate care of the patient with hypoparathyroidism.

PTS: 1 CON: Metabolism

11. ANS: 1

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 5. Describe nursing interventions for the emergency care of patients with endocrine disorders.

Page: 400

Heading: Safe and Effective Nursing Care > Clinical Pearl > Chvostek Sign

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is correct. Assessment of hyperreflexia of the muscles can be performed by tapping on the facial nerve. If a spasm occurs in the facial muscles, then a positive Chvostek sign has occurred. This confirms that the child has muscle spasms, pain, cramping, and twitches.

2	This is incorrect. Seizure activity is possible with a diagnosis of hypoparathyroidism, and recognition and management is essential. However, a seizure does not indicate the presence of pain.
3	This is incorrect. Assessment for hyperreflexia of the muscles should be performed frequently while calcium levels are unstable. However, this assessment alone does not indicate the presence of pain.
4	This is incorrect. The diagnosis of hypoparathyroidism can cause prolonged QT interval confirmed by 12-lead electrocardiogram (EKG) related to low calcium levels. Cardiac telemetry may be indicated, and the nurse should carefully monitor cardiovascular status. However, this action does not indicate the presence of pain.

PTS: 1 CON: Metabolism

12. ANS: 4

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 5. Describe nursing interventions for the emergency care of patients with endocrine disorders.

Page: 402

Heading: Critical Component: Administering Corticosteroids and Mineralocorticoids

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Moderate

	Feedback
1	This is incorrect. Elevated serum 17-OHP at birth, along with other presenting symptoms, is a potential for CAH. If the levels continue to be elevated after birth, CAH is expected because normal growth and development will decrease 17-OHP as the infant matures. However, this testing is not expected for this patient.
2	This is incorrect. The cause of the febrile condition is not as important at this time as recognition and treatment of a possible adrenal crisis.
3	This is incorrect. The patient is not likely to need or benefit from being placed in a quiet, cool environment.
4	This is correct. Emergency administration of corticosteroids given via injection should be taught to the parents for use when the child is in a crisis, such as febrile illness, surgery, trauma, or severe stress. The nurse expects a prescription to continue the medication. The doses will need to be doubled or tripled during the period of crisis. This is referred to as “stress dosing.”

PTS: 1 CON: Metabolism

13. ANS: 4

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 6. Describe nursing interventions for the acute and chronic care of the patient with endocrine disorders.

Page: 403

Heading: Adrenal Insufficiency (Addison's Disease) > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is incorrect. Glucagon is administered to increase blood glucose for a patient with hypoglycemia.
2	This is incorrect. Placing the patient in a side-lying position in case of vomiting is not the appropriate intervention for this patient.
3	This is incorrect. In an Addisonian crisis, time is lost by calling parents for permission to have the adolescent transported for acute medical attention.
4	This is correct. The nurse will administer Solu-Cortef IM, because the patient is unconscious. Careful assessment of signs and symptoms of hypovolemic shock in the severely dehydrated patient is critical. IV fluids are needed as quickly as possible; the paramedics are notified immediately.

PTS: 1

CON: Metabolism

14. ANS: 3

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 6. Describe nursing interventions for the acute and chronic care of the patient with endocrine disorders.

Page: 404

Heading: Type 1 Diabetes Mellitus (Insulin-Dependent Diabetes) > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Levels: Synthesis [Creating]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1	This is incorrect. The patient and parents need to understand the actions that are necessary for preventing hypoglycemia during exercise. Teaching will include eating an extra complex carbohydrate and protein serving at least 30 minutes to 1 hour before engaging in exercise or sports. Another topic is most important.
2	This is incorrect. The patient and parents should have a dietary consult so that the patient can be placed on a diabetic diet or be taught to count carbohydrate exchanges and adjust insulin requirements based on dietary intake. This an important action, but one other is more important.
3	This is correct. Because the patient is insulin dependent, insulin management is crucial and procedures for insulin administration are vital; this is the most important topic for the nurse to present in patient and parent teaching.

4	This is incorrect. The second most important teaching for the nurse to present is the methods by which the patient and parents will monitor the patient's blood glucose levels and test for the presence of urine ketones.
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PTS: 1 CON: Metabolism

15. ANS: 2

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 8. Identify possible alternative therapy interventions for patients with endocrine disorders.

Page: 411

Heading: Critical Component > Risk Factors for Type 2 Diabetes Mellitus

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Metabolism

Difficulty: Moderate

	Feedback
1	This is incorrect. Certain race/ethnicity groups such as African American, Latino, Asian American, American Indians, and Pacific Islanders have a higher risk for type 2 diabetes mellitus. However, there is no alternative intervention that will diminish this risk.
2	This is correct. A BMI greater than the 85th percentile for age and weight is an indication of childhood obesity, which is a major contributor to the increasing numbers of type 2 diabetes mellitus in children. The alternative intervention the nurse will recommend is increased exercise and a decrease in sedentary lifestyle.
3	This is incorrect. Maternal history of gestational diabetes or diabetes during pregnancy puts the child at higher risk for type 2 diabetes mellitus. However, there is no alternative intervention that will diminish this risk.
4	This is incorrect. A family history of type 2 diabetes mellitus places the child at a greater risk for the condition. The nurse may or may not be able to suggest alternative interventions that will diminish this risk.

PTS: 1 CON: Metabolism

MULTIPLE RESPONSE

16. ANS: 1, 2, 3, 4, 5

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 1. Identify the anatomy and physiology of the endocrine system.

Page: 387

Heading: Anatomy and Physiology

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Health Promotion

Difficulty: Difficult

	Feedback
1.	This is correct. The endocrine system regulates growth and development.
2.	This is correct. The endocrine system regulates sexual development.
3.	This is correct. The endocrine system regulates energy use and storage.
4.	This is correct. The endocrine system regulates an individual's response to stress.
5.	This is correct. The endocrine system regulates the levels of glucose, fluid, and sodium in the blood.

PTS: 1 CON: Health Promotion

17. ANS: 1, 2, 3

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 9. Develop a family teaching plan that will optimize therapy outcomes for patients with endocrine disorders.

Page: 394

Heading: Hyperpituitarism (Growth Hormone Excess) > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is correct. Information about home medications such as somatostatin analogs, dopamine agonists, and GH receptor antagonist will optimize the outcomes for a patient with hyperpituitarism.
2.	This is correct. Education about the disorder and treatment options will broaden the patient's and family's understanding of the condition and promote better informed decisions.
3.	This is correct. The severity of long-term complications related to noncompliance is likely to optimize the outcomes for a patient with hyperpituitarism. Complications include hypertension, cardiomegaly, subsequent cardiovascular disease, diabetes mellitus, osteoarthritis, sleep apnea, and early death.
4.	This is incorrect. Signs of excess bone growth and other features of gigantism are manifestations of the condition, and information is not likely to optimize the outcomes for the patient with hyperpituitarism.
5.	This is incorrect. If the patient has a tumor on or near the hypothalamus or pituitary gland, the patient may have precocious puberty in conjunction with the other manifestations of hyperpituitarism. This information is not likely to optimize the outcomes for the patient.

PTS: 1 CON: Metabolism

18. ANS: 3, 4, 5

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 10. Use critical thinking concepts to evaluate care of the patient with an endocrine disorder.

Page: 394

Heading: Syndrome of Inappropriate Antidiuretic Hormone > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is incorrect. If the parents express concern about the patient “loving salt,” the nurse will remind the parents that a diet high in sodium and protein is encouraged.
2.	This is incorrect. When the parents share that popsicles have become a favorite daytime snack, the nurse needs to be concerned that fluid restrictions are not being observed, because the parents do not understand hidden fluid sources.
3.	This is correct. The nurse will determine the child with SIADH is getting good care when the parents recognize the signs of sodium depletion and seek IV fluid replacement.
4.	This is correct. The nurse will determine the child with SIADH is getting good care when the parents pay attention to unusual behaviors and associate them with a manifestation of the child’s condition.
5.	This is correct. The nurse will determine the child with SIADH is getting good care when the parents have the child wear a medical alert bracelet.

PTS: 1

CON: Metabolism

19. ANS: 1, 2

Chapter: Chapter 17 Endocrine Disorders

Chapter Learning Objective: 10. Use critical thinking concepts to evaluate care of the patient with an endocrine disorder.

Page: 398

Heading: Safe and Effective Nursing Care > Understanding Medication > Medications for Hyperthyroidism

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is correct. Calling for refills of antithyroid medications and beta-blocking agents indicates to the nurse that the patient is managing health care appropriately and understands medication compliance.

2.	This is correct. Going to an urgent care facility over the weekend for a sore throat and fever indicates the patient understands the manifestations of agranulocytosis, which can be fatal.
3.	This is incorrect. Beta-blocking agents do not decrease the amount of thyroid hormone they provide comfort for the patient who is experiencing tachycardia, restlessness, and tremors. Experiencing these symptoms for a week indicates a lack of medication compliance.
4.	This is incorrect. Laboratory results indicating a high level of T4 are a sign of noncompliance with antithyroid medications.
5.	This is incorrect. Stopping the medication for 1 month once a year to promote hair regrowth is indicative of medication noncompliance and lack of knowledge regarding medication side effects.

PTS: 1 CON: Metabolism

20. ANS: 3, 4

Chapter: Chapter 17 Endocrine disorders

Chapter Learning Objective: 8. Identify possible alternative therapy interventions for patients with endocrine disorders.

Page: 400

Heading: Hypoparathyroidism > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is incorrect. Encouraging dietary compliance, such as avoiding caffeine and limiting the intake of carbonated beverages, is not considered an alternative intervention for dietary management.
2.	This is incorrect. Encouraging foods high in calcium and vitamin K is not considered an alternative intervention for dietary management.
3.	This is correct. Providing dietary instruction regarding alternative dietary supplements in addition to calcium and vitamin D, such as magnesium, boron, and vitamin K, is a suggestion for an alternative intervention.
4.	This is correct. Instructing parents to give calcium and vitamin D with acidic substances such as orange juice or with salads that contain lemon juice in the dressing is an alternative intervention aimed at increasing absorption.
5.	This is incorrect. The patient will need calcium and vitamin D in order to maintain health; however, no preference for the source of calcium is given.

PTS: 1 CON: Metabolism

Chapter 18 Reproductive and Genetic Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The school nurse is preparing a teaching plan for 13-year-old female students about anatomy, puberty, and reproduction. Which information does the nurse recognize as being most important?
1. Fallopian tubes are at risk for blockage from sexually transmitted infections (STIs).
 2. Endometrium is shed during menstrual cycles.
 3. Menstruation indicates an ability to become pregnant.
 4. The body will undergo observable physical changes.
- _____ 2. The nurse in a pediatric clinic is assessing an 11-year-old patient who is experiencing vaginal itching, soreness, and painful urination. Upon physical assessment, the nurse notices two ulcers, each 1 to 1.5 cm in diameter. Which additional assessment does the nurse perform?
1. Asks the patient about any sexual contact
 2. Makes inquiries about recent viral infections
 3. Inspects for the presence of a foreign vaginal body
 4. Determines whether the patient has had pinworms
- _____ 3. The nurse in a pediatric clinic is obtaining medical information from a female patient who is 18 years of age. The patient expresses concern of a vaginal infection because of the presence of fishy-smelling, thin, whitish-gray discharge. Which information from the nurse is accurate?
1. The condition is identified as bacterial vaginosis.
 2. The self-limiting infection will resolve in a week.
 3. The existence of the condition is indicative of HIV.
 4. The infection will require treatment for all sex partners.
- _____ 4. The school nurse is conducting a class for 13-year-old male students. After covering male anatomy, a student asks the nurse if the terms “sperm” and “semen” are interchangeable. Which information does the nurse provide?
1. Semen consists of sperm and various other body fluids.
 2. Sperm and semen are considered the same substance.
 3. Semen is produced by the each of two epididymides.
 4. Sperm is not present until a male reaches late teen years.
- _____ 5. The nurse is interviewing a 13-year-old male who reports a recent inability to retract his foreskin. The patient is uncircumcised and is also experiencing bleeding from the preputial orifice and pain with urination. Which initial action does the nurse take because of the indications of pathological phimosis?
1. Ask the patient about circumcision.
 2. Obtain a prescription for steroids.
 3. Consult the parents about surgery.
 4. Seek a pediatric urological consultation.
- _____ 6. A parent brings a 15-year-old adolescent to the emergency department because of severe lower abdominal pain, with nausea and vomiting. Physical examination reveals a swollen scrotum and tenderness of one testicle. Which additional assessment finding supports the possibility of testicular torsion?
1. The symptoms have been intermittent since the patient’s birth.

2. The testis lies horizontally and there is a reactive hydrocele.
3. The unaffected testicle is lifted higher in the scrotal sac.
4. The symptoms are intermittent and resolve suddenly.

_____ 7. An adolescent female, age 17 years, independently seeks care at the public health department. The adolescent reports lower abdominal pain with walking but no vaginal discharge. The adolescent also shares being sexually active with multiple partners. After confirming a pelvic inflammatory infection (PID), which is the most important teaching by the nurse?

1. Information about the probability of infertility with PID
2. Instructions about taking and completing antibiotic therapy
3. Reasons for contacting sexual partners for necessary treatment
4. Importance of not being sexually active with multiple partners

_____ 8. The school nurse is teaching a high school class about sexually transmitted infections (STIs) and specifically covers gonorrhea. A student states, "We all practice safe sex; no vaginal intercourse ever." Which reply by the nurse directly addresses the student's comment?

1. "That is good practice but still requires a condom."
2. "The only advantage is a low risk for pregnancy."
3. "Gonorrhea is spread by multiple sexual practices."
4. "I'm surprised that adolescents will admit that."

_____ 9. The nurse is preparing to teach a class about genetics at a workshop for nurses. The nurse will remind the attending nurses that genes are responsible for determining our physical attributes and biological functions. Which other function will the nurse accredit to genes?

1. Organizing the chromosomes and the genetic code
2. Maintaining cell cytoplasm for the preservation of cell organelles
3. Ensuring the development of all the different types of body cells
4. Producing products necessary for the organism's function

_____ 10. The school nurse is answering questions about anatomy and physiology in a middle school science class. One student asks, "How does our body become either a girl or a boy?" Which answer will the nurse provide?

1. "Girls are born more frequently because the mother's sex chromosome is bigger."
2. "Fathers always give a male chromosome and the mother can give one of either sex."
3. "Depending on the sex chromosome from the father, the baby will be a boy or a girl."
4. "There is no single chromosome that makes the sex determination; it's by chance."

_____ 11. The nurse is counseling parents of an infant who have just learned their child has a cancerous tumor on a kidney. The parents keep repeating that nothing like this has happened in either of their families. Which explanation does the nurse provide to the parents?

1. Errors or changes in the process of cell division accounts for diseases such as cancers.
2. Some cells just randomly begin to divide differently and create a health risk or change.
3. Somewhere in one of the parents, a genetic defect has gone unidentified until now.

4. Most commonly defects such as cancer come equally from the genes of both parents.
- _____ 12. The nurse is presenting a workshop about reproduction to other nurses. In which manner will the nurse explain the reason why all offspring from the same parents do not look exactly alike?
1. The rapid process of meiosis can result in some loss or gain of genetic material.
 2. The meiosis in the oocyte will change as the age of the contributing female increases.
 3. Meiosis in a male creates a spermatocyte, which is extremely genetically unstable.
 4. Meiosis commonly causes some chromosomes to be normal and others abnormal.
- _____ 13. The nurse is counseling a couple who has a neonate exhibiting the manifestations of trisomy 21. This is the couple's first child, and they have no identifiable risks. Which explanation does the nurse provide to the parents?
1. Advanced maternal age will cause an alteration in an oocyte before reproduction.
 2. An exposure to a highly contagious viral infection is responsible for the defect.
 3. Errors or abnormalities in a child can occur at any time during the process of development.
 4. Fertilization is likely to have occurred with an immature spermatocyte from the male.
- _____ 14. The nurse shares genetic information with the parents of a 12-year-old female with a lack of pubertal development. The child is missing an X chromosome and is diagnosed with Turner syndrome. Which information will the nurse provide regarding secondary sexual characteristics?
1. Estrogen therapy will reverse infertility.
 2. Offspring will be positive for Turner syndrome.
 3. Height and weight will remain within normal parameters.
 4. Estrogen supplementation will prompt development.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. The nurse in a pediatric clinic is performing a scheduled check-up for a 6-year-old child diagnosed with Williams syndrome. The child will be mainstreamed during the first year of school. Which information will the nurse provide to the parent of this child? *Select all that apply.*
1. The child is most likely to have a low normal IQ score.
 2. Frequently attention deficit–hyperactivity disorder (ADHD) is present.
 3. Appearance-wise, the child should fit in with other children.
 4. Stairs and other uneven surfaces may be difficult for the child to negotiate.
 5. The child has a strong memory for auditory information.
- _____ 16. The nurse at a pediatric clinic is examining a 1-month-old infant. The nurse notices eight café au lait spots on the infant's skin and two nodelike lesions on the upper extremities. When questioned, the parent tells the nurse, "All three of my kids have those. In fact I have some myself." Which conclusion will the nurse make based on assessment findings and the parent's comments? *Select all that apply.*
1. The parent, the infant, and the infant's siblings will have the same manifestations.

2. The infant displays two of seven criteria for diagnosing neurofibromatosis type 1.
3. The parent needs to have testing to identify an autosomal dominant genetic defect.
4. The siblings of the infant are likely to be diagnosed with neurofibromatosis type 1.
5. The affected family members are at a greater lifelong risk for malignancies.

_____ 17. The pediatric nurse conducts a blood sampling for PKU, which is a standard policy in most states of the United States. The parents want to void a heel stick for the blood screening and ask why the test is so important. Which answers will the nurse provide? *Select all that apply.*

1. PKU stands for a genetic mutation of a single base pair of chromosomes.
2. PKU results in an accumulation of phenylketonuria in the kidneys.
3. PKU is a required newborn screening for metabolic disorders.
4. Phenylalanine is an essential protein present in many foods.
5. Once PKU is diagnosed, the child's diet can resume to normal.

_____ 18. The nurse is providing care for a neonate with a port-wine stain on the left side of the face, which involves the eye. The nurse recognizes which nursing interventions are likely to be initiated as the neonate grows? *Select all that apply.*

1. Obtaining genetic testing before planning another pregnancy
2. Assisting parents to locate treatment for the facial deformity
3. Stressing the importance of early and regular eye examinations
4. Teaching about the management of seizure activity, if present
5. Reinforcing the reasons for an MRI of the brain and blood vessels

_____ 19. The nurse is providing care for a neonate born to a mother with a history of alcoholism. The nurse is concerned the neonate will need special attention because of fetal alcohol spectrum disorder. Which specific concerns will the nurse relay to the assigned case worker? *Select all that apply.*

1. The probability of delay in physical growth
2. Adequate feeding opportunities and amounts
3. Expected overreactivity to the environment
4. The possibility of developmental delay
5. Susceptibility to respiratory disorders

_____ 20. The nurse in a pediatrician office has been providing care for a patient with Angelman syndrome since birth. The nurse performs a general physical assessment, reviews documentation of dysmorphic features or obvious abnormalities, and assesses for the presence or absence of developmental milestones. Which information shared by the parent indicates appropriate care of the patient?

1. The parent expresses concern about the cost of special therapies.
2. The parent and child are using basic sign language to communicate.
3. The parent tells the nurse that sleep cycles are still 2 hours long.
4. The parent responds in like to the child's happy, laughing demeanor.
5. The parent informs the nurse of the child's dyspnea during playtime.

Chapter 18 Reproductive and Genetic Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 3

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 1. Identify the anatomy of the male and female reproductive systems.

Page: 413

Heading: Female Anatomy and Physiology

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Sexuality

Difficulty: Difficult

	Feedback
1	This is incorrect. It is true that fallopian tubes are at risk for blockage from STIs; however, this is not the most important information to be shared by the nurse.
2	This is incorrect. When explaining menstruation, the nurse will include information about the shedding of the endometrium during menses; however, this is not the most important information to be shared by the nurse.
3	This is correct. The most important information for the nurse to share is about the ability to become pregnant once ovulation and menstruation begins. Older adolescents need to be aware of the risk for pregnancy and STIs.
4	This is incorrect. The nurse will discuss the observable physical changes in the body, along with the information that each individual will go through puberty at a certain pace, and there are many definitions of “normal.” This is not the most important information at this point.

PTS: 1

CON: Sexuality

2. ANS: 2

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 2. Discuss common pediatric reproductive health concerns.

Page: 416

Heading: Common Pediatric Problems of the Female Genital Tract > Vulvovaginitis > Specific Causes of Vulvovaginitis > Vulvar Ulcers

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Sexuality

Difficulty: Difficult

	Feedback
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1	This is incorrect. The patient presents with the symptoms of vulvovaginitis, but the diagnosis is narrowed because of the presence of vulvar ulcers. Vulvar ulcers are typically non-sexually transmitted. The nurse needs to perform other assessments before asking the 11-year-old patient about sexual activity.
2	This is correct. Vulvar ulcers are most typically associated with viral infections such as influenza A, Epstein–Barr virus, or cytomegalovirus. If the patient has had no recent viral infections, the nurse may test for herpes simplex virus types 1 and 2. This is the additional assessment the nurse needs to make.
3	This is incorrect. With a diagnosis of vulvovaginitis, the nurse would inspect for the presence of a vaginal foreign body. The presence of vulvar ulcers changes the focus of the assessment.
4	This is incorrect. Pinworms can be a cause of vulvovaginitis but not of vulvar ulcers.

PTS: 1 CON: Sexuality

3. ANS: 1

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 2. Discuss common pediatric reproductive health concerns.

Page: 416

Heading: Adolescent Vaginal Complaints > Bacterial Vaginosis > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is correct. Bacterial vaginosis (BV) is the most common cause of vaginal discharge in postmenarchal adolescents and women of childbearing age. The patient's description is consistent with the diagnosis. The nurse needs to clarify the difference between vaginosis and vaginitis.
2	This is incorrect. BV is not an infection and only up to one-third of the cases will resolve spontaneously.
3	This is incorrect. The presence of BV is associated with HIV acquisition, but BV is not always indicative of HIV.
4	This is incorrect. Treatment is considered only for the partners of women who have sex with other women.

PTS: 1 CON: Sexuality

4. ANS: 1

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 1. Identify the anatomy of the male and female reproductive systems.

Page: 417

Heading: Male Anatomy and Physiology

Integrated Process: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is correct. Semen is the creamy discharge secreted through the penis during ejaculation. It <i>consists of sperm and fluids secreted by the prostate gland and seminal vesicles, specific epithelial cells, and other gland secretions.</i>
2	This is incorrect. Sperm and semen are not considered the same substance. Semen contains sperm and other body secretions.
3	This is incorrect. Epididymides are the long, coiled tubes that rest on the backside of each testicle and transport and store sperm cells produced in the testes. Epididymides function to bring the sperm to maturity, because the sperm that emerge from the testes are immature and incapable of fertilization.
4	This is incorrect. After puberty, the testes produce the male hormone testosterone and the reproductive cells (sperm).

PTS: 1 CON: Sexuality

5. ANS: 4

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 2. Discuss common pediatric reproductive health concerns.

Page: 419

Heading: Common Pediatric and Adolescent Problems of the Male Genital Tract > Pathological Phimosis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is incorrect. The patient is an adolescent and asking his opinion about circumcision does not address the problem of pathological phimosis.
2	This is incorrect. The patient may or may not require a prescription for steroid therapy.
3	This is incorrect. The nurse needs to address the issue about pathological phimosis; it is not the nurse's role to discuss surgery with the patient's parents.
4	This is correct. Because the pathological phimosis is a recent development and the patient is exhibiting signs of a possible infection, the nurse will seek a pediatric urologic consultation.

PTS: 1 CON: Sexuality

6. ANS: 2

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 2. Discuss common pediatric reproductive health concerns.

Page: 420

Heading: Development of the Testes > Testicular Torsion

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Reduction of Risk Potential

Concept: Communication

Difficulty: Moderate

	Feedback
1	This is incorrect. Peak incidence is in boys 12 to 18 years old; it is a smaller group in neonatal period. Torsion is corrected regardless of the age when it is first diagnosed.
2	This is correct. Along with being tender and swollen, the testis may lie horizontally and may have a reactive hydrocele.
3	This is incorrect. With scrotal edema, the affected testis is tender, swollen, and slightly elevated. The unaffected testis is not elevated.
4	This is incorrect. Intermittent torsion can present with intermittent sharp pain and swelling, then rapid resolution; however, there is no information in the scenario that validates the patient having prior experience with the condition.

PTS: 1 CON: Communication

7. ANS: 2

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 3. Describe the signs and symptoms and interventions for common sexually transmitted infections.

Page: 422

Heading: Sexually Transmitted Infections > Pelvic Inflammatory Disease

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is incorrect. PID may cause a variety of pelvic organ infections. Repeated infections can result in infertility. This teaching is important; however, other information is most important.
2	This is correct. Because PID is a serious infection caused most often by genital gonorrhea or the effects of other infections, the nurse's most important teaching is focused on instructions about taking and completing antibiotic therapy. Failure to treat the infection correctly will result in complications.
3	This is incorrect. Because PID is caused most often by genital gonorrhea and the effect of other infections, the patient needs to inform her sexual partners of the likelihood of them having an STI. This is important, but the nurse's first focus is on the patient.

4	This is incorrect. The importance of not being active with multiple partners is good information; however, the patient needs to know that even one sexual partner can be a source of STIs. The better information is about safe sex and/or abstinence.
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PTS: 1 CON: Sexuality

8. ANS: 3

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 3. Describe the signs, symptoms, and interventions for common sexually transmitted infections.

Page: 423

Heading: Sexually Transmitted Infections > Gonorrhea

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is incorrect. Rather than encouraging sexual activity among high school students, the nurse needs to address the risks and prevention.
2	This is incorrect. The only advantage to non-vaginal sex is not about the prevention of pregnancy. The nurse needs to address the other risks related to non-vaginal sex.
3	This is correct. Infection from STIs can occur in the cervix, vagina, and fallopian tubes in women but also the eyes, throat, urethra, and anus of males and females. Gonorrhea may also spread to the blood or the joints, causing painful arthritis with red, swollen joints. Gonorrhea is spread by contact with infected sites, most often through sexual contact in adolescents.
4	This is incorrect. The nurse expressing surprise about the student's information is not therapeutic and will shut down communication between the nurse and the students about the topic.

PTS: 1 CON: Sexuality

9. ANS: 4

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 4. Explain the human genome and its function.

Page: 425

Heading: Genetics and Genetic Disorders

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Synthesis [Creating]

Concept: Cellular Regulation

Difficulty: Difficult

	Feedback
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1	This is incorrect. The cell nucleus houses the chromosomes, the highly organized structures that contain the genetic code, DNA. Genes are not involved in organizing chromosomes and the genetic code.
2	This is incorrect. The cell is composed of a central enclosed core (nucleus) and the outer area (cytoplasm), which contains fluid and other cell organelles. The genes do not have a role in maintaining the cytoplasm for any purpose.
3	This is incorrect. Humans are composed of almost a trillion cells of different types, each of which derives from a single cell. This single cell then differentiates, or develops, into highly specific cell types. The genes do not play a role in cell differentiation.
4	This is correct. Under the direction of genes, the cell cytoplasm makes products necessary for the organism's functions, such as growth, release of energy, and elimination of waste products at the cellular level.

PTS: 1 CON: Cellular Regulation

10. ANS: 3

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 4. Explain the human genome and its function.

Page: 425

Heading: Genetics and Genetic Disorders > Genetic Inheritance

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Concept: Sexuality

Difficulty: Moderate

	Feedback
1	This is incorrect. The female sex chromosome is larger, but the sex of a baby is actually determined by the male.
2	This is incorrect. The female always gives an X chromosome (XX), and the male (XY) will give either an X chromosome (XX = female) or a Y chromosome (XY = male).
3	This is correct. The male, with two different sex chromosomes, always determines the sex of the child.
4	This is incorrect. The sex of a child is not determined by chance; XX for female and XY for male offspring.

PTS: 1 CON: Sexuality

11. ANS: 1

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 5. Describe errors in reproduction, including mitosis, meiosis, and their contribution to genetic disorders.

Page: 425

Heading: Genetics and Genetic Disorders > Genetic Errors

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Cellular Regulation
Difficulty: Moderate

	Feedback
1	This is correct. It is important that the nurse inform the parents that errors or changes in the process of cell division accounts for diseases such as cancers; however, genetic errors do not contribute to inherited disorders.
2	This is incorrect. Defects can occur when the ongoing process of cell reproduction is altered, which occurs by the process of mitosis—cell division into identical sister cells, each with 46 chromosomes or 23 pairs of alleles.
3	This is incorrect. The presence of cancer in a baby is not the result of a genetic defect; it is caused by a process error during mitosis.
4	This is incorrect. Cancer is not a genetic disorder caused by defective DNA from either parent. Errors during mitosis will manifest as illnesses.

PTS: 1 CON: Cellular Regulation

12. ANS: 1

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 5. Describe errors in reproduction, including mitosis, meiosis, and their contribution to genetic disorders.

Page: 425

Heading: Genetics and Genetic Disorders > Meiosis

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Cellular Regulation

Difficulty: Difficult

	Feedback
1	This is correct. The rapid process of meiosis can result in some loss or gain of genetic material, which is the reason for genetic variance.
2	This is incorrect. Increased maternal age can result in increased gene errors, because these oocytes have been present for the lifespan of the mother; however, this is not the reason for genetic variance.
3	This is incorrect. Although meiosis in a male does produce spermatocytes, spermatocytes are not genetically unstable.
4	This is incorrect. Rarely, some of the cell's chromosomes are normal and others are abnormal, a condition called mosaicism.

PTS: 1 CON: Cellular Regulation

13. ANS: 3

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 6. Explain differences and similarities among autosomal dominant, recessive, and X-linked disorders.

Page: 426

Heading: Genetic and Genetic Disorders > Alterations in Chromosomes and Development
Integrated Processes: Teaching and Learning
Client Need: Physiological Integrity: Physiological Adaptation
Cognitive Level: Application [Applying]
Concept: Cellular Regulation
Difficulty: Moderate

	Feedback
1	This is incorrect. Advanced maternal age can cause an alteration in an oocyte before reproduction; however, the scenario states no identifiable risks were associated with the parents.
2	This is incorrect. No information in the scenario identifies an exposure to a highly contagious viral infection.
3	This is correct. Errors or abnormalities can occur at any time during the process of development, from meiosis, to mitotic division after fertilization, to cell differentiation, and organ formation and development.
4	This is incorrect. Spermatocytes are immature sperm cells that are not capable of fertilization.

PTS: 1 CON: Development

14. ANS: 4

Chapter: Chapter 18 Reproductive and Genetic Disorders
Chapter Learning Objective: 7. Describe a variety of common genetic syndromes.
Page: 428
Heading: Genetics and Genetic Disorders > Turner Syndrome
Integrated Processes: Nursing Process
Client Need: Physiological Integrity: Physiological Adaptation
Cognitive Level: Application [Applying]
Concept: Growth and Development
Difficulty: Moderate

	Feedback
1	This is incorrect. Girls with Turner syndrome are missing an entire X chromosome, because the father did not contribute any sex chromosome. The female offspring will be infertile; no treatment is available for the infertility.
2	This is incorrect. The female with Turner syndrome will not be able to produce offspring.
3	This is incorrect. Height and weight are usually within normal parameters at birth, and then deceleration of height velocity occurs.
4	This is correct. Lack of pubertal development can be treated with estrogen supplementation so that girls can develop secondary sexual characteristics; however, they will remain infertile.

PTS: 1 CON: Growth and Development

MULTIPLE RESPONSE

15. ANS: 1, 2, 4, 5

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 7. Describe a variety of common genetic syndromes.

Page: 429

Heading: Genetics and Genetic Disorders > Williams Syndrome

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is correct. IQ scores on standard tests range from severe intellectual delay to low normal.
2.	This is correct. Attention deficit–hyperactivity disorder (ADHD) is present in about 70% of the children with Williams syndrome.
3.	This is incorrect. The child with Williams syndrome will have a characteristic pattern of dysmorphic facial features, including broad forehead, bitemporal narrowing, low nasal root, and periorbital fullness, to name a few.
4.	This is correct. The child with Williams syndrome will exhibit difficulty with gross motor function and depth perception; it will be very difficult to negotiate uneven surfaces and stairs.
5.	This is correct. The child with Williams syndrome will have difficulty with handwriting, drawing, buttoning, and pattern construction, as well as poor math skills. However, there is a strong memory for auditory information, such as instructions read out loud.

PTS: 1

CON: Growth and Development

16. ANS: 2, 3, 4, 5

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 7. Describe a variety of common genetic syndromes.

Page: 431

Heading: Genetics and Genetic Disorders > Neurofibromatosis, Type 1 (Von Recklinghausen's Disease)

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Patient-Centered Care

Difficulty: Difficult

	Feedback
1.	This is incorrect. Individuals in the same family will present with unique manifestations.

2.	This is correct. The infant does present with 2 of the 7 criteria for diagnosing neurofibromatosis type 1: eight café au lait spots and two nodelike lesions on the upper extremities.
3.	This is correct. Because the parent and three older siblings to the infant are exhibiting the symptoms of the disorder, the parent needs to have testing to identify an autosomal dominant genetic defect.
4.	This is correct. If the infant's siblings exhibit the manifestations of neurofibromatosis type 1, it is safely assumed they also have the disease.
5.	This is correct. The lifelong risk for malignancy in affected individuals is increased. Malignant peripheral nerve sheath tumors represent the most common neoplasm. The family will be closely monitored for this and multiple other manifestations.

PTS: 1 CON: Patient-Centered Care

17. ANS: 1, 3, 4

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 7. Describe a variety of common genetic syndromes.

Page: 431

Heading: Genetics and Genetic Disorders > Phenylketonuria

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Metabolism

Difficulty: Difficult

	Feedback
1.	This is correct. Phenylketonuria (PKU) is caused by a single mutation at a single site or base pair mutation. PKU results in an abnormality in the production of phenylalanine hydroxylase, the enzyme that breaks down the protein phenylalanine.
2.	This is incorrect. The result is an accumulation of phenylalanine in the blood and the brain, causing brain damage with progressive intellectual disability.
3.	This is correct. All 50 states now screen for PKU as part of the standard newborn screening. The nurse needs to ensure the screen is performed when a child has been fed full-strength formula or breast milk, ideally for 48 hours.
4.	This is correct. Phenylalanine is an essential protein; it cannot be removed completely from the human diet, but caregivers need to provide a phenylalanine-free formula and low-protein diet with very close monitoring by a nutrition specialist.
5.	This is incorrect. Once brain injury occurs it is irreversible, so the restrictive diet is recommended for life. Once screened as abnormal, individuals require immediate referral to a geneticist and a metabolic nutritionist for information on protein restriction.

PTS: 1 CON: Metabolism

18. ANS: 2, 3, 4, 5

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 7. Describe a variety of common genetic syndromes.

Page: 434

Heading: Genetics and Genetic Disorders > Sturge-Webber Syndrome

Integrated Process: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
1.	This is incorrect. Sturge-Weber syndrome is a rare condition caused by a spontaneous genetic mutation in the GNAC gene that affects the skin and the brain. This mutation cannot be transmitted from parent to child, and there is no reason for genetic testing of the parents.
2.	This is correct. In some cases, laser surgery can be done for infants as young as 1 month to reduce the size (and therefore the long-term effects) of the port-wine stain lesion.
3.	This is correct. Glaucoma is also common, usually in the eye affected by the port-wine stain. The eye may sometimes become enlarged. Glaucoma can be managed with medications; early diagnosis is important.
4.	This is correct. If brain lesions are present, they can cause seizures, usually occurring before the first birthday. The seizures tend to worsen with age and may be convulsions on the opposite side of the body from the skin discoloration.
5.	This is correct. Blood vessel growths called angiomas can occur on the brain and cause seizures. These angiomas may or may not be surgically repairable, depending on their size and location. An MRI of the brain and the brain blood vessels should be performed to identify angiomas.

PTS: 1

CON: Growth and Development

19. ANS: 1, 2, 3, 4

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 8. Explain the recommended age-appropriate care and pediatric surveillance for common genetic syndromes.

Page: 438

Heading: Genetics and Genetic Disorders > Fetal Alcohol Spectrum Disorder

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Synthesis [Creating]

Concept: Growth and Development

Difficulty: Difficult

	Feedback
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1.	This is correct. Growth deficiency in the child with fetal alcohol spectrum disorder starts at infancy and lasts throughout life. Affective interventions are not likely.
2.	This is correct. Neonates with fetal alcohol spectrum disorder have difficulties with suck/swallow/breath coordination and are prone to aspiration. These manifestations may cause a mother with a history of alcoholism to be impatient with feeding the neonate. The mother may be negligent during periods of drinking.
3.	This is correct. Neonates with fetal alcohol spectrum disorder are over-reactive to environmental stimuli.
4.	This is correct. Neonates diagnosed with any of the FASD subtypes are at risk for significant cognitive impairment and physical anomalies. Impulsivity, attention issues, and frank ADHD are common.
5.	This is incorrect. Fetal alcohol spectrum syndrome primarily affects the brain. There is no specific concern over the susceptibility to respiratory disorders.

PTS: 1 CON: Growth and Development

20. ANS: 2, 4, 5

Chapter: Chapter 18 Reproductive and Genetic Disorders

Chapter Learning Objective: 8. Explain the recommended age-appropriate care and pediatric surveillance for common genetic syndromes.

Page: 438

Heading: Genetics and Genetic Disorders > Angelman Syndrome

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Family Dynamics

Difficulty: Difficult

	Feedback
1.	This is incorrect. When the parent expresses concern about the cost of special therapies, it may be an indication to the nurse that the parent is experiencing financial stress, which may impact the care of the child.
2.	This is correct. The child with Angelman syndrome will have intellectual disabilities and severe speech and language impairment. Using sign language as a means of communication is indicative of effort to meet the child's needs.
3.	This is incorrect. When the parent states that the child's sleep cycles are still 2 hours long, the parent is expressing fatigue related to the short sleep cycles of the child with Angelman syndrome. Caregiver fatigue can affect the care of the child.
4.	This is correct. Children with Angelman syndrome are often laughing and happy in demeanor; when the parent responds in like, the parent is embracing the child's behavior.
5.	This is correct. Some chromosomal disorders associated with Angelman syndrome may cause multiple malformations, such as heart defects or kidney defects. The fact that the parent is involved enough to notice a possible physical problem indicates appropriate care of the patient.

PTS: 1

CON: Family Dynamics

Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The nurse in a pediatric clinic is assessing an infant who is 6 months of age. The infant is pale with poor muscle tone. Auscultation reveals tachycardia; in addition, the infant is at the 35th percentile for both height and weight. The physician orders laboratory testing. Which laboratory value does the nurse expect?
1. Erythropoietin level of 1.9 mIU
 2. Serum iron 24 mcg/dL
 3. Normal RBC index in CBC
 4. RBCs small in size and pale
- _____ 2. A 5-month-old infant is brought to the pediatrician's office, and the nurse is collecting information from the mother. The mother reports the infant is irritable, eats poorly, and is less active. The nurse identifies tachycardia and a systolic murmur. Which comment by the mother helps the nurse identify the infant's condition?
1. "I am pleased that he sleeps well at night."
 2. "Up until recently he was a happy baby."
 3. "Formula made him gassy and constipated."
 4. "He has not been what I call really sick."
- _____ 3. The nurse is providing care to two children on a pediatric unit. One child is diagnosed with iron-deficiency anemia, and the other has sickle cell disease. Which manifestation does the nurse recognize as being different between the two children?
1. A child with iron deficiency expresses significant pain and discomfort.
 2. A child with sickle cell disease experiences varying amounts of joint pain.
 3. A child with iron-deficiency anemia experiences normal physical growth.
 4. Sickle cell disease is transmitted as a dominant trait from one parent.
- _____ 4. The nurse is preparing teaching materials for parents with children diagnosed with anemia. Which information will be marked as being specifically for the child diagnosed with sickle cell anemia?
1. Follow a balanced nutritious diet.
 2. Encourage drinking as much fluids as possible.
 3. Allow low-energy activities and rest periods.
 4. Make sure that immunizations are up to date.
- _____ 5. The nurse is counseling a couple with a 3-year-old toddler diagnosed with hemophilia after experiencing excessive bleeding after a minor injury from a fall. Which teaching is most important for the nurse to provide to the parents?
1. Stressing the need for preventing injury in the environment with supervision, helmet use, and activity restrictions
 2. Sharing the availability of resources from the National Hemophilia Foundation publications

3. Preparing them for the possibility of altered family dynamics and how illness may impact financial resources
4. Explaining the importance of allowing the toddler to grow up as normally as possible

_____ 6. A 16-year-old adolescent is diagnosed with osteosarcoma and has evidence of metastasis. The nurse calculates the absolute neutrophil count (ANC) daily. The patient asks the nurse to explain the ANC. Which information does the nurse provide?

1. The ANC is a measure of how well treatment is being tolerated by the body.
2. The ANC is total percentage of eosinophils plus the total percentage of basophils divided by the RBC.
3. The ANC provides a daily count of the number of antibodies in the circulating blood.
4. ANC measures of three types of white blood cells. A lower ANC indicates vulnerability to infection.

_____ 7. A 12-year-old patient is experiencing pain in the abdomen after receiving chemotherapy. When developing a care plan, for which interventions will the nurse plan for pain management?

1. Application of heating pads to the abdomen
2. Application of cold packs to the chest area
3. Alternative therapy such as aromatherapy
4. Medication for alleviation of abdominal pain

_____ 8. An 8-year-old child arrives at the emergency department with abdominal pain and fever. The child has a medical history of leukemia in remission. The nurse receives medical orders for the child. Which order does the nurse recognize as the priority?

1. Draw blood work for a CBC and blood culture.
2. Complete a thorough physical assessment.
3. Set up neutropenic precautions for the child.
4. Prepare the child to be transported to radiology.

_____ 9. A 19-year-old college student is being treated for non-metastatic Hodgkin's lymphoma with chemotherapy. Which recommendation by the nurse was made prior to beginning chemotherapy in order to promote a level of health?

1. Gain weight before treatment to offset weight loss.
2. Consider sperm banking because of expected sterilization.
3. Move in with parents to initiate quarantine protocols.
4. Withdraw from all college courses because of fatigue.

_____ 10. The nurse is providing care for a 4-year-old child diagnosed with non-Hodgkin's lymphoma. The nurse is aware that multiple blood samples are ordered, and the child is scheduled for a lumbar puncture with bone marrow aspiration. Which intervention will the nurse plan to prepare the patient for the testing?

1. Use simple terms and provide a needleless syringe for treatment on a stuffed animal.
2. Provide information to the child's parents and encourage them to explain procedures.
3. Withhold details from the patient until right before testing procedures are performed.

4. Expect that a 4-year-old patient is incapable of understanding or cooperating during procedures.
- _____ 11. The nurse is providing patient teaching for a school-age patient and parents. Chemotherapy is prescribed for cancer treatment. Before the first dose is administered on an outpatient basis, which teaching is most important for the nurse to provide?
1. Prompt recognition of adverse effects after the therapy.
 2. Explain how to immediately initiate protective precautions.
 3. Promote nutrition with the preparation of favorite foods.
 4. Have icy drinks available to improve oral fluid intake.
- _____ 12. The nurse in a pediatric oncology unit understands that painful tests and treatments are common for children with cancer. Which interventions does the nurse decide to use to help manage pain for a 6-year-old patient scheduled for diagnostic testing?
1. Avoid too many details that may scare the child.
 2. Use language that is age appropriate for the child.
 3. Promise the child a toy for cooperating and not crying.
 4. Explain why the test is performed without pain medication.
- _____ 13. A 9-year-old child is being treated for a brain tumor. The patient asks the nurse why there is pain in his head. Which reply by the nurse includes pathophysiology and age-appropriate communication?
1. Explain there is a blockage of blood flow to the brain.
 2. State the cancer puts pressure on the neck and causes nerve pain.
 3. Explain the growing brain tumor presses on some nerves in the head.
 4. State the medications being given are causing the headache.
- _____ 14. The parents of a 4-year-old toddler bring the child to the pediatrician because a lump is found in the toddler's waist area. Diagnostic testing verifies the toddler has a Wilms' tumor on the right kidney. The toddler is to be sent home until scheduled surgery. Which teaching is essential for the parents regarding preoperative care?
1. Promote hydration by increasing fluid intake.
 2. Maintain a side position with pillows for sleeping.
 3. Avoid palpating the tumor and pushing or lifting in the area.
 4. Provide information about postoperative care.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. During the treatment of a preschool child for anemia, laboratory tests reveal the child is also positive for lead poisoning. The child is currently living in an older home being renovated by the parents. Which teaching does the nurse provide to the parents? *Select all that apply.*
1. The possibility of removing the child from the environment
 2. Methods to remove paint chips or dust from the environment
 3. Details about behavior changes indicating additional exposure
 4. Recommendation of foods that will decrease absorption of lead
 5. The need for a child development specialist to evaluate the child

- _____ 16. The nurse is providing care for an adolescent female who just gave birth to a neonate. The mother tests positive for HIV; however, the mother did not receive prenatal care. Based on the nurse's understanding about HIV/AIDS, which interventions does the nurse expect? *Select all that apply.*
1. Placement of the neonate in foster care because of deficient parenteral care
 2. Performance of HIV polymerase chain reaction (PCR) test on the neonate
 3. Recommended virological diagnostic testing for the neonate
 4. Immediate immunization of the neonate against common childhood illnesses
 5. Nutritional management that includes high-calorie, nutrient-dense foods
- _____ 17. The nurse case manager is providing care and support to a mother with AIDS who has an infant who also tests positive for the condition. Which statement by the mother indicates to the nurse that additional teaching is needed? *Select all that apply.*
1. "I wish I could control the baby's pain without so much medication."
 2. "I am so looking forward to the time when the baby is no longer infectious."
 3. "I absolutely do not want the day-care staff to know about the baby's condition."
 4. "If I am to become a better mother, I need to work on my self-esteem and self-value."
 5. "I keep hoping every day that a cure for my baby and me can be found."
- _____ 18. A toddler who is 2 years of age is cared for by a grandmother because of the death of the toddler's mother from AIDS. A critical nursing intervention is for the nurse to provide home-care instructions. Which points should the nurse emphasize? *Select all that apply.*
1. Hand hygiene precautions during hospitalization
 2. Importance of keeping up to date with all childhood immunizations
 3. Proper nutrition for a toddler diagnosed with the disease
 4. Monitoring playmates in order to avoid childhood viruses
 5. Prevention of bacterial infections as the treatment focus
- _____ 19. A pediatric patient has acute myelocytic leukemia, and patient and family are considering treatment options aimed at long-term success. Which contemplated treatment involves using the patient's stem cells collected before radiation of cancerous bone marrow? *Select all that apply.*
1. Allogenic
 2. Autologous
 3. Peripheral blood stem cells (PBSC)
 4. Umbilical cord stem cells
 5. Platelet transfusion
- _____ 20. An adolescent 16 years of age and the parents and health-care team agree that the patient is to undergo HSCT. Which specific nursing interventions will be included for the care of this patient? *Select all that apply.*
1. Sterility of the central line must be maintained.
 2. High-dose chemotherapy and/or total body irradiation is administered.
 3. IV for the administration of stem cells is established and maintained.
 4. Fluid loss from urine, vomiting, and/or diarrhea is closely monitored.
 5. Anti-T-cell immunotoxins are administered immediately after the transfusion.

Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 4

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 1. Identify the classifications of anemia.

Page: 443

Heading: Hematological Disorders > Iron Deficiency Anemia

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Hematologic Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. Erythropoietin level of 1.9 mIU is within normal limits.
2	This is incorrect. Serum iron 24 mcg/dL is within normal limits.
3	This is incorrect. CBC indicating a normal RBC index is not a sign of iron deficiency anemia.
4	This is correct. When an infant is diagnosed with iron deficiency anemia, the RBCs will appear small and pale in color.

PTS: 1

CON: Hematological Regulation

2. ANS: 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 1. Identify the classifications of anemia.

Page: 443

Heading: Hematological Disorders > Iron Deficiency Anemia

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Hematological Regulation

Difficulty: Difficult

	Feedback
1	This is incorrect. The nurse does not identify any cause of the infant's condition from a comment about the infant sleeping well at night.
2	This is incorrect. The mother is confirming a change in the infant's behavior from a happy infant to one that is irritable.

3	This is correct. The infant is exhibiting manifestations related to iron deficiency anemia. The mother's comment is expressing a negative reaction to formula at 4 months of age. The nurse now needs to identify what the infant is being fed; cow's milk is an option but should not be introduced until 12 months old. This change can cause iron deficiency anemia.
4	This is incorrect. The mother's comment about the infant not being identifiably sick does not help the nurse identify the infant's condition.

PTS: 1 CON: Hematological Regulation

3. ANS: 2

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 2. Compare iron deficiency and sickle cell anemia.

Page: 445

Heading: Hematological Disorders > Sickle Cell Anemia

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Hematological Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. A child with iron-deficiency anemia may experience irritability, anorexia, tachycardia, systolic murmur, brittle and concave nails, and poor muscle tone, and may be prone to infection. Significant pain and discomfort is not expected.
2	This is correct. Vaso-occlusive crisis (obstruction of blood flow causing tissue hypoxia and necrosis) is a painful episode with hand-foot syndrome (dactylitis), causing symmetrical infarct in the bones of the hands and feet along with very painful swelling of soft tissue.
3	This is incorrect. A child with either iron-deficiency anemia or sickle cell disease may experience delayed growth and development.
4	This is incorrect. When both parents have the recessive sickle cell trait, there is a 1 in 4 chance with each pregnancy that the child will have sickle cell disease (SCD). About 1 in 13 African American babies are born with this sickle cell trait.

PTS: 1 CON: Hematological Regulation

4. ANS: 2

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 2. Compare iron deficiency and sickle cell anemia.

Page: 446

Heading: Hematological Disorders > Sickle Cell Anemia > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Comprehension [Understanding]

Concept: Nutrition

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse recommends a balanced nutritious diet for all children, especially those with anemias.
2	This is correct. The child with sickle cell anemia is encouraged to drink as much fluid as possible to help avoid the clumping of sickled cells. Hydration can be accomplished with IV therapy and oral fluids. Children with iron-deficiency anemia need a normal daily fluid intake.
3	This is incorrect. Anemias can cause fatigue; children should participate in low-level activities, as able. Caretakers will need to provide for adequate rest periods.
4	This is incorrect. Anemias put children at risk for infections; immunizations need to be up to date. Influenza and pneumonia vaccines are strongly recommended.

PTS: 1 CON: Nutrition

5. ANS: 1

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 6. Develop a caregiver education plan for high-risk oncology clients.

Page: 447

Heading: Hematological Disorders > Hemophilia > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. Preventing injury in the environment with supervision, helmet use, and activity restrictions is the most important teaching for the nurse to provide to the family. The toddler will be especially challenging because of normal development, which places the toddler at a high risk for injury.
2	This is incorrect. Sharing the availability of resources from the National Hemophilia Foundation publications is important; however, another topic of teaching is more important.
3	This is incorrect. Helping the parents prepare for the possibility of altered family dynamics and how the illness may impact financial resources is important; however, the prevention of injury is most important.
4	This is incorrect. It is important for the nurse to teach the parents about allowing the toddler to grow up as normally as possible, but the nature of hemophilia will grossly alter the toddler's future activities, such as the avoidance of contact sports. There is another topic of more importance.

PTS: 1 CON: Patient-Centered Care

6. ANS: 4

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 3. Identify the major pathophysiology associated with the care of a child with cancer.

Page: 452

Heading: Critical Component > Absolute Neutrophil Count and Calculation

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Cellular Regulation

Difficulty: Difficult

	Feedback
1	This is incorrect. The ANC does not specifically measure how the body is tolerating treatment. When the ANC is less than 500, the client is at high risk for infection.
2	This is incorrect. The ANC is a measure of bands, segmented cells, and white blood cells, which does not include eosinophils.
3	This is incorrect. Antibodies are not factored into the ANC.
4	This is correct. Calculating the ANC requires three numbers from the CBC and differential. The following formula calculates the ANC: $(\% \text{ bands} + \% \text{ segmented cells}) \times \text{number of WBC} \times 10 = \text{ANC}$. The result indicates the patient's vulnerability to infection and directs the level of isolation required.

PTS: 1

CON: Cellular Regulation

7. ANS: 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 5. Develop a nursing care plan for the child with cancer experiencing pain.

Page: 453

Heading: Safe and Effective Nursing Care > Promoting Safety > Nursing Care Priorities for Complex Pediatric Hematology and Oncology Patients

Integrated Processes: Nursing Process

Cognitive Level: Synthesis [Creating]

Client Need: Safe and Effective Care Environment: Management of Care

Concept: Cellular Regulation

Difficulty: Difficult

	Feedback
1	This is incorrect. Applying heating pads will increase circulation to the area and increase pain.
2	This is incorrect. Cold packs to the abdomen may resolve abdominal issues but will not be effective if applied to the chest.

3	This is correct. Aromatherapy is the therapeutic use of essential oils from plants (flowers, herbs, or trees) for the improvement of physical, emotional, and spiritual well-being.
4	This is incorrect. Antacids neutralize acids, which are not the direct cause of the patient's abdominal pain.

PTS: 1 CON: Cellular Regulation

8. ANS: 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 3. Identify the major pathophysiology associated with the care of a child with cancer.

Page: 454

Heading: Oncological Disorders > Leukemia

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Cellular Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. The nurse will avoid invasive procedures and use aseptic technique to draw from central lines if present. Laboratory outcomes are important, but this action is not the priority.
2	This is incorrect. The nurse will complete a thorough physical assessment. Vital signs, duration of current symptoms, and systems evaluations are of special interest. This information is important and will guide the physician for additional orders; however, this action is not the priority.
3	This is correct. Patients with a history of leukemia need to be triaged quickly and placed in a treatment room as soon as possible. Neutropenic precautions (protective isolation) need to be initiated. The cancer cells, along with the cancer treatments, place a burden on the child's immune system. The disease pathophysiology makes observation and prevention of infection a priority.
4	This is incorrect. Radiological studies may be in order after the initial assessments and antibiotics have begun. The administration of antibiotics within 60 minutes of assessment for the presence of fever is of greater importance.

PTS: 1 CON: Cellular Regulation

9. ANS: 2

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 4. Identify nursing assessments and interventions that promote health during the care of children with cancer and their families.

Page: 456

Heading: Oncological Disorders > Lymphomas > Caregiver Education

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Cellular Regulation
Difficulty: Moderate

	Feedback
1	This is incorrect. The patient is likely to be experiencing weight loss from anorexia triggered by the disease process. Weight gain is unlikely; other nutrition recommendations are needed.
2	This is correct. Treatment with chemotherapy is likely to cause sterilization; the nurse needs to recommend sperm banking. Patients may not be able to think about a future and children; the nurse will explain the importance of having sperm at a future time.
3	This is incorrect. If the patient is living independently, there is no need to move in with parents unless care assistance is needed. The patient can initiate quarantine measures if necessary, which may be easier in an independent environment. However, other care needs may be met if living with caregivers.
4	This is incorrect. It is likely the patient will experience fatigue from chemotherapy. However, the patient may benefit from activities focused on something other than the illness and treatment. The patient may want to consider one class a term and the availability of computerized classes.

PTS: 1 CON: Cellular Regulation

10. ANS: 1

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 5. Develop a nursing care plan for the child with cancer who is experiencing pain.

Page: 457

Heading: Oncological Disorders > Lymphomas > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Cellular Regulation

Difficulty: Moderate

	Feedback
1	This is correct. The nurse needs to allow for medical play when age appropriate; a needleless syringe for stuffed animal treatment is a good choice for a 4-year-old child.
2	This is incorrect. The nurse will explain procedures to the patient's parents; however, it is the nurse's responsibility to provide teaching to the patient and ask for parenteral support.
3	This is incorrect. Even at the age of 4 years, the patient is much more likely to be cooperative with some preparation prior to the testing.
4	This is incorrect. The nurse can teach the patient on an age-appropriate level and elicit more cooperation than without teaching and preparation.

PTS: 1 CON: Cellular Regulation

11. ANS: 1

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 4. Identify nursing assessments and interventions that promote health during the care of children with cancer and their families.

Page: 458

Heading: Safe and Effective Nursing Care > Understanding Medication > Special Considerations in Chemotherapy and Biotherapy Administration

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Cellular Regulation

Difficulty: Difficult

	Feedback
1	This is correct. It is most important for the patient and family to recognize adverse effects to the chemotherapy and seek medical assistance as directed.
2	This is incorrect. Protective precautions are explained only as needed.
3	This is incorrect. Promoting the preparation of the patient's favorite food may or may not be necessary. Some favorite foods can be available after the first treatment if needed or desired.
4	This is incorrect. During chemotherapy, patients may experience mouth sores and have difficulty eating and drinking. Some patients like cold liquids, and others prefer warm liquids. It is unlikely that this is an issue after the first chemotherapy treatment.

PTS: 1

CON: Cellular Regulation

12. ANS: 2

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 5. Develop a nursing care plan for the child with cancer who is experiencing pain.

Page: 458

Heading: Safe and Effective Nursing Care > Clinical Pearl > Preparing Children for Diagnostic Testing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Synthesis [Creating]

Concept: Cellular Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. Consider the developmental level of the child and use words he or she can understand.
2	This is correct. State what the child will hear, see, feel, touch, and smell. The unknown is more frightening to a child than experiencing expected happenings, equipment, and procedures.

3	This is incorrect. Provide a special box of toys to have a treat to look forward to after the procedure. Tell the child that it is okay to cry.
4	This is incorrect. Anesthesia may put the child to sleep with a mask or IV medication. Administer medication for any pain, including headache, as needed.

PTS: 1 CON: Cellular Regulation

13. ANS: 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 3. Identify the major pathophysiology associated with the care of a child with cancer.

Page: 459

Heading: Oncological Disorders > Brain Tumors

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Cellular Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. Vaso-occlusive crisis (obstruction of blood flow causing tissue hypoxia and necrosis) is a painful episode with hand-foot syndrome (dactylitis), causing symmetrical infarct in the bones of the hands and feet along with very painful swelling of soft tissue in sickle cell crisis.
2	This is incorrect. A hematoma will cause pain, but there is no information in the scenario confirming the presence of a hematoma. The patient is experiencing pain in the head, not the neck.
3	This is correct. The patient is experiencing pain from a neoplasm in the brain directly affecting nerve receptors. The nurse's answer is pathologically correct and age appropriate.
4	This is incorrect. The medications are not a likely the cause of the patient's headache. Medication therapy is aimed at decreasing the size of the tumor and pain.

PTS: 1 CON: Cellular Regulation

14. ANS: 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 4. Identify nursing assessments and interventions that promote health during the care of children with cancer and their families.

Page: 460

Heading: Oncological Disorders > Wilms' Tumor

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Cellular Regulation

Difficulty: Moderate

	Feedback
1	This is incorrect. The parents need to provide the toddler with a balanced and healthy diet. There is no reason to expect dehydration or to increase fluids.
2	This is incorrect. There is no identifiable benefit to keeping the toddler in a side-lying position with pillow placement during sleep; keeping a 4-year-old toddler in one position may be challenging.
3	This is correct. Do not palpate the tumor, because this can cause the proliferation or spread of cancer cells. Avoid pushing or lifting in tumor area when handling and bathing the child. This is essential teaching before surgery.
4	This is incorrect. The nurse will provide information about the toddler's postoperative care; however, this is not the essential teaching before surgery.

PTS: 1

CON: Cellular Regulation

MULTIPLE RESPONSE

15. ANS: 1, 2, 5

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 6. Develop a caregiver education plan for high-risk oncology clients.

Page: 447

Heading: Hematological Disorders > Lead Poisoning

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Synthesis [Creating]

Concept: Safety

Difficulty: Difficult

	Feedback
1.	This is correct. It is very important that the child diagnosed or being treated for lead poisoning be removed from the contaminated environment.
2.	This is correct. It is very important that the parents know how to remove lead from the environment. The recommended method is to remove paint chips or dust with a damp paper towel and discard it in the trash. Use another wet paper towel to clean the surface and discard the paper towel.
3.	This is incorrect. Central nervous system signs of lead poisoning include hyperactivity, impulsiveness, lethargy, irritability, loss of developmental progress, hearing impairment, and learning difficulties. However, the goal is to avoid exposure.
4.	This is incorrect. Iron-deficiency anemia causes a greater level of lead absorption, but correcting the anemia does not prevent the child in the event of additional exposure.
5.	This is correct. Because of the impact of lead poisoning on the development of the child, the parents need to know the importance of having a child development specialist evaluate the child as needed.

PTS: 1 CON: Safety

16. ANS: 2, 3, 5

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 7. Demonstrate an understanding of the pathophysiology of and nursing interventions for the child with immunodeficiency disorders.

Page: 451

Heading: Immunological Disorders > Acquired Immunodeficiency Syndrome

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Synthesis [Synthesizing]

Concept: Immunity

Difficulty: Difficult

	Feedback
1.	This is incorrect. Neonates born to HIV-positive mothers are not routinely placed in foster care. The scenario does not present enough information to determine deficient parenteral care of the neonate.
2.	This is correct. HIV polymerase chain reaction (PCR) is used for detection in infants born to HIV-infected mothers because of the presence of maternal antibodies transferred transplacentally; preferred virological assays include HIV DNA PCR and HIV RNA assays.
3.	This is correct. Virological diagnostic testing is recommended at birth in infants at high risk for HIV infection (e.g., infants born to HIV-infected mothers who did not receive prenatal care or prenatal antiretroviral therapy).
4.	This is incorrect. Immediate immunization is not recommended; however, immunization against common childhood illnesses is strongly recommended if exposed to HIV. The immunizations will be given when age appropriate.
5.	This is correct. The nurse will expect initiation of nutritional management that includes high-calorie, nutrient-dense foods.

PTS: 1 CON: Immunity

17. ANS: 1, 2, 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 7. Demonstrate an understanding of the pathophysiology of and nursing interventions for the child with immunodeficiency disorders.

Page: 451

Heading: Immunological Disorders > Acquired Immunodeficiency Syndrome

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Immunity

Difficulty: Difficult

	Feedback
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1.	This is correct. The nurse needs to teach pain management techniques and reinforce that tolerance to opioids may require increased dosing. The nurse also needs to teach and encourage use of nonpharmacological pain interventions.
2.	This is correct. When the mother expresses the expectation that the infant will eventually be noninfectious, additional teaching by the nurse is needed.
3.	This is correct. When the mother expresses opposition to the day-care staff knowing about the infant's condition, additional teaching is needed. The nurse needs to support the mother with making complex day-care arrangements. The nurse also has a responsibility to educate day-care and school staff on current AIDS information.
4.	This is incorrect. The nurse needs to encourage positive self-concept and suggestions of how to avoid the HIV-related stigma. The nurse needs to provide support but not necessarily teaching.
5.	This is incorrect. The hope expressed by the mother for a cure for herself and her infant does not require additional teaching by the nurse.

PTS: 1 CON: Immunity

18. ANS: 2, 3, 4

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 7. Demonstrate an understanding of the pathophysiology of and nursing interventions for the child with immunodeficiency disorders.

Page: 451

Heading: Immunological Disorders > Acquired Immunodeficiency Syndrome

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Immunity

Difficulty: Difficult

	Feedback
1.	This is incorrect. The nurse will emphasize the importance of following standard precautions, such as hand hygiene, at all times when providing care.
2.	This is correct. Immunization against common childhood illnesses is required if the toddler is exposed to HIV/AIDS or tests positive for either condition.
3.	This is correct. Nutritional management with high-calorie, nutrient-dense foods is critical to putting the infant's immune system in the best possible state to fight off infections.
4.	This is correct. Prevention and management of opportunistic infections (OPs) is essential for children with severe immune suppressions. Playmates will be excluded from contact with the toddler if the playmates are sick.
5.	This is incorrect. Administration of highly active antiretroviral therapy drug (HAART) combination therapy (a strategy analogous to the treatment of infectious diseases) has improved efficacy, minimized toxicity, and delayed drug resistance. The treatment of the toddler is not just focused on bacterial infections.

PTS: 1 CON: Immunity

19. ANS: 2, 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 8. Identify the categories of hematopoietic stem cell transplant.

Page: 461

Heading: Hematopoietic Stem Cell Transplantation

Integrative Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Hematological Regulation

Difficulty: Difficult

	Feedback
1.	This is incorrect. Allogenic transplantation comes from another person rather than from the patient.
2.	This is correct. Autologous cells are the cells that were drawn from the patient.
3.	This is correct. Peripheral blood stem cells can be drawn from the patient and are therefore autologous.
4.	This is incorrect. Umbilical cord stem cells come from a newborn infant and not the patient.
5.	This is incorrect. Stem cells are those initial (parent) cells from which other blood cells are grown. Platelets are one of the products of cell formation.

PTS: 1

CON: Hematological Regulation

20. ANS: 1, 2, 3

Chapter: Chapter 19 Hematological, Immunological, and Neoplastic Disorders

Chapter Learning Objective: 8. Identify the categories of hematopoietic stem cell transplant.

Page: 461

Heading: Hematopoietic Stem Cell Transplantation

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Hematological Regulation

Difficulty: Difficult

	Feedback
1.	This is correct. Strict asepsis with central venous catheter care is a critical intervention that all health-care providers must follow.
2.	This is correct. Chemotherapy will treat the diagnosed cancer and will also reduce the immune response. The reduction of immune responses will aid in acceptance of the transplanted stem cells.
3.	This is correct. It is important to carefully initiate, monitor, and maintain the IV transfusion of the stem cells harvested from bone marrow, peripheral blood, or the umbilical vein of the placenta.

4.	This is incorrect. Monitoring both intake and output will provide information about kidney function and fluid balance. Vomiting and diarrhea is not expected during HSCT; however, signs and symptoms of chronic GVHD occur about 100 days after transplant and affect the liver, gastrointestinal system, oral mucosa, and lungs.
5.	This is incorrect. High-level nutritional intake must be provided to encourage healing and the body's acceptance of the transplanted cells.

PTS: 1 CON: Hematological Regulation

Chapter 20 Musculoskeletal Disorders

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. A 4-year-old who had a broken arm is preparing to have the cast removed after 4 weeks. The child's parent states, "I had a cast on my arm a lot longer than that for it to heal. Are you sure the cast has been on long enough?" Which is the best response to the parent?
1. "Children's bones heal faster than adults, so they don't need to wear a cast as long as an adult."
 2. "Children's bones are different, but the cast should be on about as long as yours was."
 3. "You're right. The cast hasn't been on nearly as long as an adult. I'll recheck with the health-care provider."
 4. "We don't worry about children's bones as much; they will remodel well on their own."
- _____ 2. An 11-year-old child who had been treated for a broken leg is having his cast removed. After removal, the child asks the nurse about playing basketball for his local league. Which should be included in the response to the child?
1. The child should avoid playing basketball for up to 4 weeks but can then resume playing.
 2. The child can play basketball as soon as the stiffness in this leg subsides, which is typically within 3 to 5 days.
 3. The child should avoid playing basketball until he has stopped growing due to risk of reinjury.
 4. The child may play basketball immediately, but should stop if the extremity develops swelling and pain.
- _____ 3. The nurse is caring for a child who reports he has pain in his ankle after "twisting" it during play. Which would be the most appropriate follow-up assessment to this report?
1. Color and temperature of the skin around the ankle
 2. The patient's gait and range-of-motion of all extremities
 3. Swelling and deformity of the ankle joint
 4. Palpation of the joints proximal to the ankle
- _____ 4. The nurse is preparing to complete assessments on children in a homeless shelter. Which assessment will assist the nurse to identify possible musculoskeletal conditions each child?
1. Eating and sleeping patterns

2. Height and weight growth patterns
3. Motor development patterns
4. Sensory development patterns

- _____ 5. The nurse is caring for a child in the emergency department. The child's arm is abnormally positioned, and the child is holding the arm near that area and is crying with pain. The child's parents explain that the child fell off the bicycle when riding on the streets of this subdivision. Which is the priority care to provide to the child?
1. Apply ice to the painful area.
 2. Elevate the child's arm.
 3. Encourage the child to rest.
 4. Protect the child from more injury.
- _____ 6. A school-aged child is refusing to use her right hand to write or type in the classroom. The child states that her wrist hurts. There is no apparent deformity, but the wrist is swollen. The child reports crashing her bike and landing on her hand 2 days prior. If fractured, which type of fracture does the child likely suffer from?
1. Greenstick fracture
 2. Elbow fracture
 3. Torus fracture
 4. Toddler fracture
- _____ 7. A child is being treated with a hip Spica cast and preparing to be discharged home. Which statement by the parents demonstrates effective education and readiness to care for the child?
1. "We will turn our daughter every day and watch for skin irritation."
 2. "Placing the absorbent part of the diaper toward the skin is best."
 3. "We will let our daughter choose her own snacks to help her be happy."
 4. "If our daughter is in pain, we will use only oxycodone to treat pain."
- _____ 8. The nurse is providing care to a preschool-aged child who has been placed in a hip Spica cast for treatment of a femur fracture. The child is given narcotic pain medication for the first 48 hours before being sent home. When providing discharge instructions to the child's parents, which is the most important?
1. Assist the child to find ways to play while in the cast.
 2. Ensure the cast is not too tight around the abdomen.
 3. Keep the cast clean by using absorbent pads when toileting.
 4. Avoid giving the child sugary snacks while being treated.
- _____ 9. A 2-year-old child is being evaluated for a musculoskeletal disorder. The child's laboratory results indicate hypophosphatemia, a normal erythrocyte sedimentation rate, and negative rheumatoid factor. Which disorder is the child likely experiencing?
1. Juvenile arthritis
 2. Genu varum (bowlegs)
 3. Septic arthritis
 4. Avascular necrosis

- _____ 10. The mother of a 3-month-old returns for a visit with the physician who is caring for the infant's hip dysplasia. However, the nurse notes that upon arrival, the infant is not wearing the prescribed Pavlick harness. The mother indicates she thought the infant was uncomfortable so she removed it. Which is the best response by the nurse?
1. Inform the mother how to check for capillary refill in the toes.
 2. Assess the infant for trauma or swelling to the lower extremities.
 3. Report the findings to the physician before entering the examination room.
 4. Ask the mother to demonstrate how to place the Pavlick harness.
- _____ 11. The nurse is providing care to a 7-year-old child who has been diagnosed with avascular necrosis. The patient's guardians ask the nurse what to expect due to this diagnosis. Which would be appropriate information to provide to the parents?
1. The child will be hospitalized and placed in traction.
 2. The child will be treated by surgically placing a femoral screw.
 3. The child will be placed in a Pavlik harness for several weeks.
 4. The child will be given medication while on bedrest.
- _____ 12. The mother of a 6-week-old patient calls the physician's office and reports her infant is irritable, running a fever, holds his hip in an "unusual" position, and was recently given antibiotics for an ear infection. Which response by the nurse is most appropriate?
1. "Administer acetaminophen every 4 hours for the fever and call back if he still has a fever in 24 hours."
 2. "This can easily be managed at home and will most likely resolve on its own."
 3. "The antibiotics need more time to work. Finish the antibiotics prescribed and call back if the symptoms continue."
 4. "You need to take him to the walk-in clinic or emergency room for evaluation."
- _____ 13. A 12-year-old girl is noted to walk with an uneven gait, and her shoulders appear uneven. The left shoulder sits lower than the right shoulder, and her waist appears to be tilted from side to side. Which condition is the girl most likely experiencing?
1. Internal femoral torsion
 2. Hip dysplasia
 3. Club foot
 4. Scoliosis
- _____ 14. The nurse is providing care for a 14-year-old patient who is within the first 24 hours postoperative for scoliosis treatment. Which intervention, if performed, would demonstrate appropriate care measures?
1. Removing the indwelling urinary catheter immediately upon arrival to the care unit
 2. Repositioning the patient every 2 to 4 hours while in bed during the recovery period
 3. Encouraging the patient to walk postoperatively once daily during pain-free periods
 4. Monitoring oxygen levels and bowel sounds once daily until patient is discharged to home

- _____ 15. The nurse is caring for a 13-year-old patient diagnosed with adolescent idiopathic scoliosis. The curve in her back was treated by spinal fusion with rod insertion. Which is the priority nursing intervention in the postoperative period?
1. Promote adequate oral fluid intake.
 2. Keep oxygen saturation within normal limits.
 3. Ensure placement of indwelling urinary catheter.
 4. Maintain a straight back, no bending.
- _____ 16. The child is visiting the clinic for a pre-sports-participation physical. Which should be included as part of the patient education to prevent musculoskeletal injury during participation?
1. Manage any pain with acetaminophen
 2. Consume extra fluids during warm weather
 3. Perform stretching exercises before play
 4. Seek emergency treatment for injuries
- _____ 17. A 3-year-old, well-known to the nursing staff, is admitted to the hospital frequently for fracture treatment secondary to osteogenesis imperfecta. It is known by staff and parents that the child is fragile. The nurse develops a long-term care plan to review with the parents. Which is the priority intervention for this child's plan of care?
1. Observe the child's height and weight growth.
 2. Monitor for bluish colored sclera development.
 3. Teach methods to prevent falls and injury.
 4. Inform parents about the OI foundation.
- _____ 18. A teenaged patient arrives in the emergency department where the physician diagnoses a dislocated shoulder. It is the patient's first dislocation. Which should the nurse prepare for treatment of the dislocation?
1. Make sedative medication available.
 2. Prepare a minimally invasive surgery room.
 3. Anticipate a physical therapist consultation.
 4. Get portable radiology equipment.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 19. The family of an infant is present in the hospital when the diagnosis of osteogenesis imperfecta is given to the infant. The nurse is responsible for care of the infant and family. Which hospital care is appropriate? *Select all that apply.*
1. Avoid excessive force when monitoring blood pressure.
 2. Consult a social worker for concerns of child abuse.
 3. Teach the parents how to gently handle the infant.
 4. Inform ancillary staff about handling precautions.
 5. Prepare for the use of a riser cast for treatment.
- _____ 20. A 14-year-old student reports to the school nurse that he has a persistent pain and soreness in his legs. The student is part of the football team. There is no evidence of injury to the student. Which information should the nurse provide to the student? *Select all that apply.*

1. Continue to practice despite tiredness and pain.
2. Do stretching exercises before playing or being active.
3. Ask the coach to sit out of practice for a week.
4. Ensure shoes and equipment fit properly and are stable.
5. Learn proper body mechanics for the activities.

Chapter 20 Musculoskeletal Disorders

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 1. Describe the anatomy and physiology of the musculoskeletal system.

Page: 465

Heading: Anatomy and Physiology > Skeleton

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Easy

	Feedback
1	This is correct. A child's bones tend to heal faster than adults. The immobilization times are shorter.
2	This is incorrect. The child's bones are different; however, the timing of casting is usually shorter in children due to the faster healing times.
3	This is incorrect. There is no need to check with the health-care provider based on the extent of time the child was placed in the case. Children heal faster than adults.
4	This is incorrect. Although children's bones do remodel well if there is more than 2 years of growth expected, it is not correct to indicate that this prevents health-care providers from worrying about children's bones.

PTS: 1

CON: Mobility

2. ANS: 1

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 1. Describe the anatomy and physiology of the musculoskeletal system.

Page: 465

Heading: Anatomy and Physiology > Skeleton > Unique Properties of the Immature Skeleton

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity

Cognitive Level: Application [Applying]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is correct. Children need to avoid contact activities for 2 to 4 weeks once out of a cast.

2	This is incorrect. Stiffness may resolve in 1 to 3 weeks, which isn't long enough to reduce the risk of harm if returning to contact activity.
3	This is incorrect. The fracture needs to be well healed if the child returns to this type of activity, but he does not need to wait until he is done growing.
4	This is incorrect. The child needs to wait 2 to 4 weeks before returning to contact activity. The swelling and pain should resolve in about 1 to 3 weeks.

PTS: 1 CON: Mobility

3. ANS: 3

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 3. Identify specific physical assessment skills for evaluation of the musculoskeletal system.

Page: 465

Heading: Assessment > General assessment

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. Color and temperature informs about the perfusion around the area.
2	This is incorrect. Gait and range of motion of all extremities informs about all areas rather than focus on the injured joint. Pain will affect a patient's gait.
3	This is correct. Swelling and deformity will inform about injury to the joint.
4	This is incorrect. Palpation of the proximal joints will not inform about the injured area.

PTS: 1 CON: Mobility

4. ANS: 3

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 3. Identify specific physical assessment skills for evaluation of the musculoskeletal system.

Page: 465

Heading: Assessment

Integrated Processes: Nursing Process

Client Need: Physiologic Integrity

Cognitive Level: Application [Applying]

Concept: Mobility

Difficulty: Easy

	Feedback
1	This is incorrect. Eating and sleeping patterns give insight to nutrition and rest, not musculoskeletal conditions.

2	This is incorrect. Height and weight growth patterns give insight into the ability of the child to meet nutrition needs.
3	This is correct. Motor development delays can be related to musculoskeletal conditions due to the potential impact on movement.
4	This is incorrect. Sensory development provides insight into neurological conditions.

PTS: 1 CON: Mobility

5. ANS: 4

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 5. Explain the applicable nursing interventions for musculoskeletal disorders.

Page: 465

Heading: Emergent and Initial Nursing Care of the Musculoskeletal System

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Application [Applying]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. Ice application is the third level priority in care of emergent musculoskeletal injuries.
2	This is incorrect. Elevating the extremity is the last level priority in care of emergent musculoskeletal injuries.
3	This is incorrect. Rest is the second level priority in care of emergent musculoskeletal injuries.
4	This is correct. Protecting the patient from further injury is the priority care during emergent care of musculoskeletal injuries.

PTS: 1 CON: Mobility

6. ANS: 3

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 2. Describe the common disorders of the musculoskeletal system.

Page: 469

Heading: Fractures > Torus Fractures

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. Greenstick fractures commonly occur to the long bones and mid-diaphyseal area.

2	This is incorrect. The elbow fracture would demonstrate pain in the elbow, not the wrist.
3	This is correct. The torus fracture occurs when the bone buckles but does not break. It is often diagnosed days after the injury and is often in the radius or ulna.
4	This is incorrect. A toddler fracture occurs in children younger than 2 years of age. The child typically refuses to bear weight on a leg.

PTS: 1 CON: Mobility

7. ANS: 2

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 9. Describe strategies to support patient/caregiver education about musculoskeletal disorders.

Page: 471

Heading: Fractures > Femur fractures > Care of Child in a Spica Cast

Integrated Processes: Nursing Process

Client Need: Physiological Integrity > Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Difficult

	Feedback
1	This is incorrect. The child must be turned every 2 to 4 hours while awake.
2	This is correct. Placing the absorbent part of a diaper against the skin of the child in a Spica cast assists in protecting the patient's skin from urine or feces collecting around the cast.
3	This is incorrect. The child need snacks to promote healing and prevent constipation.
4	This is incorrect. The child needs to have acetaminophen and ibuprofen used as an adjunct to the oxycodone therapy.

PTS: 1 CON: Mobility

8. ANS: 2

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 5. Explain the applicable nursing interventions for musculoskeletal disorders.

Page: 471

Heading: Fractures > Femur Fractures > Care of Child in a Spica Cast

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
--	----------

1	This is incorrect. Play is important to psychological well-being; however, it is not the priority in this situation.
2	This is correct. Preventing injury from the cast due to reducing circulation or placing pressure on the abdominal area is the priority.
3	This is incorrect. Keeping the cast clean is important; however, it is not the priority in this situation.
4	This is incorrect. Avoiding sugary treats are important; however, it is not the priority in this situation.

PTS: 1 CON: Mobility

9. ANS: 2

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 4. Describe the diagnostic tests and lab values typically monitored in musculoskeletal disorders.

Page: 474

Heading: Congenital Musculoskeletal Abnormalities > Genu Varum (Bowlegs)

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. Juvenile arthritis demonstrates a positive rheumatoid factor.
2	This is correct. Genu varum (bowlegs) can be associated with hypophosphatemia.
3	This is incorrect. Septic arthritis is associated with inflammation evidenced by an elevated erythrocyte sedimentation rate (ESR).
4	This is incorrect. Avascular necrosis is associated with abnormal radiologic examinations.

PTS: 1 CON: Mobility

10. ANS: 1

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 9. Describe strategies to support patient/caregiver education about musculoskeletal disorders.

Page: 477

Heading: Congenital Musculoskeletal Abnormalities > Hip Dysplasia > Treatment

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
--	----------

1	This is correct. This is done when the child is wearing the harness.
2	This is incorrect. Assessing for trauma or injury does not address the need to wear the harness as prescribed.
3	This is incorrect. The nurse is responsible to teach about care of the patient and has the responsibility to teach the mother rather than report to the physician.
4	This is incorrect. This allows the nurse to determine whether the patient's mother can effectively apply the harness and assess what teaching needs completed to promote proper treatment.

PTS: 1 CON: Mobility

11. ANS: 1

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 6. Integrate acute hospital care concepts for musculoskeletal disorders.

Page: 478

Heading: Congenital Musculoskeletal Abnormalities > Avascular Necrosis > Treatment

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Difficult

	Feedback
1	This is correct. The child may be treated by being placed in traction (Buck's or Bryant's) while hospitalized
2	This is incorrect. This is treatment for a slipped capital femoral epiphysis.
3	This is incorrect. This is treatment for hip dysplasia.
4	This is incorrect. This is not appropriate treatment for most musculoskeletal disorders.

PTS: 1 CON: Mobility

12. ANS: 4

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 5. Explain the applicable nursing interventions for musculoskeletal disorders.

Page: 480

Heading: Congenital Musculoskeletal Abnormalities > Septic Arthritis and Osteomyelitis of the Hip

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Application [Applying]

Concept: Infection

Difficulty: Moderate

	Feedback
--	----------

1	This is incorrect. This only addresses the fever, which is not the complete health picture.
2	This is incorrect. This condition should be evaluated and likely requires hospital management.
3	This is incorrect. Finishing the antibiotics does not address the symptoms described by the mother. The antibiotics will treat the ear infection but are not adequate to treat the full condition.
4	This is correct. The infant is demonstrating symptoms of possible septic arthritis of the hip and needs immediate evaluation for the possible condition.

PTS: 1 CON: Infection

13. ANS: 4

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 2. Describe the common disorders of the musculoskeletal system.

Page: 480

Heading: Congenital Musculoskeletal Abnormalities > Scoliosis

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. Internal femoral torsion causes laxity in the ligaments, and the child usually chooses the “W” sitting position.
2	This is incorrect. Hip dysplasia presents with limited hip abduction, asymmetrical thigh skin folds, a positive Barlow test, and Galeazzi signs.
3	This is incorrect. Club foot is the inward turning of either one or both feet. The calf often looks smaller as well as the foot may be smaller than usual.
4	This is correct. Scoliosis often presents with uneven waist line and shoulder height. The scoliosis can create an uneven gait.

PTS: 1 CON: Mobility

14. ANS: 2

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 5. Explain the applicable nursing interventions for musculoskeletal disorders.

Page: 482

Heading: Congenital Musculoskeletal Abnormalities> Scoliosis > Adolescent Idiopathic Scoliosis

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Moderate

	Feedback
1	This is incorrect. The indwelling urinary catheter should be maintained for at least the first 24 hours postoperatively.
2	This is correct. Repositioning the patient every 2 to 4 hours postoperatively prevents tissue damage to pressure areas.
3	This is incorrect. Walking should be done at least three times daily and the patient may not be pain free; however, walking should be done when pain is well-controlled.
4	This is incorrect. Oxygen levels may need monitored more than once per shift, especially if oxygenation is a problem. The patient's bowel sounds need monitored at least every 4 hours.

PTS: 1 CON: Mobility

15. ANS: 2

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 6. Integrate acute hospital care concepts for musculoskeletal disorders.

Page: 482

Heading: Congenital Musculoskeletal Abnormalities > Scoliosis > Adolescent Idiopathic Scoliosis

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Mobility

Difficulty: Difficult

	Feedback
1	This is incorrect. While adequate intake is important to reduce the risk of dehydration, it is not the priority in this list.
2	This is correct. Oxygen saturation is the priority in this list. It is the ability to maintain an open airway and breathe effectively (ABCs) that makes this the priority.
3	This is incorrect. Placement of the indwelling urinary catheter is related to the ability to excrete waste during the immediate postoperative period, which is not a priority over oxygenation.
4	This is incorrect. Maintaining a straight back, no bending, relates to the integrity of the surgical intervention and healing needed. Although important, another intervention is a higher priority.

PTS: 1 CON: Mobility

16. ANS: 3

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 5. Explain the applicable nursing interventions for musculoskeletal disorders.

Page: 484

Heading: Sports-Related Injuries

Integrated Processes: Nursing Process
 Client Need: Physiological Integrity: Reduction of Risk Potential
 Cognitive Level: Analysis [Analyzing]
 Concept: Patient-Centered Care
 Difficulty: Moderate

	Feedback
1	This is incorrect. Managing pain relates to caring for injuries or discomforts rather than preventing injury.
2	This is incorrect. Consuming extra fluids assists in preventing dehydration rather than musculoskeletal injury.
3	This is correct. Performing stretching exercises will assist the child in preventing musculoskeletal injury.
4	This is incorrect. To seek emergency treatment is managing injury after it occurs rather than preventing injury.

PTS: 1 CON: Patient-Centered Care

17. ANS: 3

Chapter: Chapter 20 Musculoskeletal Disorders
 Chapter Learning Objective: 7. Integrate chronic hospital care concepts for musculoskeletal disorders.
 Page: 484
 Heading: Osteogenesis Imperfecta > Nursing Interventions
 Integrated Processes: Nursing Process
 Client Need: Physiological Integrity: Basic Care and Comfort
 Cognitive Levels: Application
 Concept: Safety
 Difficulty: Difficult

	Feedback
1	This is incorrect. Height and weight monitoring are important; however, they do not address safety for the patient.
2	This is incorrect. Bluish sclera are noted at times; however, this is not a priority to keep the patient safe.
3	This is correct. Preventing falls and injury is the priority, as it keeps the patient safe and may reduce growth deformities.
4	This is incorrect. The OI foundation can provide support to the family and child; however; informing the parents about the OI does not address safety, which is the priority care need.

PTS: 1 CON: Safety

18. ANS: 1

Chapter: Chapter 20 Musculoskeletal Disorders
 Chapter Learning Objective: 8. Integrate emergency care or trauma concepts for musculoskeletal disorders.

Page: 484

Heading: Sports-Related Injuries > Dislocations and Subluxations

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Trauma

Difficulty: Moderate

	Feedback
1	This is correct. A dislocation is treated by relocation of the joint using a sedative medication.
2	This is incorrect. Surgery is used for repeated dislocation conditions.
3	This is incorrect. Physical therapy may be used to prevent the injury or prevent recurrence.
4	This is incorrect. Radiology equipment may be used in initial diagnosis rather than treatment.

PTS: 1

CON: Trauma

MULTIPLE RESPONSE

19. ANS: 1, 3, 4

Chapter: Chapter 20 Musculoskeletal Disorders.

Chapter Learning Objective: 6. Integrate acute hospital care concepts for musculoskeletal disorders.

Page: 483

Heading: Osteogenesis Imperfecta

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Mobility

Difficulty: Difficult

	Feedback
1.	This is correct. Excessive pressure can produce fractures in the infant. Care must be taken when monitoring blood pressure.
2.	This is incorrect. Osteogenesis imperfecta is a medical condition and does not require child-protective interventions.
3.	This is correct. Parents must be aware of proper handling to prevent injury to the infant.
4.	This is correct. Ancillary staff must be aware of the patient's condition to prevent injury to the infant.
5.	This is incorrect. A riser cast is not indicated in the treatment of osteogenesis imperfecta.

PTS: 1

CON: Mobility

20. ANS: 2, 4, 5

Chapter: Chapter 20 Musculoskeletal Disorders

Chapter Learning Objective: 9. Describe strategies to support patient/caregiver education about musculoskeletal disorders.

Page: 484

Heading: Sports-Related Injuries

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Application [Applying]

Concept: Mobility

Difficulty: Moderate

	Feedback
1.	This is incorrect. Practicing despite tiredness and pain can cause additional injury.
2.	This is correct. Stretching exercises can prevent injury for athletes.
3.	This is incorrect. Sitting out of practice provides rest, but it does not address likely causes of the condition.
4.	This is correct. Shoes and equipment must fit properly and be stable to assist in preventing injury.
5.	This is correct. Proper body mechanics assist to prevent injury in athletic activities.

PTS: 1

CON: Mobility

Chapter 21 Dermatological Diseases

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. The clinical nursing instructor gives a lecture regarding maintenance of skin integrity while caring for a hospitalized child. Later, one of the nursing students is caring for a hospitalized child. Which action by the nursing student demonstrates that the teaching was effective?
1. Changes the location of the pulse oximeter every 2 hours
 2. Trims the infant's fingernails and toenails
 3. Scrubs the infant vigorously with every diaper change
 4. Holds the infant in a rocking chair for an hour
- _____ 2. A school nurse is examining a third-grade student complaining of a small lump on her head. After parting the hair, it is evident that a tick has embedded into the scalp. Which nursing intervention should the school nurse take?
1. Apply mineral oil to the site of attachment and wait for the tick to back out.
 2. Squeeze the tick and pull it off with bare fingers.
 3. Spray DEET directly on to the child's skin where the tick is embedded.
 4. Use tweezers or forceps to remove the tick, pulling up with steady pressure.

- _____ 3. An 86-month-old child is brought to the well-baby clinic for her immunizations. The nurse observes that the child's extremities are bright red, with a few blisters. The nurse questions the mother about the extent of sun exposure and determines that the child has a second-degree sunburn. After administering the immunization, the nurse provides education about sunburn prevention to the infant's mother. Which statement by the mother indicates that she needs more teaching?
1. "I make sure to avoid the midday sun."
 2. "I apply sunscreen right after we get to the pool."
 3. "At the pool, we do not sit close to the water."
 4. "I will try to buy clothes that have SPF protection."
- _____ 4. An eighth-grade student appears in the school infirmary with a skin manifestation that itches. He states that it began on the way to school after he took a shortcut through a field. The nurse performs a skin assessment and finds areas of erythema with streaks and patches on both legs. She determines that he is exhibiting signs of contact dermatitis. What is the first action that the nurse should take?
1. Alert the child's parents.
 2. Apply cool, wet compresses.
 3. Wash the area thoroughly with soap and water.
 4. Contact medical help by notifying the child's health-care provider.
- _____ 5. A 15-year-old male is very distressed about the skin problems on his face, neck, and back. He has whiteheads on his face and some blackheads. The nurse educates him about acne and designs a plan of care to use to treat and prevent skin eruptions. Which statement should the nurse include when teaching about skin care?
1. Chocolate should be eliminated from the diet.
 2. It is important to use oil-based lotions.
 3. Squeezing whiteheads should be done only after a thorough cleansing.
 4. Keep hair clean and off the face.
- _____ 6. A 7-year-old with a myelomeningocele is paralyzed from the waist to her feet. She is either in bed or in a chair day and night. A physical evaluation is performed regularly to check skin integrity. While conducting a skin assessment, the nurse finds a partial-thickness skin loss with exposed dermis on the sacrum. How does the nurse classify this pressure ulcer on the assessment form?
1. First-degree pressure ulcer
 2. Second-degree pressure ulcer
 3. Deep tissue pressure ulcer
 4. Unstageable ulcer
- _____ 7. A 6-month-old infant presents to the clinic with a diaper rash. The nurse observes a well-demarcated rash and satellite lesions on buttocks and legs. Based on the assessment, which treatment will the nurse recommend?
1. An antiviral medication
 2. Baby powder application at each diaper change
 3. An ointment containing nystatin (Mycostatin)
 4. A hydrocortisone ointment

- _____ 8. A 15-month-old toddler has a strange rash on his nose and mouth. It contains papules and vesicles and has a honeylike, glazed look. The toddler has a history of eczema, but the nurse differentiates the features of the presenting rash and explains that he has developed a secondary infection. What is the treatment of choice for this type of dermatological disease?
1. Bath the skin daily with hydrogen peroxide.
 2. Apply Caladryl lotion to dry the lesions.
 3. Use an antiviral lotion as ordered for 7 to 10 days.
 4. Administer topical antibiotics as ordered until the skin is clear.
- _____ 9. An adolescent is brought to the health-care provider's office for evaluation of an arm lesion that is swollen, red, and warm. There is also a fluid weeping from the wound. The nurse should anticipate which test will most likely be ordered?
1. X-ray of the extremity
 2. Culture and sensitivity of the fluid
 3. Comprehensive metabolic panel (CMP)
 4. Skin biopsy
- _____ 10. A teenager is being cared for in the emergency department after being trapped in a garage fire at her home. Partial-thickness burns are present on the side of the face, as well as on both arms. Which nursing action is the highest priority?
1. Apply cool compresses to the burns.
 2. Remove all clothing from the patient.
 3. Give pain medication as ordered.
 4. Give 100% humidified oxygen.
- _____ 11. A 5-year-old child is brought to the urgent care center for treatment of a bite. The nurse asks what happened, and the parent explains that the child disturbed their pet cat while it was sleeping. The cat was started and bit the child. Which intervention should the nurse perform first?
1. Question the mother about the cat's rabies vaccination status.
 2. Wash the wound with large amounts of sterile saline under pressure.
 3. Apply a butterfly adhesive dressing.
 4. Determine when the child last received a tetanus immunization.
- _____ 12. A teenager suffered second-degree burns of the face and hands while at a campfire that burned out of control. Emergency treatment is provided, and the patient is then admitted to a burn treatment center. Which nursing action should the nurse perform first?
1. Place a nasogastric tube for feeding.
 2. Monitor intake and output.
 3. Check wound for infection.
 4. Maintain adequate airway and oxygen supply.
- _____ 13. A junior high school student is cooking in the home economics room when some oil splashes on her fingers and hand. She immediately rinses her hand with cool tap water. The skin is not broken, but soon, a blister begins to form. The teacher sends the student to the school nurse for evaluation and treatment. Which nursing intervention is most appropriate?
1. Apply ice to the burned area for 20 minutes.
 2. Soak the burn in cool water and apply aloe vera cream.
 3. Puncture the blister with a sterile needle.

4. Apply baby oil and wrap with a sterile dressing.

- _____ 14. Parents of a toddler with eczema are concerned about alleviating discomfort from itching and preventing infection, since constant scratching has been an issue. They ask the nurse what measures they can take to care for their child. The nurse creates a teaching sheet to provide information regarding the care of a patient with eczema. Which concept should the nurse include on the teaching?
1. Maintain hydration of the skin.
 2. Bathe frequently with warm to hot water using mild soaps.
 3. Apply cool, wet compresses four times daily.
 4. Avoid use of fabric softeners.

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. The nurse educator of a burn unit is developing a counseling module for an adolescent who was burned in a house fire. Which of the following should be included in the discharge counseling for the adolescent and the adolescent's parents? *Select all that apply.*

1. Feelings of anxiety, guilt, and depression may be experienced.
2. Post-traumatic stress declines over the first 2 to 3 months.
3. Post-traumatic stress is often at its worse immediately after the incident.
4. There may be a lack of support groups in which to participate.
5. Patients may have body image disturbance 2 years after the injury.

- _____ 16. The nurse is providing care for an 8-year-old child who was treated for a cat bite. Before discharging the child home, the nurse plans a play session, with the parents included, that focuses on safety around animals and pets in the home. What points would the nurse present during the session? *Select all that apply.*

1. Avoidance of animals while the wound is healing
2. Importance of not disturbing animals while they are sleeping or eating
3. Diseases that pets can transmit to humans, such as tetanus
4. Supervision of young children and pets
5. Stopping antibiotics when the redness and swelling go down

- _____ 17. The school nurse is educating preschool parents about the management of lice infestation, which has currently become a problem among the students. Which recommendations should the nurse include in the caregiver education? *Select all that apply.*

1. Use a pediculicide shampoo every day and let the hair air dry.
2. Use a fine-toothed comb to remove nits from wet hair.
3. Apply mayonnaise to the hair and cover with a plastic cap.
4. Wash clothing and bedding in hot soapy water and dry on hot cycle.
5. Wash hair with a regular shampoo daily and blow dry with a cool setting.

- _____ 18. During an outside gym class, a high school student was stung on the lip by a bee. After determining that the student was not allergic to bee stings, which treatment should the school nurse recommend to soothe the sting? *Select all that apply.*

1. Apply vinegar and oil drops to the site of the sting.

2. Create a paste of baking soda and water and apply to the lip.
3. Wash the lip with cool water and apply ice to the sting.
4. Take an over-the-counter antihistamine.
5. Apply a hot pack to the lip.

_____ 19. A high school freshman visits the school nurse to ask how she can get rid of the warts on her hands. She is embarrassed what her friends will think if they notice them, and she fears spreading them to others. Which of the following should the nurse include in the plan of care? *Select all that apply.*

1. Plantar or palmar warts are caused by human papillomavirus (HPV).
2. Hand hygiene should be performed after touching warts.
3. Old warts can be picked or scraped off as they become dry.
4. Cryosurgery is an option once she reaches age 18.
5. A patch with salicylic acid (Duofilm) is helpful to dissolve warts.

_____ 20. The nurse is preparing a home-care plan for a child whose leg wound cultured positive for MRSA. Which information should the nurse include? *Select all that apply.*

1. Do not share towels or any personal care items.
2. Wash hands with soap, rubbing for 30 seconds before rinsing.
3. Line dry clothes after washing.
4. Disinfect surfaces with bleach effective for MRSA.
5. Leave the wound open to air.

Chapter 21 Dermatological Diseases

Answer Section

MULTIPLE CHOICE

1. ANS: 1

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 1. Identify causes and precipitating factors of skin conditions and skin injuries seen in children and adolescents.

Page: 490

Heading: Critical Component > Skin Integrity and the Hospitalized Child

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Evaluation [Evaluating]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is correct. Newborns and infants have skin issues because their skin is thinner and weaker; therefore, skin probes must be carefully monitored for skin breakdown, and changing the site is recommended every 2 to 6 hours.
2	This is incorrect. Trimming of infants' fingernails is not recommended.
3	This is incorrect. Scrubbing the infant's skin is not advised. It can contribute to irritation and skin breakdown.
4	This is incorrect. Although it is helpful to change an infant's position to prevent skin breakdown, holding an infant for 1 hour will not prevent pressure from the pulse oximeter, which can cause injury to the thin skin.

PTS: 1

CON: Skin Integrity

2. ANS: 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 1. Identify causes and precipitating factors of skin conditions and skin injuries seen in children and adolescents.

Page: 502

Heading: Safe and Effective Nursing Care > Clinical Pearl > How to Remove Ticks

Integrated Processes: Nursing Process

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Skin Integrity

Difficulty: Moderate

	Feedback
1	This is incorrect. This procedure is not recommended. There is no evidence that this will prevent injury from the exposure to disease organisms.
2	This is incorrect. Squeezing is not recommended because the tick may not be removed intact. Also, tick feces can contain disease organisms.

3	This is incorrect. DEET should never be sprayed directly on the child's skin.
4	This is correct. The tick should be removed with tweezers. A steady pressure should be applied while pulling upward.

PTS: 1 CON: Skin Integrity

3. ANS: 2

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 1. Identify causes and precipitating factors of skin conditions and skin injuries seen in children and adolescents.

Page: 505

Heading: Disorders Related to Injuries > Sunburns > Caregiver Education > Prevention

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Evaluation [Evaluating]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is incorrect. No further teaching is needed. Infants older than 6 months and older children should not be exposed to direct sunlight between 10 a.m. and 4 p.m.
2	This is correct. Sunscreen should be applied 30 minutes before going out in the sun. This statement demonstrates that the mother needs more teaching.
3	This is incorrect. No further teaching is needed. Sunlight reflecting off of water can damage the skin.
4	This is incorrect. No further teaching is needed. Buying clothing with SPF protection can help prevent overexposure to damaging rays.

PTS: 1 CON: Skin Integrity

4. ANS: 3

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 5. Describe emergency care given for acute infections and infestations and various injuries to skin.

Page: 492

Heading: Dermatitis > Contact Dermatitis > Clinical Presentation > Caregiver Education > Home Care

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Evaluation [Evaluating]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is incorrect. This is not the priority action.
2	This is incorrect. Although this is a valid treatment, it is not the first action.

3	This is correct. Washing the exposed area with soap and water should be done immediately to remove the resin oils that are causing the irritation.
4	This is incorrect. Medical help is necessary only if the rash is extensive or involves the face.

PTS: 1 CON: Skin Integrity

5. ANS: 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 7. Develop a home-based plan of care for a child or adolescent with a chronic skin condition.

Page: 494

Heading: Acne > Caregiver Education > Home Care

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Skin Integrity

Difficulty: Moderate

	Feedback
1	This is incorrect. Changes to the diet will not affect acne.
2	This is incorrect. Successful management of acne includes washing the skin with a gentle cleanser and avoiding oil-based lotions, creams, or makeup.
3	This is incorrect. Whiteheads (closed comedones) should never be squeezed. Squeezing them can cause inflammation, infection, and scarring.
4	This is correct. Hair should be kept clean and off the face.

PTS: 1 CON: Skin Integrity

6. ANS: 2

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 3. Describe the clinical presentation of acquired skin diseases, skin infections and infestations, and various injuries to the skin.

Page: 490

Heading: Acquired Disorders > Pressure Ulcers > Clinical Presentation of Pressure Ulcers

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Analysis [Analyzing]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is incorrect. Stage 1 is seen as nonblanchable erythema of intact skin.
2	This is correct. Stage 2 is manifested by a partial-thickness skin loss with exposed dermis.
3	This is incorrect. Deep tissue injury is manifested as a persistent nonblanchable deep red, maroon, or purple discoloration.

4	This is incorrect. Unstageable is obscured by full-thickness skin and tissue loss.
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PTS: 1 CON: Skin Integrity

7. ANS: 3

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 3. Describe the clinical presentation of acquired skin diseases, skin infections and infestations, and various injuries to the skin.

Page: 491

Heading: Dermatitis > Clinical Presentation of Diaper Dermatitis > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Skin Integrity

Difficulty: Moderate

	Feedback
1	This is incorrect. The rash is not viral. Viral infections are not well demarcated but rather present as a fine rash.
2	This is incorrect. Baby powder is not recommended due to possible respiratory inhalation. It would not be used to treat this rash.
3	This is correct. Mycostatin is the treatment of choice for a <i>Candida</i> infection. Infection with <i>Candida albicans</i> , a yeastlike fungus, results in candidiasis of the diaper area. Erythematous areas are well demarcated, and satellite lesions may be noted on the buttocks, legs, and abdomen.
4	This is incorrect. Hydrocortisone may be ordered for atopic rashes, but this rash is consistent with a <i>Candida</i> infections.

PTS: 1 CON: Skin Integrity

8. ANS: 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 4. Explain diagnostic and laboratory studies used for skin conditions.

Page: 495

Heading: Bacterial Infections > Impetigo > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Pharmacological and Parenteral Therapies

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is incorrect. Bathing with hydrogen peroxide is not recommended.
2	This is incorrect. Caladryl is used to relieve the pruritus of contact dermatitis such as poison ivy.
3	This is incorrect. The superimposed infection is not viral.

4	This is correct. A topical antibiotic is the treatment of choice for impetigo. In severe cases, an oral antibiotic will be given in addition to the ointment. <i>Staphylococcus aureus</i> or group A beta-hemolytic streptococcus are causative factors for impetigo.
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PTS: 1 CON: Infection

9. ANS: 2

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 4. Explain diagnostic and laboratory studies used for skin conditions.

Page: 497

Heading: Bacterial Infections > Cellulitis > Diagnostic Testing

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is incorrect. An x-ray would be appropriate if there was a need to noninvasively see below the skin's surface. For example, it allows visualization of bones, tumors, and foreign bodies. It would not be of diagnostic value in this scenario.
2	This is correct. Diagnostic testing of a skin lesion includes culture and sensitivity tests on any drainage present, which would identify the organism causing the symptoms of cellulitis.
3	This is incorrect. A CMP is used to assess a person's metabolism and chemical balance. It would not be useful in this scenario.
4	This is incorrect. A skin biopsy provides information about the cellular growth of skin cells. It is not typically the first step.

PTS: 1 CON: Infection

10. ANS: 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 5. Describe emergency care given for acute infections, bites, and burns to the skin.

Page: 505

Heading: Disorders Related to Injuries > Minor and Major Burns > Nursing Interventions > Emergency Care

Client Need: Safe and Effective Care Management: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Skin Integrity

Difficulty: Moderate

	Feedback
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1	This is incorrect. Applying cool, sterile, normal saline compresses to cool and protect the skin is done after airway, breathing (oxygen), and circulation is established.
2	This is incorrect. Although it is important to remove clothing that is burned, hot in temperature, or contaminated with chemicals, this is not the highest priority.
3	This is incorrect. Although pain medication should be administered as needed, this is not the priority action.
4	This is correct. The priority action is establishing airway, breathing (oxygen), and circulation. The nurse should give 100% oxygen if the individual has been burned severely or if the fire was in a confined space. A pulse oximeter is used to monitor oxygen saturation levels.

PTS: 1 CON: Skin Integrity

11. ANS: 2

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 5. Describe emergency care given for acute infections, bites, and burns to the skin.

Page: 503

Heading: Disorders Related to Injuries > Bites (Animal, Human) > Nursing Interventions

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection

Difficulty: Moderate

	Feedback
1	This is incorrect. Although it is important to ask about rabies vaccinations, this is not the first action.
2	This is correct. The nurse should give immediate wound care. Wounds may be irrigated under pressure with large amounts of sterile saline.
3	This is incorrect. Tissue adhesive dressings are not used for closing bite wounds because of reduced drainage and increased chance of infection.
4	This is incorrect. The nurse should check the child's immunization record to determine whether tetanus immunizations are up to date, but this is not the first action.

PTS: 1 CON: Infection

12. ANS: 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 6. Describe care in the hospital for a child or adolescent with an acute or chronic skin condition or with an injury to the skin.

Page: 506

Heading: Disorders Related to Injuries > Minor and Major Burns > Nursing Interventions > Acute Hospital Care

Integrated Processes: Nursing Process

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Evaluation [Evaluating]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is incorrect. A feeding tube may be ordered if the child cannot be fed orally or cannot maintain increased caloric requirement due to change in metabolic state. However, there is nothing in this scenario to suggest that the patient is unable to eat.
2	This is incorrect. Although monitoring intake and output is an important part of care, it is not the first nursing action.
3	This is incorrect. This is an ongoing assessment but is not the first nursing action.
4	This is correct. The nurse should assess for signs of respiratory distress: dyspnea, wheezing, stridor, tachypnea, decreased breath sounds, retractions. A pulse oximeter is used to monitor oxygen saturation levels. The nurse should administer oxygen or assist with endotracheal intubation as needed and keep the head of bed elevated.

PTS: 1 CON: Skin Integrity

13. ANS: 2

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 5. Describe emergency care given for acute infections, bites, and burns to skin.

Page: 508

Heading: Critical Component > First Aid for Burns

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Analysis [Analyzing]

Concept: Skin Integrity

Difficulty: Moderate

	Feedback
1	This is incorrect. The application of ice could cause additional injury.
2	This is correct. Soaking in cool water and applying aloe vera cream is the appropriate first aid for superficial burns. Do not use butter, oils, or ice on the burn. These may cause additional injury.
3	This is incorrect. Blisters should not be broken purposely. The intact blister forms a barrier against infection. With partial-thickness burns with blistering, the patient may soak the burn in cool water but should not break the blisters.
4	This is incorrect. Applying oil to a burn will cause additional injury by trapping in the heat.

PTS: 1 CON: Skin Integrity

14. ANS: 1

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 7. Develop a home-based plan of care for a child or adolescent with a chronic skin condition.

Page: 493 Heading: Atopic Dermatitis (Eczema) > Caregiver Education > Home Care

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Skin Integrity

Difficulty: Difficult

	Feedback
1	This is correct. The key to successful treatment is hydration of the skin.
2	This is incorrect. Someone with eczema should limit bathing to every other day. The water should be warm, not hot, and a mild soap should be used.
3	This is incorrect. Cool, wet compresses applied four times daily is the appropriate care for contact dermatitis, not eczema.
4	This is incorrect. Fabric softeners do not have to be eliminated completely. However, any fabric softeners used should be mild and fragrance free.

PTS: 1

CON: Skin Integrity

MULTIPLE RESPONSE

15. ANS: 1, 3, 5

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 9. Develop a teaching plan for caregivers of children or adolescents with skin conditions or injuries to the skin.

Page: 508

Heading: Evidenced-Based Practice > Burn Injuries and Post-traumatic Stress

Integrated Processes: Caring

Client Need: Psychosocial Integrity

Cognitive Level: Synthesis [Creating]

Concept: Self

Difficulty: Difficult

	Feedback
1.	This is correct. Children and families who have suffered from burn injuries may experience anxiety, guilt, depression, and post-traumatic stress disorder (PTSD).
2.	This is incorrect. Improved pain management and support of the family and child decreases symptoms and likelihood of long-term PTSD. However, patients often have psychological stress 2 years after a burn injury.
3.	This is correct. Most patients will show symptoms of PTSD, especially early after the initial injury.

4.	This is incorrect. It is important for nurses to offer emotional support to children and their families following burn injuries and to refer both children and families to therapy or to support groups as part of their discharge planning.
5.	This is correct. Patients often have psychological stress and body image disturbances 2 years after a burn injury.

PTS: 1 CON: Self

16. ANS: 2, 3, 4

Chapter: Chapter 21 Dermatological Disorders

Chapter Learning Objective: 9. Develop a teaching plan for caregivers of children or adolescents with skin conditions or injuries.

Page: 504

Heading: Bites (Animal and Human) > Care Giver Education > Home Care > Prevention

Integrated Processes: Teaching and learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Synthesis [Creating]

Concept: Infection

Difficulty: Moderate

	Feedback
1.	This is incorrect. The child can interact with animals while healing. The nurse should, however, teach caregivers to watch for signs of cellulitis or localized infection: erythema, warmth, red streaking, edema, purulent drainage, fever, increased pain.
2.	This is correct. Children should be taught to treat pets with respect and kindness.
3.	This is correct. The nurse should explain that some diseases can be transmitted from animals to humans. The nurse should also explain the importance of keeping tetanus immunizations up to date and keep rabies vaccinations up to date for pets.
4.	This is correct. Young children should not be left alone with a pet.
5.	This is incorrect. The entire course of oral antibiotics should be completed.

PTS: 1 CON: Infection

17. ANS: 2, 3, 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 8. Discuss alternate therapies used for various skin conditions or injuries to the skin.

Page: 499

Heading: Disorders Related to Contact with Insects > Pediculosis Capitis (Head Lice) > Caregiver Education > Complementary and Alternative Therapies

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection

Difficulty: Moderate

	Feedback
1.	This is incorrect. The nurse should assess for history of asthma or allergies before recommending pediculicide shampoos. Nix (permethrin or pyrethrin) is contraindicated for children with ragweed allergies. These products may cause an allergic reaction or an exacerbation of asthma. If use is safe for the individual, it is only used every 7 to 10 days.
2.	This is correct. A fine-toothed comb can help remove nits from the hair.
3.	This is correct. Application of mayonnaise, olive oil, or petroleum jelly has been recommended to suffocate the lice. Parents are instructed to cover the child's hair with the oil-based product, cover with a shower cap overnight, and shampoo the next morning.
4.	This is correct. Washing clothing and bedding in hot soapy water and drying on a hot cycle is recommended. Hot air with the removal of existing nits additionally has shown some promise in studies.
5.	This is incorrect. Regular shampoo will not kill the insects. A cool blow dryer will not affect them either. Lice are killed by a pesticide and heat.

PTS: 1 CON: Infection

18. ANS: 2, 3, 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 8. Discuss alternate therapies used for various skin conditions or injuries to the skin.

Page: 501

Heading: Insect Bites and Stings > Caregiver Education > Home Care > Complementary and Alternative Therapies

Integrated Processes: Nursing Process

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Medication

Difficulty: Moderate

	Feedback
1.	This is incorrect. Vinegar and oil will not draw out the sting.
2.	This is correct. A baking soda paste made by mixing baking soda with cold water may be soothing to a sting site. This is considered a complementary and alternative therapy.
3.	This is correct. The immediate care of a sting is to wash the site of the sting. To reduce pain and swelling, the patient can apply ice to the sting.
4.	This is correct. The child can be given an over-the-counter oral antihistamine such as Benadryl.
5.	This is incorrect. A hot pack is contraindicated. It will not relieve pain and swelling.

PTS: 1 CON: Medication

19. ANS: 1, 2, 5

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 9. Develop a teaching plan for caregivers of children or adolescents with skin conditions or injuries to the skin.

Page: 497

Heading: Viral Infections > Papillomavirus (Verruca or Warts) > Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Management of Care

Cognitive Level: Analysis [Analyzing]

Concept: Infection

Difficulty: Moderate

	Feedback
1.	This is correct. Plantar warts on the soles of the feet or the palms of the hands are due to HPV-1.
2.	This is correct. Warts may be mildly contagious. Hand hygiene is critical.
3.	This is incorrect. The patient should be advised to avoid scratching or picking at warts. This may lead to a secondary bacterial infection and scarring.
4.	This is incorrect. Cryosurgery may be required for warts that are resistant to other therapy. This treatment is an option for adolescents.
5.	This is correct. Salicylic acid in the form of liquid, gel, or a patch (Duofilm) may be effective in removing warts.

PTS: 1

CON: Infection

20. ANS: 1, 2, 4

Chapter: Chapter 21 Dermatological Diseases

Chapter Learning Objective: 9. Develop a teaching plan for caregivers of children or adolescents with skin conditions or injuries to the skin.

Page: 496

Heading: Safe and Effective Nursing Care > Promoting Safety

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Nursing Care Environment: Safety and Infection Control

Cognitive Level: Synthesis [Creating]

Concept: Infection

Difficulty: Difficult

	Feedback
1.	This is correct. Towel and personal care items can be contaminated and spread MRSA to other family members.
2.	This is correct. Careful hand hygiene is essential to prevent infection.
3.	This is incorrect. Use of a clothes dryer is recommended. The high temperature is more effective to eliminate MRSA contamination.
4.	This is correct. Surfaces need to be disinfected with bleach. Labels should be checked for effectiveness against MRSA and should include an Environmental Protection Agency (EPA) registration number.
5.	This is incorrect. Wounds, boils, and any draining sores should be covered.

PTS: 1 CON: Infection

Chapter 22 Communicable Diseases

Multiple Choice

Identify the choice that best completes the statement or answers the question.

- _____ 1. A parent brings a 2-month-old baby in for a routine wellness examination. Which vaccination should the nurse prepare to administer to this patient?
1. Hepatitis A
 2. Hepatitis B
 3. Inactivated poliovirus
 4. Measles, mumps, rubella
- _____ 2. The nurse learns that a 16-year-old patient has not received the human papilloma virus vaccine (HPV-Gardasil). Which should the nurse explain to the patient at this time about the vaccination?
1. "The complete vaccine can be given today."
 2. "You can wait a few years before needing the vaccination."
 3. "A dose can be given today with the final dose in 6 months."
 4. "A dose should be given today with follow-up doses in 1 to 2 months, and the last in 6 months."
- _____ 3. A new mother contacts the health-care provider's office to ask for guidance regarding a change in the infant's health. For which reason should the nurse direct the mother to seek immediate medical attention?
1. Difficult to arouse
 2. Wetting six diapers a day
 3. Breastfeeding every 3 hours
 4. Sleeping for several hours in the afternoon
- _____ 4. A parent brings his school-age child to the clinic for a rash that developed over the face, trunk, and extremities. Which question should the nurse ask the parent when assessing the patient?
1. "Has the child been nauseated or has the child vomited?"
 2. "How often is the child given acetaminophen, or Tylenol?"
 3. "Has the child eaten any food that was not properly cooked?"
 4. "Was the child exposed to anyone with a respiratory infection?"
- _____ 5. A school-age patient with rubella is placed in droplet precautions. Which action should the nurse take when implementing these precautions?
1. Use a mask with a HEPA filter.
 2. Instruct to cough into the hands.
 3. Wear a mask when providing care.
 4. Assign to a negative air pressure room.
- _____ 6. A parent arrives at the family clinic seeking medical attention for his 14-year-old child. The child has a fever, malaise, nausea, and abdominal pain. Which finding should indicate to the nurse that the patient is experiencing hepatitis A?
1. Elevated serum bilirubin

2. Greater than 10% atypical lymphocytes
3. Presence of the virus in nasal secretions
4. Positive Paul-Bunnell heterophile antibody test

- _____ 7. The nurse is caring for a school-age patient diagnosed with hepatitis A. For which reason should the nurse begin implementing contact precautions for this patient?
1. Incontinent of feces
 2. Evidence of dehydration
 3. Development of dark urine
 4. Severe nausea and vomiting
- _____ 8. The nurse is caring for a 15-year-old patient who has been diagnosed with mononucleosis. The patient asks how the infection was obtained. Which information should the nurse include when responding to the patient?
1. Drinking from the cup of a person with the infection
 2. Eating contaminated food
 3. Eating meat that is undercooked
 4. Inhaling airborne germs, such as after someone coughs
- _____ 9. An adolescent develops a fever, cough, and a maculopapular rash. Which question should the nurse ask when completing the health history with this patient?
1. "Did you receive any vaccinations recently?"
 2. "Do you usually eat cold pizza left on the counter overnight?"
 3. "Do you forget to wash your hands after using the bathroom?"
 4. "Have you been around anyone with a cold over the last 3 weeks?"
- _____ 10. An older patient with weeping lesions caused by herpes zoster asks if a trip to visit small grandchildren can still occur this upcoming weekend. Which should the nurse say to the patient in response?
1. "There is no reason for you to cancel your trip."
 2. "You should not travel until all of the lesions are healed."
 3. "Be sure to keep the lesions covered until they crust over."
 4. "As long as the lesions are kept uncovered, you can travel without any issues."
- _____ 11. A nurse is caring for a school-age child who has inflammation of the right eye. Which finding, if present, would indicate to the nurse that the child has bacterial conjunctivitis rather than viral conjunctivitis?
1. Eye swelling
 2. Red conjunctiva
 3. Watery discharge
 4. Purulent drainage
- _____ 12. The nurse is providing care to an 8-week-old infant who has symptoms of an upper respiratory infection. Which assessment finding should indicate to the nurse that the patient is experiencing pertussis?
1. An expiratory wheeze
 2. A "whoop" sound after coughing
 3. Three wet diapers each day

4. Bulging fontanelles

- _____ 13. A parent brings her school-age child to the physician's office for evaluation of a sore throat, fever, headache, and fine red rough rash over both arms and abdomen. For which health problem should the nurse plan care for this patient?
1. Croup
 2. Tonsillitis
 3. Epiglottitis
 4. Scarlet fever with strep throat
- _____ 14. A school-age child is brought to the clinic to be evaluated for a headache and stomachache. For which reason should the nurse prepare the patient to have a rapid strep test?
1. Runny nose
 2. Productive cough
 3. Strawberry tongue
 4. Jaundiced conjunctiva

Multiple Response

Identify one or more choices that best complete the statement or answer the question.

- _____ 15. A school-age child is diagnosed with hand, foot, and mouth disease. Which should the nurse instruct the parent to do when caring for the patient at home? *Select all that apply.*
1. Provide bland foods and fluids.
 2. Offer generous amounts of oral fluids.
 3. Wash clothes with hot soapy water and bleach.
 4. Use acetaminophen or ibuprofen for pain control.
 5. Instruct to flush the mouth with an alcohol-based mouthwash.
- _____ 16. The nurse is visiting the home of a patient with hepatitis B who is 1 week postpartum. Which information should the nurse include when teaching this patient? *Select all that apply.*
1. "Hepatitis B is only transmitted through sexual contact."
 2. "The baby may have contracted hepatitis B through the pregnancy."
 3. "There are no medications appropriate for children with hepatitis B."
 4. "After the baby has received treatment, there is no need for follow-up."
 5. "The baby should have received the hepatitis B immunization and hepatitis B immune globulin."
- _____ 17. An adolescent is diagnosed with mononucleosis. Which teaching should the nurse provide to the parents when providing care at home? *Select all that apply.*
1. Encourage ample fluids.
 2. Avoid all contact sports for 6 to 8 weeks.
 3. Encourage rest with quiet activities and play.
 4. Limit the amount of caloric intake until recovered.
 5. Provide ibuprofen or acetaminophen for elevated temperature.
- _____ 18. A 17-year-old patient with influenza tells the nurse, "I can't believe I got sick. I've been taking vitamin C, vitamin D, echinacea, and some other herbal remedies to boost my immune system." Which should the nurse say in response to this patient? *Select all that apply.*

1. "You probably weren't taking enough of the supplements."
2. "Those supplements help with focus and attention, not disease prevention."
3. "The safe use of supplements and herbs has not been determined."
4. "The vitamins you were taking may have reduced the effectiveness of the herbs."
5. "The effectiveness of supplements and herbs against influenza has not been determined."

_____ 19. The nurse is writing teaching material about caring for a child with influenza. Which information should the nurse include? *Select all that apply.*

1. Zanamivir is only prescribed for children over 13 years old.
2. Only one antiviral has been approved by the FDA as of 2017.
3. Tamiflu can be given to children older than one on weight-based dosing.
4. Manifestations of the flu include fever, chills, headache, and myalgia (aching).
5. Medications for influenza must be given within 48 hours of the onset of symptoms.

_____ 20. A parent with a school-age child with mumps asks for information about the illness and treatment. Which information should the nurse provide? *Select all that apply.*

1. "There is no need to isolate the child."
2. "There are no laboratory tests to detect the mumps virus."
3. "Sometimes the complications of mumps involve the testicles."
4. "Mumps are seen by observation of swelling around the ears and jaw."
5. "Provide food and drink that is nonirritating to the mouth and surrounding glands."

Chapter 22 Communicable Diseases

Answer Section

MULTIPLE CHOICE

1. ANS: 2

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 5. Outline recommended immunizations for infants, children, and adolescents.

Page: 514

Heading: Immunizations > Immunizations From Birth to Age 18 Years > Hepatitis B

Integrated Processes: Nursing Process: Planning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Health Promotion; Safety

Difficulty: Moderate

	Feedback
1	This is incorrect. The hepatitis A vaccine is given to all children 12 months and older.
2	This is correct. The hepatitis B vaccine is provided to all newborns. Three doses of hepatitis B are given before age 2 years: at birth, at 1 to 2 months of age, and at 9 to 12 months of age.
3	This is incorrect. The inactivated poliovirus IPV is given in a series of four doses: at 2, 4, and 6 to 18 months as well as 4 to 6 years.
4	This is incorrect. The minimum age for receiving the measles, mumps, and rubella immunization is 12 months.

PTS: 1

CON: Health Promotion | Safety

2. ANS: 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 5. Outline recommended immunizations for infants, children, and adolescents.

Pages: 520–521

Heading: Immunizations > Immunizations From Birth to Age 18 Years > Human Papillomavirus (HPV-Gardasil)

Integrated Processes: Teaching and Learning

Client Need: Health Promotion and Maintenance

Cognitive Level: Application [Applying]

Concept: Health Promotion; Safety

Difficulty: Difficult

	Feedback
1	This is incorrect. The vaccination for HPV is given as either two doses or three doses, depending on the patient's age.

2	This is incorrect. The HPV vaccination is recommended to be given at 11 to 12 years of age (minimum age is 9 years old) in a two-dose series with a minimum of 6 months between dosing.
3	This is incorrect. If vaccination is not begun by age 15, the patient requires a three-dose series: first injection, second injection in 1 or 2 months, and the third injection 6 months after the first.
4	This is correct. Because the patient is over age 15, a first dose should be given now, with follow-up doses in 1 to 2 months, and the final dose in 6 months.

PTS: 1 CON: Health Promotion | Safety

3. ANS: 1

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 8. Provide emergency nursing care of children with complications of communicable diseases.

Page: 524

Heading: Critical Component: Caregiver Education

Integrated Processes: Caring

Client Need: Health Promotion and Maintenance

Cognitive Level: Analysis [Analyzing]

Concept: Health Promotion; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. A change in responsiveness or being difficult to arouse could indicate a change in oxygenation or perfusion. The infant should be seen immediately for medical attention.
2	This is incorrect. Wetting six diapers a day is an expected behavior for an infant.
3	This is incorrect. An infant feeding every 3 hours is expected behavior.
4	This is incorrect. Infants have a great need for sleep. Sleeping for several hours in the afternoon is expected behavior.

PTS: 1 CON: Health Promotion | Patient-Centered Care

4. ANS: 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 1. Describe the source and transmission of various communicable diseases.

Page: 524

Heading: Viral Communicable Diseases > Erythema Infectiosum (Fifth Disease) > Disease Process

Integrated Processes: Nursing Process: Assessment

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. This patient's symptoms suggest fifth disease, which is not associated with nausea and vomiting.
2	This is incorrect. Acetaminophen use would not result in these symptoms.
3	This is incorrect. Eating improperly prepared food would not cause these symptoms.
4	This is correct. These symptoms are indicative of fifth disease, which is transmitted through respiratory secretions. Assessing whether the patient was exposed to anyone with a respiratory infection would be appropriate.

PTS: 1 CON: Infection | Patient-Centered Care

5. ANS: 3

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 7. Plan appropriate isolation techniques to prevent the spread of communicable disease in the hospital.

Page: 525

Heading: Table 22-2: Isolation Guidelines and Standard Precautions

Integrated Processes: Nursing Process: Implementation

Client Need: Safe and Effective Care Environment: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. A mask with a HEPA filter is needed when caring for a patient with tuberculosis.
2	This is incorrect. The patient should be instructed to cover the mouth with a tissue when coughing, properly dispose of the used tissue, and wash the hands.
3	This is correct. For droplet precautions, health-care providers should wear a mask.
4	This is incorrect. A negative air pressure room is needed for the patient with tuberculosis.

PTS: 1 CON: Infection | Patient-Centered Care

6. ANS: 1

Chapter: Chapter 22 Communicable diseases

Chapter Learning Objective: 4. Explain diagnostic and laboratory studies used with communicable diseases.

Page: 526

Heading: Viral Communicable Diseases > Hepatitis A > Diagnostic Testing

Integrated Processes: Nursing Process: Assessment

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Difficult

	Feedback
1	This is correct. Abnormal laboratory values that indicate the presence of hepatitis A include an elevated serum bilirubin level.
2	This is incorrect. The presence of greater than 10% of atypical lymphocytes is a finding associated with mononucleosis, not hepatitis A.
3	This is correct. The presence of the virus in nasal secretions is associated with influenza, not hepatitis A.
4	This is incorrect. A positive Paul-Bunnell heterophile antibody test is a finding associated with mononucleosis, not hepatitis A.

PTS: 1 CON: Infection | Patient-Centered Care

7. ANS: 1

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 6. Describe care in the hospital for an infant, child, or adolescent with a communicable disease.

Page: 527

Heading: Viral Communicable Diseases > Hepatitis A > Nursing Interventions

Integrated Processes: Nursing Process: Implementation

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Apply [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. A patient with hepatitis A should be placed in contact precautions in the event of fecal incontinence.
2	This is incorrect. Dehydration is not a reason to place the patient in contact precautions.
3	This is incorrect. Development of dark urine is not a reason to place the patient in contact precautions.
4	This is incorrect. Severe nausea and vomiting is not a reason to place the patient in contact precautions.

PTS: 1 CON: Infection | Patient-Centered Care

8. ANS: 1

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 1. Describe the source and transmission of various communicable diseases.

Page: 528

Heading: Viral Communicable Diseases > Mononucleosis > Disease Process

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is correct. The virus that causes mononucleosis is transmitted through person-to-person contact, sharing personal objects such as cups or toothbrushes, and through saliva.
2	This is incorrect. Contaminated food does not cause mononucleosis.
3	This is incorrect. Mononucleosis is not transmitted through undercooked meat.
4	This is incorrect. Mononucleosis is not airborne; that is, it is not transmitted through respiratory droplets or respiratory secretions.

PTS: 1 CON: Infection | Patient-Centered Care

9. ANS: 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 2. Outline critical components of history for a child or adolescent with a communicable disease.

Page: 533

Heading: Viral Communicable Diseases > Varicella Zoster (Chickenpox) > Disease Process

Integrated Processes: Nursing Process: Assessment

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Receiving a vaccination does not cause the symptoms of a fever, cough, and maculopapular rash.
2	This is incorrect. Foodborne viruses do not cause a cough, fever, or maculopapular rash.
3	This is incorrect. Infections transmitted through the fecal-oral route do not cause a fever, cough, and maculopapular rash.
4	This is correct. Foodborne viruses manifest with gastrointestinal distress. Chickenpox is a respiratory-borne viral disease with an incubation period of about 21 days.

PTS: 1 CON: Infection | Patient-Centered Care

10. ANS: 3

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 1. Describe the source and transmission of various communicable diseases.

Page: 534

Heading: Critical Component > Shingles (Herpes Zoster)

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care
Difficulty: Moderate

	Feedback
1	This is incorrect. Shingles lesions must be covered to prevent spread. It is contagious until all lesions are crusted over.
2	This is incorrect. The patient can travel as long as the lesions are covered until they crust over.
3	This is correct. The lesions of shingles must be covered until they crust over to prevent the transmission of chickenpox to the grandchildren.
4	This is incorrect. Weeping lesions caused by herpes zoster must be covered to prevent transmission of chickenpox to others.

PTS: 1 CON: Infection | Patient-Centered Care

11. ANS: 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 3. Describe the clinical presentation of major communicable diseases.

Page: 535

Heading: Bacterial Communicable Diseases > Conjunctivitis (Pinkeye) > Clinical Presentation

Integrated Processes: Nursing Process: Assessment

Client Need: Safe and Effective Care Environment; Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Eye swelling or edema occurs with both bacterial and viral conjunctivitis.
2	This is incorrect. Pink or red conjunctiva occurs in both bacterial and viral conjunctivitis.
3	This is incorrect. Viral conjunctivitis causes a watery discharge from the effected eye.
4	This is correct. Purulent drainage is a symptom of bacterial conjunctivitis.

PTS: 1 CON: Infection | Patient-Centered Care

12. ANS: 2

Chapter: Chapter 22 Viral Communicable Diseases

Chapter Learning Objective: 3. Describe the clinical presentation of major communicable diseases.

Page: 536

Heading: Bacterial Communicable Diseases > Pertussis (Whooping Cough) > Clinical Presentation

Integrated Processes: Nursing Process: Assessment

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Analysis [Analyzing]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Expiratory wheezing is a symptom of asthma.
2	This is correct. The characteristic “whoop” that occurs in pertussis occurs after coughing.
3	This is incorrect. For young infants, fewer than five or six wet diapers in 24 hours indicates dehydration.
4	This is incorrect. Bulging fontanelles indicates overhydration, which is unrelated to pertussis.

PTS: 1 CON: Infection | Patient-Centered Care

13. ANS: 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 3. Describe the clinical presentation of major communicable diseases.

Page: 537

Heading: Bacterial Communicable Diseases > Strep Throat/Scarlet Fever > Clinical Presentation

Integrated Processes: Nursing Process: Planning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. Croup causes a harsh “barky” cough and does not cause a rash.
2	This is incorrect. Tonsillitis does not cause a fine red rough rash.
3	This is incorrect. Epiglottitis does not cause a fine red rough rash.
4	This is correct. Strep throat and a fine red rough rash are main symptoms of scarlet fever.

PTS: 1 CON: Infection | Patient-Centered Care

14. ANS: 3

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 4. Explain diagnostic and laboratory studies used with communicable diseases.

Page: 537

Heading: Bacterial Communicable Diseases > Strep Throat/Scarlet Fever > Diagnostic Testing

Integrated Processes: Nursing Process: Assessment

Client Need: Physiological Integrity: Reduction of Risk Potential

Cognitive Level: Analysis [Analyzing]

Concept: Infection; Patient-Centered Care

Difficulty: Moderate

	Feedback
1	This is incorrect. A rapid strep test would not be used to assess for a runny nose.
2	This is incorrect. A rapid strep test is not done because of a productive cough.

3	This is correct. A rapid strep test would be done if a strawberry tongue is assessed, an indication of strep throat or scarlet fever.
4	This is incorrect. A rapid strep test would not be done for jaundiced conjunctiva.

PTS: 1

CON: Infection | Patient-Centered Care

MULTIPLE RESPONSE

15. ANS: 1, 2, 4

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 9. Develop a home-based plan of care for a child or adolescent with a communicable disease.

Page: 526

Heading: Viral Communicable Diseases > Hand, Foot, and Mouth Disease > Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Health Promotion

Difficulty: Difficult

	Feedback
1.	This is correct. The patient with hand, foot, and mouth disease has oral lesions. The patient should consume bland foods and fluids to reduce pain when eating and drinking.
2.	This is correct. The patient with hand, foot, and mouth disease should have ample fluids to ensure adequate hydration.
3.	This is incorrect. Clothes do not need to be washed with hot soapy water and bleach for hand, foot, and mouth disease.
4.	This is correct. The patient with hand, foot, and mouth disease should be provided with mild over-the-counter analgesics such as acetaminophen or ibuprofen.
5.	This is incorrect. An alcohol-based mouthwash is irritating to oral tissues and should not be used.

PTS: 1

CON: Infection | Health Promotion

16. ANS: 2, 5

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 11. Develop a teaching plan for caregivers of children and adolescents with communicable diseases.

Page: 527

Heading: Viral Communicable Diseases>Hepatitis B>Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care; Health Promotion

Difficulty: Moderate

	Feedback
1.	This is incorrect. Hepatitis B virus (HBV) is commonly transmitted through blood or body fluids.
2.	This is correct. Newborns may acquire HBV prenatally. Postexposure prophylaxis is needed to prevent the development of chronic hepatitis B.
3.	This is incorrect. The newborn receives the hepatitis B vaccination and if the mother is positive for the infection, the newborn should also receive hepatitis B immune globulin.
4.	This is incorrect. The newborn should receive adequate care and follow-up.
5.	This is correct. The hepatitis B vaccine is administered to all newborns. If the mother is positive for hepatitis B surface antigen (HBsAg), 0.5 mL of hepatitis B immune globulin (HBIG) is also given.

PTS: 1 CON: Infection | Patient-Centered Care | Health Promotion

17. ANS: 1, 2, 3, 5

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 9. Develop a home-based plan of care for a child or adolescent with a communicable disease.

Page: 528

Heading: Viral Communicable Diseases > Mononucleosis > Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Health Promotion

Difficulty: Moderate

	Feedback
1.	This is correct. The patient with mononucleosis needs to ingest ample fluids to prevent the development of dehydration.
2.	This is correct. To prevent injury to the spleen, contact sports are not permitted for 6 to 8 weeks if the spleen is enlarged. Even though the patient's status of the spleen is not identified, the patient should avoid these types of sports until approved by the health-care provider. Examples of contact sports include basketball, football, soccer, rugby, baseball, boxing, ice hockey, rodeo, wrestling, martial arts, lacrosse, and water polo.
3.	This is correct. The patient with mononucleosis is easily fatigued and should engage in quiet activities and play.
4.	This is incorrect. There is no reason to limit the patient's caloric intake. Adequate calories should be provided to ensure for the patient's nutritional status.
5.	This is correct. The patient should be provided with acetaminophen or ibuprofen for fever control.

PTS: 1 CON: Infection | Health Promotion

18. ANS: 3, 5

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 10. Discuss alternative and complementary therapies used with communicable diseases.

Page: 528

Heading: Viral Communicable Diseases > Influenza > Caregiver Education > Complementary and Alternative Therapies

Integrated Processes: Communication and Documentation

Client Need: Physiological Integrity: Basic Care and Comfort

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care; Safety

Difficulty: Moderate

	Feedback
1.	This is incorrect. Additional research is needed regarding alternative supplements to identify safety and efficacy of these therapies.
2.	This is incorrect. The vitamins and supplements listed are ones that people commonly take for disease prevention. However, that does not mean they have been proved effective or safe.
3.	This is correct. The safety of supplements and herbs have not been determined and need additional study.
4.	This is incorrect. There is no evidence that taking vitamins and herbs concurrently reduces the effectiveness of herbs.
5.	This is correct. The effectiveness of supplements and herbs for a specific health problem has not been determined and needs additional study.

PTS: 1

CON: Infection | Patient-Centered Care | Safety

19. ANS: 3, 4, 5

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objectives: Develop a teaching plan for caregivers of children and adolescents with communicable diseases.

Page: 528

Heading: Safe and Effective Nursing Care > Understanding Medication > Treating Influenza

Integrated Processes: Teaching and Learning

Client Need: Safe and Effective Care Environment: Safety and Infection Control

Cognitive Level: Application [Applying]

Concept: Infection; Patient-Centered Care; Health Promotion

Difficulty: Difficult

	Feedback
1.	This is incorrect. Zanamivir is recommended for children older than 7 years of age.
2.	This is incorrect. Antiviral medications for influenza approved by the Food and Drug Administration (FDA) include oseltamivir (Tamiflu) and zanamivir (Relenza).
3.	This is correct. Oseltamivir (Tamiflu) may be given to children older than 1 year and should be given on weight-based dosing.
4.	This is correct. Symptoms of influenza include fever, chills, headache, sneezing, cough, malaise, conjunctivitis, and myalgia (aching).

5.	This is correct. Medications for influenza must be given within 48 hours of the onset of symptoms.
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PTS: 1 CON: Infection | Patient-Centered Care | Health Promotion

20. ANS: 3, 4, 5

Chapter: Chapter 22 Communicable Diseases

Chapter Learning Objective: 11. Develop a teaching plan for caregivers of children and adolescents with communicable diseases.

Page: 529

Heading: Viral Communicable Diseases > Mumps (Parotitis) > Caregiver Education

Integrated Processes: Teaching and Learning

Client Need: Physiological Integrity: Physiological Adaptation

Cognitive Level: Application [Applying]

Concept: Infection; Health Promotion; Patient-Centered Care

Difficulty: Moderate

	Feedback
1.	This is incorrect. Droplet isolation is required to prevent the spread of the infection.
2.	This is incorrect. IgM enzyme immunoassay is used to detect the mumps virus.
3.	This is correct. Boys may have painful swelling of the testicles (orchitis) from the mumps virus.
4.	This is correct. Mumps is manifested by swelling of parotid salivary glands in front of the ear, below the ear, and/or under the jaw.
5.	This is correct. The patient with mumps should be provided with bland, soft foods and bland liquids. Citrus juices should be avoided. The patient should be kept well-hydrated.

PTS: 1 CON: Infection | Health Promotion | Patient-Centered Care